DBT: Deb's Board Training Domain Packet

# Area III: Intervention and Practice

353 original practice questions, grouped by sub-area. Questions come first; answers and rationales for every option are in a separate section at the back, so no answer appears on the same page as its question.

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All questions are original practice items written for this study set; they are not ASWB questions. The ID beside each answer lets you report a problem with a question in the app.

## III.A Practice Concepts

**1.** A client with schizophrenia who lives in supportive housing tells the social worker, 'I want a real job, like stocking shelves.' The client's symptoms have been stable on medication, but the client has not worked in 8 years. Housing staff say a job is unrealistic for now. What is the social worker's **BEST** response?

**A.** Refer the client to a supported employment program now

**B.** Enroll the client in a prevocational work-readiness group first

**C.** Explore whether the client's job goal is realistic right now

**D.** Ask the treatment team to decide when the client is ready

**2.** In Solomon's empowerment framework, a clinic rule that turns away people who lack a fixed address is an example of which concept?

**A.** Indirect power block

**B.** Direct power block

**C.** Learned helplessness

**3.** An 82-year-old resident of an assisted living facility tells the visiting social worker that the monthly bill includes a charge for services the resident never received. The resident is cognitively intact and says, 'I hate making a fuss.' The worker has a good working relationship with the business office. What should the social worker do **FIRST**?

**A.** Coach the resident to raise the charge with the business office

**B.** Call the business office directly to dispute the charge for the resident

**C.** Contact the long-term care ombudsman about the billing error

**4.** During a home visit, a parent tells a family services social worker that the school denied the child's request for a classroom aide at an IEP meeting. The parent says, 'I feel like the only one fighting this school.' The parent also mentions that several other families in the program have had the same request denied. What is the **MOST** appropriate action?

**A.** Attend the next IEP meeting and present the parent's case to staff

**B.** Teach the parent relaxation skills to use in school meetings

**C.** File a complaint with the state education agency for the parent

**D.** Connect the parent with a parent-led disability advocacy network

**5.** Last week, a client was released from prison after 5 years and enrolled in a reentry program. The social worker plans to open the first meeting with a strengths-based assessment. Which question is **BEST** to ask?

**A.** What led to the offense that sent you to prison?

**B.** What barriers do you expect when you look for work?

**C.** What helped you get through your time inside?

**6.** Two months after a parent's children entered foster care because of the parent's methamphetamine use, the parent has stayed sober, attended every visit, and has a relative willing to help. At a family team meeting, the parent says, 'Everyone only talks about what I did wrong.' The team then turns to drafting the case plan. Which approach to the plan is **BEST**?

**A.** List the parent's strengths in a separate section of the plan

**B.** Build each safety goal on a strength the parent already shows

**C.** Ask the parent how the meeting has felt so far

**7.** A client who uses a wheelchair has been turned down after four job interviews. Two of the employers had offices reached only by stairs, so those interviews took place in a lobby. The client says, 'Maybe I'm just not good enough to hire.' Which response from the social worker is **MOST** helpful?

**A.** Help the client look at how barriers shaped these outcomes

**B.** Rehearse interview answers to build the client's confidence

**C.** Contact the employers to raise the access problem

**D.** Refer the client for counseling on self-esteem

**8.** Which case management model works mainly in the community and builds on neighbors, landlords and employers to reach goals the client chooses?

**A.** Broker model

**B.** Clinical case management

**C.** Strengths model

**D.** Assertive community treatment

**9.** In the fifth session, a client who has been engaged suddenly says, 'You're like everyone else. You don't really listen.' The comment follows the social worker's suggestion to try a new coping exercise. What should the social worker do **FIRST** to repair the working relationship?

**A.** Invite the client to say more about feeling unheard

**B.** Explain the reasons for suggesting the coping exercise

**C.** Reassure the client that the social worker is on their side

**D.** Suggest a transfer to a social worker who is a better fit

**10.** By the fourth session, a social worker notices growing impatience each time a client describes relying on relatives for help. The social worker recognizes that the reaction echoes long-standing feelings about a dependent family member in the social worker's own life. What should the social worker do **NEXT**?

**A.** Tell the client about the reaction to keep the relationship genuine

**B.** Explore the reaction with a supervisor or consultant

**C.** Transfer the client to a colleague to protect the alliance

**11.** A hospital patient recovering from a leg amputation says, 'Everyone keeps telling me how lucky I am to be alive.' The social worker responds, 'Part of you may be grieving a loss that nobody around you has made room for yet.' Which skill does the social worker's response demonstrate?

**A.** Reciprocal empathy

**B.** Interpretation

**C.** Additive empathy

**D.** Paraphrasing

**12.** In weekly telehealth sessions, a client with panic attacks tells the social worker, 'You really get me.' Their plan centers on the client's wish to finish a grocery trip without fleeing the store in panic. Each week, though, the client skips the breathing practice, saying, 'I don't see how that connects to anything.' Which part of the working alliance is **MOST** likely weakest?

**A.** Agreement on tasks

**B.** Emotional bond

**C.** Agreement on goals

**13.** A 70-year-old client, widowed for 2 years, has met with the social worker weekly for 4 months. Near the end of a session, the client says, 'I think I've fallen in love with you. You're the only one who understands me.' What is the social worker's **BEST** response?

**A.** Arrange a transfer to a colleague before the next session

**B.** Explore the feelings in sessions while holding a clear role

**C.** Remind the client that the relationship is strictly professional

**14.** A social worker in a court-ordered treatment program feels intense disgust toward a client convicted of harming a child. For 6 weeks, the worker has discussed the reaction in weekly supervision and in personal therapy. The disgust persists, the worker dreads each session, and the client says, 'You barely look at me anymore.' What is the **MOST** appropriate next step?

**A.** Keep meeting while continuing to process the reaction in supervision

**B.** Tell the client about the reaction so the work feels honest

**C.** Ask the client to describe the change they have noticed

**D.** Arrange a planned transfer to another qualified social worker

**15.** In a juvenile detention center, a 16-year-old describes assaulting another youth. The social worker says plainly that the assault caused real harm. In the same meeting and every meeting after, the worker treats the youth as a person of worth who deserves care. Which element of Rogers' person-centered approach does the worker's stance toward the youth **MOST** likely show?

**A.** Congruence

**B.** Accurate empathy

**C.** Unconditional positive regard

**16.** A social worker at a community mental health center canceled two sessions in a row, each with only an hour's notice. At the next session, a client who is usually talkative gives one-word answers and looks at the floor. What is the social worker's **BEST** response?

**A.** Name the late cancellations and take responsibility for them

**B.** Ask the client why they seem so quiet today

**C.** Move ahead with the planned agenda to restore a sense of routine

**17.** During the first week of an intensive outpatient program for alcohol use, a client wipes away tears while saying, 'Honestly, I'm doing great. This place is just a formality for my job.' Which response by the social worker is **MOST** helpful at this point?

**A.** Point out the mismatch between the tears and the words

**B.** Gently reflect the sadness that shows as the client speaks

**C.** Accept the statement and continue with the intake questions

**D.** Suggest that the tears may come from grief the client avoids

**18.** A 6-year-old placed in an emergency foster home meets a child welfare social worker for the first time. The child hides behind a chair and will not answer questions. Crayons and toy animals sit on a small table in the room. What should the social worker do **FIRST** to build rapport?

**A.** Ask the child gentle questions about life at home

**B.** Explain in simple words why the child is in this home

**C.** Sit nearby and begin playing quietly with the toys

**19.** A client in outpatient treatment for alcohol use disorder grew up with a parent who withdrew affection after any mistake. In the fourth month, the client admits to hiding a relapse and waits to be dropped from the program. The social worker responds calmly and keeps working with the client. Weeks later, the client says, 'You didn't pull away, and now I can admit mistakes at work.' Which concept **MOST** likely explains this change?

**A.** Holding environment

**B.** Corrective emotional experience

**C.** Transference

**20.** In the tenth month of weekly psychodynamic therapy, a client whose parent died when the client was 8 misses the session after the social worker announces a 3-week leave. Coverage plans were given at the time of the announcement. At the next session, the client shrugs and says, 'It doesn't matter. You'll be gone anyway.' What is the social worker's **BEST** response?

**A.** Review how to reach the covering social worker during the leave

**B.** Interpret the reaction as grief for the parent who died

**C.** Reassure the client that the leave is short and the work will go on

**D.** Invite the client to say more about the coming break

**21.** At the second session in a community clinic, a client with a history of brief, intense relationships says the social worker is 'the first one who really gets me.' The client describes two former workers as cold and useless. What is the **MOST** appropriate response?

**A.** Thank the client and invite feedback when the worker gets it wrong

**B.** Accept the praise warmly and move on to the client's goals

**C.** Point out that the client may be idealizing the worker

**22.** On an inpatient psychiatric unit, a newly admitted client with schizophrenia stands near the door, refuses to sit, and says staff record thoughts through the ceiling lights. The social worker is meeting the client for the first time to begin a working relationship. What should the social worker do **FIRST**?

**A.** Explain calmly that the lights cannot record thoughts

**B.** Ask what the client has noticed about the ceiling lights

**C.** Introduce self briefly and let the client choose the distance

**23.** At a hospital care conference, the physician and nurse recommend discharging a patient with heart failure to a skilled nursing facility. The social worker's assessment shows that the patient has decision-making capacity, wants to return home, and has a relative willing to help with home services. What is the social worker's **BEST** action?

**A.** Accept the physician's recommendation as the team's decision

**B.** Encourage the patient privately to refuse the facility placement

**C.** Refer the disagreement to the hospital ethics committee

**D.** Present the patient's goals and a home-support plan to the team

**24.** A social worker co-leads an anger-management group for adolescents at a residential treatment center. The colleague often contradicts the social worker in front of members, and members have begun taking sides. The two discussed the pattern privately after two sessions, but nothing changed. What should the social worker do **NEXT**?

**A.** Name the disagreement openly with members during the next session

**B.** Ask the program director to assign a different co-leader

**C.** Take the conflict to a joint supervision meeting

**25.** Before the annual IEP meeting for an 8-year-old autistic student, the parent tells the school social worker, 'There will be eight of them and one of me. I'll just sign whatever they put in front of me.' The parent has strong views about the child's reading support. What is the social worker's **BEST** response?

**A.** Offer to present the parent's reading concerns to the team

**B.** Help the parent write down priorities and questions to raise

**C.** Reassure the parent that the team wants what is best for the child

**26.** In an early intervention home-visiting program, one social worker is the only provider who visits a toddler's family. A speech pathologist and an occupational therapy practitioner coach the social worker, who then teaches their strategies to the parents during visits. The three plan goals together with the family. Which team model is this program **MOST** likely using?

**A.** Interdisciplinary

**B.** Multidisciplinary

**C.** Transdisciplinary

**D.** Intradisciplinary

**27.** On a hospital medical unit, a nurse stops the social worker in the hallway and complains that the attending physician talks down to a patient's family and ignores their questions. The nurse asks the social worker to 'say something' to the physician. What is the **MOST** appropriate response by the social worker?

**A.** Encourage the nurse to raise the concern with the physician directly

**B.** Bring the nurse's complaint to the physician on the nurse's behalf

**C.** Report the physician's conduct to the unit's medical director

**28.** A hospital social worker learns that a patient recovering from hip surgery will be discharged home this afternoon. The patient lives alone up a flight of stairs, cannot yet climb them, and home health services cannot start for 4 days. The patient says they are afraid of falling. The social worker shared these facts and the physical therapy evaluation with the attending physician, who says the order stands. What should the social worker do **NEXT**?

**A.** Ask the physician again to reconsider the discharge time

**B.** Take the concern up the chain of command today

**C.** Document the concern and complete the discharge plan

**29.** In an intensive care unit, a patient's family has heard different predictions about recovery from the cardiology and neurology teams. The family asks for a meeting, and the social worker is asked to arrange it. What should the social worker do **FIRST**?

**A.** Convene a brief team meeting to align the medical message

**B.** Ask the family to list the questions they most want answered

**C.** Schedule the family meeting so each team can present its view

**30.** At a homeless shelter, a client cannot apply for jobs because a stolen wallet held the client's ID and birth certificate. The social worker and client have agreed that replacing the ID is the problem to work on first. What should the social worker do **NEXT**?

**A.** Pick the quickest way to replace the ID and tell the client

**B.** List possible ways to replace the ID with the client

**C.** Request a shelter residency letter to take to the ID office

**D.** Explore how losing the wallet has affected the client

**31.** At a family resource center, a parent lists four concerns: finishing a GED, a car that needs repairs, a child's behavior at school, and an electricity shutoff notice for Friday. It is January, the home is heated by electricity, and an infant lives there. The parent says the child's school behavior worries them most. What should the social worker do **FIRST**?

**A.** Begin with the child's school behavior that the parent named

**B.** Ask the parent to rank the four concerns by importance

**C.** Work with the parent to stop the utility disconnection

**D.** Build one plan that covers the four concerns together

**32.** Before a parent's hospital discharge, an adult child asks the social worker, 'Should I move my parent in with me or choose a nursing facility? Just tell me what to do.' The parent has moderate neurocognitive disorder and has said they want to stay with family. The adult child works full time and has two teenagers at home. What is the social worker's **BEST** response?

**A.** Recommend the nursing facility given the caregiving demands described

**B.** Help the family weigh both options, including the patient's wishes

**C.** Schedule a family meeting so the care team can recommend a plan

**33.** A client at a community agency brings an eviction summons that requires a written response to the court within 5 days. The social worker listens as the client describes fear of losing the apartment. The client says the landlord's claims are false and asks what to write in the response. What should the social worker do **NEXT**?

**A.** Ask how the client is coping with the threat of eviction

**B.** Connect the client today with a legal aid office

**C.** Help the client draft a reply disputing the landlord's claims

**D.** Help the client list other places to live if evicted

**34.** A patient with advanced lung cancer is continuing chemotherapy aimed at slowing the disease. A team now also visits to manage the patient's pain and breathlessness and to support the family's stress. Which type of care is the patient **MOST** likely receiving from this team?

**A.** Palliative care

**B.** Hospice care

**C.** Caregiver respite care

**35.** A patient with metastatic colon cancer has received home hospice care for 6 weeks. The oncologist mentions a clinical trial of a new treatment, and the patient wants to try it. The patient's adult child tells the social worker they fear that leaving hospice means the patient can never return to it. How should the social worker respond?

**A.** Confirm that revoking hospice ends the patient's future eligibility

**B.** Explain that a return is possible once the current benefit period ends

**C.** Explain that hospice can be revoked now and elected again later

**36.** A patient with advanced heart failure is told that no further treatment will extend life. The patient speaks openly about the diagnosis and has updated a will. The patient then starts following a strict low-salt diet for the first time and asks the palliative care social worker whether 'being perfect from now on' could earn enough time to attend a grandchild's graduation in 2 years. Which of Kubler-Ross's stages is **MOST** likely reflected?

**A.** Bargaining

**B.** Denial

**C.** Acceptance

**D.** Depression

**37.** A home hospice patient with end-stage lung disease has said many times that they want to die at home. The patient's partner, the only caregiver, has slept little for 2 weeks. As the social worker arrives for a visit, the partner says quietly, 'I don't know how much longer I can do this.' What should the social worker do **FIRST**?

**A.** Recommend moving the patient to a nursing facility for the remaining time

**B.** Arrange a short inpatient respite stay to relieve the partner

**C.** Explore with the partner what feels unmanageable right now

**38.** During a home hospice visit, the social worker finds the patient moaning and grimacing with each movement. The patient's adult child, the main caregiver, says they have been skipping some prescribed pain medicine doses because they fear it will cause addiction or hasten death. What should the social worker do **FIRST**?

**A.** Ask the on-call nurse to reassess the pain now

**B.** Reassure the caregiver that pain medicine rarely hastens death

**C.** Explore the caregiver's fears about the patient's pain medicine

**39.** On a palliative care unit, a patient with advanced pancreatic cancer asks the social worker, 'How long do I have?' The oncologist reviewed the latest scan results with the patient yesterday. No family members are present. What should the social worker do **FIRST**?

**A.** Tell the patient the physician is the one to answer that

**B.** Share the typical survival range for this stage of cancer

**C.** Arrange a family meeting with the oncologist this week

**D.** Invite the patient to say what they understand and hope to learn

**40.** In home hospice, a patient with end-stage heart failure and the patient's partner have both been told the illness is terminal. In separate visits, each tells the social worker they understand the situation. Together, they talk only about next summer's garden and change the subject whenever either mentions the hospice nurse. Which concept **MOST** likely describes this pattern?

**A.** Mutual pretense awareness

**B.** Open awareness

**C.** Denial

**41.** In an in-home parenting program, a parent agreed to ignore their 4-year-old's whining for snacks and to praise polite requests. Four days later, the parent reports that the whining has become louder and longer and asks whether the plan is failing. The child is otherwise healthy and safe. What should the social worker do **NEXT**?

**A.** Explore with the parent how the louder whining is affecting them

**B.** Replace planned ignoring with a short time-out for whining

**C.** Explain the expected surge and coach continued consistency

**42.** In a parenting group, a parent says that when their 5-year-old screams in the checkout line, the parent buys the child candy and the screaming stops at once. Over several months, the parent has started buying candy as soon as the child begins to fuss. Which learning principle **MOST** likely explains the change in the parent's behavior?

**A.** Positive reinforcement

**B.** Extinction

**C.** Positive punishment

**D.** Negative reinforcement

**43.** A child welfare social worker supports a parent whose 3-year-old is in foster care. The parent completed a 10-week parenting class and earned a certificate. During supervised visits, the parent gives sharp commands and rarely praises, and the child cries and pulls away. The case plan review is in 6 weeks, and the parent wants to reunify. Which action is **BEST** before the review?

**A.** Enroll the parent in a second class on parenting young children

**B.** Coach the parent in real time during their play together

**C.** Record the class certificate as meeting the parenting goal

**44.** A parent in a home-visiting program made early progress with a 2-year-old's bedtime routine. For the past 6 weeks, the parent has skipped practice, sleeps much of the day and says nothing feels worth doing. A depression screen today falls in the moderately severe range, and the parent denies thoughts of self-harm when asked directly. Which action is **MOST** appropriate?

**A.** Refer the parent for depression treatment and keep visiting

**B.** Pause home visits until the parent's depression improves

**C.** Increase practice assignments to rebuild the bedtime routine now

**D.** Explore with the parent what has lowered their motivation

**45.** During a structured play task in a parenting group, the social worker watches a parent say, 'Can you maybe start putting the blocks away? Let's clean up, okay?' The parent repeats this 5 times while the 6-year-old keeps playing. Which coaching suggestion would be **MOST** helpful?

**A.** Explain to the child why cleaning up matters first

**B.** Ignore the child's refusal to avoid a power struggle

**C.** Give one clear, specific command phrased as a statement

**D.** Promise the child a treat before the cleanup begins

**46.** In a home-visiting program, a parent asks the social worker how to use time-out for a 3-year-old who hits a younger sibling. The parent says most of their time together is spent giving commands or corrections, with little relaxed time. The hitting has caused no injuries. What should the social worker do **FIRST**?

**A.** Teach a consistent time-out procedure for each hitting incident

**B.** Explore how the parent was disciplined as a child

**C.** Coach brief daily child-led play with specific praise

**47.** A foster parent in a parenting group says praise 'doesn't work' for an 8-year-old who leaves chores unfinished. Asked to describe it, the foster parent says they tell the child 'good job' at bedtime on days when the chores were done. What is the **MOST** appropriate suggestion?

**A.** Add a larger weekly reward to the bedtime praise

**B.** Respond right after each chore by naming what was done

**C.** Take away screen time on any day a chore is left unfinished

**48.** An adult with schizophrenia has been clinically ready to leave a state psychiatric hospital for 5 months and wants to live in the community. The treatment team agrees. The only barrier is a waitlist for state-funded supported housing. A nursing facility has an open bed, and hospital administrators suggest transferring the client there. What is the social worker's **BEST** action?

**A.** Support the nursing facility transfer as a less restrictive setting

**B.** Press the state for supported housing under disability rights law

**C.** Keep the client in the state hospital until housing opens

**D.** Discharge the client to a shelter with outpatient follow-up

**49.** Which practice change does the federal Family First Prevention Services Act of 2018 encourage in child welfare agencies?

**A.** Evidence-based in-home help so children can stay safely at home

**B.** Faster filing to end parental rights for children in long-term care

**C.** Wider use of group homes for youth with complex needs

**50.** A child welfare social worker places a 6-year-old in emergency foster care after finding the child alone at home for 2 days. The parent says the child is an enrolled member of a federally recognized tribe but has no enrollment card on hand. The agency plans to file a petition for foster care placement. What does federal law require of the agency regarding the tribe?

**A.** Wait for proof of enrollment before contacting the tribe

**B.** Bring in the tribe once reunification efforts have failed

**C.** Notify the tribe and apply the law's protections now

**51.** A 24-year-old client in outpatient therapy finishes graduate school next month and will start a part-time job with no health benefits. The client has been covered under a parent's employer plan and plans to stop therapy at graduation because the insurance will 'run out.' What is the social worker's **BEST** response?

**A.** Explain that the coverage can continue until the client turns 26

**B.** Help the client apply for Medicaid before graduation

**C.** Help the client compare marketplace plans before graduation

**52.** A grant proposal includes the statement 'By the end of year one, 75% of enrolled caregivers will show reduced stress on a standardized scale.' In which proposal section does this statement belong?

**A.** Statement of need

**B.** Program goals

**C.** Program objectives

**D.** Evaluation plan

**53.** In a year-end report to a funder, which statement describes a program outcome rather than an output?

**A.** One hundred twenty caregivers attended 8 or more group sessions

**B.** The program held 48 support groups at 3 community sites

**C.** Two bilingual facilitators were hired and trained this year

**D.** Caregivers' average strain scores fell 25% by 6 months

**54.** A school social worker is writing a foundation grant proposal for an anxiety skills group for middle school students. The funder's guidelines ask for a statement of need. Which content would **BEST** support this section?

**A.** District data on anxiety rates and current waitlists for services

**B.** National statistics showing rising anxiety among adolescents

**C.** A history of the agency's 30 years serving local youth

**D.** A session-by-session outline of the planned group curriculum

**55.** A social worker is drafting a year-end report to a foundation that funded a caregiver support program. The grant target was 100 enrolled caregivers; 72 enrolled, and 40 more called for information but did not enroll. The program director suggests counting the callers as clients served to meet the target. How should the social worker report enrollment?

**A.** Report the callers as enrolled and add a footnote

**B.** Omit the enrollment figure and lead with outcome gains

**C.** Report 72 enrolled and explain the gap and next steps

## III.B Intervention Methods and Techniques

**56.** A community clinic is redesigning its intake. Currently, staff seat each new client, then ask a fixed list of trauma history questions in the same order for everyone. Which change is **MOST** consistent with a trauma-informed approach?

**A.** Offer clients choices about pacing and what to share

**B.** Collect a detailed trauma history at the first visit

**C.** Refer clients who report trauma to a trauma specialist

**57.** A client with a history of repeated childhood abuse starts outpatient therapy and asks to 'go through the memories' right away. In the past month, the client has cut their arms several times, has been sleeping in their car, and often loses track of time during sessions. The client says reading about trauma therapy gave them hope. What should the social worker focus on **FIRST**?

**A.** Begin detailed processing of the abuse memories as requested

**B.** Work with the client on safety and stabilization

**C.** Refer the client to an inpatient trauma program

**58.** A program director at a child advocacy center learns that several interviewers and family advocates have come to see most strangers as dangerous, check on their own children many times a day, and describe the world as unsafe. The staff say they still believe in the work and want to stay. What is the **BEST** organizational response?

**A.** Build regular trauma-focused supervision into each staff member's week

**B.** Refer each affected staff member to the employee assistance program

**C.** Hold a staff wellness day with yoga and chair massage

**59.** In a hospital, a patient who disclosed a past sexual assault becomes panicky before a needed physical exam, gripping the bed rail and saying, 'I can't do this if I don't know what's coming.' The physician asks the social worker for help. What is the **MOST** appropriate action?

**A.** Suggest that the physician order a sedative first

**B.** Ask the physician to postpone the exam until the patient feels ready

**C.** Reassure the patient that the staff are skilled professionals

**D.** Arrange for each step to be explained and a way to pause

**60.** A behavioral health clinic serving a tribal nation offers a weekly PTSD skills group that few people attend. Community members tell the social worker that their grandparents were taken to boarding schools and that outside agencies have rarely kept their promises. Several elders lead traditional healing gatherings that are well attended. What is the **BEST** step to make the clinic more trauma-informed?

**A.** Ask the elders to promote the PTSD group at gatherings

**B.** Hire tribal community staff to lead the clinic's PTSD group

**C.** Invite the elders to help shape the clinic's services

**61.** During a video session, a client describing a carjacking stops mid-sentence, stares past the camera, and does not respond when the social worker says the client's name. After a long pause, the client whispers, 'I'm not really here.' What should the social worker do **FIRST**?

**A.** Help the client notice objects and sounds in the room

**B.** Ask the client what feelings are coming up about the carjacking

**C.** Encourage the client to keep going and finish the story

**D.** End the session early so the client can rest

**62.** A family service agency's director says the agency is now trauma-informed because two clinicians completed TF-CBT training. Meanwhile, front-desk staff call out clients' reasons for visits in the waiting room, and a no-show policy closes cases after two missed visits without outreach. Which recommendation is **MOST** consistent with a trauma-informed approach?

**A.** Train more clinicians in TF-CBT so every child is served

**B.** Train staff in each role and revise intake and attendance rules

**C.** Have intake staff screen every new client for trauma

**63.** In a hospital emergency department, a social worker meets an adult patient who was sexually assaulted by an acquaintance about 6 hours ago. The patient is tearful, wants to shower and go home, and does not want police involved. The patient asks the social worker, 'Is there any reason I should stay?' What is the **BEST** response?

**A.** Support the decision to leave and arrange a call tomorrow

**B.** Explain the care and exam options that need no police report

**C.** Encourage a police report to protect others from the acquaintance

**64.** On an inpatient psychiatric unit, policy requires a body check for injuries and contraband at admission. A newly admitted adult with a history of childhood sexual abuse starts shaking and crying when told about the check and backs into a corner. The nurse asks the social worker for help. What is the **MOST** appropriate action?

**A.** Ask the nurse to skip the check for this patient

**B.** Explain the safety reasons for the check, then proceed

**C.** Plan with the patient and nurse how the check will be done

**65.** In a hospice program, a patient is expected to die within days. The patient's partner asks the social worker how to tell their 5-year-old and plans to say the patient is 'going to sleep for a long time' because it sounds gentle. What is the **BEST** guidance?

**A.** Use the gentle phrase, since young children cannot grasp death

**B.** Wait to explain the death until the child starts asking

**C.** Use the words 'dying' and 'died,' explained in simple terms

**66.** An employee assistance client is struggling after a close friend died of an overdose 3 weeks ago. The friend's family did not invite the client to the funeral, the employer denied bereavement leave because the friend was not a relative, and coworkers have said, 'It was bound to happen.' Which concept **MOST** likely describes the client's experience?

**A.** Anticipatory grief

**B.** Disenfranchised grief

**C.** Prolonged grief disorder

**D.** Ambiguous loss

**67.** A college student whose parent died 3 months ago describes a typical day to a campus social worker. Mornings are spent looking at old photos and crying. Afternoons go to classes, estate paperwork, and learning to cook the parent's recipes for younger siblings. The student worries that this back-and-forth means something is wrong. Which framework **MOST** likely explains this pattern?

**A.** Dual process model

**B.** Worden's tasks of mourning

**C.** Kübler-Ross stages of dying

**D.** Continuing bonds

**68.** A home health social worker is assigned to a 78-year-old whose spouse of 50 years died 6 weeks ago. Before the first visit, an adult child calls, says the client 'cries all the time and talks to the photo every morning,' and asks the worker to arrange antidepressant medication. What should the social worker do **FIRST**?

**A.** Refer the client to a prescriber for an antidepressant evaluation

**B.** Begin structured treatment for prolonged grief

**C.** Help the family see these reactions as expected grief

**D.** Meet with the client to explore how they are living with the loss

**69.** A client whose partner died in a car crash 16 months ago has seen a social worker for supportive counseling for 3 months. The worker's assessment finds intense daily yearning, a sense that part of the client has died, avoidance of the crash route, a feeling that life is meaningless, and missed work most weeks. The worker has no training in treatments designed for this condition, and a nearby program offers one. What should the worker do **NEXT**?

**A.** Continue weekly supportive counseling and track the symptoms

**B.** Refer the client for a therapy that targets this grief

**C.** Help the client accept that grief has no set timeline

**70.** A client's partner, a commercial fisher, went missing at sea 14 months ago, and no body was found. Relatives urge the client to hold a funeral and 'get closure.' The client says, 'If I bury an empty coffin, I'm giving up on them, but I can't keep waiting like this either.' What is the **MOST** appropriate intervention?

**A.** Encourage the memorial service so the client can reach closure

**B.** Refer the client for prolonged grief disorder treatment

**C.** Support the client's hope that the partner may be found

**D.** Help the client accept holding both hope and grief at once

**71.** A social worker in a state prison meets with a client whose parent died last week. The facility denied the client's request to attend the funeral. In their first meeting since the death, the client sits quietly for a while, then says, 'It doesn't feel real. I never got to say goodbye.' What should the social worker do **FIRST**?

**A.** Invite the client to talk about the relationship and the loss

**B.** Request a psychiatric evaluation for depression after the loss

**C.** Help the client plan a private memorial ritual on site

**72.** In a hospital family room, a social worker meets the parent of a 19-year-old who died in a motorcycle crash an hour ago. The parent sits rigidly and says, 'I told them to sell that bike. I should have taken the keys.' What should the social worker do **FIRST**?

**A.** Invite the parent to keep talking about what they feel

**B.** Assure the parent that they are blameless for the crash

**C.** Provide information about bereavement support groups

**73.** At intake, the social worker explains the court's attendance requirement and what will be reported to the probation officer. The client says, 'I don't have an anger problem. I'm only here so the judge stays off my back.' Which response by the social worker is **BEST**?

**A.** Review the police report with the client to address the denial

**B.** Repeat the court's requirements and the reporting rules

**C.** Ask what the client would want to gain from finishing the program

**74.** A physician asks a hospital social worker to assess home safety for a 79-year-old patient admitted after a fall. When the social worker arrives, the patient says, 'I didn't ask for a social worker. I'm going home, period.' The patient is alert and oriented. What should the social worker do **FIRST**?

**A.** Explain that the visit aims to help the patient get home safely

**B.** Leave and document that the patient declined social work services

**C.** Begin the home safety questions the physician requested

**75.** A parent must complete a parenting class under a court-ordered child welfare case plan. A few minutes into the first meeting, the parent says, 'Nobody asked what I need. I'm not sitting in some class.' What should the social worker do **FIRST**?

**A.** Describe what the court may do if the class goes unfinished

**B.** Reflect the parent's frustration at having no say in the plan

**C.** Separate the required steps from those the parent can choose

**76.** A client who sought telehealth counseling for work stress has canceled 2 of the last 3 sessions by text, each time citing a busy schedule. The agency discharges clients after 3 missed sessions. What should the social worker do **FIRST**?

**A.** Send a letter explaining the agency's discharge policy

**B.** Reach out to discuss what is making sessions hard to keep

**C.** Wait to see whether the client attends the next scheduled session

**77.** A school requires a 15-year-old to meet with the social worker after a suspension. When the social worker lists the sessions the student must attend and the topics they will cover, the student refuses to talk. At the next meeting, the worker names what is required and lets the student pick the topics and meeting times, and the student begins to participate. Which concept **MOST** likely explains the student's first response?

**A.** Psychological reactance

**B.** Learned helplessness

**C.** Cognitive dissonance

**D.** Transference

**78.** On a mild evening, a street outreach social worker approaches an adult who has slept in the same park for months. The person accepts a bottle of water but says, 'I don't need your programs,' and turns away. What should the social worker do **FIRST**?

**A.** Leave on friendly terms and say when the worker will return

**B.** Describe the shelter's open beds and services before leaving

**C.** Complete the housing intake assessment with the person

**79.** A voluntary client who has never had counseling comes to an outpatient clinic about job stress. A few minutes into the first session, the client asks, 'So do you just tell me what to fix? I've never done this before.' What should the social worker do **FIRST**?

**A.** Offer two or three practical tips for handling job stress

**B.** Ask the client to describe the job stress in detail

**C.** Explain how sessions work and ask what the client expects

**80.** A client on probation attends a court-ordered program. When the client mentions arriving on time for a new job all week, the social worker gives specific praise. When the client jokes about 'beating the system,' the worker calmly offers a different view and moves on. The worker also arrives on time and keeps every promise. Which approach is the social worker **MOST** likely using?

**A.** Motivational interviewing

**B.** Prosocial modeling

**C.** Contingency management

**81.** A client who injects opioids says they are not ready to stop using but are worried after a friend overdosed. The client wants to keep meeting with the social worker. What is the **MOST** appropriate intervention?

**A.** Provide naloxone and teach overdose prevention steps

**B.** Make continued services contingent on abstinence

**C.** Refer the client to a residential treatment program this week

**82.** A community outreach client uses cocaine at weekend parties and says they have no plans to stop. After two friends overdosed on cocaine that contained fentanyl, the client asks the social worker, 'How do I keep this from happening to me?' What is the **BEST** response?

**A.** Advise the client that quitting is the safest choice

**B.** Explore what the client might gain from quitting cocaine

**C.** Urge the client to enroll in an intensive treatment program

**D.** Go over practical ways to make each use less risky

**83.** An employee assistance client drinks 4 to 5 beers most weeknights and wants to cut back to 2 drinks on weekend nights only. The client says quitting entirely is not the goal right now but is willing to keep a daily log. What is the **MOST** appropriate step?

**A.** Set the drinking limit with the client and review it weekly

**B.** Have the client log drinks for 2 weeks before setting a limit

**C.** Support the goal if the client agrees to attend AA meetings

**84.** A street outreach social worker has met for months with an adult who has schizophrenia and alcohol use disorder and has lived outdoors for 6 years. The client declines treatment but says, 'I'd take an apartment if I didn't have to quit drinking.' The worker's program follows a Housing First model. Which action is consistent with that model?

**A.** Refer the client to a sober transitional apartment first

**B.** Keep building rapport until the client accepts treatment

**C.** Offer a permanent apartment with services the client may choose

**85.** At a syringe services program, a participant shows the social worker a swollen, hot area on the forearm where the redness has spread since yesterday. The participant feels feverish and explains clearly that they will not go to the emergency department after being mistreated there. The program partners with a walk-in wound clinic two blocks away that serves people who use drugs. What is the **BEST** action for the social worker?

**A.** Give wound care supplies and check the arm next week

**B.** Go with the participant to get wound care today

**C.** Call an ambulance despite the participant's objection

**D.** Explore the participant's readiness to stop injecting

**86.** A client has attended an opioid treatment program for 8 months, taking methadone daily and holding a steady job. Recent urine tests show benzodiazepines that were not prescribed. The client says the pills help with sleep and fears losing methadone. At a team meeting, a staff member proposes discharge under the program's 'no illicit use' rule. What is the **BEST** response by the social worker?

**A.** Urge keeping the client enrolled while the team addresses the pill use

**B.** Support discharge with a referral to an abstinence-based residential program

**C.** Tell the client that staying enrolled requires benzodiazepine-free tests

**87.** A client in outpatient counseling uses methamphetamine a few times a month and says they are not interested in stopping. The client carries condoms but says they rarely use them during sex while high, then asks, 'Is there anything that can lower my chances of getting HIV?' What is the **BEST** response?

**A.** Link the client to a clinic offering preventive HIV medication

**B.** Explore the client's readiness to cut back on methamphetamine

**C.** Provide condoms and go over safer-sex steps that lower HIV risk

**88.** A 72-year-old client with COPD receives a notice that Medicaid home care hours will be cut in half in 10 days. The client relies on the aide for bathing and meals and wants to fight the cut. The notice explains how to request a fair hearing. What should the social worker do **FIRST**?

**A.** Call the Medicaid caseworker to request that the cut be reversed

**B.** Gather medical records that document the client's care needs

**C.** Help the client file an appeal before the cut takes effect

**D.** Arrange a private-pay aide to cover the lost hours

**89.** A social worker from a community aging services agency visits a client who lives in a nursing facility. Soon after the client complained about slow call-light response, the facility issued a 30-day discharge notice. The client and social worker met with the administrator, who refused to reconsider. The client wants to stay. What should the social worker do **NEXT**?

**A.** Help the client contact the long-term care ombudsman program

**B.** Help the client tour other facilities before the notice period ends

**C.** Report the facility to adult protective services for neglect

**90.** A school social worker learns that the front office turned away 3 families staying in a motel this semester because they could not show proof of a home address. Each family enrolled only after the social worker stepped in. Which action is **BEST**?

**A.** Keep helping each family enroll as new cases come up

**B.** Give front-office staff a written copy of the enrollment rules

**C.** Raise the enrollment practice with the district's homeless liaison

**91.** Which practice role involves pressing an agency, on a client's behalf, to provide a service the client is entitled to receive?

**A.** Broker

**B.** Advocate

**C.** Mediator

**D.** Enabler

**92.** A client with schizoaffective disorder receives a lease warning after neighbors complain of loud talking at night. The client links it to voices that worsened during a medication change, which the prescriber is now adjusting. The client wants to keep the apartment and agrees to let the social worker help. What is the **MOST** appropriate way for the social worker to assist?

**A.** Help the client ask the landlord for a reasonable accommodation

**B.** Help the client look for a unit with fewer shared walls

**C.** Help the client file a fair housing complaint right away

**D.** Send the landlord a letter describing the client's diagnosis

**93.** A parent of two arrives upset, saying the family's food assistance 'just got cut off out of nowhere.' The parent holds a letter from the benefits office but has not read it closely and asks the social worker to call the office director right away. What should the social worker do **FIRST**?

**A.** Call the office director to object to the cutoff

**B.** Review the notice together to find the stated reason

**C.** Help the parent submit a written fair hearing request today

**94.** In an Illness Management and Recovery group at a community mental health center, the social worker has been reading each module's handout aloud. A member with schizophrenia who reads at a basic level says the material 'goes right over my head' and has started missing sessions. What is the **MOST** helpful adjustment?

**A.** Assign the full handout as homework between sessions

**B.** Refer the member to individual therapy instead

**C.** Teach interactively with pictures and frequent review

**D.** Give a printed summary written at a lower reading level

**95.** After a young adult's first hospitalization for schizophrenia, the parents meet with the inpatient social worker. One parent asks, 'Did our strict parenting cause this?' What is the **BEST** response?

**A.** Explain that the illness comes from biological vulnerability plus stress

**B.** Explore the parent's guilt about past parenting decisions in detail

**C.** Recommend family therapy to change the household's parenting style

**96.** In an early psychosis program, a 22-year-old client who smokes cannabis daily asks whether it is 'really that bad.' The social worker first asks what the client has heard, then briefly shares research on heavy cannabis use and psychotic relapse. What should the social worker do **NEXT**?

**A.** Advise the client to cut back before the next visit

**B.** Ask what the client makes of this information

**C.** Ask whether the client is willing to cut back now

**D.** Give a handout that summarizes the research findings

**97.** A client has gone to the emergency department three times for a racing heart and chest tightness. Cardiac testing was normal each time, and the physician diagnosed panic disorder and referred the client to the social worker. The client asks, 'If my heart is fine, why does my body do this?' What should the social worker do **FIRST**?

**A.** Explain how the body's alarm response produces these sensations

**B.** Begin exercises that bring on a racing heart in session

**C.** Ask the physician to order further heart tests for reassurance

**98.** During a home visit, an adult child caring for a parent with major neurocognitive disorder says the parent asks the same question 20 times an hour. The social worker acknowledges how draining that sounds. The caregiver replies, 'I think it's on purpose, to get under my skin,' and says they have started snapping at the parent. What is the **MOST** appropriate response?

**A.** Suggest gently reminding the parent that the question was answered

**B.** Explain that memory loss keeps the parent from recalling asking

**C.** Reflect the caregiver's frustration and exhaustion

**99.** A social worker leads an illness management group at a community mental health center for adults with schizophrenia. During a session on stress and symptoms, a member says, 'This is a waste of time. I'm not sick.' Earlier, the member said they want to keep their apartment and get a job. What is the **BEST** response?

**A.** Review the member's symptoms with the group to build insight

**B.** Note the member's view and continue with the planned material

**C.** Connect the skills to what the member wants for work and housing

**D.** Suggest individual therapy to work on accepting the diagnosis

**100.** A client with bipolar I disorder, stable for 6 months after a manic episode, asks for help avoiding another hospitalization. The client's partner joins the session and asks for a complete list of symptoms to watch for each day. The client says the partner's constant questions about mood feel like being watched. Which psychoeducation approach is **MOST** helpful?

**A.** Give the partner a daily checklist of bipolar symptoms to watch for

**B.** Meet with the client alone to protect the client's privacy

**C.** Help the client name their own early signs and the partner's role

**D.** Teach the partner to watch for reduced sleep as the key warning sign

**101.** Which treatment objective for a client with depression is written in measurable terms?

**A.** Client will feel less depressed and more hopeful about the future

**B.** Client will increase pleasant activities as often as possible

**C.** Client will gain insight into the causes of the low mood

**D.** Client will walk 20 minutes on 5 days each week by week 6

**102.** At a primary care clinic, a client recently diagnosed with type 2 diabetes also lost a job last month. Early in their first meeting, the client says, 'I'm supposed to change my diet, test my blood, exercise, and find work. I can't do any of it.' What should the social worker do **FIRST**?

**A.** Reflect how overwhelming the many new demands feel

**B.** Create a full plan that covers each of the four demands

**C.** Agree with the client on one small step to take this week

**103.** A client at a family service agency agreed to an 8-session task-centered contract focused on conflict with a teenage child. At session 5, the client arrives with an eviction notice and says it is all they can think about. The original goals are about half met. What is the social worker's **BEST** response?

**A.** Keep the original contract and plan to address housing afterward

**B.** Explore how the eviction connects to the conflict at home

**C.** Revise the contract with the client to add the housing problem

**104.** In a telehealth employee assistance session, a client says the main goal is for their partner to stop drinking. The assessment shows the client is losing sleep and phoning the partner's employer to excuse missed shifts. The social worker and client are now setting goals. Which goal is **MOST** appropriate?

**A.** Client will let the partner handle their own work absences for 4 weeks

**B.** Partner will attend a mutual-help meeting each week by the second month

**C.** Client will get the partner to agree to an assessment by week 4

**D.** Client will accept that the partner's drinking is outside their control

**105.** In an integrated primary care clinic, a client with moderate depression says, 'My license is suspended, so I can't get to work. That has to come first.' The social worker believes the depression should be treated before anything else. What is the **BEST** way to set the first goal?

**A.** Make treating the depression the first goal of the contract

**B.** Ask the client to rank the problems in order of importance

**C.** Work on the license and the depression as equal goals

**D.** Start with the steps to reinstate the license

**106.** By telehealth, a parent says the main goal is to stop yelling at the children, which happens most evenings. The parent and the social worker are writing a measurable objective for the next 6 weeks. Which objective follows goal-setting guidelines?

**A.** Use a calm voice for evening directions on 5 of 7 days

**B.** Stop yelling at the children completely within 6 weeks

**C.** Yell less often at the children over the next 6 weeks

**D.** Become a calmer, more patient parent by the end of 6 weeks

**107.** At a senior center, a client caring for a spouse with major neurocognitive disorder is using structured problem solving to tackle exhaustion. Together they defined the problem and listed options, and the client chose an adult day program 2 days a week. The client has now used it for 3 weeks. What should the social worker do **NEXT**?

**A.** Brainstorm more respite options in case the program falls short

**B.** Review with the client how well the program is meeting the goal

**C.** Begin structured problem solving on another stressor

**108.** A social worker leads a problem-solving skills group at a clubhouse for adults with serious mental illness. Members are suggesting ways to get to a new job site after a bus route ended. One member responds to each idea as it is offered, saying, 'That won't work.' What should the social worker do **FIRST**?

**A.** Ask members to list every idea before judging any of them

**B.** Invite the critical member to offer an idea of their own

**C.** Ask the member to explain what makes each idea unworkable

**109.** At a reentry program, a social worker and client sign an agreement to work on the two problems the client ranked highest, over 8 sessions. Each session opens with a review of progress on those problems. Each session closes with the client choosing an action step, rehearsing it, and planning for obstacles. Which approach is the social worker **MOST** likely using?

**A.** Solution-focused brief therapy

**B.** Problem-solving therapy

**C.** Behavioral activation

**D.** Task-centered practice

**110.** At a senior center, an older adult in problem-solving therapy chose to ask a neighbor for rides to medical visits. At the next session, the client sighs, reports that the neighbor said no, and says, 'This method just doesn't work for me.' What should the social worker do **FIRST**?

**A.** Acknowledge the client's disappointment about the refusal

**B.** Arrange agency transportation for the client's visits

**C.** Revisit the earlier list of options with the client

**111.** A client in a primary care clinic says, 'Drinking is the only thing that helps me sleep. But my liver tests are getting worse, and that scares me.' The client has not decided whether to cut back. Which response by the social worker is **MOST** appropriate?

**A.** For your liver's sake, it's time to find another way to sleep.

**B.** Drinking gets you to sleep, and your liver results worry you.

**C.** Let's set a date to cut back on drinking, and I can help.

**112.** A client seeks help because their partner drinks heavily most nights and refuses to discuss treatment. The client feels safe at home and wants the partner to get help. A relative suggests gathering the family for a surprise confrontation. What is the **BEST** approach for the social worker to offer?

**A.** Help the client plan a family confrontation with clear consequences

**B.** Coach the client to reward sober behavior and ask at calm moments

**C.** Refer the client to Al-Anon to focus on their own coping

**113.** An outpatient client stopped drinking 4 months ago. In session, the client reports having three beers at a cousin's wedding last weekend and none since. The client says, 'I blew it. I'm just someone who can't stay sober, so why keep trying?' What should the social worker do **FIRST**?

**A.** Help the client see the slip as a lapse to learn from

**B.** Recommend moving the client to a more intensive level of care

**C.** Review the wedding situation to plan for future high-risk events

**114.** A client has taken buprenorphine for 8 months and is stable at work and home. In a telehealth session, the client mentions that people at a mutual-help meeting call medication 'not real recovery' and says, 'Maybe I should just stop it next week.' What should the social worker do **FIRST**?

**A.** Accept the plan and schedule weekly check-ins during the stop

**B.** Ask what the client heard and how they now see the medication

**C.** Suggest a mutual-help group that welcomes members on medication

**D.** Help the client meet with the prescriber before any change

**115.** In a primary care clinic, a client who has drunk about a fifth of liquor daily for years says they will quit alone this weekend. Last year, the client had a seizure two days after stopping suddenly. The client asks the social worker for tips to get through the weekend. What should the social worker do **FIRST**?

**A.** Plan coping strategies and supports to use over the weekend

**B.** Explore what is motivating the client to quit drinking now

**C.** Arrange medically supervised withdrawal before the client stops

**116.** Which treatment has the **MOST** consistent research support for helping adults stop using cocaine or methamphetamine?

**A.** Contingency management plus community reinforcement

**B.** Cognitive-behavioral relapse prevention therapy

**C.** Twelve-step facilitation with peer sponsorship

**D.** Supportive-expressive psychodynamic therapy

**117.** In an emergency department, a social worker meets with a 52-year-old patient who fell on icy steps after a party. A screening questionnaire shows 5 or 6 drinks on most weekend nights, and brief follow-up questions point to at-risk drinking rather than an alcohol use disorder. What should the social worker do **NEXT**?

**A.** Discuss how alcohol relates to the injury and advise cutting down

**B.** Give the patient a handout on low-risk drinking limits

**C.** Complete a full biopsychosocial assessment before discussing alcohol

**118.** A client in early recovery from alcohol use disorder starts taking a new route home from work 'to beat traffic.' The route passes a liquor store the client used to visit daily. The next week, the client stops there 'just to buy a soda' and drinks again that night. Which relapse prevention concept **MOST** likely describes the route change?

**A.** An apparently irrelevant decision

**B.** The abstinence violation effect

**C.** Positive outcome expectancies

**D.** A lapse

**119.** A client in outpatient treatment for alcohol use disorder wants peer support between sessions. After attending two 12-step meetings, the client says, 'I'm an atheist. All the talk about a higher power made me feel like I didn't belong.' What is the **MOST** appropriate response?

**A.** Encourage the client to keep attending and define a higher power freely

**B.** Share secular mutual-help options, such as SMART Recovery

**C.** Suggest relying on counseling sessions alone for now

**120.** A case manager on a community mental health team makes a home visit to a client with schizophrenia. The client says the neighbors are poisoning the tap water. Six sealed water jugs are stacked by the door, and the refrigerator is empty. Which progress-note entry **BEST** documents the visit?

**A.** Client appeared paranoid about the water and hoarded jugs; refrigerator empty, likely off medications

**B.** Client said neighbors are poisoning the tap water; six jugs by door; refrigerator empty

**C.** Client's fear that neighbors use poison seemed worse than last month; self-care looked poor

**121.** In a SOAP progress note, which section holds the social worker's clinical interpretation of what the client reported and what the worker observed?

**A.** Subjective

**B.** Objective

**C.** Plan

**D.** Assessment

**122.** A client with schizophrenia has been hospitalized 4 times in the past year and is staying in a shelter. The client misses most monthly office visits with an outpatient case manager and stops medication between admissions. The client says they want to stay out of the hospital. What is the **MOST** appropriate step for the case manager?

**A.** Increase to weekly office visits with a reminder call before each

**B.** Ask the psychiatrist to consider a long-acting injectable medication

**C.** Refer the client to an assertive community treatment team

**123.** During a home visit for a job placement program, a client chats about weekend plans and mentions that a sibling who lives in another state is undocumented. The client's service plan centers on finding warehouse work. Which way of handling the sibling's status in the case record is **BEST**?

**A.** Record the sibling's status to keep the social history complete

**B.** Leave the sibling's status out of the case record

**C.** Ask the client's permission before writing the status down

**124.** A social worker providing case management helped a client with schizoaffective disorder schedule a first primary care visit for high blood pressure. The appointment was last Tuesday. The social worker has not heard from the client or the clinic since. What should the social worker do **NEXT**?

**A.** Record the primary care linkage as complete

**B.** Begin the dental referral that is next on the plan

**C.** Check with the client about whether the visit happened

**D.** Ask the clinic to send the notes from the visit

**125.** A client with schizophrenia lives in a supportive housing program. The landlord calls the social worker and says the client yelled at neighbors in the hallway on several nights this week. Staff have not seen this happen. How should the social worker record the call?

**A.** Record that the client has been aggressive toward neighbors

**B.** Record the landlord's report and name the landlord as its source

**C.** Leave out the landlord's secondhand report

**126.** A social worker recommends that a client with schizophrenia join an intensive outpatient program after a recent rise in hearing voices. The client declines because the program hours conflict with a valued job. After going over the likely benefits and dangers, the social worker judges that the client understands them. Which entry **BEST** documents this decision?

**A.** The recommendation, risks discussed and the client's reasons

**B.** That the client was noncompliant with the treatment plan

**C.** A signed statement releasing the agency from responsibility

**D.** That the client declined the referral

**127.** In a supportive housing program, two roommates have argued for weeks over the shared kitchen, and each now demands that the other move out. The social worker meets with both, sets ground rules for the talk, and has each describe the conflict. What should the social worker do **NEXT**?

**A.** Ask each roommate what they need from the kitchen

**B.** Propose a schedule that splits kitchen hours evenly

**C.** Ask each roommate to propose a fair solution

**128.** In the Thomas-Kilmann model of conflict, which mode combines high assertiveness with high cooperativeness?

**A.** Compromising

**B.** Accommodating

**C.** Competing

**D.** Collaborating

**129.** At a drop-in center for adults experiencing homelessness, two clients begin shouting about a missing backpack. One stands inches from the other with fists clenched, and other clients have stopped to watch. What should the social worker do **FIRST**?

**A.** Bring both to a table to give their sides

**B.** Separate them calmly and speak with each apart

**C.** Warn both that more shouting will get them barred

**130.** A social worker at a family services agency is helping separated co-parents settle a dispute over their child's weekday schedule. In the second joint session, one parent agrees to each proposal within seconds whenever the other parent's voice rises and keeps glancing at the door. After the session, that parent asks whether anything said privately would be shared with the other parent. What should the social worker do **FIRST**?

**A.** Ask both parents in the next joint session whether either feels unsafe

**B.** Switch to separate-room sessions with each parent from now on

**C.** Meet alone with that parent to screen for abuse and fear

**131.** During a video session, the social worker glances several times at case records on a second monitor. The client stops mid-sentence and says, 'You keep looking off to the side. I guess I'm pretty boring.' What is the social worker's **BEST** response?

**A.** Ask what leads the client to see themselves as boring

**B.** Explain that the notes help track the client's goals

**C.** Acknowledge reading notes and set them aside

**132.** On a stroke rehabilitation unit, a 68-year-old patient has expressive aphasia. The patient follows conversation and nods appropriately but struggles to find words. The social worker needs the patient's preferences for discharge. The patient's adult child offers to answer for the patient. What is the **MOST** appropriate way for the social worker to proceed?

**A.** Ask the adult child to describe the patient's preferences

**B.** Use short, simple questions and allow ample time to answer

**C.** Finish the patient's sentences when the patient pauses

**D.** Give the patient written questions to answer later

**133.** In an integrated primary care clinic, a client with diabetes and depression has just agreed to a plan for daily walks and a mood log. While agreeing, the client speaks softly, looks at the floor, and rubs the back of their neck. What should the social worker do **NEXT**?

**A.** Ask which part of the plan might be hardest to do

**B.** Ask the client why they seem unsure about the plan

**C.** Give the client a written copy of the agreed plan

**134.** In Carl Rogers' person-centered approach, which term describes a helper whose outward responses fit what the helper is actually experiencing?

**A.** Unconditional positive regard

**B.** Accurate empathy

**C.** Congruence

**D.** Immediacy

**135.** During a first meeting at a community agency, a social worker leans forward in an open posture to show interest. Each time the social worker leans in, the client slides back in the chair, folds their arms, and gives shorter answers. How should the social worker respond?

**A.** Shift to less personal questions for now

**B.** Point out the folded arms and ask what they mean

**C.** Sit back and allow more personal space

**136.** Which term describes the vocal qualities that accompany spoken words, such as tone, pitch, pace and volume?

**A.** Kinesics

**B.** Paralanguage

**C.** Proxemics

**D.** Haptics

**137.** A probation officer sends a client to anger management after an arrest for shoving a partner. The client says the partner 'pushes my buttons.' The client also checks the partner's phone each night and controls all the household money. Coworkers describe the client as calm and easygoing. Which plan is **MOST** appropriate?

**A.** Enroll the client in a general anger management group

**B.** Refer the client to a partner-abuse intervention program

**C.** Offer joint sessions with the partner to improve communication

**138.** A teacher refers a 10-year-old to the school social worker for anger management. The student has thrown books and torn up worksheets several times this month. The teacher's log shows that most outbursts happen when the class reads aloud, and the student's reading grades have dropped all year. What should the social worker do **FIRST**?

**A.** Enroll the student in a small anger management skills group

**B.** Teach the student a calm-down routine to use during reading

**C.** Request an evaluation of the student's reading skills

**D.** Set up a daily reward chart for calm behavior

**139.** A client in an outpatient anger management group has started taking time-outs during arguments by walking out of the room. The client reports that the adult sibling they live with follows them, shouting, 'Don't walk away from me,' and the argument gets worse. The client says the sibling would be willing to talk about it at a calm time. How can the social worker **BEST** help the client handle these arguments?

**A.** Agree on a time-out signal and return plan with the sibling

**B.** Announce a return time to the sibling while walking out

**C.** Leave without a word to the sibling until fully calm

**140.** A client asks an employee assistance social worker for help with anger. For 3 months, a coworker has made slurs about the client's background, and two other staff members have heard them. The client has not acted aggressively but lies awake furious and wants the remarks to stop. How should the social worker **BEST** help with the anger?

**A.** Teach slow breathing for the moments the coworker speaks

**B.** Help the client plan an assertive report through work channels

**C.** Help the client consider other reasons for the coworker's remarks

**141.** In family sessions, whenever the parents try to set a curfew, their 14-year-old and one parent join forces against the other parent, and the discussion collapses. Which intervention is **MOST** consistent with structural family therapy?

**A.** Construct a three-generation genogram with the family

**B.** Coach the parents to decide the curfew together in session

**C.** Direct the parents to argue about the curfew each evening at home

**142.** Parents seek help for their teen's school refusal. The social worker meets with the parents alone, stays calm while they argue, asks each parent what they think rather than feel about the conflict, and avoids siding with either one. Which family therapy model is the social worker **MOST** likely using?

**A.** Bowen family systems therapy

**B.** Structural family therapy

**C.** Strategic family therapy

**D.** Experiential family therapy

**143.** A family is referred for in-home therapy after a 15-year-old's school suspension. At the first visit, a parent sits with arms crossed and says, 'The last provider told us everything we were doing wrong.' The social worker uses structural family therapy. What should the social worker do **FIRST**?

**A.** Ask the family to act out a recent argument in session

**B.** Seat the parents together and the teen across the room

**C.** Learn the family's routines and language and follow their lead

**144.** Parents of a 16-year-old report that the teen has stopped doing homework. Every evening they remind, lecture, and check the teen's work, and each week the teen does less. Which intervention is **MOST** consistent with the Mental Research Institute (MRI) brief therapy model?

**A.** Help the parents stop the nightly reminders and try something new

**B.** Build a three-generation genogram of the family's school achievement

**C.** Have the parents and teen enact a homework argument in session

**D.** Ask the teen when homework gets done without any reminders

**145.** In a family session about a 12-year-old who refuses to go to school, the social worker asks the older sibling, 'When your grandparent and your parent argue about getting to school, who steps in first? Who seems most worried afterward?' Which technique is the social worker **MOST** likely using?

**A.** Process questioning

**B.** Circular questioning

**C.** Tracking

**D.** Relative influence questioning

**146.** During a family session about a teen's recent arrest, one parent speaks in a flat, even tone. The parent cites studies on teen brain growth and says feelings 'only cloud the issue.' According to Satir, which communication stance is this parent **MOST** likely showing?

**A.** Blaming

**B.** Super-reasonable

**C.** Placating

**D.** Irrelevant

**147.** A 38-year-old client moved across the country 10 years ago and has had almost no contact with their family since. The client reports a series of intense, short romantic relationships and wants help with them. The social worker uses Bowen family systems therapy. Which intervention is **BEST**?

**A.** Invite the parents to a session to confront them about the past

**B.** Explore the pattern in the client's romantic relationships

**C.** Coach the client to plan brief visits and stay calm during them

**148.** On an inpatient unit, a 24-year-old patient with schizophrenia is in their second admission this year and will return home to live with their parents. The parents describe frequent criticism and say, 'We watch everything they do.' The patient agrees to family involvement. Which family intervention is **BEST** for reducing the risk of relapse?

**A.** Give the parents one education session before discharge

**B.** Refer the patient's parents to a family peer support group

**C.** Start months-long psychoeducation with the parents and patient

**D.** Coach the parents to examine patterns in their own families

**149.** A 15-year-old on probation has two new arrests, skips school, and spends time with peers who steal cars. Six months of weekly clinic-based family therapy have not changed the pattern. The probation officer raises residential placement, and the family wants the teen to stay home. What should the social worker do **NEXT**?

**A.** Recommend placement in a residential treatment center

**B.** Add school-based mentoring alongside the current family therapy

**C.** Refer the family to a multisystemic therapy program

**150.** On a home visit, a single parent in treatment for depression reports returning to work and feeling better. During the session, the 12-year-old answers questions put to the parent and tells the younger siblings when to sit down. The parent looks to the 12-year-old before speaking. Using a structural approach, which response is **BEST**?

**A.** Thank the 12-year-old for helping and let them keep guiding the siblings

**B.** Arrange individual sessions to address the 12-year-old's stress

**C.** Pause family sessions until the parent's depression fully remits

**D.** Ask the parent to direct the siblings while the 12-year-old rests

**151.** At the first family session, the parents of a 16-year-old who is skipping school say, 'You're the expert. Just tell us what to do and fix our kid.' The social worker replies that only the family can decide what they want to change and how hard they will work at it. This response **MOST** likely reflects which concept?

**A.** Battle for structure

**B.** Restraining change

**C.** Battle for initiative

**D.** Positive connotation

**152.** A 14-year-old with anorexia nervosa is discharged home after medical stabilization, with weekly weight checks by a pediatrician. The family begins family-based treatment with the social worker. In the second session, the parents say they want to let their teen choose and plan their own meals so that eating does not become a battle. Which response is **BEST**?

**A.** Coach the parents to plan and supervise every meal for now

**B.** Support the parents' plan to let the teen choose meals

**C.** Start individual sessions on the teen's need for control

**153.** A family in which three generations share a home seeks help for a 10-year-old who ignores household rules. By family tradition, the grandparent makes major household decisions. The parent works nights, and in session the parent and grandparent contradict each other's rules in front of the child. Using a structural approach, what is the **MOST** appropriate intervention?

**A.** Strengthen the parent's authority by limiting the grandparent's role

**B.** Help the parent and grandparent agree on rules as a team

**C.** Explore the child's feelings about the two sets of rules

**154.** A parent has returned home after a year-long military deployment. In family sessions, the partner and two teens say everyone feels distant, but they struggle to put the feeling into words. The social worker uses Satir's experiential approach. Which intervention is **MOST** consistent with this approach?

**A.** Prescribe that the family keep its distance this week

**B.** Ask circular questions about who notices the distance first

**C.** Build a three-generation genogram of military service

**D.** Have members place one another to show closeness

**155.** During in-home structural family therapy for a 13-year-old's aggression, one parent asks to speak privately. The parent says the other parent controls the money, checks the parent's phone, and pushed the parent into a wall last week. The parent asks the social worker to keep family sessions going, fearing that stopping them would anger the other parent. What is the **BEST** response?

**A.** Connect the parent with a domestic violence advocate to plan safely

**B.** Continue family sessions as asked, keeping the focus on the teen

**C.** Stop family sessions and explain the reason to the family

**D.** Ask the other parent privately about the pushing

**156.** A family therapy team meets with parents, a grandparent, and a 16-year-old who has stopped eating meals with the family. At the end of the session, the team says the teen's refusal keeps the parents united and the grandparent's frequent advice keeps everyone close. Each member's behavior is described as a sacrifice for the family's unity. Which technique is the team **MOST** likely using?

**A.** Prescribing the symptom

**B.** Unbalancing

**C.** Circular questioning

**D.** Positive connotation

**157.** Parents bring their 9-year-old to family therapy for tantrums. The family used a behavior chart consistently for 4 weeks with no change. In sessions, the child starts acting out whenever the parents begin to disagree about money, and the parents then stop arguing and work together to calm the child. Both parents say they 'haven't been a team lately.' What is the **BEST** intervention at this point?

**A.** Coach the parents to stay calm and united during tantrums

**B.** Meet with the parents as a couple about their conflict

**C.** Tell the family that the tantrums protect the parents from fighting

**158.** In a family session, one parent makes every decision. This parent talks over the other parent, who rarely speaks. For a few minutes, the social worker takes the quiet parent's side in a fight about bedtime. The goal is to shift who holds power in the family. Which structural technique is the social worker using?

**A.** Joining

**B.** Enactment

**C.** Boundary making

**D.** Unbalancing

**159.** A client asks for family therapy because of escalating conflict with their partner, who refuses to attend any sessions. The social worker practices from a Bowen family systems approach. The client wants to start now and says, 'I'm the one who keeps losing it.' What is the **BEST** way to proceed?

**A.** Wait to begin until the partner agrees to come to sessions

**B.** Refer the client for individual psychodynamic treatment

**C.** Coach the client alone to lower their own reactivity

**160.** Which family therapy model focuses on hidden loyalties and on what each generation owes and is owed?

**A.** Contextual therapy

**B.** Bowenian therapy

**C.** Symbolic-experiential therapy

**D.** Milan systemic therapy

**161.** An inpatient social worker is planning discharge for an adult with schizophrenia. The patient has had three admissions this year, each after missing the first outpatient appointment. The patient is stable and agrees to outpatient care. Which discharge step is **BEST**?

**A.** Give the patient a list of clinics to call after discharge

**B.** Extend the stay until the patient shows full insight

**C.** Schedule a clinic visit within 7 days and arrange a warm handoff

**162.** A client with bipolar disorder was discharged from a psychiatric unit 3 days ago after a suicidal crisis. The client does not arrive for the first appointment at the outpatient clinic and has not called. What should the outpatient social worker do **FIRST**?

**A.** Phone the client today to check in and reschedule

**B.** Mail a letter offering a new appointment time

**C.** Close the referral under the clinic's no-show policy

**D.** Request a police welfare check at the client's home

**163.** On an inpatient psychiatric unit, a patient with bipolar disorder is set for discharge tomorrow after an overdose attempt. While reviewing aftercare plans, the patient says, 'I still have a month of pills at home. If I lose my job, I'll finish what I started.' The patient's employer has announced layoffs. When the social worker asks directly, the patient says this means taking all the pills to end their life. What should the social worker do **NEXT**?

**A.** Ask the treatment team to reassess discharge readiness today

**B.** Add removal of the stored pills to the discharge safety plan

**C.** Explore the patient's worries about losing the job

**164.** A 15-year-old has received wraparound services for 3 months to avoid residential placement. After a second school suspension, the school asks the team to recommend a residential program. The parent and youth want to keep working on the home-based plan. Which response by the wraparound facilitator is **MOST** consistent with wraparound principles?

**A.** Ask the school to hold its request until the plan's next review

**B.** Add a separate anger management program while the plan continues

**C.** Reconvene the team with the family to revise the plan

**165.** A 78-year-old patient is admitted to a hospital medical unit after a stroke that weakened one side of the body. The patient lives with a partner in a two-story house. The medical team has not set a discharge date. What should the social worker do **FIRST**?

**A.** Ask the physician for an expected discharge date to guide planning

**B.** Give the patient's partner a list of inpatient rehabilitation facilities

**C.** Request a home safety evaluation from occupational therapy

**D.** Meet with the patient and partner to assess needs and supports

**166.** On a medical unit, a patient being treated for pneumonia insists on leaving today, before the planned discharge, to attend a parent's funeral in another city. The physician has explained the risks, the patient has decision-making capacity, and the patient still plans to leave. What is the **MOST** appropriate action by the social worker?

**A.** Explain that insurance may deny payment if the patient leaves early

**B.** Arrange follow-up care and prescriptions before the patient leaves

**C.** Explore whether the patient could delay the trip by one day

**167.** A patient with schizophrenia, admitted after a suicide attempt, is ready for discharge. The team finds the patient no longer at imminent risk and able to make treatment decisions. The patient declines any outpatient appointment, saying the hospital stay was enough, and plans to return home. Which follow-up plan is **MOST** appropriate?

**A.** Delay discharge until the patient agrees to an appointment

**B.** Give a list of local clinics and close the case at discharge

**C.** Offer to send brief, caring check-in messages after discharge

**168.** A client with schizophrenia was discharged from an inpatient unit 4 days ago to supported housing and missed the first clinic appointment yesterday. The social worker has called twice this afternoon with no answer. Housing staff report the client has not left the room in 2 days, has refused meals, and can be heard shouting when no one else is there. What should the social worker do **NEXT**?

**A.** Notify the outpatient prescriber about the missed appointment

**B.** Request a mobile crisis team visit to the client today

**C.** Ask housing staff to urge the client to call the clinic

**D.** Schedule a home visit with the client for later this week

**169.** Two days after discharge from a psychiatric unit, a client with bipolar disorder tells the social worker on a scheduled follow-up call that the pharmacy would not fill the new mood stabilizer. The insurer needs a prior authorization form from the prescriber. The client has called the insurer twice, wants to restart the medication, and says, 'I've been on the phone all day. I don't have the energy to keep calling.' What should the social worker do **FIRST**?

**A.** Move the client's outpatient appointment to an earlier date

**B.** Coach the client to ask the prescriber's office for the form

**C.** Call the prescriber's office today about the form

**D.** Ask how the client feels about taking a mood stabilizer

**170.** In the third meeting of a closed anger management group, several members complain that the attendance rules are unfair and question the leader's competence. The social worker calmly restates the group agreements, but the complaints continue. What should the social worker do **NEXT**?

**A.** Restate the rules firmly and move on to the planned skill

**B.** Invite the group to discuss its reactions to the rules

**C.** Meet outside the group with the members who complain

**171.** In week 4 of a closed support group for adults with chronic pain, one member talks at length each session about medical appointments. Others have started checking their phones, and two arrived late last week. No one has interrupted the talkative member. What should the social worker do **FIRST**?

**A.** Meet privately with the talkative member after group

**B.** Ask the group how it has been sharing session time

**C.** Set a fixed time limit for each member's turn

**D.** Ask the member to save medical updates for later

**172.** In a residential treatment group for adolescents, the social worker announces that a planned outing is canceled because of staffing. Members begin blaming one quiet member, saying the outing was canceled because of that member's 'drama,' and the comments grow harsher. Which response is **BEST**?

**A.** Stop the blaming and name the group's shared frustration

**B.** Explain to the group the staffing reasons for canceling

**C.** Coach the blamed member privately on coping with peers

**173.** As an outpatient substance use group begins, a member arrives smelling of alcohol, slurring words, and says they drove to the session. The member asks to stay and talk about the relapse, and several members say they want to help. What should the social worker do **FIRST**?

**A.** Let the member stay and discuss the relapse with the group

**B.** Discharge the member from the program for drinking

**C.** Excuse the member from group and arrange a safe ride

**174.** In month 3 of an outpatient process group, a member often makes jokes when others share painful stories. Two members tell the member that the jokes leave them feeling brushed off. Over the next weeks, the member says a partner and coworkers have complained of the same habit, and starts responding seriously in group. Which therapeutic factor is **MOST** likely at work?

**A.** Interpersonal learning

**B.** Catharsis

**C.** Universality

**D.** Imitative behavior

**175.** A social worker is screening applicants for a 12-week interpersonal process group at a community mental health center. One applicant with schizophrenia stopped taking medication a month ago, hears critical voices most of the day, and believes neighbors are watching through the walls. The applicant hopes the group will help them make friends. What is the **MOST** appropriate action?

**A.** Admit the applicant and pair them with a peer buddy

**B.** Defer group entry and link the applicant to psychiatric care

**C.** Explore what the applicant hopes to gain from the group

**176.** In a closed school group for 9th graders whose parents are divorcing, three members who share a lunch period start sitting together. During sessions, they trade glances and laugh quietly when others speak, and two members who used to share have gone quiet. Which response is **BEST**?

**A.** Name the pattern aloud and invite members to discuss it

**B.** Meet with the three members privately and ask them to stop

**C.** Wait a session to see whether the pattern fades

**177.** A closed 16-week grief group for older adults meets at a senior center. In week 12, the social worker reminded members that the group ends in 4 weeks. In week 14, two members miss without notice, and a member who had been sleeping well says the sleep problems are back. What should the social worker do **NEXT**?

**A.** Offer to extend the group by 4 more weeks

**B.** Refer the member with sleep problems for evaluation

**C.** Invite members to talk about the approaching last session

**178.** A social worker is asked to start a daily group on an acute inpatient psychiatric unit. Most patients stay 4 to 7 days, and new patients arrive every day. Which group format fits this unit?

**A.** Closed group meeting weekly for 8 sessions

**B.** Open group with each session complete in itself

**C.** Open group whose themes build week to week

**179.** In the working stage of a process group at a veterans' center, one member describes feeling like a stranger at home. Another member turns to the social worker and says, 'I think what they mean is they feel invisible to their family.' What is the **MOST** appropriate response?

**A.** Ask the member to say that directly to the other member

**B.** Agree and add the social worker's own reading of it

**C.** Thank the member and move on to the next person

**D.** Restate the speaker's feelings in the leader's own words

**180.** In a hospital support group for adults newly diagnosed with cancer, a member in the second week of chemotherapy listens as another member describes finishing treatment 5 years ago and returning to work. Afterward the newer member says, 'If they got through it, maybe I can too.' Which therapeutic factor is **MOST** likely at work for the newer member?

**A.** Instillation of hope

**B.** Universality

**C.** Imitative behavior

**D.** Imparting information

**181.** In week 5 of a middle school group for students who have a parent in prison, a member's grandparent calls the school social worker. At lunch, another group member told classmates about the member's parent's charges, which the member had shared in group. The member is upset and now refuses to return. What should the social worker do **FIRST**?

**A.** Meet with the member to hear the impact and plan next steps

**B.** Address the breach at the next session and restate group rules

**C.** Meet first with the member who shared the information

**182.** In a caregiver support group at a senior center, one member has attended 5 sessions, listens closely, and speaks only when asked a direct question. During a break, the member explains that in their family, talking about relatives' problems with outsiders brings shame, but hearing the others 'helps a lot.' What is the **MOST** appropriate response by the social worker?

**A.** Use go-around turns in which any member may pass

**B.** Invite the member to share one struggle at the next session

**C.** Suggest individual counseling as a better fit for the member

**183.** In week 4 of a 10-week hospice bereavement group, a member whose partner died 3 months ago stays after the session. In response to the social worker's questions, the member says they now drink most of a bottle of wine every night to sleep, have missed 6 workdays this month, and their adult child has asked them to cut back. What should the social worker do **NEXT**?

**A.** Help the member arrange a substance use evaluation and stay in group

**B.** Suggest a bedtime routine and check on the drinking next week

**C.** Explore further how the loss has affected the member's sleep

**D.** Ask the member to pause group until the drinking is under control

**184.** In the first session of a closed outpatient group for adults coping with anxiety, introductions begin. One member starts describing childhood sexual abuse in vivid detail and is crying. Other members look at the floor, and one moves their chair back. What should the social worker do **FIRST**?

**A.** Allow the member to finish the account without interruption

**B.** Gently pause the member and suggest returning to it later

**C.** Talk privately with the member after the session ends

**185.** A social worker at a rural aging services agency finds that several older adult clients miss medical appointments because the county has no public bus service. The county has several active congregations, a senior center and a hospital system. The agency director proposes buying a van and hiring a driver. What should the social worker do **FIRST**?

**A.** Write a grant proposal to purchase an agency van

**B.** Ask county commissioners to fund a public bus route

**C.** Inventory existing transportation options and possible partners

**D.** Arrange taxi vouchers for clients one appointment at a time

**186.** A hospital emergency department social worker refers patients treated for opioid overdose to a community clinic that offers medication for opioid use disorder. For 2 years, a clinic nurse personally scheduled each referred patient. Since that nurse left 3 months ago, 7 of 9 referred patients never received an intake. Clinic leaders say they want the partnership to work. What is the social worker's **BEST** response?

**A.** Develop a written referral agreement with clinic leadership

**B.** Find a new staff member at the clinic to schedule intakes

**C.** Phone the clinic to book each patient's intake before discharge

**187.** A community mental health agency sees almost no clients from a recently resettled refugee community, although resettlement staff report high stress among its families. Translated flyers mailed last spring produced no calls. The community has an active mutual assistance association whose elected leaders families consult on many decisions. What should the social worker do **FIRST**?

**A.** Ask community leaders to hand out the translated flyers

**B.** Plan the outreach jointly with the community's leaders

**C.** Host an open house at the agency with interpreters

**188.** An 84-year-old client who lives alone in a rural area needs help with meals, rides to the store, and a daily check-in while recovering from a hip fracture. The regional aging agency's in-home program has an 8-month waitlist. The client has belonged to a local congregation for 50 years, and two neighbors already stop by now and then. The client wants to stay home. What is the social worker's **BEST** plan for the coming months?

**A.** Recommend a move to an assisted living facility

**B.** Add the client to the in-home program waitlist and check in monthly

**C.** Turn the client's informal contacts into a weekly helper schedule

**189.** At a community health center, a client asks the social worker for help 'being more assertive' with their partner. The client says the partner checks their phone, gets 'scary' when they disagree, and last month broke the client's phone during an argument. The client wants to practice speaking up at home. What should the social worker do **FIRST**?

**A.** Role-play a calm I-statement for the next disagreement

**B.** Assess the danger posed by the partner and plan for safety

**C.** Suggest joint sessions so both partners can practice

**190.** A client tells the social worker that a cousin keeps pressing them to co-sign a car loan. Each time, the client explains why they can't, the cousin argues with every reason, and the client ends up saying they will 'think about it.' Which assertiveness technique is **BEST** for the social worker to teach?

**A.** Calmly repeat the same short refusal each time

**B.** Give a stronger reason the cousin cannot dispute

**C.** Offer to help the cousin find a different co-signer

**191.** A client at a family services agency wants to ask their parent-in-law, who lives with the family, to stop giving the client's 6-year-old candy at bedtime. When the social worker offers a handout with an I-statement script, the client frowns and says, 'That's not how we talk to elders in my family.' What should the social worker do **FIRST**?

**A.** Coach the client to deliver the handout script as written

**B.** Help the client plan a respectful request made in private

**C.** Ask how the client would usually raise such a concern at home

**192.** A client with a lifelong pattern of giving in to others starts assertiveness training at a housing agency. In 3 days, the client meets with a landlord who is holding back a security deposit the client needs for a new apartment. Only one session fits before the meeting, and the client asks to use it to prepare. Which practice situation is **BEST** for that session?

**A.** The upcoming talk to get the money returned

**B.** A low-stakes request, such as returning a store item

**C.** A long-standing conflict with a relative

**193.** Before discharge from an inpatient unit, a client with social anxiety must call a landlord to ask about a lease. The client watches the social worker's confident manner and says, 'I could never sound as smooth as you.' How should the social worker **BEST** demonstrate the call?

**A.** Model the call smoothly and confidently from start to finish

**B.** Model the call with some hesitation and spoken coping self-talk

**C.** Describe each step aloud and let the client try the call first

**194.** In a middle school group on handling peer pressure, a 13-year-old accurately lists and acts out the refusal steps the social worker modeled. Later, the student says that when an older cousin turned down a vape at a party, friends mocked the cousin for weeks. Since then, the student laughs along when offered a vape. Which part of observational learning is **MOST** likely blocking use of the skill?

**A.** Attention

**B.** Retention

**C.** Reproduction

**D.** Motivation

**195.** In an outpatient substance use group, a member who is 3 weeks sober dreads a coworker's retirement party where alcohol will be served. When the social worker offers to demonstrate refusing a drink, the member says, 'Easy for you, you've never been in my shoes.' Another member, sober for 2 years, has handled many similar work events. Which approach to teaching drink refusal is **MOST** effective?

**A.** Have the social worker demonstrate the refusal anyway

**B.** Give the member a list of refusal phrases to rehearse

**C.** Invite the member in longer recovery to demonstrate it

**196.** In a parenting group for families working toward reunification, the social worker models giving a 4-year-old a calm, specific instruction. The next week, a parent accurately lists every step of the demonstration and says they want to use it. The parent reports that at visits the words come out rushed and harsh. What should the social worker do **NEXT**?

**A.** Model the instruction again, more slowly this time

**B.** Offer the parent a reward for using the skill at visits

**C.** Have the parent practice the instruction with coaching

**D.** Explain the reasons the calm instruction works

**197.** During a couples session about household spending, one partner rolls their eyes and says in a mocking tone, 'Oh, here comes the financial genius again.' Which of Gottman's Four Horsemen does this behavior show?

**A.** Criticism

**B.** Defensiveness

**C.** Stonewalling

**D.** Contempt

**198.** A couple asks for help with frequent arguments. In the individual screening session, one partner says the other checks their phone daily, controls all the money, and once blocked the door to keep them from leaving an argument. The client says they fear the partner's reaction to anything said in joint sessions but wants to 'work on the relationship.' What is the **BEST** response?

**A.** Begin joint sessions with firm ground rules for respectful talk

**B.** Explore the client's hopes for the relationship in more depth

**C.** Refer the client to separate services addressing relationship abuse

**199.** In emotionally focused couples therapy, during which stage does the social worker help partners see their pattern of pursuing and withdrawing as a shared enemy rather than either partner's fault?

**A.** Cycle de-escalation

**B.** Restructuring interactions

**C.** Consolidation

**200.** During a video session, one partner raises a complaint about household chores. The other partner goes silent, looks away from the camera, and says, 'My heart is pounding. I can't do this right now.' The social worker uses the Gottman Method. What should the social worker do **FIRST**?

**A.** Ask the quiet partner to share their view now

**B.** Call a break of at least 20 minutes to self-soothe

**C.** Teach both partners a structured listening exercise

**201.** At intake for couples therapy, one partner says they have nearly decided to divorce and came only because the other partner pleaded. The other partner wants to save the marriage and asks to start work on communication right away. Both agree to a few sessions. Which approach is **BEST**?

**A.** Begin couples therapy focused on communication skills

**B.** Refer the partner leaning toward divorce for individual therapy

**C.** Help both partners reach clarity about their next step

**202.** A couple seeks help for frequent arguments about parenting. In separate screening interviews, each partner describes one argument 8 months ago in which both pushed each other, and each takes responsibility for their part. Both say they feel safe at home, make money and social decisions together, and want to stay together. What is the **MOST** appropriate plan?

**A.** Refer each partner to a separate violence intervention program first

**B.** Begin couples sessions once both sign a no-violence agreement, with no further screening

**C.** Begin couples sessions that include periodic individual safety check-ins

**203.** During a video couples session, one partner opens a talk about chores by saying, 'You never do anything around here unless I nag you. You only think about yourself.' The other partner sits back and calmly starts to explain their week. The social worker uses the Gottman Method. What is the **BEST** intervention?

**A.** Ask the listening partner to repeat back what they heard

**B.** Suggest a 20-minute break so both partners can calm down

**C.** Coach the speaker to restate the complaint as a feeling and need

**D.** Explore how chores were divided in each partner's childhood home

**204.** A client plans to ask a supervisor at work for a schedule change to care for a relative. The client says they freeze whenever they speak with people in authority. Which approach is **BEST** for helping the client prepare during the session?

**A.** Write a script for the client to read to the supervisor

**B.** Rehearse the request with the social worker acting as supervisor

**C.** Offer to call the supervisor and explain the client's needs

**205.** In a role-play, which technique has the client portray the other person in a conflict while the social worker portrays the client?

**A.** Role reversal

**B.** Doubling

**C.** Mirroring

**D.** Empty chair

**206.** In a residential program, a 15-year-old practices asking staff for an extra phone call instead of demanding one. In the first role-play, the teen uses a calm voice and says 'please,' but stands very close and talks over the staff member's reply. What should the social worker do **NEXT**?

**A.** Praise what worked, coach one change, and run it again

**B.** Model the whole request again with the right distance

**C.** Assign the teen to try the request with staff later today

**207.** A client with schizophrenia in a supported employment program has had five job interviews without an offer. The client says the interviews 'went fine' and cannot say what might have gone wrong. What should the social worker do **FIRST** to help the client improve?

**A.** Teach a standard set of tips for answering interview questions

**B.** Ask the client to role-play an interview while the worker observes

**C.** Help the client request feedback from the interviewers

**208.** In a jail reentry program, a client says, 'Things got messed up with my probation officer last week.' The social worker responds, 'When you say things got messed up, what happened?' Which listening skill is the social worker using?

**A.** Paraphrase

**B.** Clarification

**C.** Interpretation

**209.** In a first session at a community mental health center, an adult client talks rapidly about eviction worries, a recent breakup, poor sleep, and conflict with a sibling, jumping between topics. Twenty minutes remain in the session. What is the **BEST** next move to begin setting priorities?

**A.** Focus the rest of the session on the eviction threat

**B.** Reflect how overwhelmed the client seems to feel right now

**C.** Ask closed questions to cover each problem in turn

**D.** Summarize the concerns and ask which one matters most

**210.** At an adult day program, a 79-year-old client says a new sleep medicine 'is working fine.' During a 30-minute meeting with the social worker, the client's head drops forward twice and the client loses track of questions. What should the social worker do **FIRST**?

**A.** Share what was observed and ask about daytime drowsiness

**B.** Contact the prescriber about a possible medication side effect

**C.** Accept the client's report and continue the meeting

**D.** Suggest the client take the medicine earlier in the evening

**211.** In a senior housing building, a resident tells the social worker, 'My kids keep pushing me to move to assisted living. I've lived in this apartment for 40 years.' Which response **BEST** shows accurate reflective listening?

**A.** 'Your children are worried and just want you to be safe.'

**B.** 'Why don't you want to consider assisted living?'

**C.** 'Leaving this home would mean losing a big part of your life.'

**D.** 'Many people your age feel that same way about moving.'

**212.** An 80-year-old client comes to a senior center for help with a benefits renewal form that is due tomorrow. For 20 minutes the client has talked about past jobs and old friends, and the form is still blank. The client seems to enjoy the visit and has no other appointments today. Which response is **MOST** helpful?

**A.** Keep listening warmly and let the client set the pace of the visit

**B.** Briefly reflect the stories' meaning, then return to the form

**C.** Explain that the form comes first and the stories can wait

**213.** During a video session from home, a 67-year-old client who lives alone suddenly begins slurring words. The social worker sees that one side of the client's mouth droops and that the client cannot lift their right arm to adjust the camera. When asked how they feel, the client says, 'I'm just tired. I'll lie down.' What should the social worker do **FIRST**?

**A.** Ask about the client's recent sleep and medication changes

**B.** Call the client's emergency contact to check on them

**C.** Suggest the client rest and reconnect in one hour

**D.** Request an ambulance to the client's home

**214.** A client in weekly outpatient therapy has begun calling the agency 4 to 6 times a day to talk through daily conflicts. The calls started 2 weeks ago, after months with no contact between sessions. They are not about emergencies, and front desk staff are struggling to keep up. What should the social worker do **FIRST**?

**A.** Return each call promptly to protect the working relationship

**B.** Set a scheduled check-in time and crisis plan with the client

**C.** Ask the client what has changed for them recently

**215.** In a supportive housing program, a client with schizoaffective disorder invites the social worker to a public gallery showing of the client's paintings. Showing work in public was a recovery goal the two set a year ago. The event is an evening opening at a community center. What is the **MOST** appropriate response?

**A.** Decline, explaining that attending would create a dual relationship

**B.** Ask the client to bring photos of the paintings to a session

**C.** Attend briefly after agreeing on boundaries with the client

**D.** Attend and join the client's celebration dinner afterward

**216.** A client often runs late, and the social worker has been holding firm on ending sessions on time. With 2 minutes left today, the client says their partner shoved them last night, and they are going home after the session. Another client is waiting. What is the **BEST** action?

**A.** Briefly extend the session to check the client's safety tonight

**B.** End on time and make this the first topic next session

**C.** Give a hotline number and end the session on time

**217.** During home-based services, a client has made comments about the social worker's body. At the last visit, the social worker clearly stated that such comments must stop. Today the client repeats them and briefly stands between the social worker and the door. What should the social worker do **FIRST**?

**A.** Restate the limit firmly and continue with the visit

**B.** End the visit and review safety with the supervisor

**C.** Explore what the comments mean to the client

**218.** In Landreth's ACT model for setting limits in child-centered play therapy, what does the social worker do **FIRST**?

**A.** State the limit on the behavior

**B.** Offer an acceptable alternative

**C.** Acknowledge the feeling behind the behavior

**D.** Name the consequence if it continues

**219.** A client with a trauma history yelled profanities at the front desk staff after a long wait. In the next session, the client explained that crowded waiting rooms feel unsafe, and the clinic began offering a quiet room for waiting. This week, after a 5-minute delay in the quiet room, the client swore at a receptionist again. What should the social worker do **NEXT**?

**A.** Explore again what waiting brings up for the client

**B.** Set clear conduct expectations and state what happens next time

**C.** Arrange for the client to skip the waiting area entirely

**220.** A crisis stabilization program offers up to 6 sessions. A client seen after a sudden job loss and panic attacks is, by session 5, sleeping again, applying for jobs, and using a coping plan. The client now asks to work on feelings of worthlessness that go back to childhood. What is the **MOST** appropriate step?

**A.** Extend crisis sessions to address the childhood pattern

**B.** Close the case now that functioning has returned

**C.** Refer for ongoing therapy and set a follow-up contact

**221.** A crisis line social worker takes a call from an adult who is crying and talking fast. The worker asks about suicide and violence; the caller has no such thoughts, has no weapons, and is staying at a friend's apartment. The caller has now calmed, says they feel heard, and wants help sorting out 'everything falling apart.' What should the social worker do **NEXT**?

**A.** Help the caller brainstorm options for the coming week

**B.** Teach the caller a slow-breathing exercise to settle down

**C.** Ask what happened today that prompted reaching out

**D.** Work out a written step-by-step plan for tonight

**222.** Two hours after an apartment fire, a parent sits in a hospital emergency department while nurses treat the parent's two children for minor injuries. The parent keeps repeating, 'I don't know what to do,' stares at the floor, and cannot answer when asked whom to call. What is the **BEST** approach for the social worker?

**A.** Give brief, concrete directions for the immediate next steps

**B.** Reflect the parent's feelings and wait for the parent to choose

**C.** Complete a full psychosocial history for the record

**223.** A high school social worker completes a suicide risk assessment with a 16-year-old who reported fleeting thoughts that 'it would be easier not to wake up.' The thoughts come at night when the student worries about grades, and the student names a parent and a coach as supports. An assistant principal asks the worker to have the student sign a no-harm agreement before returning to class. What is the **MOST** appropriate step?

**A.** Have the student sign the agreement the administrator requested

**B.** Build a written safety plan with the student and family

**C.** Send the student back to class and check in next week

**224.** A crisis that arises when a normal, expected life change, such as becoming a parent or retiring, overwhelms a person's usual coping is which type?

**A.** Situational crisis

**B.** Existential crisis

**C.** Environmental crisis

**D.** Developmental crisis

**225.** On a text-based crisis line, a person writes that they swallowed a full bottle of a prescription sleep medication 30 minutes ago and now feel 'calm and ready.' Earlier in the chat they gave their home address. When the social worker raises getting help, the person replies, 'Please don't send anyone.' What should the social worker do **FIRST**?

**A.** Send emergency medical services to the person now

**B.** Ask what happened today that led to taking the pills

**C.** Work with the person on a safety plan for tonight

**226.** At a public benefits office, a client whose food assistance was denied is pacing near the front desk, shouting that staff 'never help anyone.' The client's hands are empty, and other people in the lobby have moved away. The social worker approaches. What should the social worker do **FIRST**?

**A.** Tell the client security will be called unless they sit down

**B.** Speak calmly from a safe distance and offer a quieter room

**C.** Ask the client to explain why the application was denied

**D.** Ask security staff to escort the client out of the building

**227.** Late at night, an 8-year-old waits at a child welfare office after being removed from home. The required investigative interview is already done. The child is crying and keeps asking, 'Where am I going to sleep? Who's going to feed my dog?' What would be **MOST** helpful for the social worker to do?

**A.** Teach the child a slow-breathing exercise to calm down

**B.** Promise the child they will go home within a few days

**C.** Tell the child plainly what will happen tonight, step by step

**228.** Ten days after a fatal crane accident, a social worker on an employer's crisis response team meets again with an employee who saw it happen. The employee sleeps about 2 hours a night, eats little, and has not been able to drive near the site. The employee says the breathing exercises, walks, and talks with their partner that they planned together last week have not helped. What should the social worker do **NEXT**?

**A.** Review the coping plan again and meet next week

**B.** Ask the employee to go over the accident in more detail

**C.** Link the employee to a mental health provider for assessment

**D.** Encourage a gradual return to the site alongside a coworker

**229.** On the morning their home was condemned, a client arrives at a community crisis center too shaken to speak. An hour later, the client is calm, has listed what must happen today, and mentions usually handling the family's bills and appointments. The client then asks the social worker to 'just make all the calls.' What is the **BEST** response?

**A.** Make the calls for the client while the client rests

**B.** Plan which calls the client will make, with help as needed

**C.** Refer the client to a housing agency for case management

**230.** Who studied survivors of the Cocoanut Grove nightclub fire and described acute grief as a normal reaction that runs its course?

**A.** Gerald Caplan

**B.** Erich Lindemann

**C.** Elisabeth Kubler-Ross

**D.** Naomi Golan

**231.** A high school student died by suicide over the weekend. The family has confirmed the cause of death and agreed that it may be shared. The principal asks the school social worker how to inform students on Monday morning. Which approach is **MOST** helpful?

**A.** Read a brief statement over the school intercom

**B.** Hold an assembly where staff share facts and resources

**C.** Say a student died and decline to discuss the cause

**D.** Have trained staff share the news in homeroom-size groups

**232.** Hours after a shooting at a warehouse, a social worker is providing psychological first aid at a family assistance center. An employee who hid during the shooting is trembling but answers questions clearly. The employee says their phone died and their family does not yet know they are alive. What is the **BEST** response?

**A.** Give the employee information on common stress reactions

**B.** Guide the employee through a brief breathing exercise

**C.** Help the employee contact loved ones right away

**233.** In a residential treatment program, a 15-year-old comes out of a phone call with a parent shaking, panting, and saying they want to punch the wall. The teen has been learning DBT skills. What should the social worker help the teen do **FIRST**?

**A.** Splash cold water on their face and slow their breathing

**B.** Check the facts about what the parent said on the call

**C.** Talk through what upset them during the call

**234.** Through an employee assistance program, a client meets with a social worker by video. The client says they handle anger in staff meetings by clamping down and showing nothing, but later get headaches and snap at their children. Which emotion-regulation skill is **BEST** for the social worker to teach?

**A.** Vent the anger fully at home to release the pressure

**B.** Reframe triggering events before anger builds

**C.** Practice muscle relaxation each evening after work

**235.** A 6-year-old in a new foster home has daily meltdowns. When the child welfare social worker asks how the child felt before a meltdown, the child shrugs or says 'bad.' The foster parent wants help so the child can calm down. What should the social worker do **FIRST** to build the child's regulation skills?

**A.** Ask the child to tell the story of leaving home

**B.** Set up a sticker chart that rewards calm days

**C.** Teach the child to name feelings using a faces chart

**236.** A client in a DBT skills group feels intense shame and wants to stay home after stumbling over words in a work presentation. With the social worker, the client has already checked the facts and concluded the shame does not fit, since no personal values were violated. Which skill is **BEST** to teach next?

**A.** Radical acceptance

**B.** Interpersonal effectiveness

**C.** Opposite action

**D.** Self-soothing

**237.** Which DBT skills help a client feel less easily upset by caring for sleep, food, exercise, and illness?

**A.** Opposite action

**B.** PLEASE skills

**C.** TIP skills

**D.** Accumulating positive emotions

**238.** A 15-year-old in a school-based DBT program uses emotion regulation skills well in sessions. Diary cards show that the teen's worst outbursts follow times when a parent says things like 'You're overreacting, just get over it.' The parent attends sessions and wants to help. What is the **MOST** appropriate next step?

**A.** Coach the parent in validating the teen's feelings

**B.** Add home distress tolerance practice the parent can prompt

**C.** Refer the teen to a residential DBT program

**239.** A client in psychodynamic treatment arrives late and apologizes repeatedly, saying the social worker must be angry. The social worker has shown no displeasure. The client adds that a parent used to explode over small mistakes. What is the **BEST** response within a psychodynamic approach?

**A.** Reassure the client that being late is fine

**B.** Assign a thought record to test the belief about anger

**C.** Explore how this fear may echo the client's past relationships

**240.** A client changes the subject each time a sibling's death comes up. The social worker points out the pattern and suggests that avoiding the topic may protect the client from guilt carried since childhood. Which treatment approach is the social worker **MOST** likely using?

**A.** Cognitive behavioral therapy

**B.** Psychodynamic therapy

**C.** Eye movement desensitization and reprocessing

**241.** A retired client has been in weekly psychodynamic therapy exploring lifelong conflicts with authority figures. The client's spouse died suddenly 2 weeks ago. The client now arrives disheveled, has been skipping meals, and says they cannot think clearly enough to deal with the funeral bills. What is the **BEST** way for the social worker to adjust the approach?

**A.** Interpret how the loss revives early conflicts with authority

**B.** Shift to ego-supportive work on daily coping and tasks

**C.** Refer the client to a grief support group instead

**242.** A client in treatment informed by self psychology often describes successes at work and brightens when the social worker responds with interest. In one session, the social worker glances at a phone message while the client is speaking. The client falls silent, then says nothing they do really matters. What is the **MOST** appropriate response within this approach?

**A.** Explore what at work has left the client feeling unimportant

**B.** Reassure the client that their work successes are impressive

**C.** Acknowledge the lapse in attention and how it affected the client

**243.** A client with a history of chaotic early caregiving often arrives furious and accuses the social worker of not caring. The social worker keeps the same session time and room, responds calmly without retaliating, and reliably resumes the work the following week. Which psychodynamic concept is the social worker's stance **MOST** likely drawing on?

**A.** Holding environment

**B.** Transitional object

**C.** Mirroring transference

**D.** Working through

**244.** A social worker and client agree to 16 sessions. Early on, they identify a recurring pattern: the client longs for closeness, expects rejection, and pulls away first. The social worker actively keeps sessions on this pattern, tracing it with the client's partner, with coworkers, and with the social worker. Which approach is the social worker **MOST** likely using?

**A.** Interpersonal psychotherapy

**B.** Task-centered practice

**C.** Solution-focused therapy

**D.** Time-limited dynamic psychotherapy

**245.** A client in a hospital's outpatient cardiac rehabilitation program has medical clearance to exercise but avoids the treadmill. In a CBT session, the social worker helps the client identify the thought 'If I push myself, my heart will stop,' and the client rates belief in it at 90%. What should the social worker do **NEXT**?

**A.** Remind the client that the cardiologist approved the exercise plan

**B.** Use guided questions to test how accurate the prediction is

**C.** Begin graded treadmill sessions starting at a slow speed

**D.** Explore early experiences that shaped the client's fear of illness

**246.** A client with PTSD after a car crash meets with a social worker trained in cognitive processing therapy (CPT) but not in EMDR. After hearing about both treatments, the client declines CPT, saying that writing about the crash feels unbearable, and asks for EMDR. Another practice in the area offers EMDR and accepts the client's insurance. What is the **MOST** appropriate action?

**A.** Refer the client elsewhere for EMDR

**B.** Begin EMDR after studying a published protocol manual

**C.** Encourage the client to try a few CPT sessions first

**247.** A patient on an inpatient psychiatric unit is in a third admission this year for self-harm and has a diagnosis of borderline personality disorder. Between admissions, the patient attended weekly supportive therapy with a provider untrained in DBT. A standard outpatient DBT program has an opening. What is the **BEST** discharge plan?

**A.** Resume weekly supportive therapy with an updated safety plan

**B.** Step the patient down to partial hospital, then supportive therapy

**C.** Refer the patient for comprehensive DBT

**D.** Add DBT skills worksheets to the current supportive therapy

**248.** In an integrated primary care clinic, a client with type 2 diabetes tells the social worker, 'I bought walking shoes over the weekend and went around the block twice this week.' In motivational interviewing, which type of change talk is this?

**A.** Commitment

**B.** Ability

**C.** Taking steps

**D.** Desire

**249.** A hospital social worker meets with a patient admitted for heart failure who keeps adding salt to meals brought from home. The patient asks, 'Why does salt even matter for my heart?' Which response is **MOST** consistent with motivational interviewing?

**A.** Explain how salt causes fluid buildup and strains the heart

**B.** Ask what the patient already knows about salt and the heart

**C.** Reflect that the diet limits seem frustrating to the patient

**250.** In EMDR therapy, the social worker explains what to expect and teaches a calm-place exercise. This happens before any target memory is chosen. Which phase is this?

**A.** Preparation

**B.** Assessment

**C.** Installation

**D.** Closure

**251.** A group program was first built at a medical center for patients with chronic pain and illness. It uses 8 weekly classes of body scan, sitting meditation, and gentle yoga. What is it called?

**A.** Mindfulness-based cognitive therapy

**B.** DBT skills training group

**C.** Acceptance and commitment therapy

**D.** Mindfulness-based stress reduction

**252.** A client with social anxiety in ACT-based treatment says, 'I'm boring, and everyone at work can tell.' The client has been skipping team lunches because of this. Which response is **MOST** consistent with acceptance and commitment therapy?

**A.** Ask the client to list evidence for and against being boring

**B.** Point out qualities that coworkers likely value in the client

**C.** Invite the client to observe the thought as passing words

**253.** A client staying in an emergency shelter meets with a social worker who uses solution-focused brief therapy. Today the client says the week was awful, nothing improved, and the job search went nowhere. Which question is **MOST** appropriate for the social worker to ask?

**A.** 'How have you kept things from getting even worse?'

**B.** 'If a miracle happened tonight, what would be different?'

**C.** 'What do you think is keeping you stuck right now?'

**254.** After being stuck in an elevator, a client now avoids medical appointments on upper floors. Using systematic desensitization, the social worker and client have built a ranked list of elevator situations, and the client has learned progressive muscle relaxation. What should the social worker do **NEXT**?

**A.** Have the client ride to the top floor right away

**B.** Pair relaxation with imagining the least distressing item

**C.** Pair relaxation with imagining the most distressing item

**255.** Two months after moving into assisted living, a client with depression stays in their room most of the day. The client says, 'I'll join the activities once I feel like myself again.' The social worker is using cognitive behavioral therapy. Which intervention is **BEST** to start with?

**A.** Use thought records to track the client's negative thoughts

**B.** Explore the client's grief about the move to assisted living

**C.** Schedule one brief, enjoyable activity for each day

**256.** In the fourth month of psychodynamic therapy, a client makes a joke each time the conversation turns to a parent's recent cancer diagnosis, then changes the subject. The client has never remarked on this pattern. What should the social worker do **FIRST**?

**A.** Suggest that the joking guards against fear of losing the parent

**B.** Point out that the jokes appear whenever the illness comes up

**C.** Ask the client to stop the jokes and stay with the feeling

**257.** A young adult in weekly video sessions describes finishing a first art exhibit, then pauses and looks eagerly at the social worker for a reaction. In earlier sessions, the client seemed deflated when accomplishments drew little response. The social worker uses a self psychology approach. Which response is **MOST** consistent with this approach?

**A.** Point out that the client seems to need approval to feel worthwhile

**B.** Keep a neutral stance so the wish for praise can be analyzed

**C.** Respond warmly to the pride the client is showing

**258.** Early in psychodynamic therapy, a client realized that they keep choosing partners who dismiss them, much as a parent did. Over the next 8 months, the client spots the same expectation with a boss, a close friend, and the social worker. Each time, the client feels less upset and tries a new response. Which process is **MOST** likely taking place?

**A.** Working through

**B.** Repetition compulsion

**C.** Corrective emotional experience

**D.** Abreaction

**259.** After 8 months of psychodynamic therapy for relationship problems, a client starts having panic attacks and now avoids driving on highways. The client asks to focus on the panic. When the social worker explores the request, the client says the attacks feel urgent and are hurting the client's job. The social worker is trained in CBT for panic disorder. What is the **BEST** response?

**A.** Shift to treating the panic with interoceptive exposure

**B.** Explore how the panic connects to the relationship themes

**C.** Add a CBT panic workbook while continuing the current sessions

**260.** A client in standard outpatient DBT reports at the start of an individual session that they cut their forearm 2 days ago after an argument. The social worker checks the wound, which needed no medical care, and confirms the client has no current urge or intent to self-harm. What should the social worker do **NEXT**?

**A.** Teach a new distress tolerance skill to use after arguments

**B.** Turn to the argument and explore what made it so painful

**C.** Do a chain analysis of the events that led to the cutting

**261.** In the third week of an 8-week MBSR group, a member with PTSD grips the chair and breathes rapidly during the body scan. Afterward, the member says the exercise brought back the assault and the room felt far away. The member wants to stay in the group. The social worker is trained in MBSR. What is the **BEST** response?

**A.** Encourage the member to keep attention on body sensations as they arise

**B.** Help the member process the assault memory in a private session

**C.** Arrange individual trauma therapy in place of the group sessions

**D.** Offer options such as eyes open and an outside focus during practice

**262.** A 78-year-old hospital patient recovering from a fall has agreed to talk with the social worker about safety at home. When the social worker mentions grab bars and a shower chair, the patient snaps, 'You sound just like my kids. Everyone is telling me what to do.' Which response is **MOST** appropriate for a social worker using motivational interviewing?

**A.** Explain the fall risks again in simpler, clearer terms

**B.** Emphasize that changes at home are the patient's decision

**C.** Ask what it would take for the patient to accept the changes

**263.** Which PTSD treatment has the client write an impact statement and then examine 'stuck points,' such as self-blame for the trauma?

**A.** Prolonged exposure

**B.** EMDR

**C.** Cognitive processing therapy

**D.** Trauma-focused CBT

**264.** After a layoff, a client at a community job center tells the social worker they lie awake panicking about a utility shutoff notice due in 10 days. The client has not yet contacted the utility or applied for bill assistance. Which coping skill is **BEST** for the social worker to teach?

**A.** Break the problem into concrete steps to act on

**B.** Practice slow breathing when panic rises at night

**C.** Write about the worries in a journal before bed

**265.** A client with schizophrenia is moving into a supported apartment and wants to learn to cook simple meals safely. The social worker has explained why each step matters, such as turning off the stove. What should the social worker do **NEXT**?

**A.** Give the client a written checklist to follow at home

**B.** Demonstrate cooking a meal while the client watches

**C.** Have the client try cooking a meal with coaching

**266.** At a senior center, a 74-year-old client tells the social worker that most nights they lie awake in bed for an hour or more, watching TV and checking the news on a tablet until sleep comes. A physician found no medical or medication cause. Which self-care skill is **BEST** for the social worker to teach?

**A.** Swap the TV and tablet for reading in bed until drowsy

**B.** Get up when unable to sleep and return when sleepy

**C.** Go to bed earlier to allow more total time for sleep

**267.** A 74-year-old caring at home for a spouse with Parkinson's disease has worked with the social worker for 6 weeks on relaxation, sleep habits, and short daily breaks. The spouse now needs help turning and toileting several times a night, and the caregiver sleeps about 3 hours. The caregiver says, 'I can't keep this up, but letting strangers in feels like breaking my promise to care for my spouse myself.' What should the social worker do **FIRST**?

**A.** Teach a new set of relaxation exercises for the caregiver

**B.** Explore the caregiver's feelings about accepting outside help

**C.** Suggest the caregiver ask a physician about a sleep aid

**D.** Help arrange overnight in-home respite so the caregiver can sleep

**268.** An employee assistance program refers a 52-year-old client for stress management by video. The client describes long hours and a new manager. The client also mentions chest tightness when climbing stairs at work that eases after resting, and has not seen a physician in 5 years. The client asks to learn relaxation skills. What is the social worker's **BEST** response?

**A.** Urge the client to get a prompt medical evaluation

**B.** Teach slow breathing to use when the tightness starts

**C.** Ask the client to log when the tightness occurs this week

**269.** In Meichenbaum's stress inoculation training, during which phase does the client practice coping skills under graded imagined and real-life stressors?

**A.** Conceptualization

**B.** Skills acquisition and rehearsal

**C.** Application and follow-through

**270.** In a hospital ICU waiting room, a patient's adult child tells the social worker they have not slept in 2 days. They are pacing, their hands are shaking, and they say, 'I can't think straight.' The medical team will meet with the family in half an hour to discuss treatment choices. What should the social worker do **FIRST**?

**A.** Guide a few minutes of slow, paced breathing

**B.** Help write down questions for the medical team

**C.** Explore their fears about the parent's prognosis

**271.** A graduate student tells a campus social worker that worries about loans and grades pop up all day, breaking concentration during study time and lectures. The student has tried to push the worries out of mind, but they come back stronger. Which stress management technique is **BEST** to teach?

**A.** Shift to a quick task whenever worries start

**B.** Talk worries over with a friend right away

**C.** Postpone worries to a set daily worry period

## III.C Practice Evaluation and Research

**272.** A social worker records a client's daily anxiety ratings for 2 weeks, then begins cognitive behavioral therapy. Ratings fall steadily over the next 6 weeks. During the same period, the client starts a new job they enjoy. Which conclusion is **BEST** supported by these data?

**A.** The cognitive therapy caused the drop in the client's anxiety

**B.** The new job, rather than therapy, caused the change

**C.** The data are unusable because the baseline was too short

**D.** Anxiety improved, but the design cannot isolate the cause

**273.** A school social worker plans to use a token reward system to reduce a 7-year-old's classroom outbursts. The social worker wants data showing whether the system works. What should the social worker do **FIRST**?

**A.** Start the token system and count outbursts weekly

**B.** Ask the teacher to rate progress at the end of the term

**C.** Count the outbursts daily for a period before starting

**274.** An outpatient social worker gives a client with depression the PHQ-9 at each session of cognitive behavioral therapy. After 8 sessions, the totals remain in the moderate range with no downward trend. The client says the sessions are helpful and would like to keep going the same way. What should the social worker do **FIRST**?

**A.** Continue the current approach because the client finds it helpful

**B.** Go over the results with the client and revise the plan

**C.** Ask a psychiatric prescriber to evaluate the client for medication

**275.** A home-based case management program for older adults serves clients with very different aims. One wants to attend a senior center weekly, another to pay bills on time, and a third to keep medical appointments. The director wants one evaluation method that tracks each client's own targets and yields scores that can be compared across clients. Which method is **BEST**?

**A.** Self-anchored rating scale

**B.** Geriatric Depression Scale

**C.** Goal Attainment Scaling

**D.** Behavioral frequency count

**276.** A residential treatment social worker plans a point system to reduce a 15-year-old's verbal aggression on the living unit, in the classroom and during family visits. The team wants evidence that the point system, rather than outside events, produces any change. Staff will not remove the point system once it starts working. Which evaluation design is **MOST** appropriate?

**A.** Reversal (ABAB) design

**B.** Multiple-baseline design

**C.** AB design

**277.** A social worker providing telehealth services plans to evaluate exposure work for a client with social anxiety. During a 2-week baseline, the client records how many conversations they start each day. The counts rise steadily across the 14 days, from 1 a day to 5 a day. The client is not in crisis. What should the social worker do **NEXT**?

**A.** Keep collecting daily counts until they level off

**B.** Begin the planned exposure sessions on schedule

**C.** Replace the baseline counts with a standardized anxiety scale

**278.** A client with alcohol use disorder attends a weekly outpatient program. Four weeks ago, after drinking rose, the social worker and client revised the plan to add motivational interviewing and a second weekly session. Since then, drinking days have increased from 2 to 5 per week, the client has missed 3 workdays, and the client says, 'I want to stop, but I can't do it on this schedule.' Which action is **BEST**?

**A.** Explore the client's ambivalence about drinking once more

**B.** Add a peer recovery coach to the current outpatient plan

**C.** Refer the client for a higher level of care assessment

**279.** A 14-year-old in a residential treatment program has been banging their head against walls several times a week. Staff incident reports from the past 6 weeks record the date and time of each episode. The team is ready to start a new behavior plan and wants to judge whether it works. Which approach is **BEST**?

**A.** Count episodes for 2 more weeks before starting the plan

**B.** Start the plan and log episodes in new incident reports

**C.** Use the past 6 weeks of incident reports as the baseline

**D.** Plan to pause the behavior plan later to confirm its effect

**280.** An adult client with panic disorder has logged panic attacks daily since intake. Over 3 baseline weeks the count held steady at about 6 per week. During 6 weeks of exposure-based treatment, it has fallen steadily to 2 per week. The client read about a newer therapy online and asks about switching to it. Which response is **BEST**?

**A.** Agree to switch to the newer therapy as the client requests

**B.** Review the progress data together and discuss continuing

**C.** Blend the newer therapy's techniques into current sessions

**281.** A single-system design sets a goal for one behavior. The goal then moves in steps, and each new level starts only after the client meets the current one. What is this design called?

**A.** Multiple-baseline design

**B.** Changing-criterion design

**C.** ABC design

**D.** ABAB reversal design

**282.** A social worker is treating a 15-year-old for anxiety about attending school and tracks the teen's daily anxiety ratings on a 0-10 scale. Over 3 weeks the ratings fall from 8 to 2. In this session, the teen mentions that a parent recently arranged online classes so the teen can study at home most days. What should the social worker do **NEXT**?

**A.** Examine whether staying home accounts for the lower ratings

**B.** Praise the progress and continue the current treatment plan

**C.** Add a second anxiety scale to confirm the lower ratings

**283.** A client with depression has completed the PHQ-9 at the start of each session for 10 weeks. The social worker records the scores in the chart but has not discussed them with the client. The client now asks to stop, saying, 'I fill it out and never hear about it again.' What is the social worker's **BEST** response?

**A.** Stop the measure and rely on discussion in sessions

**B.** Explain why the scores matter and ask the client to continue

**C.** Switch to completing the measure once a month

**D.** Begin reviewing a graph of the scores with the client

**284.** A social worker reviews the service plan of a 79-year-old client who lives alone and receives home-delivered meals to address weight loss. After 2 months, the client has lost another 8 pounds. The client says they eat every meal, and the home aide's log confirms the meals are finished. What should the social worker do **NEXT**?

**A.** Ask the client's doctor to check promptly for a medical cause

**B.** Ask a dietitian to review the calorie content of the meals

**C.** Continue the current plan and reweigh next month

**285.** A workforce program for young adults is writing objectives for a new grant. The team needs one objective that states the change participants should achieve. Which objective is **BEST**?

**A.** By month 12, 60% of participants will keep a job for 90 days

**B.** Staff will hold 40 job-readiness workshops during the grant year

**C.** Participants will gain the confidence they need to succeed at work

**D.** The program will help young adults become self-sufficient

**286.** An after-school program serves 40 middle school students in a city of 300,000. Its activities are tutoring, mentoring and family outreach. The new board chair wants the program's main objective to be a 50% drop in citywide juvenile arrests within 1 year. What is the **MOST** appropriate response from the program's social worker?

**A.** Keep the arrest objective but extend the deadline to 5 years

**B.** Report students served and hours of tutoring and mentoring

**C.** Propose student outcomes that its tutoring and mentoring can change

**287.** A middle school program sets this objective: 'By June, participating students will miss 25% fewer school days than last year.' To measure progress, staff plan to give students a survey asking how much they enjoy the program. Which indicator would **MOST** directly measure progress on the objective?

**A.** Official school attendance records for each participant

**B.** A student survey asking how often they miss school

**C.** The number of program sessions each student attends

**288.** A reentry program is writing outcome objectives for a new grant. A draft reads: 'Within 12 months, increase stable housing among participants by 30%.' Staff have no figures on how many people like these participants are stably housed now. What should the social worker do **FIRST**?

**A.** Lower the target to 10% so it is achievable

**B.** Find the current housing rate for comparable participants

**C.** Reword it to promise better housing stability for participants

**289.** A supported employment program's new contract pays the agency only for clients still employed 6 months after enrollment. Within a quarter, the social worker who manages the program sees intake staff steering applicants with criminal records or recent hospitalizations to a waitlist. Placement rates have risen. Intake staff completed eligibility training last month and say they fear losing the contract. What action is **BEST**?

**A.** Collect another quarter of intake data to confirm the pattern

**B.** Raise the pattern with leaders and seek fairer performance measures

**C.** Remind intake staff of the eligibility rules at a staff meeting

**290.** A year-end evaluation of a home-based depression program shows strong overall results, and the board will vote next week on expanding it unchanged to three counties. When the social worker breaks the data down by age, clients 65 and older showed no improvement and dropped out at twice the rate of younger clients. What is the **MOST** appropriate action?

**A.** Support the board's plan, since overall results are strong

**B.** Hold the age results until a follow-up study confirms them

**C.** Present the age findings to the board before the vote

**291.** An agency director announces that an outside firm will evaluate the family support program this spring. Several case managers tell the program's coordinator, a social worker, that they fear the results will be used to cut positions, and some say they may not cooperate. Planning has not begun. What should the coordinator do **FIRST**?

**A.** Involve staff in shaping the evaluation questions and uses

**B.** Assure staff that the evaluation results will stay confidential

**C.** Start data collection and address concerns as they come up

**292.** A youth shelter invites line staff to help design an evaluation of its case management. While drafting measures, months before data collection begins, staff start defining what a 'stable exit' means, discussing outcomes in team meetings and changing how they set goals with youth. Which type of evaluation use does this **MOST** likely show?

**A.** Instrumental use

**B.** Symbolic use

**C.** Conceptual use

**D.** Process use

**293.** To meet a new funder requirement, a community mental health program added a 45-minute questionnaire battery to every intake. Over the next quarter, missed second appointments doubled. Exit calls with clients who left point to the long first visit spent on paperwork. The funder requires data on 3 outcomes. Which action is **BEST**?

**A.** Trim the battery to brief versions of the funder's measures

**B.** Pause the funder's battery until attendance recovers

**C.** Survey clients who dropped out about the intake visit

**294.** Which type of evaluation examines whether a program is being delivered as designed while the program is still under way?

**A.** Formative evaluation

**B.** Summative evaluation

**C.** Needs assessment

**D.** Cost-benefit analysis

**295.** At the end of a 3-year grant, an agency analyzes whether participants in its supportive housing program stayed housed and made fewer emergency room visits. The board will use the results to decide whether to continue the program. Which type of evaluation is the agency conducting?

**A.** Summative evaluation

**B.** Formative evaluation

**C.** Needs assessment

**296.** A family services agency is expanding into a new county and wants to learn which services residents lack. Staff meet one at a time with a pastor, two school principals, a clinic physician and the police chief to hear what problems they see in the community. Which needs assessment method is the agency using?

**A.** Community forum

**B.** Focus group

**C.** Key informant approach

**297.** A board member of a rural senior center returns from a conference excited about a meal-delivery program that succeeded in a large city. The board asks the center's social worker to begin planning to launch the same program next year. What should the social worker do **FIRST**?

**A.** Gather data on unmet needs among local older adults

**B.** Write a grant proposal to fund a local version

**C.** Pilot the program in one town and track its use

**298.** A youth mentoring program runs at four sites, and its funder asks for an outcome study within the year. Site reports show that two sites hold weekly mentor meetings as designed. The other two meet about once a month and have dropped mentor training. The program director asks the social worker to plan the evaluation. What should the social worker do **FIRST**?

**A.** Start measuring youth outcomes at every site now

**B.** Compare outcomes between high- and low-fidelity sites

**C.** Examine and correct how each site delivers the model

**D.** Ask the funder to drop the outcome requirement

**299.** A county commissioner asks a jail diversion program to show whether the money it saves the county exceeds what it costs. The program director assigns the task to a social worker whose evaluation training covers outcome tracking but not converting outcomes into dollar values. Which action is **BEST**?

**A.** Report cost-effectiveness as cost per person diverted

**B.** Consult an evaluator experienced in cost-benefit analysis

**C.** Decline the task because cost studies are outside social work

**300.** Which needs assessment approach uses public data, such as census and health figures, to compare needs across neighborhoods?

**A.** Key informant approach

**B.** Social indicators approach

**C.** Community forum approach

**D.** Rates-under-treatment approach

**301.** A county parenting program is 5 months into its first year. Process data show that parents who speak Spanish come to the first session but rarely come back. Exit calls reveal that sessions are held in English with English handouts. The funder's summative outcome evaluation is scheduled for month 12. Which action is **BEST**?

**A.** Keep the model unchanged until the outcome evaluation is done

**B.** Interview the Spanish-speaking parents about why they stop attending

**C.** Add Spanish-language sessions and track retention by language

**302.** An agency compares the cost per youth served of two mentoring models. One model uses paid mentors. The other relies on volunteer mentors and meets in space a church donates. The draft analysis counts budgeted expenses alone, so the volunteer model looks far cheaper. How should the social worker revise the cost analysis?

**A.** Add a market value for unpaid mentor hours and the room

**B.** Keep budgeted expenses, since unpaid mentors cost the agency nothing

**C.** Convert both models' outcomes into dollar benefits

**303.** A risk scale is completed at intake. Two years later, clients who scored high at intake have returned to the hospital far more often than clients who scored low. Which measurement property does this show?

**A.** Concurrent validity

**B.** Predictive validity

**C.** Test-retest reliability

**D.** Content validity

**304.** Two direct support staff at a group home count one resident's aggressive episodes during the same evening shifts. On the same evenings, one records 2 episodes and the other records 7. When asked, one counts only hitting, while the other also counts shouting and slamming doors. Which step is **BEST** for getting usable data?

**A.** Average the two staff members' counts for each evening

**B.** Use the counts from the staff member with more experience

**C.** Switch to a standardized aggression rating scale

**D.** Write a shared definition with examples and train both staff

**305.** A parenting program screens 400 parents with a stress scale and offers places only to those in the top 10%. Because of a staffing delay, these parents are rescored 3 weeks later, before any sessions begin. Their scores have fallen noticeably as a group. Parents with moderate scores who were rescored at the same time changed little. What **MOST** likely explains the drop?

**A.** Statistical regression toward the mean

**B.** A testing effect from completing the scale twice

**C.** History, such as an outside event during the wait

**306.** A community clinic begins serving many adults who recently resettled from Afghanistan. Staff want to track depression with a scale developed and normed with English-speaking U.S. adults, and a staff member has translated it into Dari. Early scores look low, even for clients who describe poor sleep, hopelessness and withdrawal. What is the **MOST** appropriate response?

**A.** Compare the clients' scores with the scale's published norms

**B.** Seek a version with validity evidence for this population

**C.** Have a second translator check the Dari wording

**D.** Stop using standardized measures with this group

**307.** An agency director shares a study of a new parenting program that randomly assigned 3,000 families. Program families scored 1 point lower than controls on a 100-point child stress scale, and the difference was statistically significant. The director proposes replacing the agency's current parenting program with the new one. How should the social worker respond?

**A.** Support the change, since the result was statistically significant

**B.** Ask for the study to be repeated with a larger sample

**C.** Ask whether a change that small matters in practice

**308.** Which type of validity is supported when a panel of experts judges that a scale's items represent every part of the construct it is meant to measure?

**A.** Content validity

**B.** Face validity

**C.** Construct validity

**D.** Concurrent validity

**309.** At the end of each final session, outpatient social workers hand their clients a satisfaction survey with the worker's name printed at the top. Each worker waits while the client fills it out and then collects it. Nearly every rating is 'very satisfied.' Which change would **MOST** improve the accuracy of the results?

**A.** Add more rating items to the survey for detailed feedback

**B.** Have staff outside the client's care collect anonymous surveys

**C.** Remove the worker's name but keep workers collecting it

**310.** An agency evaluates a 12-week program for adults with alcohol use disorder by comparing intake and final scores. Of 60 clients who started, 25 left before week 12, most of them clients with the most severe drinking at intake. Final scores among those who finished are much better than the group's intake scores. Which threat to internal validity **MOST** likely affects this conclusion?

**A.** History

**B.** Maturation

**C.** Attrition

**D.** Selection

**311.** An agency can offer a new caregiver support group to only 20 people at a time, and 60 eligible caregivers are waiting. Burden scores are in the moderate range for nearly all of them, and caregivers in crisis are routed to urgent services. The director wants to learn whether the group reduces burden and proposes starting with the 20 highest scorers. What should the social worker **BEST** recommend?

**A.** Start with the 20 highest scorers and compare their pre and post scores

**B.** Set start order by lottery and compare early and later groups

**C.** Start in list order and measure each group before and after

**312.** A new scale is given to a group. Its items correlate strongly with one another. Which property does this result support?

**A.** Internal consistency reliability

**B.** Test-retest reliability

**C.** Construct validity

**313.** A community mental health agency wants to learn how satisfied its 900 active clients are. Last year it surveyed clients who attended the annual picnic, and about 150 responded. A board member suggests repeating the picnic survey with more volunteers to collect more forms. Which change would **MOST** improve how well the results represent all active clients?

**A.** Make the picnic survey anonymous so answers are more candid

**B.** Double the number of picnic surveys collected

**C.** Survey a random sample drawn from the full client list

**314.** During an external licensing review, a state reviewer found that 30% of sampled charts at a community mental health agency lacked treatment plan updates within the required period. The agency's quality coordinator, a social worker, has since fixed the cited charts and trained staff on the requirement. What should the coordinator do **NEXT**?

**A.** Require supervisor sign-off on each chart's plan update

**B.** Repeat the staff training at the next staff meeting

**C.** Audit a new sample of charts over the next few months

**315.** A social worker doing the quarterly internal chart audit at a counseling agency reviews 10 sampled charts for one colleague. Nine document sessions on dates when the electronic check-in system shows the client never arrived, and all nine were billed. Agency policy names a compliance officer to receive billing concerns. What should the social worker do **NEXT**?

**A.** Ask the colleague to explain the mismatched dates

**B.** Expand the review before reporting to the compliance officer

**C.** Correct the billing for the nine sessions

**D.** Report the findings to the compliance officer

**316.** Which external review checks a whole organization against published standards and grants it a recognized status for a set period?

**A.** Utilization review

**B.** Accreditation survey

**C.** Peer review

**D.** Financial audit

**317.** An accreditation survey of an outpatient behavioral health clinic results in three cited deficiencies. The director asks the quality manager to draft a plan of correction and to decide which deficiency to correct before the others. Which finding should the plan address **FIRST**?

**A.** Client signatures missing on 40% of treatment plans

**B.** Consent forms omitting current confidentiality limits

**C.** Six positive suicide screens lacking reassessment

## III.D Supervision and Administration

**318.** An associate social worker sees an adult client by telehealth. The client, angry after losing a custody hearing, says the former partner 'will regret this' and mentions keeping a handgun at home. When asked directly, the client says the comment meant an appeal and denies intent to harm anyone. The associate remains unsure whether protective action is needed, and supervision is scheduled in 6 days. What should the associate do **NEXT**?

**A.** Contact the supervisor today to review the risk together

**B.** Bring detailed notes to the scheduled supervision

**C.** Revisit intent and firearm access at the next session

**319.** A newly hired associate social worker at a community mental health center is drafting goals for the supervision contract. The associate is skilled in individual cognitive behavioral work but has little experience with family sessions, which make up a third of the assigned caseload. The supervisor will also complete the associate's licensure evaluations. Which approach to the goals is **BEST**?

**A.** Let the supervisor set goals from the first licensure evaluation

**B.** Make family session work a priority learning goal

**C.** Ask that the family cases go to more experienced colleagues

**320.** An associate social worker at a family counseling agency records sessions, with client consent, for review in supervision. The supervisor asks the associate to bring one recording from this week. Which recording would be **MOST** useful for the associate's learning?

**A.** A session in which a new technique worked well

**B.** The most recent session, whatever happened in it

**C.** A session that felt stuck or went poorly

**321.** A hospital social worker covering two medical units falls 3 days behind on discharge assessments and progress notes. Two patients on the list have unsafe home situations, and several others need only routine paperwork. The social worker has been staying late, which has not closed the gap. What is the **MOST** appropriate action?

**A.** Rank the caseload by risk and review it with the supervisor

**B.** Keep working late until every assessment is current

**C.** Shorten notes on routine cases to free up time

**D.** Ask a colleague on another unit to take the two complex patients

**322.** An associate social worker at a substance use treatment program has two supervisors. The program manager provides administrative supervision, and an outside licensed social worker provides clinical supervision for licensure. For one client, the manager directs weekly drug screens, while the clinical supervisor advises pausing screens to rebuild trust. Neither supervisor knows of the other's direction. What should the associate do **FIRST**?

**A.** Follow the program manager, who represents the employer

**B.** Follow the clinical supervisor, who oversees licensure hours

**C.** Seek an outside consultant's opinion on drug screening

**D.** Ask both supervisors to resolve the conflict together

**323.** An associate social worker at a hospital realizes that a patient's psychosocial assessment, sent an hour ago by secure email, went to the wrong family's case manager at a nursing facility. The associate's supervisor is on site today, and weekly supervision is in 4 days. What should the associate do **FIRST**?

**A.** Inform the supervisor of the error now

**B.** Raise the error with the supervisor at the scheduled session

**C.** Ask the recipient to delete the message and confirm by reply

**324.** A child welfare associate is completing a self-assessment before a 6-month evaluation. The associate feels confident about court reports, but the supervisor returned three of the last five reports for missing safety details. The agency form asks for a rating and an example for each skill. Which approach to the self-assessment is **MOST** useful?

**A.** Rate court reports a strength, based on the associate's confidence

**B.** Give every skill a middle rating to avoid seeming overconfident

**C.** Rate court reports a growth area, noting the specific feedback

**325.** A social worker practicing under clinical supervision for licensure presents a case at a monthly peer consultation group. The group recommends a different treatment approach from the one the clinical supervisor directed. What should the social worker do **FIRST**?

**A.** Switch to the approach the consultation group recommended

**B.** Continue the supervisor's plan without raising the group's view

**C.** Discuss the group's recommendation with the supervisor

**326.** A child welfare supervisor meets with a caseworker each week. In one meeting, the supervisor reviews the caseworker's cases and moves two families to a coworker who has fewer. The supervisor also checks that court reports will be filed on time. Which function of supervision is the supervisor **MOST** likely carrying out?

**A.** Educational

**B.** Administrative

**C.** Supportive

**327.** A nursing home's charge nurse asks a social work consultant for help with a resident who refuses meals. The consultant learns the nurse has managed similar residents well, but this resident reminds the nurse of a parent, and the nurse now pleads and argues at mealtimes. The consultant works on the nurse's reactions to this resident. Which type of consultation is this **MOST** likely?

**A.** Client-centered case consultation

**B.** Program-centered administrative consultation

**C.** Consultee-centered administrative consultation

**D.** Consultee-centered case consultation

**328.** An integrated primary care clinic hires 5 new associate social workers at once. All of them must learn the clinic's brief behavioral health protocol, and the one licensed supervisor has limited hours. Each associate already has a weekly individual meeting for case-specific risk questions. Which added format is **MOST** appropriate for teaching the protocol?

**A.** Group supervision for the associates as a cohort

**B.** A peer consultation group the associates run themselves

**C.** Live supervision of each associate's patient sessions

**329.** A social worker with 6 years of independent practice in adult outpatient therapy transfers to a pediatric oncology unit. In supervision, the social worker asks for scripts for explaining procedures to children and wants every family meeting plan reviewed. With adult caregivers, the social worker works confidently and needs little input. Which supervisory approach is **MOST** appropriate?

**A.** Keep the autonomy-focused approach that suits the worker's experience

**B.** Explore with the worker why confidence has dropped since the move

**C.** Give more structure and direction on the child-focused work

**D.** Give close direction across the worker's whole caseload

**330.** A first-year school social worker co-leads a social skills group for 8-year-olds. Sessions keep unraveling when two members begin taunting each other, and the social worker freezes. Discussing session notes in weekly supervision has produced sound plans that fall apart in the moment. Which supervision format is **BEST** suited to this need?

**A.** Live supervision with in-session coaching

**B.** Review of session recordings afterward

**C.** Peer consultation with other school staff

**D.** Group supervision with case presentations

**331.** In supervision, an associate describes rushing to end sessions whenever clients cry. The supervisor asks what the associate notices in themself at those moments and how those reactions shape the work, keeping the focus on the associate's practice rather than personal history. In Bernard's discrimination model, which supervisor role does this **MOST** likely show?

**A.** Teacher role

**B.** Counselor role

**C.** Consultant role

**332.** To cut costs, a nonprofit's director proposes ending individual supervision for 4 associate social workers and replacing it with a monthly peer consultation group made up of the associates. The associates carry caseloads with frequent suicide risk. The director asks the clinical manager for a recommendation. What is the **MOST** appropriate recommendation?

**A.** Adopt the peer group, since associates can advise one another

**B.** Adopt the peer group with the manager reviewing notes monthly

**C.** Keep designated supervisors and add the peer group as a supplement

**D.** Keep individual supervision but reduce it to monthly sessions

**333.** A supervisee who is usually confident becomes hesitant in supervision and repeatedly asks the supervisor to say exactly what to do. The supervisee is working with a client who feels helpless and keeps asking the supervisee for answers. What is the supervisor's **BEST** response?

**A.** Give step-by-step directions for the next session

**B.** Recommend personal therapy for the supervisee's dependence

**C.** Explore whether the case dynamic is being replayed between them

**D.** Note a performance concern in the supervisee's evaluation

**334.** A supervisee who usually welcomes feedback starts rejecting every suggestion the supervisor offers about one case, saying, 'That won't work either.' In that case, the client turns down each resource the supervisee proposes. The supervisee stays open to feedback on other cases. What is **MOST** likely happening?

**A.** Transference toward the supervisor

**B.** Countertransference toward the client

**C.** Parallel process

**D.** Burnout

**335.** An associate social worker in a jail mental health program came from a job where a supervisor publicly criticized every mistake. The new supervisor gives mild, specific feedback, yet the associate apologizes repeatedly, leaves errors out of case reviews and braces for reprimand at each meeting. What is **MOST** likely occurring?

**A.** Supervisee transference

**B.** Parallel process

**C.** Supervisor countertransference

**336.** A hospice social work supervisor has been quietly rewriting a new staff member's incomplete bereavement risk screens instead of returning them, as is done for other staff. In consultation, the supervisor traces the rewriting to a protective pull: the staff member's recent loss of a parent closely resembles the supervisor's own. What should the supervisor do **NEXT**?

**A.** Keep examining the protective reaction in consultation

**B.** Give the staff member the same corrective feedback others receive

**C.** Share the similar loss with the staff member to build trust

**337.** An emergency department social work supervisor is under pressure to shorten patient stays. The supervisor has started cutting supervision short and giving quick orders. Now a supervisee has begun ending patient assessments abruptly and rushing patients through instructions. Which response by the supervisor is **BEST**?

**A.** Coach the supervisee to slow assessments with patients

**B.** Review the supervisee's recent assessments for missed details

**C.** Slow down and model an attentive pace with the supervisee

**338.** A social worker with 8 years of hospice experience recently joined a new hospice team. In supervision, the social worker praises every suggestion, repeats the supervisor's phrasing word for word and recently said the supervisor is 'just like my first mentor, who always knew best.' The social worker has stopped offering their own case formulations. What is the **MOST** appropriate response by the supervisor?

**A.** Offer more direction while the social worker settles in

**B.** Name the pattern and explore it with the social worker

**C.** Suggest personal therapy to work through the mentor relationship

**339.** A clinical director supervises an associate who publicly questioned the director's plan at a team meeting. Since then, the director has drafted lower ratings for the associate's annual evaluation than the case reviews support. On reflection, the director recognizes resentment about the meeting as the source. The evaluation is due in 5 days. What should the director do **NEXT**?

**A.** Keep monitoring the reaction over the coming month

**B.** Tell the associate about the resentment to clear the air

**C.** Seek consultation on the reaction before finishing the evaluation

**D.** Transfer the evaluation and supervision to another manager

**340.** At a youth residential program, a resident left the building at night and was injured. Required 15-minute checks had been logged but not done. The director asks the clinical social work manager to lead the response and revise the overnight safety procedures. What should the manager do **FIRST**?

**A.** Draft stricter overnight check rules for board approval

**B.** Review the incident with involved staff to find system causes

**C.** Retrain overnight staff on the check procedure right away

**D.** Gather sample procedures from similar programs

**341.** Six months ago, an outpatient clinic adopted a policy requiring a suicide risk screen at every intake, with a same-day safety plan for positive screens. All clinicians signed the policy and finished training. The quality committee now wants to know whether the policy is reducing risk. What should the committee do **FIRST**?

**A.** Audit a sample of charts for completed steps

**B.** Compare suicide attempts among clients before and after adoption

**C.** Survey clinicians about how useful they find the screen

**D.** Report the signature and training totals to the board

**342.** An agency committee lists risk areas such as consent and boundaries. It compares daily practice with written rules, rates each risk and plans fixes. What is this process called?

**A.** Utilization review

**B.** Root cause analysis

**C.** Ethics audit

**D.** Peer review

**343.** A family services agency will begin evening home visits next quarter. The director asks a senior social worker to draft a field safety policy. Over the past 2 years, staff have filed several incident reports about daytime visits, and some workers have described near misses at staff meetings. What should the social worker do **FIRST**?

**A.** Review past visit problems with frontline staff

**B.** Adapt a neighboring agency's evening visit policy

**C.** Require two workers on every evening visit

**D.** Schedule de-escalation training for field staff

**344.** A community agency banned text messaging with clients to reduce privacy risk. A compliance review a year later finds that many case managers text clients from personal phones anyway. In interviews, staff explain that many clients have prepaid phones with few voice minutes and answer texts but not calls. Which policy response is **BEST**?

**A.** Reinforce the ban and discipline staff texting from personal phones

**B.** Survey clients about how they prefer the agency to reach them

**C.** Allow texting on agency phones with set safeguards

**345.** In a large hospital, each social worker answers to two people. The head of social work oversees professional standards. A unit nurse manager sets daily assignments. Which organizational structure does this describe?

**A.** Flat structure

**B.** Matrix structure

**C.** Functional structure

**D.** Divisional structure

**346.** A community agency plans to replace paper charts with an electronic record. Months before any training, the director shares data on missed follow-ups caused by lost paper files, holds staff forums about problems with the current system and invites concerns. In Lewin's model, which stage is the director **MOST** likely working on?

**A.** Refreezing

**B.** Unfreezing

**C.** Moving

**347.** A new director at a large homeless shelter plans to move from rule-based discipline to a trauma-informed approach. Staff resist, and many say they will wait to see what one long-time staff member thinks, though that person holds no supervisory title. The board supports the change. What should the director do **FIRST**?

**A.** Involve that colleague in planning from the start

**B.** Schedule trauma-informed care training for the full staff

**C.** Announce a firm start date with the board's backing

**348.** At a county benefits office, intake staff spend most of each appointment checking that every form field is complete. Workers turn away applicants in urgent need over minor paperwork gaps, and staff meetings focus on audit error rates rather than whether families get help. Supervisors praise flawless files. Which organizational problem is **MOST** likely present?

**A.** Mission drift

**B.** Goal displacement

**C.** Groupthink

**D.** Creaming

**349.** A family service agency learns its funding will drop 15% next year. It runs a crisis response program, a parenting education class and a youth recreation program. The director must decide how to allocate the reduced budget. Which approach is **BEST**?

**A.** Cut each program's budget by the same 15%

**B.** Rank programs openly by client need and outcome data

**C.** Close the program that serves the fewest clients

**D.** Protect the programs that board members value most

**350.** Which budgeting system calculates cost per outcome achieved, such as the cost for each client who stays housed for a full year?

**A.** Line-item budgeting

**B.** Functional budgeting

**C.** Program budgeting

**351.** A family housing program's grant pays for two case managers. One position has been vacant for 4 months, leaving unspent salary funds. Meanwhile, several client families face eviction over small rent debts. The grant agreement requires the funder's written consent for spending outside its line items. Which action by the program director is **BEST**?

**A.** Use the savings for rent aid and inform the funder at year end

**B.** Ask the funder to approve moving the savings to rent aid

**C.** Use the savings for rent aid once the board approves it

**352.** A crisis stabilization program manager reviews the monthly budget report at midyear. Overtime spending is 40% above its year-to-date budget, and the program is projected to end the year in deficit. The executive director asks for a plan by next week. What should the manager do **FIRST**?

**A.** Draft a staffing schedule that caps overtime spending

**B.** Find out what is driving the overtime spending

**C.** Ask the board to raise the overtime budget

**353.** A family support program gives grocery gift cards to clients in need. One case manager orders the cards, keeps them in a desk drawer, hands them out and records each distribution in a spreadsheet. A routine review finds the records complete. The program director wants to reduce the risk of error or misuse. Which control is **MOST** effective?

**A.** Give purchasing, custody and recordkeeping to different staff

**B.** Have a supervisor count the cards against the log each month

**C.** Refer the arrangement to the compliance officer for investigation

# Answers and rationales

## III.A Practice Concepts

### 1. INT-1508 (rev 1) · Best answer: A

A client with schizophrenia who lives in supportive housing tells the social worker, 'I want a real job, like stocking shelves.' The client's symptoms have been stable on medication, but the client has not worked in 8 years. Housing staff say a job is unrealistic for now. What is the social worker's **BEST** response?

**A.** Refer the client to a supported employment program now
**B.** Enroll the client in a prevocational work-readiness group first
**C.** Explore whether the client's job goal is realistic right now
**D.** Ask the treatment team to decide when the client is ready

#### Why each option is right or wrong

**A** **Correct.** The strengths perspective assumes no upper limit on growth and lets the client's own goals lead (Rapp & Goscha, 2012). Supported employment in the Individual Placement and Support model accepts anyone who wants to work and starts the job search quickly, without readiness tests (Drake, Bond & Becker, 2012). The client is stable and has named a specific goal, so linkage now honors it. It beats B because readiness groups delay competitive work, and placing people in jobs with ongoing support has stronger employment outcomes.

**B** **Incorrect** (trap: Wrong model or concept). Training before placement sounds cautious, but the evidence favors placing people in competitive jobs quickly and supporting them on the job. Readiness groups often delay real work or replace it.

**C** **Incorrect** (trap: Overrides the client's choice). Questioning whether the goal is realistic starts from the staff's doubts rather than the client's strengths. Exploring the client's hopes and job preferences would fit; testing the goal's realism signals low expectations.

**D** **Incorrect** (trap: Overrides the client's choice). Handing the timing to the team overrides a stable client's own choice. Supported employment eligibility rests on the client's wish to work, not on staff judgments of readiness.

**Takeaway:** Strengths-based practice follows the client's goal without assumed limits; for a work goal with serious mental illness, link to supported employment.

**Sources:** Rapp, C. A., & Goscha, R. J. (2012). The Strengths Model: A Recovery-Oriented Approach to Mental Health Services (3rd ed.). Oxford University Press, principles of the strengths model; Drake, R. E., Bond, G. R., & Becker, D. R. (2012). Individual Placement and Support: An Evidence-Based Approach to Supported Employment. Oxford University Press, IPS principles: zero exclusion, rapid job search
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.1 Strengths-based and empowerment interventions · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

### 2. INT-1509 (rev 1) · Best answer: B

In Solomon's empowerment framework, a clinic rule that turns away people who lack a fixed address is an example of which concept?

**A.** Indirect power block
**B.** Direct power block
**C.** Learned helplessness

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Indirect power blocks are internal: negative messages absorbed through family and community experience that weaken self-esteem and skills. The clinic rule is an external barrier, not an internalized one.

**B** **Correct.** Solomon (1976) described two sources of powerlessness for stigmatized groups. Direct power blocks are applied by social institutions, such as rules that deny services or resources. Indirect power blocks are negative valuations absorbed through development and family life, which limit skills and self-belief. A clinic rule that excludes people without a fixed address is imposed from outside by an institution, so it is a direct block. It beats A because nothing here has been internalized; the barrier sits in the policy.

**C** **Incorrect** (trap: Wrong model or concept). Learned helplessness describes passivity after repeated uncontrollable events. It is not part of Solomon's framework, and it describes a person's response rather than an institutional rule.

**Takeaway:** Solomon: direct power blocks are institutional barriers; indirect power blocks are internalized negative valuations that erode skills and self-belief.

**Sources:** Solomon, B. B. (1976). Black Empowerment: Social Work in Oppressed Communities. Columbia University Press, direct and indirect power blocks
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.1 Strengths-based and empowerment interventions · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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### 3. INT-1510 (rev 1) · Best answer: A

An 82-year-old resident of an assisted living facility tells the visiting social worker that the monthly bill includes a charge for services the resident never received. The resident is cognitively intact and says, 'I hate making a fuss.' The worker has a good working relationship with the business office. What should the social worker do **FIRST**?

**A.** Coach the resident to raise the charge with the business office
**B.** Call the business office directly to dispute the charge for the resident
**C.** Contact the long-term care ombudsman about the billing error

**Why each option is right or wrong**

**A** **Correct.** Empowerment practice builds the client's own capacity to act and starts with self-advocacy before the worker steps in or escalates. The resident is cognitively intact and able to raise the issue; coaching addresses the reluctance ('I hate making a fuss') and leaves the resident better able to handle the next problem (Lee, 2001; NASW Code 1.02). It beats B because calling for the resident fixes this bill but does for a capable client what the client can do, which reinforces dependence.

**B** **Incorrect** (trap: Good but not best). Making the call is efficient and well-meant, but it takes over a task a capable resident can do and misses a chance to build confidence. Direct advocacy fits if the resident tries and is ignored, or asks the worker to act.

**C** **Incorrect** (trap: Right action, wrong time). The ombudsman is an appropriate outside channel if internal steps fail or the facility retaliates. Going there first skips the resident's own voice and the facility's internal process.

**Takeaway:** Empower capable clients to advocate for themselves first; the worker steps in, or escalates outside the agency, only if those efforts fail.

**Sources:** Lee, J. A. B. (2001). The Empowerment Approach to Social Work Practice: Building the Beloved Community (2nd ed.). Columbia University Press, empowerment practice: building client capacity; NASW Code of Ethics (2021), 1.02

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## 4. INT-1511 (rev 1) · Best answer: D

During a home visit, a parent tells a family services social worker that the school denied the child's request for a classroom aide at an IEP meeting. The parent says, 'I feel like the only one fighting this school.' The parent also mentions that several other families in the program have had the same request denied. What is the **MOST** appropriate action?

**A.** Attend the next IEP meeting and present the parent's case to staff

**B.** Teach the parent relaxation skills to use in school meetings

**C.** File a complaint with the state education agency for the parent

**D.** Connect the parent with a parent-led disability advocacy network

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Presenting the case is direct advocacy and may help in the short term, but it does for the parent what the parent can learn to do and leaves the parent isolated. Attending as a support while the parent speaks would fit empowerment better.

**B** **Incorrect** (trap: Wrong model or concept). Coping skills may ease stress in meetings, but they treat a shared, structural problem as a personal one and leave the parent alone against the school.

**C** **Incorrect** (trap: Overrides the client's choice). A state complaint is a legitimate formal channel, but filing it for the parent bypasses the parent's own voice and decision. It may be a later step the parent chooses with support.

**D** **Correct.** Empowerment practice reduces powerlessness by linking people who share a problem so they can act together, building knowledge, skills, and collective voice (Gutierrez, 1990). The parent feels isolated, and other families face the same denials, so a parent-led advocacy network both ends the isolation and strengthens the parent's capacity to press the school. It beats A because speaking for the parent may win this request but leaves the parent dependent and alone. The worker can still attend meetings as support.

**Takeaway:** When clients share a problem rooted in a system, empowerment links them to act together rather than having the worker act for them.

**Sources:** Gutierrez, L. M. (1990). Working with women of color: An empowerment perspective. Social Work, 35, 149-153, empowerment through connection and collective action
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.1 Strengths-based and empowerment interventions · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 5. INT-1512 (rev 1) · Best answer: C

Last week, a client was released from prison after 5 years and enrolled in a reentry program. The social worker plans to open the first meeting with a strengths-based assessment. Which question is **BEST** to ask?

**A.** What led to the offense that sent you to prison?
**B.** What barriers do you expect when you look for work?
**C.** What helped you get through your time inside?

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Understanding the offense may matter for risk planning, but opening with it frames the client by their worst act. A strengths-based assessment starts with what the client brings.

**B** **Incorrect** (trap: Good but not best). Expected barriers are real and will be addressed in planning. Starting with them makes the first conversation about problems rather than resources.

**C** **Correct.** A strengths-based assessment starts by asking about the capacities, resources, and survival skills a person already has, on the view that struggle can also build strength (Saleebey, 2013). Asking how the client coped with incarceration invites skills, relationships, and achievements, such as classes or work assignments, that can anchor reentry goals. It beats B because barriers matter for planning, but leading with them frames the client by deficits; they are explored after strengths are on the table.

**Takeaway:** A strengths-based assessment opens by asking how the client has coped and what resources they have, before turning to problems.

**Sources:** Saleebey, D. (2013). The Strengths Perspective in Social Work Practice (6th ed.). Pearson, principles of the strengths perspective
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.1 Strengths-based and empowerment interventions · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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## 6. INT-1629 (rev 2) · Best answer: B

Two months after a parent's children entered foster care because of the parent's methamphetamine use, the parent has stayed sober, attended every visit, and has a relative willing to help. At a family team meeting, the parent says, 'Everyone only talks about what I did wrong.' The team then turns to drafting the case plan. Which approach to the plan is **BEST**?

**A.** List the parent's strengths in a separate section of the plan
**B.** Build each safety goal on a strength the parent already shows
**C.** Ask the parent how the meeting has felt so far

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Recording strengths is better than omitting them, but a separate list leaves them unused. The safety goals stay framed only around past harm, and the parent's sobriety, visits and relative do not shape the path to reunification.

**B** **Correct.** The strengths perspective does not ignore danger; it uses what is already working to address it (Saleebey, 2013). Signs of Safety practice builds safety goals from existing strengths, such as a support network (Turnell & Edwards, 1999). Here the relative can anchor a safety plan, and steady sobriety and visits can become measured steps toward reunification. It beats A because listing strengths in a separate section honors them on paper while leaving the safety goals deficit-based, which is the pattern the parent is protesting.

**C** **Incorrect** (trap: Good but not best). Checking in about how the meeting feels can matter, and the parent's frustration deserves a response. It does not answer the question of how to write the plan, though, and the parent's complaint is answered most directly by a plan that is built on what the parent has achieved.

**Takeaway:** Strengths-based child welfare plans use existing strengths as the means of meeting safety goals; they neither ignore risk nor list strengths separately.

**Sources:** Saleebey, D. (2013). The Strengths Perspective in Social Work Practice (6th ed.). Pearson, Principles of the strengths perspective; Turnell, A., & Edwards, S. (1999). Signs of Safety: A Solution and Safety Oriented Approach to Child Protection Casework. W. W. Norton, Building safety from existing strengths

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## 7. INT-1630 (rev 1) · Best answer: A

A client who uses a wheelchair has been turned down after four job interviews. Two of the employers had offices reached only by stairs, so those interviews took place in a lobby. The client says, 'Maybe I'm just not good enough to hire.' Which response from the social worker is **MOST** helpful?

**A.** Help the client look at how barriers shaped these outcomes
**B.** Rehearse interview answers to build the client's confidence
**C.** Contact the employers to raise the access problem
**D.** Refer the client for counseling on self-esteem

**Why each option is right or wrong**

**A** **Correct.** Empowerment practice links personal troubles to structural conditions, so that clients stop blaming themselves for the effects of exclusion (Gutierrez, 1990). This consciousness raising is a first step toward action: a client who sees the barrier can decide how to respond (Lee, 2001). The client blames personal inadequacy even though two interviews were moved to a lobby because the offices were inaccessible. It beats B because rehearsal treats the problem as a skills gap and can confirm the self-blame; skills work may help later, once the client sees the structural part of the picture.

**B** **Incorrect** (trap: Good but not best). Interview practice is a reasonable support, but starting there accepts the client's view that the rejections reflect personal shortcomings. It ignores the access barriers that shaped at least two of the interviews.

**C** **Incorrect** (trap: Overrides the client's choice). Contacting employers may become part of a plan, but doing so now acts for the client before the client has made sense of what happened or chosen a response. Empowerment builds the client's own capacity to act.

**D** **Incorrect** (trap: Hands off too soon). Referral for self-esteem work frames the problem as an internal deficit and sends away a concern the social worker can address directly. The client's self-doubt follows real exclusion, which belongs in this conversation.

**Takeaway:** Empowerment practice helps clients connect personal troubles to structural barriers before choosing action, reducing self-blame for the effects of exclusion.

**Sources:** Gutierrez, L. M. (1990). Working with women of color: An empowerment perspective. Social Work, 35(2), 149-153, Empowerment techniques: raising critical consciousness; Lee, J. A. B. (2001). The Empowerment Approach to Social Work Practice (2nd ed.). Columbia University Press, Consciousness raising
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.1 Strengths-based and empowerment interventions · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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## 8. INT-1631 (rev 1) · Best answer: C

Which case management model works mainly in the community and builds on neighbors, landlords and employers to reach goals the client chooses?

**A.** Broker model
**B.** Clinical case management
**C.** Strengths model
**D.** Assertive community treatment

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Broker case management links clients to formal agencies and does little direct work with them. It neither centers the community as the work setting nor favors informal, natural resources over referrals to programs.

**B** **Incorrect** (trap: Wrong model or concept). Clinical case management combines therapy with resource coordination. The description involves no therapy; it centers on client-chosen goals pursued through informal community resources.

**C** **Correct.** Rapp and Goscha's (2012) strengths model holds that the community is an oasis of resources, the client directs the helping process, and the community rather than the office is the primary setting for work. Using landlords, neighbors and employers to reach client-chosen goals is its hallmark. It beats D because ACT also works in the community, but an ACT team mainly delivers treatment and support itself; the emphasis on naturally occurring resources ahead of formal programs marks the strengths model.

**D** **Incorrect** (trap: Wrong model or concept). ACT is community based too, which makes it tempting. But ACT is a multidisciplinary team that provides most services directly, around the clock, for people with frequent hospitalizations, rather than leaning first on neighbors and employers (SAMHSA ACT EBP KIT, 2008).

**Takeaway:** The strengths model of case management works in the community, follows client-chosen goals, and favors naturally occurring resources over formal services.

**Sources:** Rapp, C. A., & Goscha, R. J. (2012). The Strengths Model: A Recovery-Oriented Approach to Mental Health Services (3rd ed.). Oxford University Press, Principles of the strengths model; SAMHSA (2008) Assertive Community Treatment Evidence-Based Practices KIT, Building Your Program: core features of ACT
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.1 Strengths-based and empowerment interventions · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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## 9. INT-0001 (rev 1) · Best answer: A

In the fifth session, a client who has been engaged suddenly says, 'You're like everyone else. You don't really listen.' The comment follows the social worker's suggestion to try a new coping exercise. What should the social worker do **FIRST** to repair the working relationship?

**A.** Invite the client to say more about feeling unheard
**B.** Explain the reasons for suggesting the coping exercise
**C.** Reassure the client that the social worker is on their side
**D.** Suggest a transfer to a social worker who is a better fit

#### Why each option is right or wrong

**A** **Correct.** A rupture in the alliance is best repaired by meeting it with curiosity rather than defense. Inviting the client to describe the experience of not being heard validates the feedback and gives the social worker information to adjust (Safran & Muran, 2000). The client's reaction to a suggestion is also useful clinical material. It beats C because reassurance tells the client how to feel about the relationship instead of exploring what went wrong, so the rupture stays unaddressed.

**B** **Incorrect** (trap: Too directive). Explaining the rationale defends the suggestion and implies the client misunderstood. It can confirm the client's sense of not being heard. The rationale can be revisited once the client's experience is understood.

**C** **Incorrect** (trap: Good but not best). Reassurance is well-meant but skips the client's actual complaint. Saying the social worker cares does not show listening; exploring the client's experience does.

**D** **Incorrect** (trap: Hands off too soon). Offering a transfer at the first sign of tension treats a normal, repairable rupture as a failed relationship. Working through ruptures often strengthens the alliance.

**Takeaway:** Meet an alliance rupture with nondefensive curiosity: invite the client's experience before explaining or reassuring.

**Sources:** Safran, J. D., & Muran, J. C. (2000). Negotiating the Therapeutic Alliance: A Relational Treatment Guide. Guilford Press, rupture resolution
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.2 Building and sustaining the helping relationship (empathy, rapport, acceptance) · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 10. INT-0100 (rev 1) · Best answer: B

By the fourth session, a social worker notices growing impatience each time a client describes relying on relatives for help. The social worker recognizes that the reaction echoes long-standing feelings about a dependent family member in the social worker's own life. What should the social worker do **NEXT**?

**A.** Tell the client about the reaction to keep the relationship genuine
**B.** Explore the reaction with a supervisor or consultant
**C.** Transfer the client to a colleague to protect the alliance

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). Disclosing countertransference shifts the focus to the social worker's feelings and can burden or confuse the client. Self-disclosure should serve the client's needs, and the reaction needs to be understood in supervision first.

**B** **Correct.** Countertransference is the social worker's own emotional reaction to a client, often rooted in the worker's history. Once it is recognized, the next step is to examine it in supervision or consultation so it informs, rather than drives, the work (Gelso & Hayes, 2007; NASW Code 2.05(a)). Here the worker has already named the source of the impatience, so exploring it with a supervisor protects the client from subtle withdrawal or judgment. It beats C because a transfer ends a relationship that is not yet harmed, before the worker has tried to understand and manage the reaction.

**C** **Incorrect** (trap: Hands off too soon). Transfer may become appropriate if supervision shows the reaction cannot be managed and is harming the work. As a next step, it hands off a client the worker has not yet tried to serve with support.

**Takeaway:** When countertransference is recognized, examine it in supervision or consultation before considering disclosure or transfer.

**Sources:** Gelso, C. J., & Hayes, J. A. (2007). Countertransference and the Therapist's Inner Experience: Perils and Possibilities. Lawrence Erlbaum, managing countertransference; NASW Code of Ethics (2021), 2.05(a)
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.2 Building and sustaining the helping relationship (empathy, rapport, acceptance) · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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### 11. INT-1500 (rev 1) · Best answer: C

A hospital patient recovering from a leg amputation says, 'Everyone keeps telling me how lucky I am to be alive.' The social worker responds, 'Part of you may be grieving a loss that nobody around you has made room for yet.' Which skill does the social worker's response demonstrate?

**A.** Reciprocal empathy
**B.** Interpretation
**C.** Additive empathy
**D.** Paraphrasing

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Reciprocal, or interchangeable, empathy reflects the feeling and content the client actually expressed, at the same depth. A reciprocal reply here would name frustration with what others keep saying. This response adds a feeling the patient did not state.

**B** **Incorrect** (trap: Wrong model or concept). Interpretation offers a new explanation, often linking present reactions to patterns or past experience. This response stays with a present, implied feeling and offers no causal frame, so it is closer to additive empathy than to interpretation.

**C** **Correct.** Additive empathy goes a step past what the client said, naming feelings or meanings that are implied but unspoken. The patient voiced irritation at other people's comments; the social worker names an unstated grief and the sense that no one has allowed it. Going beneath the surface in this way is additive. It beats A because reciprocal empathy would mirror only the expressed feeling, irritation with others, without adding the deeper meaning (Hepworth et al.).

**D** **Incorrect** (trap: Wrong model or concept). Paraphrasing restates the content of what the client said, such as others calling the patient lucky. The response here centers on an unspoken feeling rather than restating content.

**Takeaway:** Reciprocal empathy mirrors the feeling a client expressed; additive empathy names implied, unspoken feelings and fits once rapport exists.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Additive empathy, interpretation, and confrontation

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### 12. INT-1501 (rev 2) · Best answer: A

In weekly telehealth sessions, a client with panic attacks tells the social worker, 'You really get me.' Their plan centers on the client's wish to finish a grocery trip without fleeing the store in panic. Each week, though, the client skips the breathing practice, saying, 'I don't see how that connects to anything.' Which part of the working alliance is **MOST** likely weakest?

**A.** Agreement on tasks
**B.** Emotional bond
**C.** Agreement on goals

#### Why each option is right or wrong

**A** **Correct.** Bordin (1979) described the working alliance as three linked parts: agreement on goals, agreement on tasks, and the bond. Here the bond is strong ('You really get me') and the plan is built on the client's own wish, so the aim is shared. The break is the client's doubt that breathing practice leads anywhere, which is disagreement about the methods used to reach the goal. It beats C because the client accepts the goal itself and questions only the route. The repair is to discuss how each exercise links to that goal.

**B** **Incorrect** (trap: Wrong model or concept). The bond is the trust and attachment between client and worker. The client's warm comment shows a strong bond, so it is not the weak part here. Bond problems show up as mistrust, guardedness, or ruptures.

**C** **Incorrect** (trap: Wrong model or concept). Goal agreement concerns what the work is trying to achieve. The plan is built on the client's own wish to finish a grocery trip, so the aim is shared. Weak goal agreement would look like the client wanting a different outcome altogether.

**Takeaway:** Bordin's alliance has three parts: goals, tasks, and bond. A client who accepts the goal but doubts the methods signals weak task agreement.

**Sources:** Bordin, E. S. (1979). The generalizability of the psychoanalytic concept of the working alliance. Psychotherapy: Theory, Research & Practice, 16(3), 252-260, three components of the working alliance
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.2 Building and sustaining the helping relationship (empathy, rapport, acceptance) · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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## 13. INT-1502 (rev 1) · Best answer: B

A 70-year-old client, widowed for 2 years, has met with the social worker weekly for 4 months. Near the end of a session, the client says, 'I think I've fallen in love with you. You're the only one who understands me.' What is the social worker's **BEST** response?

**A.** Arrange a transfer to a colleague before the next session
**B.** Explore the feelings in sessions while holding a clear role
**C.** Remind the client that the relationship is strictly professional

**Why each option is right or wrong**

**A** **Incorrect** (trap: Hands off too soon). Transferring at the first disclosure treats a workable transference as a failure and can feel like rejection to a grieving client. Transfer becomes appropriate if the feelings cannot be managed in the work or the worker's own reactions interfere.

**B** **Correct.** Romantic feelings toward a social worker are a common form of transference and are treated as clinical material, not as a crisis. Exploring them, while the worker keeps the role clear and never reciprocates (NASW Code 1.09(a)), can show what the client longs for, here connection after losing a spouse. It beats C because a bare reminder of the rules is accurate but can leave the client ashamed and closes off useful material; the boundary is held while the meaning is explored (Hepworth et al.).

**C** **Incorrect** (trap: Good but not best). Stating the professional boundary is necessary, but stopping there can shame the client and lose the chance to understand the feelings. The boundary works best when it is paired with exploration of what the feelings mean.

**Takeaway:** Treat a client's romantic feelings as transference to explore within a clear professional role, not as grounds for immediate transfer.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Relational reactions: transference; NASW Code of Ethics (2021), 1.09(a)
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### 14. INT-1503 (rev 1) · Best answer: D

A social worker in a court-ordered treatment program feels intense disgust toward a client convicted of harming a child. For 6 weeks, the worker has discussed the reaction in weekly supervision and in personal therapy. The disgust persists, the worker dreads each session, and the client says, 'You barely look at me anymore.' What is the **MOST** appropriate next step?

**A.** Keep meeting while continuing to process the reaction in supervision
**B.** Tell the client about the reaction so the work feels honest
**C.** Ask the client to describe the change they have noticed
**D.** Arrange a planned transfer to another qualified social worker

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Processing countertransference in supervision is the correct first response, and it has been tried. After 6 weeks without change, and with the client feeling the withdrawal, staying on keeps the client in a relationship that is not working.

**B** **Incorrect** (trap: Boundary slip or collusion). Disclosing disgust would burden and shame the client and serve the worker's need to unload. Workers usually do not share countertransference feelings with clients; they work through them in supervision or therapy.

**C** **Incorrect** (trap: Good but not best). Inviting the client's view is a good repair move when the cause of a strain is unclear. Here the cause is known, the worker's persisting reaction, and exploring it with the client cannot restore the worker's capacity to stay engaged.

**D** **Correct.** When a worker's reaction keeps impairing the relationship after supervision and personal work, a planned transfer protects the client. The Code directs referral when a social worker is not being effective and other services are needed (NASW Code 1.16(a)). The client already senses the withdrawal. It beats A because supervision has been tried for 6 weeks without change, so more of the same prolongs ineffective care. Supervision remains the right first step in most cases (Hepworth et al.).

**Takeaway:** Take countertransference to supervision first; if it still impairs the work, arrange a planned transfer rather than continue ineffective care.

**Sources:** NASW Code of Ethics (2021), 1.16(a); Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Relational reactions: countertransference
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### 15. INT-1504 (rev 1) · Best answer: C

In a juvenile detention center, a 16-year-old describes assaulting another youth. The social worker says plainly that the assault caused real harm. In the same meeting and every meeting after, the worker treats the youth as a person of worth who deserves care. Which element of Rogers' person-centered approach does the worker's stance toward the youth **MOST** likely show?

**A.** Congruence
**B.** Accurate empathy
**C.** Unconditional positive regard

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Congruence is genuineness: the worker's outward responses match inner experience. Naming the harm plainly is congruent, which makes this option tempting, but the stance the question asks about is the steady valuing of the youth, not the worker's transparency.

**B** **Incorrect** (trap: Wrong model or concept). Accurate empathy is understanding the client's inner world and conveying that understanding. The stem describes how the worker values the youth, not how the worker grasps the youth's feelings.

**C** **Correct.** Rogers (1957) named unconditional positive regard as a core condition: the worker prizes the client as a person without making that acceptance depend on behavior. The worker condemns the act yet keeps treating the youth as worthy of care, which separates the person from the behavior. It beats A because naming the harm honestly reflects congruence, but the continuing valuing of the youth is the defining feature here.

**Takeaway:** Unconditional positive regard values the client as a person while the worker can still name harmful behavior honestly.

**Sources:** Rogers, C. R. (1957). The necessary and sufficient conditions of therapeutic personality change. Journal of Consulting Psychology, 21(2), 95-103, core conditions
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.2 Building and sustaining the helping relationship (empathy, rapport, acceptance) · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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### 16. INT-1505 (rev 1) · Best answer: A

A social worker at a community mental health center canceled two sessions in a row, each with only an hour's notice. At the next session, a client who is usually talkative gives one-word answers and looks at the floor. What is the social worker's **BEST** response?

**A.** Name the late cancellations and take responsibility for them
**B.** Ask the client why they seem so quiet today
**C.** Move ahead with the planned agenda to restore a sense of routine

**Why each option is right or wrong**

**A** **Correct.** When the worker's own behavior has likely strained the relationship, repair starts with the worker raising it and owning it. Two last-minute cancellations are a plausible cause of the client's withdrawal; naming them openly, without excuses, models accountability and invites the client's reaction (Safran & Muran, 2000). It beats B because asking the client to explain the silence puts the burden on the client and sidesteps the strain the worker caused, and a 'why' question tends to provoke defensiveness.

**B** **Incorrect** (trap: Misuses the technique). Inviting the client's view is often a good opening, but this version asks 'why,' which can feel accusatory, and it ignores the likely cause the worker already knows. Open curiosity fits best after the worker owns the cancellations.

**C** **Incorrect** (trap: Good but not best). Routine can steady some clients, but pressing on past a visible strain leaves it unaddressed and can deepen the client's sense that the relationship is unreliable.

**Takeaway:** When the worker's own lapse strains the alliance, raise it and take responsibility before pressing on or asking the client to explain.

**Sources:** Safran, J. D., & Muran, J. C. (2000). Negotiating the Therapeutic Alliance: A Relational Treatment Guide. Guilford Press, rupture resolution: therapist acknowledging own contribution
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## 17. INT-1506 (rev 1) · Best answer: B

During the first week of an intensive outpatient program for alcohol use, a client wipes away tears while saying, 'Honestly, I'm doing great. This place is just a formality for my job.' Which response by the social worker is **MOST** helpful at this point?

**A.** Point out the mismatch between the tears and the words
**B.** Gently reflect the sadness that shows as the client speaks
**C.** Accept the statement and continue with the intake questions
**D.** Suggest that the tears may come from grief the client avoids

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Pointing out a discrepancy is a valid technique, but it belongs later, once trust is established. In the first week, it is likely to be heard as calling the client dishonest and to increase discord.

**B** **Correct.** Early in a relationship, a gap between words and feelings is met with empathic reflection, not confrontation. Naming the sadness gently shows the client that the worker notices and accepts the feeling without challenging the words. It beats A because confronting a discrepancy works only after rapport exists; done in the first week, it can feel like an accusation and raise defensiveness. Reflection builds the alliance that later confrontation needs (Hepworth et al.; Miller & Rollnick, 2023).

**C** **Incorrect** (trap: Good but not best). Moving on respects the client's words but ignores the visible feeling and misses a chance to show understanding. Noticing and reflecting the emotion builds rapport more effectively.

**D** **Incorrect** (trap: Too directive). Offering a hypothesis about hidden grief is an interpretation, the riskiest of the interviewing techniques, and it needs a strong alliance. Offered in the first week, it is likely to feel presumptuous.

**Takeaway:** Early on, meet a gap between a client's words and feelings with gentle reflection; save confrontation and interpretation until rapport forms.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Empathic communication; confrontation; Miller & Rollnick (2023). Motivational Interviewing (4th ed.), Engaging: reflective listening
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## 18. INT-1507 (rev 1) · Best answer: C

A 6-year-old placed in an emergency foster home meets a child welfare social worker for the first time. The child hides behind a chair and will not answer questions. Crayons and toy animals sit on a small table in the room. What should the social worker do **FIRST** to build rapport?

**A.** Ask the child gentle questions about life at home
**B.** Explain in simple words why the child is in this home
**C.** Sit nearby and begin playing quietly with the toys

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Questions about home are part of the work, but asked of a hiding child they add pressure and may feel like a test. Questions about possible maltreatment also belong to a trained forensic interviewer, not a rapport visit.

**B** **Incorrect** (trap: Right action, wrong time). A simple explanation of the placement matters and should come soon. Given to a child who is hiding and silent, it is unlikely to register; engagement through play makes it possible.

**C** **Correct.** Young children build trust and communicate through play more readily than through talk. Sitting at the child's level and playing quietly lets the child approach at their own pace, which lowers fear in a frightened, newly placed 6-year-old (Webb, 2019). It beats B because an explanation, even a simple one, asks a frightened child to listen before any trust exists; it lands better once the child has engaged.

**Takeaway:** With young children, build trust through play at the child's own pace before asking questions or giving explanations.

**Sources:** Webb, N. B. (2019). Social Work Practice with Children (4th ed.). Guilford Press, engaging children; play as communication
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### 19. INT-1625 (rev 1) · Best answer: B

A client in outpatient treatment for alcohol use disorder grew up with a parent who withdrew affection after any mistake. In the fourth month, the client admits to hiding a relapse and waits to be dropped from the program. The social worker responds calmly and keeps working with the client. Weeks later, the client says, 'You didn't pull away, and now I can admit mistakes at work.' Which concept **MOST** likely explains this change?

**A.** Holding environment
**B.** Corrective emotional experience
**C.** Transference

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Winnicott's holding environment is the steady, safe frame of the relationship that contains a client's distress. It is part of the background here, but the change follows one specific event: an expected rejection that did not come. That points to a corrective emotional experience.

**B** **Correct.** Alexander and French (1946) described the corrective emotional experience: a client relives an old relational conflict with the helper, the helper responds differently from the original figure, and the new outcome loosens the old expectation. Here the client expects the parent's withdrawal after confessing a relapse, the social worker stays steady, and the client then risks admitting mistakes elsewhere. It beats A because a holding environment (Winnicott, 1965) describes the reliable, containing frame that makes such work possible, not the specific disconfirmation of an expected response that produced this change.

**C** **Incorrect** (trap: Wrong model or concept). The client's expectation of being dropped is transference: a response carried over from the parent. Transference explains the fear before the confession, not the change afterward. The shift came from the social worker responding differently than the parent did.

**Takeaway:** When a helper responds differently than early caregivers did to a replayed conflict, the corrective emotional experience can revise the client's expectations.

**Sources:** Alexander, F., & French, T. M. (1946). Psychoanalytic Therapy: Principles and Application. Ronald Press, Corrective emotional experience; Winnicott, D. W. (1965). The Maturational Processes and the Facilitating Environment. Hogarth Press, Holding environment
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### 20. INT-1626 (rev 2) · Best answer: D

In the tenth month of weekly psychodynamic therapy, a client whose parent died when the client was 8 misses the session after the social worker announces a 3-week leave. Coverage plans were given at the time of the announcement. At the next session, the client shrugs and says, 'It doesn't matter. You'll be gone anyway.' What is the social worker's **BEST** response?

**A.** Review how to reach the covering social worker during the leave
**B.** Interpret the reaction as grief for the parent who died
**C.** Reassure the client that the leave is short and the work will go on
**D.** Invite the client to say more about the coming break

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Coverage was already explained when the leave was announced. Repeating it treats the comment as a practical question and steps around the feeling behind the missed session and the shrug.

**B** **Incorrect** (trap: Right action, wrong time). The link to the parent's death may be accurate, and in long-term work an interpretation can be appropriate. Offered now, before the client has explored the present reaction, it jumps ahead of the client's awareness and is likely to be dismissed. It fits better after exploration brings the material closer.

**C** **Incorrect** (trap: Good but not best). Reassurance is kind and may ease some worry, but it answers the shrug with the worker's view instead of the client's. It closes down the feelings the missed session hints at, which are the material that matters before the break, and they may resurface as further absences.

**D** **Correct.** A client's reaction to the helper's absence is central relationship material, and psychodynamic work starts at the surface: the helper invites the client's own thoughts and feelings about the break before linking them to the past (Gabbard, 2014). The missed session and the shrug suggest the leave stirred more than the client says, and talking about it now protects the alliance before the gap. It beats B because offering the link to the parent's death before the client has looked at the present reaction is likely to feel intrusive and be brushed aside; that interpretation may fit later, once the client's own associations lead there.

**Takeaway:** A client's reaction to the helper's absence is relationship material; invite the present reaction before offering links to the past.

**Sources:** Gabbard, G. O. (2014). Psychodynamic Psychiatry in Clinical Practice (5th ed.). American Psychiatric Publishing, Individual psychotherapy: expressive-supportive continuum of interventions
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## 21. INT-1627 (rev 2) · Best answer: A

At the second session in a community clinic, a client with a history of brief, intense relationships says the social worker is 'the first one who really gets me.' The client describes two former workers as cold and useless. What is the **MOST** appropriate response?

**A.** Thank the client and invite feedback when the worker gets it wrong
**B.** Accept the praise warmly and move on to the client's goals
**C.** Point out that the client may be idealizing the worker

**Why each option is right or wrong**

**A** **Correct.** Early idealization, especially with a history of intense relationships that end badly, often swings to disappointment. Safran and Muran (2000) describe building an alliance in which ruptures can be named and worked through; inviting feedback now turns a future letdown into something to discuss rather than a reason to leave. Accepting the thanks keeps the bond warm in session two. It beats C because confronting idealization before trust exists is likely to feel like rejection and repeat the pattern the client fears; confrontation fits after rapport is established (Hepworth et al.).

**B** **Incorrect** (trap: Good but not best). Accepting the thanks keeps the bond warm in session two, and returning to goals is reasonable. Moving on without preparing for future disappointment, though, misses the chance to make ruptures discussable, which matters most for a client whose relationships tend to swing from ideal to useless and end abruptly.

**C** **Incorrect** (trap: Too directive). Naming idealization can be useful later, once the alliance can hold it. In the second session it is likely to land as criticism and may push away a client whose relationships tend to end abruptly.

**Takeaway:** Early idealization often precedes disappointment; build an alliance where future ruptures can be voiced instead of colluding with or confronting it.

**Sources:** Safran, J. D., & Muran, J. C. (2000). Negotiating the Therapeutic Alliance: A Relational Treatment Guide. Guilford Press, Ruptures and metacommunication; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Confrontation: timing after rapport

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## 22. INT-1628 (rev 2) · Best answer: C

On an inpatient psychiatric unit, a newly admitted client with schizophrenia stands near the door, refuses to sit, and says staff record thoughts through the ceiling lights. The social worker is meeting the client for the first time to begin a working relationship. What should the social worker do **FIRST**?

**A.** Explain calmly that the lights cannot record thoughts

**B.** Ask what the client has noticed about the ceiling lights

**C.** Introduce self briefly and let the client choose the distance

#### Why each option is right or wrong

**A** **Incorrect** (trap: Too directive). Disputing a fixed belief at the first meeting usually deepens mistrust. Gentle, collaborative reality testing can come later, once the client feels safe with the social worker.

**B** **Incorrect** (trap: Right action, wrong time). Curious, non-judgmental questions about a belief are part of later collaborative work, so this is a tempting move. At first contact with a frightened client standing by the door, though, opening with the feared topic adds pressure; a brief introduction and control over distance come first.

**C** **Correct.** With a frightened, suspicious client, engagement starts with predictability and control: a short, clear introduction, respect for physical distance, and no pressure to discuss the belief. Kingdon and Turkington (2005) stress building trust before examining delusional beliefs, and standing by the door signals fear. It beats A because arguing against the belief at first contact, however calmly, tends to harden it and casts the social worker as one more person who does not believe the client. Respectful contact can begin now, without waiting.

**Takeaway:** With a suspicious client, begin with brief, predictable contact that gives the client control, and avoid disputing the belief before trust exists.

**Sources:** Kingdon, D. G., & Turkington, D. (2005). Cognitive Therapy of Schizophrenia. Guilford Press, Engagement and the therapeutic relationship
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## 23. INT-0101 (rev 1) · Best answer: D

At a hospital care conference, the physician and nurse recommend discharging a patient with heart failure to a skilled nursing facility. The social worker's assessment shows that the patient has decision-making capacity, wants to return home, and has a relative willing to help with home services. What is the social worker's **BEST** action?

**A.** Accept the physician's recommendation as the team's decision
**B.** Encourage the patient privately to refuse the facility placement
**C.** Refer the disagreement to the hospital ethics committee
**D.** Present the patient's goals and a home-support plan to the team

**Why each option is right or wrong**

**A** **Incorrect** (trap: Overrides the client's choice). Deferring to the physician leaves out information the team needs and lets a plan go forward against a capable patient's wishes. Medical input matters, but placement also rests on psychosocial factors and the patient's choice.

**B** **Incorrect** (trap: Boundary slip or collusion). Coaching the patient behind the team's back splits the team before the social worker has even shared the assessment. Supporting the patient's voice works best openly, in the conference where the decision is made.

**C** **Incorrect** (trap: Right action, wrong time). An ethics committee consult is a reasonable later step if the team still overrides a capable patient's choice. Using it first skips the direct team discussion the Code expects (NASW Code 2.03(b)).

**D** **Correct.** Social workers on interdisciplinary teams contribute the social work perspective to decisions, including the client's own goals (NASW Code 2.03(a)). The decisive facts are that the patient has capacity, wants to go home, and has a workable support plan, so the social worker brings that assessment into the conference where the decision is being made, supporting self-determination (NASW Code 1.02). It beats C because the Code directs social workers to try resolving team disagreement through the team's own channels first; an ethics consult fits if the team still overrides a capable patient.

**Takeaway:** On an interdisciplinary team, bring the client's goals and the psychosocial assessment into team discussion before escalating elsewhere.

**Sources:** NASW Code of Ethics (2021), 2.03(a); NASW Code of Ethics (2021), 2.03(b); NASW Code of Ethics (2021), 1.02
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### 24. INT-1530 (rev 1) · Best answer: C

A social worker co-leads an anger-management group for adolescents at a residential treatment center. The colleague often contradicts the social worker in front of members, and members have begun taking sides. The two discussed the pattern privately after two sessions, but nothing changed. What should the social worker do **NEXT**?

**A.** Name the disagreement openly with members during the next session
**B.** Ask the program director to assign a different co-leader
**C.** Take the conflict to a joint supervision meeting

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Co-leaders can model respectful disagreement in session, but only when their relationship is sound. Raising a live rivalry with teens who are already taking sides risks deepening the split. This fits after the pair has worked the conflict through.

**B** **Incorrect** (trap: Hands off too soon). Replacing the partner skips the in-house step of joint supervision and leaves the members' side-taking unexamined. Reassignment fits if supervised work fails or the pairing proves unworkable.

**C** **Correct.** Co-leaders need a working alliance; unresolved rivalry between them splits a group and models poor conflict handling for members (Yalom & Leszcz, 2020). The pair already tried a private talk, and the pattern continues while members take sides, so the next step is outside help: a joint supervision or consultation meeting where both leaders examine the dynamic together. It beats A because airing a still-unresolved conflict in group invites members to widen the split; open disagreement helps members only when the co-leaders are working well together.

**Takeaway:** Co-leader conflict that persists after direct talk and splits the group goes to joint supervision, not to members or a reassignment.

**Sources:** Yalom & Leszcz (2020). The Theory and Practice of Group Psychotherapy (6th ed.), The therapist: co-therapy
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### 25. INT-1531 (rev 1) · Best answer: B

Before the annual IEP meeting for an 8-year-old autistic student, the parent tells the school social worker, 'There will be eight of them and one of me. I'll just sign whatever they put in front of me.' The parent has strong views about the child's reading support. What is the social worker's **BEST** response?

**A.** Offer to present the parent's reading concerns to the team
**B.** Help the parent write down priorities and questions to raise
**C.** Reassure the parent that the team wants what is best for the child

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Speaking for the parent may get the concern heard once, but it leaves the parent in a passive role at this and future meetings. Speaking on the parent's behalf fits if the parent asks for it or cannot attend.

**B** **Correct.** Under IDEA, parents are members of the IEP team, and schools must make sure they have a real chance to participate (34 CFR 300.321, 300.322). This parent has clear views but expects to be overwhelmed, so the most useful step is helping the parent organize priorities and questions to bring, which builds the parent's own voice on the team. It beats A because presenting the concerns for the parent may help once but keeps the parent passive; supporting the client's self-advocacy comes before advocating on their behalf (Hepworth et al.).

**C** **Incorrect** (trap: Good but not best). Reassurance may ease anxiety, but it does nothing to help the parent voice strong views about reading support. It can also discourage the parent from speaking up at all.

**Takeaway:** Help parents take part as full IEP team members by preparing their own priorities, rather than speaking for them or simply reassuring them.

**Sources:** IDEA Part B regulations, 34 CFR Part 300, 34 CFR 300.321(a)(1); 300.322(a); Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Advocacy and social action: case and cause advocacy
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## 26. INT-1532 (rev 1) · Best answer: C

In an early intervention home-visiting program, one social worker is the only provider who visits a toddler's family. A speech pathologist and an occupational therapy practitioner coach the social worker, who then teaches their strategies to the parents during visits. The three plan goals together with the family. Which team model is this program **MOST** likely using?

**A.** Interdisciplinary
**B.** Multidisciplinary
**C.** Transdisciplinary
**D.** Intradisciplinary

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Interdisciplinary teams meet and build an integrated plan, which fits the joint goal setting. But each professional still delivers their own services. Here only the social worker visits and uses the other professions' strategies.

**B** **Incorrect** (trap: Wrong model or concept). Multidisciplinary teams work in parallel: each profession assesses and treats separately, with little coordination. The joint planning and the single provider carrying the others' methods do not fit.

**C** **Correct.** In a transdisciplinary team, members share assessment and planning with the family and practice role release: one primary provider delivers services while teammates teach and back up that provider (King et al., 2009). Here a single social worker carries the speech and occupational therapy strategies into the home after coaching. It beats A, interdisciplinary, where members plan together and share findings but each still delivers their own profession's services. Joint planning fits both; the single provider using others' strategies is what makes this transdisciplinary.

**D** **Incorrect** (trap: Wrong model or concept). Intradisciplinary collaboration happens among members of one profession, such as two social workers co-leading a group. This team includes a speech pathologist and an occupational therapist.

**Takeaway:** Transdisciplinary teams use role release: one primary provider delivers care, using strategies other professions teach, within a shared plan.

**Sources:** King, G., Strachan, D., Tucker, M., et al. (2009). The application of a transdisciplinary model for early intervention services. Infants & Young Children, 22(3), 211-223, whole article

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## 27. INT-1533 (rev 2) · Best answer: A

On a hospital medical unit, a nurse stops the social worker in the hallway and complains that the attending physician talks down to a patient's family and ignores their questions. The nurse asks the social worker to 'say something' to the physician. What is the **MOST** appropriate response by the social worker?

**A.** Encourage the nurse to raise the concern with the physician directly

**B.** Bring the nurse's complaint to the physician on the nurse's behalf

**C.** Report the physician's conduct to the unit's medical director

#### Why each option is right or wrong

**A** **Correct.** Social workers cooperate with colleagues of other professions when it serves clients' well-being (NASW Code 2.01(c)), and team concerns are best settled by the people directly involved. Pulling a third person into a two-person conflict forms a triangle, which eases tension briefly but leaves the conflict unresolved (Bowen, 1978). The nurse saw the problem and can describe it best, so the social worker supports the nurse in speaking with the physician and can suggest a team meeting if that fails. It beats B because carrying the complaint makes the social worker a go-between, distorts the message and weakens trust between nurse and physician.

**B** **Incorrect** (trap: Boundary slip or collusion). Carrying the complaint makes the social worker a go-between: the physician hears it secondhand and the nurse never addresses it. Speaking up directly is right when the social worker has seen the problem firsthand.

**C** **Incorrect** (trap: Skips assessment). Going over the physician's head before anyone has spoken with the physician skips the direct step and can damage team trust. Formal escalation fits if direct talk fails and the family's care is being harmed.

**Takeaway:** Support teammates in raising concerns directly with each other; carrying complaints for them creates triangles that keep team conflict unresolved.

**Sources:** NASW Code of Ethics (2021), 2.01(c); Bowen, M. (1978). Family Therapy in Clinical Practice. Jason Aronson, triangles

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### 28. INT-1639 (rev 1) · Best answer: B

A hospital social worker learns that a patient recovering from hip surgery will be discharged home this afternoon. The patient lives alone up a flight of stairs, cannot yet climb them, and home health services cannot start for 4 days. The patient says they are afraid of falling. The social worker shared these facts and the physical therapy evaluation with the attending physician, who says the order stands. What should the social worker do **NEXT**?

**A.** Ask the physician again to reconsider the discharge time
**B.** Take the concern up the chain of command today
**C.** Document the concern and complete the discharge plan

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Direct communication with the physician is the right first step, and it has been taken with the supporting facts. Asking again without new information is unlikely to change the decision before the afternoon discharge.

**B** **Correct.** When a team decision raises ethical concerns, social workers try to resolve the disagreement through appropriate channels and, if that fails, pursue other avenues consistent with client well-being (NASW Code 2.03(b)). The social worker has already gone directly to the physician with the facts, and the patient faces a same-day discharge that is unsafe as planned. Taking the concern up the hospital's chain, such as the social work supervisor or a physician advisor, is the next step. It beats A because the direct conversation has already happened; repeating it without new information spends the little time left before the afternoon discharge.

**C** **Incorrect** (trap: Ignores risk). Documenting the concern is necessary, but completing an unsafe plan leaves a patient who cannot manage stairs alone at home for 4 days. Recording a risk does not end the duty to pursue the concern.

**Takeaway:** When direct discussion fails to resolve a team decision that endangers a patient, pursue the concern through the organization's escalation channels.

**Sources:** NASW Code of Ethics (2021), 2.03(b)
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### 29. INT-1640 (rev 1) · Best answer: A

In an intensive care unit, a patient's family has heard different predictions about recovery from the cardiology and neurology teams. The family asks for a meeting, and the social worker is asked to arrange it. What should the social worker do **FIRST**?

**A.** Convene a brief team meeting to align the medical message
**B.** Ask the family to list the questions they most want answered
**C.** Schedule the family meeting so each team can present its view

**Why each option is right or wrong**

**A** **Correct.** Family conferences go better when clinicians meet beforehand to share what each knows and agree on the information and goals for the meeting (Curtis & White, 2008). This family's distress comes from conflicting messages, so the teams must reconcile their views before meeting the family together. Contributing to team decisions is part of the social worker's role (NASW Code 2.03(a)). It beats B because gathering the family's questions is useful preparation, but it helps most once the team has a shared message to answer them with.

**B** **Incorrect** (trap: Right action, wrong time). Learning the family's questions is good preparation, but it does not fix the source of the confusion. Without a shared team message, the answers will conflict again in the meeting.

**C** **Incorrect** (trap: Good but not best). Bringing every team together sounds transparent, but presenting differing predictions side by side in front of the family repeats the confusion. Disagreements should be worked through among clinicians first.

**Takeaway:** Before a family conference, the team meets to reconcile information and agree on goals, especially when the family has heard conflicting messages.

**Sources:** Curtis, J. R., & White, D. B. (2008). Practical guidance for evidence-based ICU family conferences. Chest, 134(4), 835-843, Preparing for the family conference; NASW Code of Ethics (2021), 2.03(a)
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## 30. INT-1513 (rev 2) · Best answer: B

At a homeless shelter, a client cannot apply for jobs because a stolen wallet held the client's ID and birth certificate. The social worker and client have agreed that replacing the ID is the problem to work on first. What should the social worker do **NEXT**?

**A.** Pick the quickest way to replace the ID and tell the client
**B.** List possible ways to replace the ID with the client
**C.** Request a shelter residency letter to take to the ID office
**D.** Explore how losing the wallet has affected the client

**Why each option is right or wrong**

**A** **Incorrect** (trap: Too directive). Choosing for the client is efficient, but it skips generating and weighing options and puts the worker in charge of the client's problem. Choosing comes after alternatives are listed and compared with the client.

**B** **Correct.** The problem-solving sequence runs from defining the problem to generating alternatives, deciding, acting, and checking results (D'Zurilla & Goldfried, 1971). The problem is now defined and agreed, so the next step is brainstorming options with the client, such as a shelter letter, a vital records request, or a legal aid clinic. The worker's role is to support the client's own problem solving. It beats A because picking the route for the client skips the alternatives step and takes the decision away from the client.

**C** **Incorrect** (trap: Right action, wrong time). A shelter residency letter may well become part of the plan, but acting on one route now skips generating and weighing alternatives with the client. It fits once the client has compared the options and chosen that route.

**D** **Incorrect** (trap: Right action, wrong time). Exploring the impact of the theft is a worthwhile engagement move, but the pair have already defined the problem together. Returning to feelings now delays the concrete work the client needs.

**Takeaway:** In problem solving, once the problem is defined, generate alternatives with the client before choosing, acting, or evaluating.

**Sources:** D'Zurilla, T. J., & Goldfried, M. R. (1971). Problem solving and behavior modification. Journal of Abnormal Psychology, 78(1), 107-126, stages of problem solving

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## 31. INT-1514 (rev 1) · Best answer: C

At a family resource center, a parent lists four concerns: finishing a GED, a car that needs repairs, a child's behavior at school, and an electricity shutoff notice for Friday. It is January, the home is heated by electricity, and an infant lives there. The parent says the child's school behavior worries them most. What should the social worker do **FIRST**?

**A.** Begin with the child's school behavior that the parent named

**B.** Ask the parent to rank the four concerns by importance

**C.** Work with the parent to stop the utility disconnection

**D.** Build one plan that covers the four concerns together

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Starting with the client's top concern is usually right and builds engagement. Here, a heating shutoff in winter with an infant at home is an imminent safety risk with a fixed deadline, so it comes first.

**B** **Incorrect** (trap: Right action, wrong time). Asking the client to rank concerns is a sound step when priorities are unclear. The parent has already named a top concern, and one problem carries a safety deadline that ranking would not change.

**C** **Correct.** When a client has many problems, the worker helps prioritize, usually starting with what matters most to the client, but threats to safety and basic needs come first. A January power shutoff in an electrically heated home with an infant is an imminent health risk due in days, and it is concrete and solvable (Maslow, 1943; Hepworth et al.). It beats A because starting where the client is remains the general rule; the worker can explain the urgency, deal with the shutoff, then turn to the school concern.

**D** **Incorrect** (trap: Misuses the technique). Tackling everything at once overwhelms the client and loses focus. Problems are partialized and taken one at a time, starting with the most urgent.

**Takeaway:** Help clients prioritize by their own concerns, except that imminent threats to safety or basic needs are addressed first.

**Sources:** Maslow, A. H. (1943). A theory of human motivation. Psychological Review, 50(4), 370-396, hierarchy of needs; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Assessment: prioritizing problems

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### 32. INT-1515 (rev 2) · Best answer: B

Before a parent's hospital discharge, an adult child asks the social worker, 'Should I move my parent in with me or choose a nursing facility? Just tell me what to do.' The parent has moderate neurocognitive disorder and has said they want to stay with family. The adult child works full time and has two teenagers at home. What is the social worker's **BEST** response?

**A.** Recommend the nursing facility given the caregiving demands described
**B.** Help the family weigh both options, including the patient's wishes
**C.** Schedule a family meeting so the care team can recommend a plan

**Why each option is right or wrong**

**A** **Incorrect** (trap: Too directive). A recommendation answers the request but takes over a values-laden family decision and sets aside the parent's stated wishes. The worker can share information about each option without choosing for the family.

**B** **Correct.** In problem solving, the social worker's role is to strengthen the client's own capacity to decide, not to decide for them (Perlman, 1957; NASW Code 1.02). Walking through each option, including the parent's wish to stay with family, caregiving hours, the teenagers' needs, and costs, helps the adult child reach a choice they can carry out. It beats C because a family meeting can add facts, but framing it as the team recommending a plan hands the decision to professionals instead of building the family's own judgment.

**C** **Incorrect** (trap: Overrides the client's choice). A family meeting with the team is useful for medical facts and options. Setting it up so the team recommends the plan shifts the decision away from the family, who must live with it.

**Takeaway:** In problem solving, the social worker helps clients weigh options against their own values and limits rather than deciding for them.

**Sources:** Perlman, H. H. (1957). Social Casework: A Problem-Solving Process. University of Chicago Press, the problem-solving process; NASW Code of Ethics (2021), 1.02
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### 33. INT-1632 (rev 1) · Best answer: B

A client at a community agency brings an eviction summons that requires a written response to the court within 5 days. The social worker listens as the client describes fear of losing the apartment. The client says the landlord's claims are false and asks what to write in the response. What should the social worker do **NEXT**?

**A.** Ask how the client is coping with the threat of eviction
**B.** Connect the client today with a legal aid office
**C.** Help the client draft a reply disputing the landlord's claims
**D.** Help the client list other places to live if evicted

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). The social worker has already listened to the client's fear. With a court deadline days away, more exploration delays the step that protects the client's housing.

**B** **Correct.** In problem solving, part of the social worker's role is recognizing when a problem needs another profession's expertise and making a timely link. Deciding what to argue in a court filing is legal advice, outside social work competence (NASW Code 1.04(a)), and the Code directs referral when specialized knowledge is needed (1.16(a)). The 5-day deadline makes today the time to act. It beats C because helping write the court response would put the social worker in the role of legal adviser, and an error could cost the client the case.

**C** **Incorrect** (trap: Outside the social work role). Helping draft a legal reply feels supportive, but choosing what to argue and how to file is legal work. The social worker can help the client gather documents for an attorney, not shape the court filing.

**D** **Incorrect** (trap: Good but not best). A backup housing plan may be needed later, but the client wants to contest claims believed to be false. Planning a move first treats the eviction as settled and uses time the legal response requires.

**Takeaway:** When a client's problem requires legal expertise, the social worker's role is a timely link to legal help, not drafting legal arguments.

**Sources:** NASW Code of Ethics (2021), 1.04(a); NASW Code of Ethics (2021), 1.16(a)
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### 34. INT-0102 (rev 1) · Best answer: A

A patient with advanced lung cancer is continuing chemotherapy aimed at slowing the disease. A team now also visits to manage the patient's pain and breathlessness and to support the family's stress. Which type of care is the patient **MOST** likely receiving from this team?

**A.** Palliative care
**B.** Hospice care
**C.** Caregiver respite care

#### Why each option is right or wrong

**A** **Correct.** Palliative care relieves symptoms and stress and supports quality of life for people with serious illness at any stage, and it can be given alongside treatment aimed at the disease (National Consensus Project, 2018). The decisive detail is that the patient continues chemotherapy while the team manages symptoms and family stress. It beats B because hospice serves people with a prognosis of 6 months or less who choose comfort-focused care; under Medicare, electing hospice waives coverage for treatment of the terminal illness itself (42 CFR 418.3, 418.24).

**B** **Incorrect** (trap: Wrong model or concept). Hospice also manages symptoms and supports families, but it serves patients with a prognosis of about 6 months or less who have generally stopped treatment aimed at the disease. Ongoing chemotherapy points away from hospice.

**C** **Incorrect** (trap: Wrong model or concept). Respite care gives family caregivers short-term relief by providing temporary care for the patient. It does not describe a team that manages pain and breathlessness during active treatment.

**Takeaway:** Palliative care can accompany disease-directed treatment at any stage; hospice is comfort-focused care for a prognosis of about 6 months or less.

**Sources:** National Consensus Project for Quality Palliative Care (2018). Clinical Practice Guidelines for Quality Palliative Care (4th ed.). National Coalition for Hospice and Palliative Care, definition of palliative care; Medicare hospice regulations, 42 CFR Part 418, 42 CFR 418.3 (terminally ill); 418.24 (election and waiver)
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## 35. INT-1553 (rev 2) · Best answer: C

A patient with metastatic colon cancer has received home hospice care for 6 weeks. The oncologist mentions a clinical trial of a new treatment, and the patient wants to try it. The patient's adult child tells the social worker they fear that leaving hospice means the patient can never return to it. How should the social worker respond?

**A.** Confirm that revoking hospice ends the patient's future eligibility
**B.** Explain that a return is possible once the current benefit period ends
**C.** Explain that hospice can be revoked now and elected again later

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). This repeats the family's fear as fact. Revocation ends hospice coverage only for the current election period, and the patient can elect hospice again later if still eligible. Misinformation could push the patient into a choice they do not want.

**B** **Incorrect** (trap: Misreads law or ethics). This mixes up two rules. Revoking does give up the days left in the current election period, but the patient may elect hospice again at any time for a later period they qualify for; there is no wait until the old period would have ended. A false waiting period could weigh on the patient's choice.

**C** **Correct.** Under the Medicare hospice benefit, a patient may revoke the hospice election at any time and may elect hospice again for later periods they are eligible for (42 CFR 418.28). Hospice election waives coverage for treatment aimed at the terminal illness, so pursuing the trial generally means revoking first. Accurate information lets the patient make an informed choice without fearing the loss of comfort care (NASW Code 1.02). It beats B because revocation forfeits only the rest of the current period; re-election does not have to wait for that period to run out.

**Takeaway:** Give patients accurate hospice information: the election can be revoked to pursue treatment and elected again later if eligible.

**Sources:** Medicare hospice regulations, 42 CFR Part 418, 42 CFR 418.28 (revoking the election of hospice care); NASW Code of Ethics (2021), 1.02
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### 36. INT-1554 (rev 1) · Best answer: A

A patient with advanced heart failure is told that no further treatment will extend life. The patient speaks openly about the diagnosis and has updated a will. The patient then starts following a strict low-salt diet for the first time and asks the palliative care social worker whether 'being perfect from now on' could earn enough time to attend a grandchild's graduation in 2 years. Which of Kubler-Ross's stages is **MOST** likely reflected?

**A.** Bargaining
**B.** Denial
**C.** Acceptance
**D.** Depression

#### Why each option is right or wrong

**A** **Correct.** Bargaining is an attempt to postpone death by striking a deal, often with God or fate, in exchange for good behavior (Kubler-Ross, 1969). This patient accepts the diagnosis but hopes perfect diet control will earn time to reach a valued milestone. It beats C because the will and open talk suggest acceptance, yet trying to trade behavior for more time shows the patient has not made peace with the outcome. Kubler-Ross described these reactions as overlapping, not a fixed sequence.

**B** **Incorrect** (trap: Wrong model or concept). Denial shows as rejecting or ignoring the diagnosis, such as insisting the doctors are wrong. This patient names the illness openly and has updated a will, which shows the reality is recognized.

**C** **Incorrect** (trap: Wrong model or concept). Updating a will and talking openly fit acceptance, so this option is tempting. Acceptance, however, involves calm readiness rather than an effort to win more time through perfect behavior.

**D** **Incorrect** (trap: Wrong model or concept). Depression in this model involves sadness, withdrawal and grieving coming losses. This patient is energized and hopeful, focused on a deal for more time rather than on despair.

**Takeaway:** Bargaining offers good behavior in exchange for more time; it can coexist with open acknowledgment because the stages overlap.

**Sources:** Kubler-Ross, E. (1969). On Death and Dying. Macmillan, the third stage: bargaining
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### 37. INT-1555 (rev 3) · Best answer: C

A home hospice patient with end-stage lung disease has said many times that they want to die at home. The patient's partner, the only caregiver, has slept little for 2 weeks. As the social worker arrives for a visit, the partner says quietly, 'I don't know how much longer I can do this.' What should the social worker do **FIRST**?

**A.** Recommend moving the patient to a nursing facility for the remaining time
**B.** Arrange a short inpatient respite stay to relieve the partner
**C.** Explore with the partner what feels unmanageable right now

**Why each option is right or wrong**

**A** **Incorrect** (trap: Overrides the client's choice). Placement may become necessary if home care cannot be sustained, but it gives up the patient's clearly stated wish for a home death before less drastic supports have been tried (NASW Serious Illness Care Standard 1).

**B** **Incorrect** (trap: Right action, wrong time). Short inpatient respite, up to 5 consecutive days under the Medicare hospice benefit (42 CFR 418.204), may be exactly what this partner needs, and it protects the plan for a home death. Arranging it before hearing the partner out assumes the problem is only lost sleep and skips checking whether guilt, grief or hopelessness also needs attention.

**C** **Correct.** Assessment comes before intervention (Hepworth et al.). 'I don't know how much longer I can do this' could mean exhaustion from night care, guilt about the patient's wish for a home death, anticipatory grief, or something more serious such as hopelessness. Exploring what the partner means shows which help fits, whether respite, more in-home hours or support around the home-death wish, and whether the caregiver's own safety needs attention. It beats B because respite is the likely next step, but arranging it before understanding the statement could miss part of what the partner is facing.

**Takeaway:** When a hospice caregiver says they cannot go on, explore what that means before arranging respite or other help.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Assessment: exploring and understanding problems and strengths; Medicare hospice regulations, 42 CFR Part 418, 42 CFR 418.204(b) (respite care); NASW Practice Standards for Serious Illness Care: Hospice and Palliative Social Work, Standard 1: Ethics and Values

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## 38. INT-1556 (rev 1) · Best answer: A

During a home hospice visit, the social worker finds the patient moaning and grimacing with each movement. The patient's adult child, the main caregiver, says they have been skipping some prescribed pain medicine doses because they fear it will cause addiction or hasten death. What should the social worker do **FIRST**?

**A.** Ask the on-call nurse to reassess the pain now

**B.** Reassure the caregiver that pain medicine rarely hastens death

**C.** Explore the caregiver's fears about the patient's pain medicine

**Why each option is right or wrong**

**A** **Correct.** Uncontrolled pain at the end of life is an urgent symptom, and assessing pain and adjusting medication belong to the hospice nurse and physician, not the social worker (NASW Code 1.04(a)). The patient is in visible distress now, so the first step is bringing in the nurse to reassess today; the team then addresses the caregiver's fears together (NASW Serious Illness Care Standard 5). It beats C because exploring fears matters but leaves the patient in pain while the conversation unfolds.

**B** **Incorrect** (trap: Outside the social work role). Addressing the fear is worthwhile, but explaining medication effects is the clinical team's role, and reassurance does not

relieve pain that is happening now. The nurse can give accurate information while adjusting the plan.

**C** **Incorrect** (trap: Right action, wrong time). Exploring the caregiver's fears is a key part of the response, and the social worker should do it. Done first, it delays help for a patient in visible pain; it fits once the nurse has been called.

**Takeaway:** When a hospice patient's symptoms are uncontrolled, bring in the nurse or physician immediately, then address caregiver fears alongside the team.

**Sources:** NASW Code of Ethics (2021), 1.04(a); NASW Practice Standards for Serious Illness Care: Hospice and Palliative Social Work, Standard 5: Collaboration
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## 39. INT-1655 (rev 2) · Best answer: D

On a palliative care unit, a patient with advanced pancreatic cancer asks the social worker, 'How long do I have?' The oncologist reviewed the latest scan results with the patient yesterday. No family members are present. What should the social worker do **FIRST**?

**A.** Tell the patient the physician is the one to answer that
**B.** Share the typical survival range for this stage of cancer
**C.** Arrange a family meeting with the oncologist this week
**D.** Invite the patient to say what they understand and hope to learn

#### Why each option is right or wrong

**A** **Incorrect** (trap: Hands off too soon). Prognosis is medical information, and the oncologist should give it. Redirecting immediately, though, skips learning what the patient understood and wants, and may feel dismissive. The handoff fits once the patient's question is clearer.

**B** **Incorrect** (trap: Outside the social work role). Survival estimates are medical judgments for the physician. Offering general figures also assumes the patient wants numbers, which many patients do not, and may conflict with what the oncologist has said.

**C** **Incorrect** (trap: Right action, wrong time). A family meeting may help later with planning and support. Scheduling it now responds to a question the patient has not asked and brings in others before learning what the patient wants to know or share.

**D** **Correct.** Before prognosis is discussed, sound end-of-life communication learns what the patient understands and how much they want to know, the perception and invitation steps of the SPIKES protocol (Baile et al., 2000). The patient heard the scan results yesterday, so the question may reflect confusion, fear, or a wish to plan; asking first shows which, and the social worker can then bring in the oncologist for medical details. It beats A because redirecting at once can feel like a closed door at a vulnerable moment and misses what the patient is really asking.

**Takeaway:** When a dying patient asks about prognosis, first learn what they understand and want to know before arranging information.

**Sources:** Baile, W. F., Buckman, R., Lenzi, R., Glober, G., Beale, E. A., & Kudelka, A. P. (2000). SPIKES: A six-step protocol for delivering bad news: Application to the patient with cancer. The Oncologist, 5(4), 302-311, perception and invitation steps

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## 40. INT-1656 (rev 1) · Best answer: A

In home hospice, a patient with end-stage heart failure and the patient's partner have both been told the illness is terminal. In separate visits, each tells the social worker they understand the situation. Together, they talk only about next summer's garden and change the subject whenever either mentions the hospice nurse. Which concept **MOST** likely describes this pattern?

**A.** Mutual pretense awareness

**B.** Open awareness

**C.** Denial

#### Why each option is right or wrong

**A** **Correct.** Glaser and Strauss (1965) described awareness contexts around dying. In mutual pretense, the dying person and others all know death is coming, yet in each other's presence they act as though the topic does not exist, often to protect one another. Here each partner privately confirms understanding, but together they steer to future plans and away from hospice. It beats B because open awareness requires that the shared knowledge be acknowledged openly between them, which is exactly what they avoid.

**B** **Incorrect** (trap: Wrong model or concept). Open awareness means everyone knows and can speak openly about dying. Each partner knows, but they avoid the subject with each other, so the knowledge is shared without being open.

**C** **Incorrect** (trap: Wrong model or concept). Denial is the Kübler-Ross stage in which a person rejects the reality of the illness (Kübler-Ross, 1969). Both partners privately say they understand the situation, so their avoidance protects each other rather than refusing the facts.

**Takeaway:** In mutual pretense, the dying person and family both know but avoid the subject together; private acknowledgment separates it from denial.

**Sources:** Glaser, B. G., & Strauss, A. L. (1965). Awareness of Dying. Aldine, awareness contexts: mutual pretense; Kübler-Ross, E. (1969). On Death and Dying. Macmillan, stages: denial

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### 41. INT-1557 (rev 1) · Best answer: C

In an in-home parenting program, a parent agreed to ignore their 4-year-old's whining for snacks and to praise polite requests. Four days later, the parent reports that the whining has become louder and longer and asks whether the plan is failing. The child is otherwise healthy and safe. What should the social worker do **NEXT**?

**A.** Explore with the parent how the louder whining is affecting them
**B.** Replace planned ignoring with a short time-out for whining
**C.** Explain the expected surge and coach continued consistency

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). The parent's stress deserves attention, but the parent asked a direct question and the stem already shows what is happening. Exploring feelings first leaves the parent without the information needed to stay on the plan tonight.

**B** **Incorrect** (trap: Misuses the technique). Time-out can be useful, but switching tactics during a surge breaks the extinction process and teaches the child that louder whining brings change. Time-out fits behaviors that cannot safely be ignored, such as hitting.

**C** **Correct.** When a behavior's reinforcer is withheld, the behavior often gets worse before it fades, a pattern called an extinction burst. Louder, longer whining after 4 days is that expected surge, not a failed plan. Explaining it and coaching consistency, while continuing praise for polite requests, keeps the parent from giving in at the peak, which would teach the child that escalation works (Kazdin, 2005). It beats B because switching tactics now breaks the extinction process at the hardest moment.

**Takeaway:** A temporary increase after a reinforcer is withdrawn is an expected extinction burst; coach parents to stay consistent rather than change plans.

**Sources:** Kazdin, A. E. (2005). Parent Management Training: Treatment for Oppositional, Aggressive, and Antisocial Behavior in Children and Adolescents. Oxford University Press, extinction and extinction bursts
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### 42. INT-1558 (rev 1) · Best answer: D

In a parenting group, a parent says that when their 5-year-old screams in the checkout line, the parent buys the child candy and the screaming stops at once. Over several months, the parent has started buying candy as soon as the child begins to fuss. Which learning principle **MOST** likely explains the change in the parent's behavior?

**A.** Positive reinforcement
**B.** Extinction
**C.** Positive punishment
**D.** Negative reinforcement

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). This describes what happens to the child, who receives candy after screaming, so screaming is strengthened. The question asks about the parent, whose behavior grows because the screaming ends, not because the parent gains something.

**B** **Incorrect** (trap: Wrong model or concept). Extinction weakens a behavior by withholding its reinforcer. Here the parent's candy buying is increasing, and the child's screaming is still rewarded, so nothing is being extinguished.

**C** **Incorrect** (trap: Wrong model or concept). Positive punishment adds something unpleasant to reduce a behavior. The parent's candy buying is becoming more frequent, not less, and nothing aversive follows it.

**D** **Correct.** In negative reinforcement, a behavior becomes more frequent because it ends something unpleasant. Buying candy ends the child's screaming, so the parent's giving in grows stronger, which is why the parent now buys candy at the first fuss (Patterson, 1982). It beats A because positive reinforcement describes the child's side of the cycle: the child gains candy, so screaming increases. Seeing this coercive cycle helps parents understand why giving in feels effective in the moment yet makes tantrums more likely.

**Takeaway:** When giving in stops a child's tantrum, the parent's giving in is negatively reinforced; this coercive cycle strengthens both behaviors.

**Sources:** Patterson, G. R. (1982). Coercive Family Process. Castalia, coercion cycle in parent-child interaction
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.6 Building parenting capacity and skills · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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### 43. INT-1559 (rev 1) · Best answer: B

A child welfare social worker supports a parent whose 3-year-old is in foster care. The parent completed a 10-week parenting class and earned a certificate. During supervised visits, the parent gives sharp commands and rarely praises, and the child cries and pulls away. The case plan review is in 6 weeks, and the parent wants to reunify. Which action is **BEST** before the review?

**A.** Enroll the parent in a second class on parenting young children
**B.** Coach the parent in real time during their play together
**C.** Record the class certificate as meeting the parenting goal

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). A second course adds knowledge but repeats the format that left skills unchanged at visits. It might help if the parent lacked basic information; here the gap is applying skills with the child.

**B** **Correct.** Parent training works best when parents practice new skills with their own child and receive feedback, a component linked to larger effects in a meta-analysis of parent training programs (Kaminski et al., 2008). The certificate shows attendance, but the visits show the skills have not carried over. Live coaching targets praise and calm commands while the child is present. It beats A because more classroom teaching repeats the format that has not yet changed the parent's behavior with this child.

**C** **Incorrect** (trap: Misreads law or ethics). Completion is not the same as change. Recording the goal as met when visits show harsh commands and a distressed child misrepresents progress to the court and team; the record should reflect observed behavior.

**Takeaway:** Judge parenting progress by observed skills with the child, and build skills through practice with live feedback, not certificates alone.

**Sources:** Kaminski, J. W., Valle, L. A., Filene, J. H., & Boyle, C. L. (2008). A meta-analytic review of components associated with parent training program effectiveness. Journal of Abnormal Child Psychology, 36(4), 567-589, whole article

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## 44. INT-1560 (rev 1) · Best answer: A

A parent in a home-visiting program made early progress with a 2-year-old's bedtime routine. For the past 6 weeks, the parent has skipped practice, sleeps much of the day and says nothing feels worth doing. A depression screen today falls in the moderately severe range, and the parent denies thoughts of self-harm when asked directly. Which action is **MOST** appropriate?

- **A.** Refer the parent for depression treatment and keep visiting
- **B.** Pause home visits until the parent's depression improves
- **C.** Increase practice assignments to rebuild the bedtime routine now
- **D.** Explore with the parent what has lowered their motivation

**Why each option is right or wrong**

**A** **Correct.** Parental depression is a major barrier to building parenting skills, and treating it helps children too: children's symptoms fell when their parents' depression remitted (Weissman et al., 2006). The symptoms and screen show a need beyond the home visitor's role, so a referral is warranted, and continuing visits keeps the child's support in place (NASW Code 1.16(a)). It beats B because pausing removes support from the parent and child at a hard time instead of adding treatment.

**B** **Incorrect** (trap: Hands off too soon). Pausing seems kind, but it withdraws support from a struggling parent and a young child. Visits can continue, perhaps in a lighter form, while the parent starts treatment.

**C** **Incorrect** (trap: Good but not best). More assignments add pressure on a parent whose energy and interest are already low. Skill practice is likely to work better once the depression is being treated.

**D** **Incorrect** (trap: Right action, wrong time). Exploring motivation is a sound step when the cause of a stall is unclear. Here the symptoms and the screen already point to depression, so further exploration delays the help the parent needs.

**Takeaway:** When parental depression stalls parenting work, refer the parent for treatment while continuing parenting support rather than pausing or pushing harder.

**Sources:** Weissman, M. M., Pilowsky, D. J., Wickramaratne, P. J., Talati, A., Wisniewski, S. R., Fava, M., et al. (2006). Remissions in maternal depression and child psychopathology: A STAR*D-child report. JAMA, 295, 1389-1398, whole article; NASW Code of Ethics (2021), 1.16(a)

**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.6 Building parenting capacity and skills · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 45. INT-1561 (rev 1) · Best answer: C

During a structured play task in a parenting group, the social worker watches a parent say, 'Can you maybe start putting the blocks away? Let's clean up, okay?' The parent repeats this 5 times while the 6-year-old keeps playing. Which coaching suggestion would be **MOST** helpful?

**A.** Explain to the child why cleaning up matters first
**B.** Ignore the child's refusal to avoid a power struggle
**C.** Give one clear, specific command phrased as a statement
**D.** Promise the child a treat before the cleanup begins

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misuses the technique). Brief reasons can help children understand rules, but an explanation before the request delays it and invites debate. A short reason can follow once the child complies.

**B** **Incorrect** (trap: Wrong model or concept). Planned ignoring works for minor attention-seeking behavior, not for refusing an instruction. Ignoring noncompliance teaches the child that requests can be skipped.

**C** **Correct.** Effective commands are direct, specific and given once as a statement rather than a question or a 'let's' suggestion, followed by praise for compliance or a planned consequence (McMahon & Forehand, 2003). The parent's question form invites refusal, and repeating it teaches the child that the first request can be ignored. It beats A because an explanation before the command delays the request and gives the child more to argue with.

**D** **Incorrect** (trap: Misuses the technique). A reward offered before the task works like a bribe and can strengthen stalling. Praise or a privilege given after compliance reinforces following through.

**Takeaway:** Coach parents to give one clear, specific, statement-form command and follow through, rather than repeated questions or vague suggestions.

**Sources:** McMahon, R. J., & Forehand, R. L. (2003). Helping the Noncompliant Child: Family-Based Treatment for Oppositional Behavior (2nd ed.). Guilford Press, giving clear instructions
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.6 Building parenting capacity and skills · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

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### 46. INT-1657 (rev 1) · Best answer: C

In a home-visiting program, a parent asks the social worker how to use time-out for a 3-year-old who hits a younger sibling. The parent says most of their time together is spent giving commands or corrections, with little relaxed time. The hitting has caused no injuries. What should the social worker do **FIRST**?

**A.** Teach a consistent time-out procedure for each hitting incident
**B.** Explore how the parent was disciplined as a child
**C.** Coach brief daily child-led play with specific praise

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Time-out is an effective tool and the parent asked for it, so it will likely be taught. Taught first, in a relationship made up mostly of corrections, it has little positive time to contrast with and often turns into power struggles.

**B** **Incorrect** (trap: Good but not best). A parent's own history can shape discipline and may be worth discussing. As a first step it gives the parent no skill to use, and the clearest need described is too little positive interaction.

**C** **Correct.** Behavioral parent training builds a warm, positive relationship before adding discipline; in Parent-Child Interaction Therapy, the child-directed phase, with praise and attention for positive behavior, comes before the discipline phase (McNeil & Hembree-Kigin, 2010). This parent's time with the child is mostly commands and corrections, so time-out would land in a relationship with little positive attention to lose. Daily play builds that base while the parent simply separates the children if hitting occurs. It beats A because time-out works best once positive time is plentiful; taught first, it often fails and escalates conflict.

**Takeaway:** Build positive parent-child time with praise and child-led play before teaching discipline techniques such as time-out.

**Sources:** McNeil, C. B., & Hembree-Kigin, T. L. (2010). Parent-Child Interaction Therapy (2nd ed.). Springer, child-directed interaction before parent-directed interaction

**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.6 Building parenting capacity and skills · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 47. INT-1658 (rev 1) · Best answer: B

A foster parent in a parenting group says praise 'doesn't work' for an 8-year-old who leaves chores unfinished. Asked to describe it, the foster parent says they tell the child 'good job' at bedtime on days when the chores were done. What is the **MOST** appropriate suggestion?

**A.** Add a larger weekly reward to the bedtime praise

**B.** Respond right after each chore by naming what was done

**C.** Take away screen time on any day a chore is left unfinished

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Larger rewards can help, but this one keeps the same flaw: it arrives long after the behavior and does not name it. A reward system works best once praise is immediate and specific.

**B** **Correct.** Praise works as reinforcement when it comes right after the behavior and names exactly what the child did (Kazdin, 2005). A bedtime 'good job' comes hours late and is vague, so the child may not link it to the chores. Specific, immediate praise, such as thanking the child for putting the dishes away as soon as it happens, fixes the delivery rather than the size of the reward. It beats A because a bigger reward keeps the same delay and vagueness that weakened the praise.

**C** **Incorrect** (trap: Good but not best). Removing privileges punishes the problem but does not teach or strengthen the wanted behavior, and the praise has not yet had a fair test. Well-delivered reinforcement should come first.

**Takeaway:** Reinforcement works when it is immediate, specific and contingent; fix how praise is delivered before increasing rewards or adding punishment.

**Sources:** Kazdin, A. E. (2005). Parent Management Training: Treatment for Oppositional, Aggressive, and Antisocial Behavior in Children and Adolescents. Oxford University Press, positive reinforcement: delivering praise
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.6 Building parenting capacity and skills · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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## 48. INT-1562 (rev 2) · Best answer: B

An adult with schizophrenia has been clinically ready to leave a state psychiatric hospital for 5 months and wants to live in the community. The treatment team agrees. The only barrier is a waitlist for state-funded supported housing. A nursing facility has an open bed, and hospital administrators suggest transferring the client there. What is the social worker's **BEST** action?

**A.** Support the nursing facility transfer as a less restrictive setting
**B.** Press the state for supported housing under disability rights law
**C.** Keep the client in the state hospital until housing opens
**D.** Discharge the client to a shelter with outpatient follow-up

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). A nursing facility may look less restrictive than a state hospital, but it is still an institution, and the client wants community living. The integration mandate calls for the most integrated setting appropriate to the client, not a different institution.

**B** **Correct.** Under the Americans with Disabilities Act (ADA), as interpreted in Olmstead v. L.C. (1999), unjustified institutional isolation of people with disabilities is discrimination. Public entities must serve people in the most integrated setting appropriate to their needs (28 CFR 35.130(d)) when professionals judge community care appropriate, the person does not oppose it, and it can be reasonably accommodated. The first two conditions are clearly met and a state program exists, so the social worker presses the state to provide supported housing. It beats A because a nursing facility swaps one institution for another.

**C** **Incorrect** (trap: Good but not best). Staying put avoids an unsafe discharge, but months of unnecessary hospitalization is itself the problem. Waiting without pressing for housing leaves the client institutionalized indefinitely.

**D** **Incorrect** (trap: Ignores risk). A shelter ends the hospital stay but puts the client's stability at risk. The most integrated appropriate setting means community living with the supports the client needs, not the least supported option.

**Takeaway:** Policies such as the ADA integration mandate support advocating for community-based services over unnecessary institutional care.

**Sources:** Olmstead v. L.C. ex rel. Zimring (1999), 527 U.S. 581; ADA Title II regulations, 28 CFR 35.130(d)
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.7 Effects of policy, regulation, legislation on practice and services · **Qualifier** BEST · **Cognitive** application · **Difficulty** 3 of 3

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### 49. INT-1563 (rev 1) · Best answer: A

Which practice change does the federal Family First Prevention Services Act of 2018 encourage in child welfare agencies?

**A.** Evidence-based in-home help so children can stay safely at home
**B.** Faster filing to end parental rights for children in long-term care
**C.** Wider use of group homes for youth with complex needs

**Why each option is right or wrong**

**A** **Correct.** The Family First Prevention Services Act lets states use federal Title IV-E funds for evidence-based mental health, substance use and in-home parenting services that help children at imminent risk of foster care stay safely with their families. It also limits federal payments for most group care after the first 2 weeks. It beats B because faster termination timelines come from the Adoption and Safe Families Act of 1997, an earlier permanency law. Knowing which law drives which practice helps social workers explain service changes to families.

**B** **Incorrect** (trap: Wrong model or concept). Timelines for seeking termination of parental rights, such as after 15 of the most recent 22 months in care, come from the Adoption and Safe Families Act of 1997. Family First focuses on prevention and placement quality.

**C** **Incorrect** (trap: Misreads law or ethics). Family First moves the other way. It restricts federal funding for most congregate care after the first 2 weeks unless the setting meets standards such as those for a qualified residential treatment program.

**Takeaway:** Family First funds evidence-based prevention services to keep children safely with families and limits federal funding for most group care.

**Sources:** Family First Prevention Services Act of 2018 (Bipartisan Budget Act of 2018, P.L. 115-123, Div. E, Title VII), Title IV-E prevention services; limits on congregate care payments; Adoption and Safe Families Act of 1997, 42 U.S.C. 675(5)(E)
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.7 Effects of policy, regulation, legislation on practice and services · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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### 50. INT-1564 (rev 2) · Best answer: C

A child welfare social worker places a 6-year-old in emergency foster care after finding the child alone at home for 2 days. The parent says the child is an enrolled member of a federally recognized tribe but has no enrollment card on hand. The agency plans to file a petition for foster care placement. What does federal law require of the agency regarding the tribe?

**A.** Wait for proof of enrollment before contacting the tribe
**B.** Bring in the tribe once reunification efforts have failed
**C.** Notify the tribe and apply the law's protections now

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Verification matters, but federal rules treat the child as an Indian child while membership is confirmed with the tribe. Waiting for documents delays notice and the tribe's chance to take part.

**B** **Incorrect** (trap: Misreads law or ethics). The tribe has a right to notice and to intervene from the start, not only as a fallback. The Indian Child Welfare Act also

requires active efforts to keep the family together, which the tribe can help shape from the beginning.

**C** **Correct.** The Indian Child Welfare Act requires the party seeking foster care placement to notify the child's tribe of the proceeding, and no hearing may occur until at least 10 days after the tribe receives notice (25 U.S.C. 1912(a)). When there is reason to know a child is an Indian child, the child is treated as one until membership is confirmed or ruled out, and the agency works with the tribe to verify it (25 CFR 23.107). It beats A because waiting for a card delays the tribe's right to participate.

**Takeaway:** When there is reason to know a child is an Indian child, notify the tribe promptly and apply ICWA protections while membership is verified.

**Sources:** Indian Child Welfare Act of 1978, 25 U.S.C. 1912(a); Indian Child Welfare Act regulations, 25 CFR Part 23, 25 CFR 23.107(b)

**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.7 Effects of policy, regulation, legislation on practice and services · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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## 51. INT-1659 (rev 2) · Best answer: A

A 24-year-old client in outpatient therapy finishes graduate school next month and will start a part-time job with no health benefits. The client has been covered under a parent's employer plan and plans to stop therapy at graduation because the insurance will 'run out.' What is the social worker's **BEST** response?

**A.** Explain that the coverage can continue until the client turns 26

**B.** Help the client apply for Medicaid before graduation

**C.** Help the client compare marketplace plans before graduation

#### Why each option is right or wrong

**A** **Correct.** The Affordable Care Act requires plans that offer dependent coverage to keep it available until a child turns 26, whether or not the child is a student, married, or financially dependent (42 U.S.C. 300gg-14(a); 45 CFR 147.120). The client is 24, so graduation does not end coverage, and treatment can continue without a gap. Knowing how policy shapes access lets the social worker correct a mistaken belief that would end effective care. It beats B because Medicaid may or may not fit the client's income and could mean changing providers, while the current coverage already continues.

**B** **Incorrect** (trap: Good but not best). Medicaid can be a lifeline for clients who lose coverage, and eligibility could be checked if needed. Here the client keeps the parent's plan until 26, so applying solves a problem that does not exist yet and may disrupt care.

**C** **Incorrect** (trap: Good but not best). Marketplace coverage is a real option for young adults leaving a parent's plan, so this is a reasonable thought. It is not needed yet: the client can stay on the parent's plan until 26, which keeps the current coverage and provider without a gap or a new premium.

**Takeaway:** Know how laws shape access: young adults can stay on a parent's health plan until 26, regardless of student status.

**Sources:** Patient Protection and Affordable Care Act, 42 U.S.C. 300gg-14(a); Affordable Care Act regulations, 45 CFR 147.120

**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.7 Effects of policy, regulation, legislation on practice and services · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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### 52. INT-0103 (rev 1) · Best answer: C

A grant proposal includes the statement 'By the end of year one, 75% of enrolled caregivers will show reduced stress on a standardized scale.' In which proposal section does this statement belong?

**A.** Statement of need
**B.** Program goals
**C.** Program objectives
**D.** Evaluation plan

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). The statement of need documents the problem the program addresses, usually with local data and research. It describes current conditions, not the change the program promises to achieve.

**B** **Incorrect** (trap: Wrong model or concept). Goals describe the broad, long-term result a program works toward, such as better caregiver well-being. A target with a percentage and a deadline is too specific to be a goal.

**C** **Correct.** Objectives are specific, measurable, time-bound statements of the change a program will produce (Coley, Scheinberg & Levites Strekalova, 2021; Kettner, Moroney & Martin, 2022). This statement names who will change (enrolled caregivers), by how much (75%), how change will be measured (a standardized scale) and by when (end of year one), so it is an outcome objective. It beats B because goals are broad statements of intended direction, such as improving caregiver well-being, and are not written with numeric targets or deadlines.

**D** **Incorrect** (trap: Wrong model or concept). The evaluation plan explains how progress on the objectives will be measured, such as data sources, instruments and timing. It refers to the objectives rather than stating them.

**Takeaway:** In proposals, the need statement documents the problem, goals state broad intent, and objectives give measurable, time-bound targets.

**Sources:** Coley, S. M., Scheinberg, C. A., & Levites Strekalova, Y. A. (2021). Proposal Writing: Effective Grantsmanship for Funding (6th ed.). SAGE, Ch. 7, Program Objectives and Evaluation; Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 7, program goals and objectives

**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.8 Writing formal documents: grants, proposals, reports, evaluations · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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### 53. INT-1565 (rev 1) · Best answer: D

In a year-end report to a funder, which statement describes a program outcome rather than an output?

**A.** One hundred twenty caregivers attended 8 or more group sessions
**B.** The program held 48 support groups at 3 community sites
**C.** Two bilingual facilitators were hired and trained this year
**D.** Caregivers' average strain scores fell 25% by 6 months

**Why each option is right or wrong**

- **A** **Incorrect** (trap: Wrong model or concept). Attendance shows how many people received the service, which makes it an output. It shows reach but says nothing about whether caregivers' strain changed.
- **B** **Incorrect** (trap: Wrong model or concept). The number of groups and sites describes the volume of activity delivered. It is an output or activity measure that belongs in the report, but it is not evidence of change.
- **C** **Incorrect** (trap: Wrong model or concept). Hiring and training staff is an input or activity that builds program capacity. It shows the program is ready to deliver services, not that participants benefited.
- **D** **Correct.** Outcomes are changes in participants' conditions, such as reduced stress or better functioning, while outputs count the services delivered (Kettner, Moroney & Martin, 2022). A 25% drop in caregiver strain measures change in the people served, so it belongs with outcomes. It beats A because attendance is the most outcome-like distractor, yet it still counts participation rather than showing that anyone's situation improved.

**Takeaway:** Outputs count services delivered; outcomes measure change in participants. Funder reports need both, labeled correctly.

**Sources:** Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 10: Performance Measurement, Monitoring, and Program Evaluation

**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.8 Writing formal documents: grants, proposals, reports, evaluations · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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## 54. INT-1566 (rev 1) · Best answer: A

A school social worker is writing a foundation grant proposal for an anxiety skills group for middle school students. The funder's guidelines ask for a statement of need. Which content would **BEST** support this section?

- **A.** District data on anxiety rates and current waitlists for services
- **B.** National statistics showing rising anxiety among adolescents
- **C.** A history of the agency's 30 years serving local youth
- **D.** A session-by-session outline of the planned group curriculum

**Why each option is right or wrong**

- **A** **Correct.** A need statement documents the problem in the specific population and place the program will serve, using credible data that show the gap the program will fill (Coley, Scheinberg & Levites Strekalova, 2021). District anxiety data and waitlists show both the local need and the shortage of current help. It beats B because national figures show a broad trend but do not prove that students in this district need this group.
- **B** **Incorrect** (trap: Good but not best). National data can add context in a sentence or two, but funders want evidence that the problem exists locally and is unmet. Relying on national trends leaves the local need unproven.

**C** **Incorrect** (trap: Wrong model or concept). Agency history shows capacity and credibility, which belongs in the organizational background section. It does not describe the problem the program will address.

**D** **Incorrect** (trap: Wrong model or concept). A curriculum outline describes how the program will work, which belongs in the program description or methods section, not in the statement of need.

**Takeaway:** A grant's need statement uses local, credible data to show the problem and service gap in the population the program will serve.

**Sources:** Coley, S. M., Scheinberg, C. A., & Levites Strekalova, Y. A. (2021). Proposal Writing: Effective Grantsmanship for Funding (6th ed.). SAGE, Ch. 8: Writing the Need or Problem Statement
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.8 Writing formal documents: grants, proposals, reports, evaluations · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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## 55. INT-1660 (rev 1) · Best answer: C

A social worker is drafting a year-end report to a foundation that funded a caregiver support program. The grant target was 100 enrolled caregivers; 72 enrolled, and 40 more called for information but did not enroll. The program director suggests counting the callers as clients served to meet the target. How should the social worker report enrollment?

**A.** Report the callers as enrolled and add a footnote
**B.** Omit the enrollment figure and lead with outcome gains
**C.** Report 72 enrolled and explain the gap and next steps

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). A footnote makes the change visible, but the headline number would still count people who never enrolled toward an enrollment target, and readers relying on it would be misled. Inquiries can be reported separately as interest in the program.

**B** **Incorrect** (trap: Misreads law or ethics). Leading with outcomes is fine, but leaving out a required figure because it falls short misrepresents the program by omission. Funders expect results against every stated objective, including those not met.

**C** **Correct.** Formal reports must describe results accurately: social workers should report evaluation findings accurately and not falsify them (NASW Code 5.02(o)), and should not take part in deception (4.04). Reporting the 72 enrolled caregivers, with the reasons for the gap and a plan to close it, keeps the report honest and gives the funder what it needs to judge the program; the 40 inquiries can appear as a separate line. It beats A because a footnote does not fix counting callers who never enrolled toward a target defined by enrollment.

**Takeaway:** Report program results accurately against the stated target, explaining shortfalls rather than redefining measures or omitting them.

**Sources:** NASW Code of Ethics (2021), 5.02(o); NASW Code of Ethics (2021), 4.04
**Area** III Intervention and Practice · **Sub-area** III.A Practice Concepts · **Topic** III.A.8 Writing formal documents: grants, proposals, reports, evaluations · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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# III.B Intervention Methods and Techniques

### 56. INT-0112 (rev 1) · Best answer: A

A community clinic is redesigning its intake. Currently, staff seat each new client, then ask a fixed list of trauma history questions in the same order for everyone. Which change is **MOST** consistent with a trauma-informed approach?

**A.** Offer clients choices about pacing and what to share
**B.** Collect a detailed trauma history at the first visit
**C.** Refer clients who report trauma to a trauma specialist

**Why each option is right or wrong**

**A** **Correct.** A trauma-informed approach applies to everyone and builds in safety, collaboration, and empowerment, voice and choice, while resisting re-traumatization (SAMHSA, 2014). A fixed, staff-controlled trauma interview removes control from clients, which can echo the powerlessness of trauma. Offering choices about pacing and what to share returns some control and builds trust. It beats B because requiring a detailed trauma history at the first visit, before trust exists, risks re-traumatization; screening can be brief, with fuller assessment later when the client is ready.

**B** **Incorrect** (trap: Right action, wrong time). Detailed trauma questioning at the first contact, before any trust or choice, can overwhelm clients and risk re-traumatization. A brief screen now and fuller assessment later fits trauma-informed practice.

**C** **Incorrect** (trap: Hands off too soon). Trauma-informed care is an agency-wide approach, not a specialty service. Sending clients elsewhere leaves in place the intake practices that may harm them, though trauma-specific treatment can be offered when indicated.

**Takeaway:** Trauma-informed care gives clients choice and control in routine procedures and applies to everyone, whether or not trauma is disclosed.

**Sources:** SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), six key principles: empowerment, voice and choice
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.1 Trauma-informed care across trauma types (complex, vicarious) · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

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### 57. INT-1082 (rev 1) · Best answer: B

A client with a history of repeated childhood abuse starts outpatient therapy and asks to 'go through the memories' right away. In the past month, the client has cut their arms several times, has been sleeping in their car, and often loses track of time during sessions. The client says reading about trauma therapy gave them hope. What should the social worker focus on **FIRST**?

**A.** Begin detailed processing of the abuse memories as requested
**B.** Work with the client on safety and stabilization
**C.** Refer the client to an inpatient trauma program

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Trauma processing is the client's goal and a valid later step. Starting it while the client is cutting, without stable housing and dissociating risks pushing the client beyond what they can tolerate. Processing fits once safety and coping are established.

**B** **Correct.** Phase-oriented treatment of complex trauma begins with establishing safety and stabilization before remembrance and mourning (Herman, 1992). Current cutting, unstable housing and frequent dissociation in session show the client cannot yet tolerate intense memory work. The client's hope is honored by framing stabilization as the first part of trauma treatment, not a refusal. It beats A because following the request respects choice, but processing memories while the client is self-harming and dissociating risks overwhelming them; the goal is pursued in sequence.

**C** **Incorrect** (trap: Hands off too soon). Inpatient care fits imminent danger or an inability to stay safe with outpatient support. This client is engaged and seeking help, so the first step is stabilization within the current relationship, with a higher level of care considered if safety worsens.

**Takeaway:** With complex trauma, establish safety and stabilization before trauma processing; current self-harm, instability or dissociation mark the first phase.

**Sources:** Herman, J. L. (1992). Trauma and Recovery. Basic Books, stages of recovery: establishing safety
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.1 Trauma-informed care across trauma types (complex, vicarious) · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 58. INT-1083 (rev 1) · Best answer: A

A program director at a child advocacy center learns that several interviewers and family advocates have come to see most strangers as dangerous, check on their own children many times a day, and describe the world as unsafe. The staff say they still believe in the work and want to stay. What is the **BEST** organizational response?

**A.** Build regular trauma-focused supervision into each staff member's week
**B.** Refer each affected staff member to the employee assistance program
**C.** Hold a staff wellness day with yoga and chair massage

**Why each option is right or wrong**

**A** **Correct.** Vicarious trauma is a cumulative change in a helper's inner beliefs about safety, trust and other people, caused by empathic engagement with clients' trauma (Pearlman & Saakvitne, 1995). Several staff now show these shifts, so the response belongs at the program level: regular supervision that names trauma exposure, along with caseload balance and ongoing self-care, which the Code calls paramount for competent practice (NASW Code, Purpose). It beats B because individual counseling can help, but referral treats a predictable job hazard as each person's private problem and leaves exposure unchanged.

**B** **Incorrect** (trap: Hands off too soon). Counseling through the EAP can help an individual, but several staff show the same changes from the same work. Handing each person off leaves the program's exposure and supports unchanged, so the effects are likely to recur.

**C** **Incorrect** (trap: Good but not best). A wellness day is a kind gesture and may ease stress briefly. It does not address the ongoing trauma exposure or the changed beliefs, which call for steady supervision and workload practices rather than a single event.

**Takeaway:** Vicarious trauma is an expected hazard of trauma work; address it through ongoing supervision, caseload balance and self-care built into the program.

**Sources:** Pearlman, L. A., & Saakvitne, K. W. (1995). Trauma and the Therapist: Countertransference and Vicarious Traumatization in Psychotherapy with Incest Survivors. W. W. Norton, addressing vicarious traumatization: supervision, balance and self-care; NASW Code of Ethics (2021), Purpose
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.1 Trauma-informed care across trauma types (complex, vicarious) · **Qualifier** BEST · **Cognitive** application · **Difficulty** 3 of 3

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## 59. INT-1084 (rev 2) · Best answer: D

In a hospital, a patient who disclosed a past sexual assault becomes panicky before a needed physical exam, gripping the bed rail and saying, 'I can't do this if I don't know what's coming.' The physician asks the social worker for help. What is the **MOST** appropriate action?

**A.** Suggest that the physician order a sedative first
**B.** Ask the physician to postpone the exam until the patient feels ready
**C.** Reassure the patient that the staff are skilled professionals
**D.** Arrange for each step to be explained and a way to pause

**Why each option is right or wrong**

**A** **Incorrect** (trap: Outside the social work role). Medication decisions belong to the physician and patient, and a sedative skips the patient's own stated need for information. Trauma-informed steps that restore predictability and control should come first.

**B** **Incorrect** (trap: Good but not best). Postponing respects the distress, but the exam is needed and the patient has named what would make it possible: knowing what is coming. Delay leaves the fear in place and may make the next attempt harder. Postponing fits if the patient still declines once each step is explained.

**C** **Incorrect** (trap: Good but not best). Reassurance is well meant, but the panic comes from not knowing what will happen and feeling out of control. Telling the patient the staff are skilled does not supply the predictability the patient asked for.

**D** **Correct.** Trauma-informed care builds safety, transparency and a sense of control, and avoids repeating experiences of powerlessness (SAMHSA, 2014). The patient states exactly what is needed: knowing what is coming. Explaining each step before it happens and agreeing on a signal to pause gives predictability and voice. It beats C because reassurance about staff skill is kind but does not change the loss of control that drives the panic; the patient asked for information, not credentials.

**Takeaway:** Trauma-informed care restores predictability and control: explain each step in advance and give the person a way to pause.

**Sources:** SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), six key principles: safety; trustworthiness and transparency; empowerment, voice and choice
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## 60. INT-1085 (rev 2) · Best answer: C

A behavioral health clinic serving a tribal nation offers a weekly PTSD skills group that few people attend. Community members tell the social worker that their grandparents were taken to boarding schools and that outside agencies have rarely kept their promises. Several elders lead traditional healing gatherings that are well attended. What is the **BEST** step to make the clinic more trauma-informed?

**A.** Ask the elders to promote the PTSD group at gatherings
**B.** Hire tribal community staff to lead the clinic's PTSD group
**C.** Invite the elders to help shape the clinic's services

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Endorsement by elders may raise attendance, but it uses their trust to promote a model they had no part in shaping. After historical trauma and broken promises, the community needs a voice in what is offered, not just publicity.

**B** **Incorrect** (trap: Good but not best). Hiring community members can build trust and is a real trauma-informed strategy. Placing them in the existing PTSD group, though, keeps a service the community had no part in choosing. After broken promises, and with traditional gatherings already well attended, the community needs a share in deciding what is offered.

**C** **Correct.** SAMHSA's trauma-informed principles include attention to cultural, historical and gender issues: recognizing historical trauma and drawing on the healing value of traditional cultural connections (SAMHSA, 2014). Collective losses such as boarding-school removals, and broken promises by outside agencies, explain low trust in a clinic-run group (Brave Heart, 2003). Partnering with respected elders who already lead healing gatherings builds trust and fits services to the community. It beats A because asking elders to promote the existing group borrows their trust without sharing decisions; collaboration means the community helps decide what is offered.

**Takeaway:** Trauma-informed care addresses historical trauma by partnering with community leaders and drawing on traditional healing practices.

**Sources:** SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), six key principles: cultural, historical, and gender issues; Brave Heart, M. Y. H. (2003). The historical trauma response among Natives and its relationship with substance abuse: A Lakota illustration. Journal of Psychoactive Drugs, 35(1), 7-13, whole article
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## 61. INT-1086 (rev 1) · Best answer: A

During a video session, a client describing a carjacking stops mid-sentence, stares past the camera, and does not respond when the social worker says the client's name. After a long pause, the client whispers, 'I'm not really here.' What should the social worker do **FIRST**?

**A.** Help the client notice objects and sounds in the room
**B.** Ask the client what feelings are coming up about the carjacking
**C.** Encourage the client to keep going and finish the story
**D.** End the session early so the client can rest

**Why each option is right or wrong**

**A** **Correct.** When a client dissociates, the worker pauses trauma content and uses grounding to bring attention back to the present, such as naming things the client can see, hear or touch (SAMHSA TIP 57, 2014). Staring, not responding to their name and saying 'I'm not really here' show the client has left the window of tolerance (Siegel, 1999). It beats B because asking about feelings keeps the focus on the traumatic memory and needs a level of presence the client lacks right now; feelings can be explored once the client is oriented.

**B** **Incorrect** (trap: Right action, wrong time). Exploring feelings is useful when the client is present and within tolerance. Right now the client is dissociated, and turning back to the carjacking deepens the disconnection. Grounding must come first.

**C** **Incorrect** (trap: Ignores risk). Finishing the story while dissociated does not process the memory and can reinforce overwhelm. The narrative can resume, if appropriate, once the client is grounded and able to stay present.

**D** **Incorrect** (trap: Ignores risk). Ending the call leaves a dissociated client alone, which is unsafe on video where the worker cannot help in person. The client should be grounded and oriented before the session closes, with a plan for the rest of the day.

**Takeaway:** When a client dissociates during trauma work, pause the content and use grounding to reorient them to the present before anything else.

**Sources:** SAMHSA TIP 57 (2014), Part 1, Ch. 4: Screening and Assessment (Exhibit 1.4-1, Grounding Techniques); Siegel, D. J. (1999). The Developing Mind. Guilford Press, window of tolerance
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## 62. INT-1087 (rev 2) · Best answer: B

A family service agency's director says the agency is now trauma-informed because two clinicians completed TF-CBT training. Meanwhile, front-desk staff call out clients' reasons for visits in the waiting room, and a no-show policy closes cases after two missed visits without outreach. Which recommendation is **MOST** consistent with a trauma-informed approach?

**A.** Train more clinicians in TF-CBT so every child is served
**B.** Train staff in each role and revise intake and attendance rules
**C.** Have intake staff screen every new client for trauma

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). TF-CBT is an evidence-based, trauma-specific treatment, but expanding it does not make the agency trauma-informed. The approach applies to every staff member and policy, including the waiting room and the no-show rule.

**B** **Correct.** A trauma-informed approach is organization-wide: everyone, including front-desk staff, realizes trauma's impact, recognizes its signs, responds through policy and practice, and resists re-traumatization (SAMHSA, 2014). Trauma-specific treatments such as TF-CBT are interventions, not the approach itself. Announcing visit reasons aloud and closing cases without outreach undermine safety and trust. It beats C because universal screening fits within trauma-informed care, but it adds a step while leaving the practices that already harm clients in place (SAMHSA TIP 57, 2014).

**C** **Incorrect** (trap: Good but not best). Screening is part of many trauma-informed programs, but it does not fix the practices already harming clients. Asking about trauma while privacy is ignored at the front desk may also feel unsafe to clients.

**Takeaway:** Trauma-informed care is an organization-wide approach covering all staff and policies; trauma-specific therapy alone does not make an agency trauma-informed.

**Sources:** SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), the four Rs; trauma-informed approach versus trauma-specific interventions; SAMHSA TIP 57 (2014), Part 1, Ch. 4: Screening and Assessment

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## 63. INT-1161 (rev 1) · Best answer: B

In a hospital emergency department, a social worker meets an adult patient who was sexually assaulted by an acquaintance about 6 hours ago. The patient is tearful, wants to shower and go home, and does not want police involved. The patient asks the social worker, 'Is there any reason I should stay?' What is the **BEST** response?

**A.** Support the decision to leave and arrange a call tomorrow

**B.** Explain the care and exam options that need no police report

**C.** Encourage a police report to protect others from the acquaintance

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Respecting the wish to leave matters, and a follow-up call is good practice. But the patient asked whether to stay, and answering without the time-sensitive options leaves the decision uninformed.

**B** **Correct.** Trauma-informed care restores control to survivors through voice and choice, and real choice depends on accurate information (SAMHSA, 2014). The patient is asking for exactly that. Some care after an assault, such as preventive medication and evidence collection, is time-sensitive, and federal guidance says a medical forensic exam can be done without involving police (National Protocol, U.S. Department of Justice, 2013). Explaining these options lets the patient decide with full facts. It beats A because supporting the wish to leave without answering the question withholds what the patient needs to choose.

**C** **Incorrect** (trap: Overrides the client's choice). Urging a report pressures the survivor toward a step they have declined and puts others' interests ahead of the patient's control. Whether to report is the patient's decision.

**Takeaway:** Trauma-informed care supports survivor choice by giving full, accurate options, including time-sensitive care that does not require a police report.

**Sources:** SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Six key principles: empowerment, voice and choice; U.S. Department of Justice, Office on Violence Against Women (2013). A National Protocol for Sexual Assault Medical Forensic Examinations: Adults/Adolescents (2nd ed.), patient-centered care; exams without law enforcement involvement
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## 64. INT-1186 (rev 1) · Best answer: C

On an inpatient psychiatric unit, policy requires a body check for injuries and contraband at admission. A newly admitted adult with a history of childhood sexual abuse starts shaking and crying when told about the check and backs into a corner. The nurse asks the social worker for help. What is the **MOST** appropriate action?

**A.** Ask the nurse to skip the check for this patient
**B.** Explain the safety reasons for the check, then proceed
**C.** Plan with the patient and nurse how the check will be done

#### Why each option is right or wrong

**A** **Incorrect** (trap: Ignores risk). Waiving the check may calm the moment but leaves possible injuries or dangerous items undetected on a unit where other patients and staff could be harmed. Trauma-informed care adapts how safety procedures happen; it does not drop them.

**B** **Incorrect** (trap: Good but not best). Transparency is a trauma-informed principle and belongs in the response. On its own, though, it still imposes the procedure on a frightened survivor without any say, which risks re-traumatization.

**C** **Correct.** A trauma-informed approach keeps needed safety procedures while giving the person as much voice and choice as possible, in line with SAMHSA's principles of safety, collaboration and empowerment (SAMHSA, 2014). For a survivor of sexual abuse, a body check can echo the original violation, so involving the patient in how it is done, such as who conducts it, the order of steps, staying partly clothed and pausing when needed, lowers the risk of re-traumatization. It beats B because explanation alone is transparent but leaves the patient powerless in a procedure that resembles past abuse.

**Takeaway:** Trauma-informed care adapts required safety procedures with the person's input instead of dropping them or imposing them unchanged.

**Sources:** SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Six key principles; resisting re-traumatization
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### 65. INT-1088 (rev 1) · Best answer: C

In a hospice program, a patient is expected to die within days. The patient's partner asks the social worker how to tell their 5-year-old and plans to say the patient is 'going to sleep for a long time' because it sounds gentle. What is the **BEST** guidance?

**A.** Use the gentle phrase, since young children cannot grasp death
**B.** Wait to explain the death until the child starts asking
**C.** Use the words 'dying' and 'died,' explained in simple terms

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Children around age 5 are beginning to understand that death is permanent, universal and has a cause. The 'sleep' phrase can lead to fear of bedtime or hope that the parent will wake up. A gentle tone with accurate words works better.

**B** **Incorrect** (trap: Good but not best). Many young children do not ask directly even when they sense a crisis, and silence can increase anxiety. Prepared, honest explanation before and after the death supports grieving; later questions can be answered as they come.

**C** **Correct.** Young children take words literally, so euphemisms such as sleep can confuse them or make them afraid to go to sleep; using 'dead' or 'died' is recommended (Schonfeld et al., 2024). Pairing those words with a simple explanation, such as the body stopping working and not being able to come back, helps a 5-year-old begin to grasp that death is permanent. It beats B because waiting leaves the child sensing that something is wrong without explanation; children do better with honest, early, age-suited information.

**Takeaway:** Talk with young children about death using concrete words such as 'died'; euphemisms like sleep can confuse or frighten them.

**Sources:** Schonfeld, D. J., Demaria, T., Nasir, A., Kumar, S., & AAP Committee on Psychosocial Aspects of Child and Family Health, Council on Children and Disasters (2024). Supporting the grieving child and family. Pediatrics, 154(1), e2024067212, communicating with children about death: avoid euphemisms
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### 66. INT-1089 (rev 1) · Best answer: B

An employee assistance client is struggling after a close friend died of an overdose 3 weeks ago. The friend's family did not invite the client to the funeral, the employer denied bereavement leave because the friend was not a relative, and coworkers have said, 'It was bound to happen.' Which concept **MOST** likely describes the client's experience?

**A.** Anticipatory grief
**B.** Disenfranchised grief
**C.** Prolonged grief disorder
**D.** Ambiguous loss

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Anticipatory grief begins before an expected death, as with a terminal illness. This friend has already died, and the problem is how others respond to the loss, not grieving in advance.

**B** **Correct.** Disenfranchised grief is grief that is not openly acknowledged, socially supported or publicly mourned, as often happens after the death of a friend, an ex-partner or someone who died by overdose (Doka, 1989). Exclusion from the funeral, denied leave because the friend was not family, and blaming remarks all signal that others do not recognize this loss. It beats C because the grief is intense, but at 3 weeks it is far too early for prolonged grief disorder, which requires at least 12 months since the death in adults (DSM-5-TR).

**C** **Incorrect** (trap: Near-miss diagnosis). Prolonged grief disorder requires persistent, impairing grief at least 12 months after a death in adults. Three weeks after the loss, intense grief is expected; the striking feature here is the lack of social recognition.

**D** **Incorrect** (trap: Wrong model or concept). Ambiguous loss involves uncertainty, such as a missing person, or someone physically present but psychologically absent with dementia. This death is certain; what is missing is social permission to grieve.

**Takeaway:** Disenfranchised grief is grief others do not recognize or support, common after deaths of friends, ex-partners or by overdose.

**Sources:** Doka, K. J. (Ed.). (1989). Disenfranchised Grief: Recognizing Hidden Sorrow. Lexington Books, definition of disenfranchised grief; DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Prolonged Grief Disorder
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## 67. INT-1090 (rev 1) · Best answer: A

A college student whose parent died 3 months ago describes a typical day to a campus social worker. Mornings are spent looking at old photos and crying. Afternoons go to classes, estate paperwork, and learning to cook the parent's recipes for younger siblings. The student worries that this back-and-forth means something is wrong. Which framework **MOST** likely explains this pattern?

**A.** Dual process model
**B.** Worden's tasks of mourning
**C.** Kübler-Ross stages of dying
**D.** Continuing bonds

#### Why each option is right or wrong

**A** **Correct.** The dual process model describes healthy grieving as oscillation between loss-oriented coping, such as looking at photos and crying, and restoration-oriented coping, such as taking on new roles and tasks; time away from grief is part of adjustment (Stroebe & Schut, 1999). The student's daily alternation fits this closely, and the model frames the shifting as adaptive. It beats B because Worden's tasks describe what mourning must accomplish, such as processing pain and adjusting to a changed world, but they do not center on moving between the two kinds of coping.

**B** **Incorrect** (trap: Wrong model or concept). Worden's tasks (accept the reality, process the pain, adjust to a world without the person, find an enduring connection) fit parts of this picture (Worden, 2018). The defining feature here, though, is the daily alternation between grieving and restoring, the core of the dual process model.

**C** **Incorrect** (trap: Wrong model or concept). Kübler-Ross described stages such as denial, anger, bargaining, depression and acceptance, first in dying patients. The

student is not moving through a sequence of emotional stages but alternating between two kinds of coping each day.

**D** **Incorrect** (trap: Wrong model or concept). Continuing bonds describes keeping an ongoing inner connection with the person who died, as cooking the parent's recipes may show. It explains that connection, not the alternation between grieving and getting on with new tasks.

**Takeaway:** The dual process model sees healthy grieving as oscillation between loss-oriented and restoration-oriented coping; time away from grief is adaptive.

**Sources:** Stroebe, M., & Schut, H. (1999). The dual process model of coping with bereavement: Rationale and description. Death Studies, 23(3), 197-224, whole article; Worden, J. W. (2018). Grief Counseling and Grief Therapy (5th ed.). Springer Publishing, four tasks of mourning

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## 68. INT-1091 (rev 3) · Best answer: D

A home health social worker is assigned to a 78-year-old whose spouse of 50 years died 6 weeks ago. Before the first visit, an adult child calls, says the client 'cries all the time and talks to the photo every morning,' and asks the worker to arrange antidepressant medication. What should the social worker do **FIRST**?

**A.** Refer the client to a prescriber for an antidepressant evaluation

**B.** Begin structured treatment for prolonged grief

**C.** Help the family see these reactions as expected grief

**D.** Meet with the client to explore how they are living with the loss

#### Why each option is right or wrong

**A** **Incorrect** (trap: Skips assessment). A medication evaluation fits if assessment shows a major depressive episode, such as pervasive worthlessness, loss of functioning or suicidal thoughts. Arranging it from a relative's phone report skips the client's own account and may treat expected sadness as illness.

**B** **Incorrect** (trap: Near-miss diagnosis). Structured prolonged grief treatment is for grief that stays intense and impairing at least 12 months after the death in adults (DSM-5-TR). At 6 weeks it is premature, and nothing yet shows impairment.

**C** **Incorrect** (trap: Right action, wrong time). Daily crying and talking to a photo are common in early grief, and an ongoing inner bond with the person who died is often part of healthy mourning (Klass et al., 1996). Telling the family so is the right message once the worker has met the client; said first, it rests only on a relative's report and could miss depression or risk.

**D** **Correct.** Assessment comes before intervention, and the person to assess is the client. So far the worker has only the adult child's description. Exploring how the client is experiencing the loss, including mood, self-worth, sleep, daily routines, supports and any thoughts of death, separates expected grief from a major depressive episode (DSM-5-TR) and lets the client's own wishes guide what follows. It beats C because reassuring the family before meeting the client could miss depression or suicide risk; normalizing fits once the assessment supports it.

**Takeaway:** When family asks for treatment for a grieving relative, first explore the loss with the client directly; normalize or refer only after that.

**Sources:** Klass, D., Silverman, P. R., & Nickman, S. L. (Eds.). (1996). Continuing Bonds: New Understandings of Grief. Taylor & Francis, continuing bonds as part of normal grief; DSM-5-TR (APA, 2022), Depressive Disorders: Major Depressive Disorder
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## 69. INT-1092 (rev 2) · Best answer: B

A client whose partner died in a car crash 16 months ago has seen a social worker for supportive counseling for 3 months. The worker's assessment finds intense daily yearning, a sense that part of the client has died, avoidance of the crash route, a feeling that life is meaningless, and missed work most weeks. The worker has no training in treatments designed for this condition, and a nearby program offers one. What should the worker do **NEXT**?

**A.** Continue weekly supportive counseling and track the symptoms
**B.** Refer the client for a therapy that targets this grief
**C.** Help the client accept that grief has no set timeline

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Supportive counseling and monitoring would be reasonable if the client were improving. After 3 months, symptoms remain intense and work is still disrupted, which signals a need for a treatment designed for this condition.

**B** **Correct.** Intense, impairing grief more than 12 months after a death fits prolonged grief disorder in adults (DSM-5-TR), and a treatment targeting it outperformed interpersonal psychotherapy in a randomized trial (Shear et al., 2005). The worker lacks training in such methods and an accessible program offers one; social workers should refer when specialized expertise is needed or progress is not being made (NASW Code 1.16(a)). It beats A because supportive counseling has run 3 months while the client still misses work most weeks; continuing it delays effective care.

**C** **Incorrect** (trap: Near-miss diagnosis). Grief has no fixed timeline in general, but this grief is intense, impairing and has lasted over a year. Treating it as ordinary mourning would leave a treatable condition unaddressed.

**Takeaway:** When grief stays intense and impairing beyond 12 months and current help isn't working, refer for treatment that targets prolonged grief.

**Sources:** DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Prolonged Grief Disorder; Shear, K., Frank, E., Houck, P. R., & Reynolds, C. F. (2005). Treatment of complicated grief: A randomized controlled trial. JAMA, 293(21), 2601-2608, whole article; NASW Code of Ethics (2021), 1.16(a)
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### 70. INT-1177 (rev 2) · Best answer: D

A client's partner, a commercial fisher, went missing at sea 14 months ago, and no body was found. Relatives urge the client to hold a funeral and 'get closure.' The client says, 'If I bury an empty coffin, I'm giving up on them, but I can't keep waiting like this either.' What is the **MOST** appropriate intervention?

**A.** Encourage the memorial service so the client can reach closure
**B.** Refer the client for prolonged grief disorder treatment
**C.** Support the client's hope that the partner may be found
**D.** Help the client accept holding both hope and grief at once

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Rituals help many mourners, and the client may later choose one. Pressing for a funeral to reach closure assumes the loss is settled, which is exactly what the client cannot know; Boss warns that insisting on closure hinders coping with this kind of loss.

**B** **Incorrect** (trap: Near-miss diagnosis). In DSM-5-TR, prolonged grief disorder follows a confirmed death and involves intense, impairing grief well beyond expected norms. Here the death is unproven and the client's struggle is the uncertainty itself, which calls for ambiguous-loss work rather than a disorder-specific referral.

**C** **Incorrect** (trap: Good but not best). Supporting hope alone validates only one side of what the client said and leaves no room for the grief they also feel. Ambiguous-loss work holds both truths instead of siding with hope or with death.

**D** **Correct.** Boss's ambiguous loss framework holds that when a loved one is physically missing without proof of death, closure may never come, and pushing for it adds distress. The aim is tolerating the uncertainty through both/and thinking: the partner may be alive and may be dead, and the client can grieve while still hoping (Boss, 2006). The client already voices both sides. It beats A because a funeral pressed on the client as a route to closure forces a choice the facts do not support and leaves the client feeling disloyal.

**Takeaway:** With a loss that has no proof or end, help the person hold hope and grief together; pressing for closure adds distress.

**Sources:** Boss, P. (2006). Loss, Trauma, and Resilience: Therapeutic Work With Ambiguous Loss. W. W. Norton, Both/and thinking; tolerance for ambiguity; the myth of closure; DSM-5-TR (APA, 2022), Trauma- and Stressor-Related Disorders: Prolonged Grief Disorder

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### 71. INT-1182 (rev 2) · Best answer: A

A social worker in a state prison meets with a client whose parent died last week. The facility denied the client's request to attend the funeral. In their first meeting since the death, the client sits quietly for a while, then says, 'It doesn't feel real. I never got to say goodbye.' What should the social worker do **FIRST**?

**A.** Invite the client to talk about the relationship and the loss
**B.** Request a psychiatric evaluation for depression after the loss
**C.** Help the client plan a private memorial ritual on site

**Why each option is right or wrong**

- **A** **Correct.** Early grief work starts by making room for the loss: inviting the client to talk about the parent, their relationship and what being kept from the funeral means. Talking about the person who died helps a mourner begin to accept the reality of the death, Worden's first task of mourning (Worden, 2018), and shows the worker what the client needs before anything is proposed. It beats C because a substitute ritual can help the client say goodbye, but planning one in the first conversation, before the client has been heard, moves to a task before the grief has been received; it fits once the client's wishes are known.
- **B** **Incorrect** (trap: Skips assessment). One week after a death, disbelief, anger and guilt are expected grief reactions, not signs of a disorder. A psychiatric evaluation would fit if the client showed pervasive hopelessness, suicidal thinking or an inability to function.
- **C** **Incorrect** (trap: Right action, wrong time). Doka describes therapeutic rituals as a way to recognize grief that circumstances have blocked (Doka, 2002), and a memorial inside the facility may well help this client say goodbye. Proposed in the first meeting, though, before the client has talked about the parent or the loss, it skips hearing what the client needs and wants.

**Takeaway:** In a first contact after a death, invite the bereaved to talk about the person and the loss before proposing rituals or referrals.

**Sources:** Worden, J. W. (2018). Grief Counseling and Grief Therapy (5th ed.). Springer Publishing, Task I: accept the reality of the loss; funerals and rituals; Doka, K. J. (Ed.). (2002). Disenfranchised Grief: New Directions, Challenges, and Strategies for Practice. Research Press, Ch. 9: The role of ritual in the treatment of disenfranchised grief

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## 72. INT-1189 (rev 1) · Best answer: A

In a hospital family room, a social worker meets the parent of a 19-year-old who died in a motorcycle crash an hour ago. The parent sits rigidly and says, 'I told them to sell that bike. I should have taken the keys.' What should the social worker do **FIRST**?

- **A.** Invite the parent to keep talking about what they feel
- **B.** Assure the parent that they are blameless for the crash
- **C.** Provide information about bereavement support groups

**Why each option is right or wrong**

- **A** **Correct.** In the first hour after a sudden death, the priority is a supportive presence that lets the bereaved person express what they feel. Guilt and self-blame are common early grief reactions, and Worden's counseling principles include helping survivors identify and experience such feelings (Worden, 2018). The parent has just voiced self-blame, so inviting more is the first step. It beats B because reassurance, though kind, argues with the parent's feelings before they are heard and can close the conversation; checking guilt against the facts comes later.
- **B** **Incorrect** (trap: Good but not best). Reassurance feels kind, but offered now it dismisses the parent's guilt before it is fully spoken and can end the conversation. Gently testing guilt against the facts belongs later, once the feeling has been heard.

**C** **Incorrect** (trap: Right action, wrong time). Linking to support groups is a useful later step, often before the family leaves or at follow-up. An hour after the death, resources come before the parent has had any chance to be heard.

**Takeaway:** Right after a sudden death, let the bereaved voice grief and guilt before reassuring, problem-solving or offering resources.

**Sources:** Worden, J. W. (2018). Grief Counseling and Grief Therapy (5th ed.). Springer Publishing, Grief counseling principles: help the survivor identify and experience feelings, including guilt

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## 73. INT-0111 (rev 1) · Best answer: C

At intake, the social worker explains the court's attendance requirement and what will be reported to the probation officer. The client says, 'I don't have an anger problem. I'm only here so the judge stays off my back.' Which response by the social worker is **BEST**?

**A.** Review the police report with the client to address the denial

**B.** Repeat the court's requirements and the reporting rules

**C.** Ask what the client would want to gain from finishing the program

#### Why each option is right or wrong

**A** **Incorrect** (trap: Too directive). Confronting denial with the police report this early tends to increase defensiveness and discord. Discrepancies can be explored later, gently, once rapport exists.

**B** **Incorrect** (trap: Good but not best). Clarifying requirements and reporting is essential, but it was just done. Repeating it in response to the client's frustration can feel like pressure and does not invite the client into the work.

**C** **Correct.** With mandated clients, engagement grows when the social worker looks for a goal the client actually wants within the mandate's limits, such as getting the court out of their life (Rooney, 2009). The requirements and reporting rules have already been explained, so the useful move now is to invite the client's own reasons for completing the program. Offering real choice reduces reactance. It beats B because repeating rules the client just heard adds nothing new and may sound like a threat, which tends to deepen resistance rather than build engagement.

**Takeaway:** With a mandated client, after clarifying the mandate, look for a goal the client owns, such as ending outside involvement.

**Sources:** Rooney, R. H. (Ed.). (2009). Strategies for Work with Involuntary Clients (2nd ed.). Columbia University Press, engaging involuntary clients; reactance; Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, engaging; discord

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### 74. INT-1516 (rev 1) · Best answer: A

A physician asks a hospital social worker to assess home safety for a 79-year-old patient admitted after a fall. When the social worker arrives, the patient says, 'I didn't ask for a social worker. I'm going home, period.' The patient is alert and oriented. What should the social worker do **FIRST**?

**A.** Explain that the visit aims to help the patient get home safely
**B.** Leave and document that the patient declined social work services
**C.** Begin the home safety questions the physician requested

**Why each option is right or wrong**

**A** **Correct.** Nonvoluntary clients engage best when the worker clarifies the purpose of contact and ties it to a goal the client already holds (Rooney, 2009). The patient's goal is going home, and the referral exists to make that goal workable, so explaining this reframes the visit as help with the patient's own aim. It beats B because a competent patient may decline, but a refusal given before the patient knows why the worker came is not yet an informed choice; once the purpose is clear, the decision is respected.

**B** **Incorrect** (trap: Good but not best). Respecting refusal is right for a competent adult who understands what is offered. Leaving now, before explaining the purpose, may forfeit help with the very goal the patient wants.

**C** **Incorrect** (trap: Right action, wrong time). The home safety assessment is the eventual task, but starting it over the patient's objection ignores the refusal and invites pushback. Engagement and consent come first.

**Takeaway:** With nonvoluntary clients, first clarify the purpose of contact and link it to a goal the client already holds before assessing.

**Sources:** Rooney, R. H. (Ed.). (2009). Strategies for Work with Involuntary Clients (2nd ed.). Columbia University Press, engaging nonvoluntary clients: clarifying purpose and client goals
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### 75. INT-1517 (rev 2) · Best answer: B

A parent must complete a parenting class under a court-ordered child welfare case plan. A few minutes into the first meeting, the parent says, 'Nobody asked what I need. I'm not sitting in some class.' What should the social worker do **FIRST**?

**A.** Describe what the court may do if the class goes unfinished
**B.** Reflect the parent's frustration at having no say in the plan
**C.** Separate the required steps from those the parent can choose

**Why each option is right or wrong**

**A** **Incorrect** (trap: Too directive). The parent does need accurate information about consequences, but leading with them sounds like a threat and increases pushback. Consequences fit within a clear account of what is fixed and what is open.

**B** **Correct.** Work with an involuntary client starts by acknowledging the client's reaction to the mandate. Anger at lost freedom, or reactance, is an expected response to pressure, and reflecting it lowers defensiveness and shows the worker is listening (Rooney, 2009). The parent names a specific grievance, not being asked, so a reflection

that captures it opens the conversation. It beats C because clarifying what is required and what is open is the right next step, but leading with it before the parent feels heard can sound like more of the same control.

**C** **Incorrect** (trap: Right action, wrong time). Separating non-negotiable requirements from areas of choice is central to work with mandated clients and answers the complaint about having no say (Rooney, 2009). It lands best right after the parent's anger has been heard; offered first, it can sound like a sales pitch for the class.

**Takeaway:** With a mandated client's anger, reflect the reaction first; then separate non-negotiable requirements from areas of choice.

**Sources:** Rooney, R. H. (Ed.). (2009). Strategies for Work with Involuntary Clients (2nd ed.). Columbia University Press, non-negotiable versus negotiable elements of mandated contact

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## 76. INT-1518 (rev 2) · Best answer: B

A client who sought telehealth counseling for work stress has canceled 2 of the last 3 sessions by text, each time citing a busy schedule. The agency discharges clients after 3 missed sessions. What should the social worker do **FIRST**?

**A.** Send a letter explaining the agency's discharge policy

**B.** Reach out to discuss what is making sessions hard to keep

**C.** Wait to see whether the client attends the next scheduled session

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). Clients should know the attendance policy, but a letter sent before any conversation feels punitive and may push an ambivalent client away. The policy can be explained during a discussion of barriers.

**B** **Correct.** Missed sessions often signal ambivalence or practical barriers, and voluntary clients stay engaged when the worker raises the pattern openly and without blame. Asking what is making sessions hard, such as timing, fit, or doubts about the work, can lead to schedule changes or renewed goals and guards against ending care for a client who still needs it (NASW Code 1.17(b); Hepworth et al.). It beats A because the policy can be covered in that conversation; a letter alone reads as a warning.

**C** **Incorrect** (trap: Good but not best). Waiting respects the client's pace, but two cancellations in three weeks are already a pattern. Staying silent risks a third miss and automatic discharge before the barrier is ever discussed. Waiting fits after a single, clearly explained cancellation.

**Takeaway:** When a voluntary client starts missing sessions, explore barriers and ambivalence with the client before warnings or discharge.

**Sources:** NASW Code of Ethics (2021), 1.17(b); Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Managing barriers to change

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### 77. INT-1519 (rev 1) · Best answer: A

A school requires a 15-year-old to meet with the social worker after a suspension. When the social worker lists the sessions the student must attend and the topics they will cover, the student refuses to talk. At the next meeting, the worker names what is required and lets the student pick the topics and meeting times, and the student begins to participate. Which concept **MOST** likely explains the student's first response?

**A.** Psychological reactance
**B.** Learned helplessness
**C.** Cognitive dissonance
**D.** Transference

#### Why each option is right or wrong

**A** **Correct.** Reactance theory holds that when people sense a threat to their freedom, they push back to restore it (Brehm, 1966). The student shut down when every part of the work was dictated, then engaged once real choices were offered within the school's requirement. That shift with choice is the signature of reactance, and it is why workers with nonvoluntary clients maximize choice (Rooney, 2009). It beats D because transference would follow the student into the relationship whatever the session structure; here, offering choices alone changed the response.

**B** **Incorrect** (trap: Wrong model or concept). Learned helplessness is passivity after repeated experiences of having no control. The student's first response was active refusal of imposed terms, not resignation or giving up.

**C** **Incorrect** (trap: Wrong model or concept). Cognitive dissonance is discomfort from holding conflicting beliefs or acting against one's beliefs. Nothing in the stem shows a clash between the student's beliefs and actions.

**D** **Incorrect** (trap: Wrong model or concept). Transference redirects feelings from earlier relationships, such as with strict caregivers, onto the worker. It would not be expected to fade simply because the meeting structure changed, as happened here.

**Takeaway:** Threats to freedom provoke reactance; offering real choices within fixed requirements reduces it and helps engage nonvoluntary clients.

**Sources:** Brehm, J. W. (1966). A Theory of Psychological Reactance. Academic Press, reactance to threatened freedom; Rooney, R. H. (Ed.). (2009). Strategies for Work with Involuntary Clients (2nd ed.). Columbia University Press, reactance and involuntary clients

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### 78. INT-1633 (rev 1) · Best answer: A

On a mild evening, a street outreach social worker approaches an adult who has slept in the same park for months. The person accepts a bottle of water but says, 'I don't need your programs,' and turns away. What should the social worker do **FIRST**?

**A.** Leave on friendly terms and say when the worker will return
**B.** Describe the shelter's open beds and services before leaving
**C.** Complete the housing intake assessment with the person

**Why each option is right or wrong**

**A** **Correct.** Engaging a person who is wary of services begins with respect for the refusal and reliable, low-pressure contact. SAMHSA TIP 55 (2013) describes short early contacts, accepting refusals without confrontation, and returning consistently with items the person may accept. Leaving on good terms and naming a return time builds the predictability that trust grows from. It beats B because pressing shelter information right after the person declined repeats the push behind the refusal; information can come once the person shows interest.

**B** **Incorrect** (trap: Too directive). Shelter information is useful once the person is interested. Delivered right after a clear refusal, it signals that the social worker did not hear the person, and it can make the next approach harder.

**C** **Incorrect** (trap: Right action, wrong time). A detailed housing assessment belongs after rapport is built and the person wants help with housing. Starting it after a refusal treats the person's answer as an obstacle rather than a decision.

**Takeaway:** With people wary of services, engagement starts with respecting refusals and returning reliably; formal assessment follows trust and expressed interest.

**Sources:** SAMHSA TIP 55 (2013) Behavioral Health Services for People Who Are Homeless, Part 1: outreach and engagement vignettes
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## 79. INT-1634 (rev 2) · Best answer: C

A voluntary client who has never had counseling comes to an outpatient clinic about job stress. A few minutes into the first session, the client asks, 'So do you just tell me what to fix? I've never done this before.' What should the social worker do **FIRST**?

**A.** Offer two or three practical tips for handling job stress
**B.** Ask the client to describe the job stress in detail
**C.** Explain how sessions work and ask what the client expects

**Why each option is right or wrong**

**A** **Incorrect** (trap: Too directive). Giving tips confirms the idea that the social worker fixes problems for the client, and it comes before any assessment of the job stress. Advice may have a place later, once the situation is understood.

**B** **Incorrect** (trap: Right action, wrong time). Exploring the presenting problem is central to early sessions and will come soon. Starting it now passes over the client's direct question about how counseling works; clients engage more readily in assessment once they understand what to expect.

**C** **Correct.** Orienting a new client to the helping process is a core engagement task: the social worker explains how sessions work and what each person does, and draws out the client's expectations (Hepworth et al.). Expectations that do not match the service are a common reason clients drop out early. This client has just said they do not know what counseling involves. It beats B because exploring the stress before answering the question leaves the client unsure what counseling involves or why the questions matter; the exploration follows once the client knows what to expect.

**Takeaway:** Early in engagement, orient new clients to how the work proceeds and draw out their expectations; mismatched expectations can lead to early dropout.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Orienting clients to the helping process
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### 80. INT-1635 (rev 1) · Best answer: B

A client on probation attends a court-ordered program. When the client mentions arriving on time for a new job all week, the social worker gives specific praise. When the client jokes about 'beating the system,' the worker calmly offers a different view and moves on. The worker also arrives on time and keeps every promise. Which approach is the social worker **MOST** likely using?

**A.** Motivational interviewing
**B.** Prosocial modeling
**C.** Contingency management

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). MI does use affirmations, so the praise fits partly. But MI responds to a remark like the joke with reflection rather than the worker's own contrasting view, and modeling punctuality is not part of the method.

**B** **Correct.** Trotter's (2022) prosocial modeling approach for involuntary clients has the worker notice and reward prosocial comments and actions, model reliable and respectful behavior, and briefly challenge procriminal remarks. Specific praise for punctuality, a calm counter to the 'beating the system' joke, and the worker's own punctuality and kept promises are each part of it. It beats A because motivational interviewing meets a provocative remark with reflection rather than the worker's own contrasting view, and it does not center the worker's modeling of reliable behavior (Miller & Rollnick, 2023).

**C** **Incorrect** (trap: Wrong model or concept). Contingency management gives tangible rewards, such as vouchers, for verified target behaviors on a set schedule. Here the reinforcement is verbal and paired with modeling and gentle challenge, which marks a different approach.

**Takeaway:** Prosocial modeling rewards clients' prosocial words and actions, models reliability and respect, and briefly challenges procriminal remarks with involuntary clients.

**Sources:** Trotter, C. (2022). Working with Involuntary Clients: A Guide to Practice (4th ed.). Routledge, Prosocial modelling and reinforcement; Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, Affirming; responding to discord
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### 81. INT-0003 (rev 1) · Best answer: A

A client who injects opioids says they are not ready to stop using but are worried after a friend overdosed. The client wants to keep meeting with the social worker. What is the **MOST** appropriate intervention?

**A.** Provide naloxone and teach overdose prevention steps
**B.** Make continued services contingent on abstinence
**C.** Refer the client to a residential treatment program this week

**Why each option is right or wrong**

**A** **Correct.** Harm reduction meets clients where they are and reduces the risks of ongoing use without requiring abstinence first (SAMHSA, 2023). The client's worry after a friend's overdose is a real opening. Naloxone and overdose-prevention teaching, such as not using alone, protect the client's life now and keep the relationship going. It beats C because a client who says they are not ready to stop is unlikely to accept residential treatment, and pushing it risks losing the engagement the client wants.

**B** **Incorrect** (trap: Overrides the client's choice). Requiring abstinence for continued services pushes out the client most at risk. Withdrawing support from someone not ready to stop increases danger and conflicts with meeting clients where they are.

**C** **Incorrect** (trap: Hands off too soon). Residential treatment may become appropriate if the client's goals change. Referring now ignores the client's stated readiness and may end engagement with a client at high overdose risk.

**Takeaway:** With a client not ready to stop using, harm reduction such as naloxone and safer-use teaching protects life and keeps engagement.

**Sources:** SAMHSA (2023) Harm Reduction Framework, pillars and principles; SAMHSA (2018) Opioid Overdose Prevention Toolkit (SMA18-4742), naloxone and overdose prevention
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### 82. INT-1093 (rev 1) · Best answer: D

A community outreach client uses cocaine at weekend parties and says they have no plans to stop. After two friends overdosed on cocaine that contained fentanyl, the client asks the social worker, 'How do I keep this from happening to me?' What is the **BEST** response?

**A.** Advise the client that quitting is the safest choice
**B.** Explore what the client might gain from quitting cocaine
**C.** Urge the client to enroll in an intensive treatment program
**D.** Go over practical ways to make each use less risky

**Why each option is right or wrong**

**A** **Incorrect** (trap: Too directive). Quitting would remove the risk, but the client has said they will keep using. Advice to stop answers a question the client did not ask and may end the conversation before any safety steps are shared.

**B** **Incorrect** (trap: Good but not best). Exploring reasons for change fits a client who is weighing it. This client asked a direct safety question, and pivoting to quitting can feel like a lecture; it leaves the immediate overdose risk unaddressed.

**C** **Incorrect** (trap: Hands off too soon). Intensive treatment fits people who want structured help with a substance use disorder. The client has not asked for treatment, and pushing it ignores the stated goal and may cost the relationship.

**D** **Correct.** Harm reduction meets people where they are and works to reduce the harms of use without requiring abstinence (Hawk et al., 2017). The client has a clear safety question and has said they will keep using. Discussing concrete steps, such as having naloxone on hand and someone nearby who can respond, answers that question and builds trust that keeps the door open to later change (SAMHSA, 2023). It beats B because exploring change is useful, but turning to quitting after the client asked how to stay alive sidesteps the request.

**Takeaway:** When a client who plans to keep using asks how to stay safer, answer with concrete harm-reduction steps rather than redirecting to abstinence.

**Sources:** Hawk, M., et al. (2017). Harm reduction principles for healthcare settings. Harm Reduction Journal, 14, 70, Table 1: harm reduction principles (pragmatism, autonomy); SAMHSA (2023) Harm Reduction Framework, pillars and principles; overdose prevention education

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## 83. INT-1094 (rev 2) · Best answer: A

An employee assistance client drinks 4 to 5 beers most weeknights and wants to cut back to 2 drinks on weekend nights only. The client says quitting entirely is not the goal right now but is willing to keep a daily log. What is the **MOST** appropriate step?

**A.** Set the drinking limit with the client and review it weekly

**B.** Have the client log drinks for 2 weeks before setting a limit

**C.** Support the goal if the client agrees to attend AA meetings

#### Why each option is right or wrong

**A** **Correct.** Harm reduction treats any positive change as progress and lets the client set the goal, with abstinence as one option rather than a requirement (Marlatt & Witkiewitz, 2002). This client wants a specific, sizable reduction and offers to track it. Building a concrete plan with a weekly review supports autonomy and incremental change (Hawk et al., 2017). It beats B because a 2-week baseline before any limit puts off a change the client is ready to make now; the daily log works best alongside the agreed limit, as the record reviewed each week. If moderation does not work, the plan can be revisited together.

**B** **Incorrect** (trap: Right action, wrong time). Self-monitoring is useful, and the client already offers to keep a log. Holding off on the goal for 2 weeks of baseline, though, delays a specific change the client is ready to start; the log can begin now and track progress toward the agreed limit.

**C** **Incorrect** (trap: Overrides the client's choice). Mutual-help groups can be useful, but making them a condition of support overrides the client's choice. Harm reduction offers options without coercion; AA can be suggested, not required.

**Takeaway:** Harm reduction accepts client-defined goals such as moderation, supports measurable steps, and treats any reduction in harm as progress.

**Sources:** Marlatt, G. A., & Witkiewitz, K. (2002). Harm reduction approaches to alcohol use: Health promotion, prevention, and treatment. Addictive Behaviors, 27(6), 867-886, whole article; Hawk, M., et al. (2017). Harm reduction principles for healthcare settings. Harm Reduction Journal, 14, 70, Table 1: autonomy; incrementalism
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## 84. INT-1095 (rev 1) · Best answer: C

A street outreach social worker has met for months with an adult who has schizophrenia and alcohol use disorder and has lived outdoors for 6 years. The client declines treatment but says, 'I'd take an apartment if I didn't have to quit drinking.' The worker's program follows a Housing First model. Which action is consistent with that model?

**A.** Refer the client to a sober transitional apartment first
**B.** Keep building rapport until the client accepts treatment
**C.** Offer a permanent apartment with services the client may choose

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Transitional programs that require sobriety follow a treatment-first, housing-readiness approach. Housing First removes those preconditions, so making housing depend on abstinence contradicts the model the program follows.

**B** **Incorrect** (trap: Good but not best). Continued engagement matters, but waiting for treatment acceptance before offering housing is the readiness approach that Housing First replaces. Housing comes first; treatment is offered, not required.

**C** **Correct.** Housing First offers immediate access to permanent housing without requiring sobriety or treatment, and pairs it with voluntary, client-directed support services (Tsemberis, 2010). The client is ready for housing and declines treatment, the situation the model was built for. Treatment stays available and is often accepted later, once basic stability exists. It beats A because a sober transitional step is a treatment-first approach that makes housing depend on abstinence, the very condition this client rejects.

**Takeaway:** Housing First provides permanent housing without sobriety or treatment requirements, with voluntary support services offered alongside.

**Sources:** Tsemberis, S. (2010). Housing First: The Pathways Model to End Homelessness for People with Mental Illness and Addiction. Hazelden, core principles: immediate housing, consumer choice, separation of housing and treatment
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### 85. INT-1096 (rev 2) · Best answer: B

At a syringe services program, a participant shows the social worker a swollen, hot area on the forearm where the redness has spread since yesterday. The participant feels feverish and explains clearly that they will not go to the emergency department after being mistreated there. The program partners with a walk-in wound clinic two blocks away that serves people who use drugs. What is the **BEST** action for the social worker?

**A.** Give wound care supplies and check the arm next week
**B.** Go with the participant to get wound care today
**C.** Call an ambulance despite the participant's objection
**D.** Explore the participant's readiness to stop injecting

**Why each option is right or wrong**

**A** **Incorrect** (trap: Ignores risk). Supplies help with minor wounds, but redness spreading over a day with fever signals a worsening infection. Waiting a week risks serious illness. Same-day medical care is needed, with supplies as an add-on.

**B** **Correct.** Harm reduction keeps people engaged without judgment and links them to care on terms they can accept (SAMHSA, 2023). Spreading redness, heat and fever suggest a worsening infection that needs medical treatment now, and people who inject drugs often delay care after stigma or mistreatment in hospitals (Gilbert et al., 2019). Going with the participant to a nearby, trusted clinic today meets the urgency while honoring the refusal of the emergency department. It beats C because an ambulance overrides a capable adult's choice when an acceptable same-day option exists.

**C** **Incorrect** (trap: Overrides the client's choice). Emergency transport over objection fits someone who is unresponsive or unable to decide. This participant is making a clear choice about one setting, not refusing all care, and a same-day alternative exists that the participant may accept.

**D** **Incorrect** (trap: Good but not best). Talking about change has its place, but it does not address an infection that is getting worse today. Raising treatment for drug use now may also feel like a condition of help and push the participant away.

**Takeaway:** Harm reduction links people to urgent medical care on terms they accept; worsening infection signs need same-day treatment, not routine follow-up.

**Sources:** SAMHSA (2023) Harm Reduction Framework, pillars and principles; linkage to care; Gilbert, A. R., Hellman, J. L., Wilkes, M. S., Rees, V. W., & Summers, P. J. (2019). Self-care habits among people who inject drugs with skin and soft tissue infections: A qualitative analysis. Harm Reduction Journal, 16, 69, whole article
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### 86. INT-1179 (rev 2) · Best answer: A

A client has attended an opioid treatment program for 8 months, taking methadone daily and holding a steady job. Recent urine tests show benzodiazepines that were not prescribed. The client says the pills help with sleep and fears losing methadone. At a team meeting, a staff member proposes discharge under the program's 'no illicit use' rule. What is the **BEST** response by the social worker?

**A.** Urge keeping the client enrolled while the team addresses the pill use
**B.** Support discharge with a referral to an abstinence-based residential program
**C.** Tell the client that staying enrolled requires benzodiazepine-free tests

**Why each option is right or wrong**

**A** **Correct.** Harm reduction keeps people connected to protective care while risks are addressed (SAMHSA Harm Reduction Framework). In 2017 the FDA advised that methadone and buprenorphine should not be withheld from patients who use benzodiazepines, because untreated opioid addiction can outweigh the risks of the combination, and discharge is unlikely to stop the use (FDA, 2017). This client is stable and engaged, so the team can assess the sleep problem, involve the prescriber and add overdose education. It beats C because an abstinence ultimatum is likely to end in the same discharge if use continues.

**B** **Incorrect** (trap: Hands off too soon). Residential care may suit some clients, but discharge here ends a treatment that is working and leaves an opioid-dependent person without methadone. The FDA has cautioned that excluding patients for benzodiazepine use is unlikely to stop it.

**C** **Incorrect** (trap: Ignores risk). Clear expectations matter, but an ultimatum tied to test results usually ends in discharge if use continues. It repeats the harm of B on a delay and skips assessing why the client takes the pills for sleep.

**Takeaway:** Ongoing use of another substance is a reason to adjust care, not to withdraw medication for opioid use disorder.

**Sources:** FDA (2017). Drug Safety Communication: FDA urges caution about withholding opioid addiction medications from patients taking benzodiazepines or CNS depressants, September 20, 2017; SAMHSA (2023) Harm Reduction Framework, Pillar 2: embraces the inherent value of people; Pillar 5: offers most accessible and noncoercive support

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### 87. INT-1184 (rev 2) · Best answer: A

A client in outpatient counseling uses methamphetamine a few times a month and says they are not interested in stopping. The client carries condoms but says they rarely use them during sex while high, then asks, 'Is there anything that can lower my chances of getting HIV?' What is the **BEST** response?

**A.** Link the client to a clinic offering preventive HIV medication
**B.** Explore the client's readiness to cut back on methamphetamine
**C.** Provide condoms and go over safer-sex steps that lower HIV risk

**Why each option is right or wrong**

**A** **Correct.** Harm reduction meets people where they are and responds to the risk they name (SAMHSA Harm Reduction Framework). This client has declined to stop using, carries condoms but rarely uses them during sex while high, and asks directly about HIV prevention. CDC guidance supports offering pre-exposure prophylaxis (PrEP) to sexually active adults who have condomless sex, and linking the client to a prescriber is the most effective step within the social work role (CDC, 2021). It beats C because the client already has condoms and says they rarely get used while high, so relying on them alone leaves the main risk in place.

**B** **Incorrect** (trap: Good but not best). Exploring readiness for change fits when a client is weighing their use. This client has stated no plan to stop and asked a specific prevention question; steering back to the drug use sidesteps that request and can weaken trust.

**C** **Incorrect** (trap: Good but not best). Condoms and safer-sex teaching are reasonable additions, and the worker can offer them. Used alone, they depend on the very behavior the client says breaks down while high, so they protect less than linkage to medication that works without it.

**Takeaway:** Harm reduction answers the risk a client names; link clients with ongoing sexual HIV risk to PrEP care, whether or not they stop using.

**Sources:** CDC (2021). Preexposure Prophylaxis for the Prevention of HIV Infection in the United States: 2021 Update. A Clinical Practice Guideline, Identifying indications for PrEP: sexually active adults; SAMHSA (2023) Harm Reduction Framework, Pillar 6: any positive change, as defined by the person; principle: respect autonomy (meet people where they are)

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.12 Harm-reduction approaches · **Qualifier** BEST · **Cognitive** application · **Difficulty** 3 of 3

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## 88. INT-1543 (rev 1) · Best answer: C

A 72-year-old client with COPD receives a notice that Medicaid home care hours will be cut in half in 10 days. The client relies on the aide for bathing and meals and wants to fight the cut. The notice explains how to request a fair hearing. What should the social worker do **FIRST**?

**A.** Call the Medicaid caseworker to request that the cut be reversed

**B.** Gather medical records that document the client's care needs

**C.** Help the client file an appeal before the cut takes effect

**D.** Arrange a private-pay aide to cover the lost hours

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Informal negotiation is often the first rung of advocacy and may still help. Relying on it first risks missing the window that keeps services in place during an appeal. It fits alongside the appeal, not before it.

**B** **Incorrect** (trap: Right action, wrong time). Documentation will strengthen the case at the hearing, but gathering it first can push the request past the effective date. Evidence can be collected after the appeal is filed.

**C** **Correct.** Advocacy usually starts with the least adversarial step, but deadlines change the order. Under federal Medicaid rules, when a beneficiary requests a hearing before

the date of action, services generally continue until the hearing decision (42 CFR 431.230(a)). Filing now protects the bathing and meal help the client depends on, and helping the client file keeps the client in charge of the challenge. It beats A because a call to the caseworker can happen at the same time, but if it fails after the effective date, the hours are already lost.

**D** **Incorrect** (trap: Good but not best). Paying privately may be costly and accepts a cut the client wants to challenge. It is a fallback if the appeal fails or a stopgap while other options are explored.

**Takeaway:** When a benefit cut has an appeal deadline, file the appeal promptly to preserve services, then pursue negotiation and evidence.

**Sources:** Medicaid fair hearing regulations, 42 CFR Part 431 Subpart E, 42 CFR 431.230(a)
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.13 Advocating for needed services and resources · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 89. INT-1544 (rev 1) · Best answer: A

A social worker from a community aging services agency visits a client who lives in a nursing facility. Soon after the client complained about slow call-light response, the facility issued a 30-day discharge notice. The client and social worker met with the administrator, who refused to reconsider. The client wants to stay. What should the social worker do **NEXT**?

**A.** Help the client contact the long-term care ombudsman program
**B.** Help the client tour other facilities before the notice period ends
**C.** Report the facility to adult protective services for neglect

#### Why each option is right or wrong

**A** **Correct.** Advocacy moves from informal to formal and outside channels when internal efforts fail. The client already raised the issue with the administrator without success. The long-term care ombudsman program helps residents resolve complaints about their rights, and every discharge notice must list the ombudsman's contact and the resident's appeal rights (42 CFR 483.15(c)(5); 45 CFR 1324.19). The ombudsman can help the client appeal what may be a retaliatory discharge. It beats C because slow call lights alone do not establish neglect, and a protective report would not address the discharge itself.

**B** **Incorrect** (trap: Good but not best). Touring other facilities accepts a discharge the client wants to fight. It may be a sensible backup later, but the client has appeal rights and has not yet used outside help.

**C** **Incorrect** (trap: Skips assessment). Slow call-light response is a care-quality complaint, not clear evidence of abuse or neglect, and a protective report would not stop the discharge. A report fits if the visit shows signs of neglect or mistreatment.

**Takeaway:** After internal efforts fail, link a nursing facility resident to the long-term care ombudsman to contest a discharge or rights problem.

**Sources:** Requirements for long-term care facilities, 42 CFR 483.15, 42 CFR 483.15(c)(5); State Long-Term Care Ombudsman Program rule, 45 CFR 1324.19, 45 CFR 1324.19(a)
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.13 Advocating for needed services and resources · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 1 of 3

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### 90. INT-1545 (rev 2) · Best answer: C

A school social worker learns that the front office turned away 3 families staying in a motel this semester because they could not show proof of a home address. Each family enrolled only after the social worker stepped in. Which action is **BEST**?

**A.** Keep helping each family enroll as new cases come up
**B.** Give front-office staff a written copy of the enrollment rules
**C.** Raise the enrollment practice with the district's homeless liaison

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Case advocacy helped each family, but a repeated pattern signals a practice problem. Continuing one rescue at a time leaves future families facing the same barrier and delay.

**B** **Incorrect** (trap: Good but not best). Sharing the rules with front-office staff is a reasonable instinct and may help. It relies on an informal correction, though; the district liaison is the person federal law charges with ensuring immediate enrollment, and can change the practice and train staff across schools.

**C** **Correct.** When the same barrier harms several clients, the worker adds cause advocacy, seeking a change in policy or practice, to case-by-case help (Hepworth et al.). Federal law requires schools to enroll children experiencing homelessness immediately, even without records normally required, and each district must name a liaison to ensure this (McKinney-Vento Act, 42 U.S.C. 11432(g)). Three turn-aways show a front-office practice problem, so raising it with the liaison targets the cause. It beats A because one rescue at a time leaves the practice unchanged for the next family.

**Takeaway:** When one barrier keeps harming several clients, add cause advocacy through the proper channel to case-by-case help.

**Sources:** McKinney-Vento Homeless Assistance Act, 42 U.S.C. 11432, 42 U.S.C. 11432(g)(3)(C) and (g)(1)(J)(ii); Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Advocacy and social action: case and cause advocacy
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.13 Advocating for needed services and resources · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 91. INT-1546 (rev 1) · Best answer: B

Which practice role involves pressing an agency, on a client's behalf, to provide a service the client is entitled to receive?

**A.** Broker
**B.** Advocate
**C.** Mediator
**D.** Enabler

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). A broker links clients with resources they need, such as connecting a family to a food bank. Brokering assumes a willing provider; pressing a reluctant agency on the client's behalf is advocacy.

**B** **Correct.** In the advocate role, the social worker champions a client's rights and entitlements and presses a resistant agency or system to provide services the client should receive (Hepworth et al.). The definition's key features are acting on the client's

behalf and challenging a refusal. It beats A, broker, which connects clients to resources but does not involve pushing back against a provider that is withholding them.

**C** **Incorrect** (trap: Wrong model or concept). A mediator helps two parties in a dispute reach agreement and stays neutral between them. Pressing the agency on the client's side is not neutral.

**D** **Incorrect** (trap: Wrong model or concept). An enabler helps clients find and use their own strengths and coping resources to make changes. The definition describes acting toward an outside agency for the client, which is advocacy.

**Takeaway:** Advocates press systems for clients' entitlements; brokers link clients to resources; mediators stay neutral between disputing parties.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Social work roles: broker, mediator, client advocate

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.13 Advocating for needed services and resources · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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## 92. INT-1652 (rev 1) · Best answer: A

A client with schizoaffective disorder receives a lease warning after neighbors complain of loud talking at night. The client links it to voices that worsened during a medication change, which the prescriber is now adjusting. The client wants to keep the apartment and agrees to let the social worker help. What is the **MOST** appropriate way for the social worker to assist?

**A.** Help the client ask the landlord for a reasonable accommodation

**B.** Help the client look for a unit with fewer shared walls

**C.** Help the client file a fair housing complaint right away

**D.** Send the landlord a letter describing the client's diagnosis

#### Why each option is right or wrong

**A** **Correct.** The Fair Housing Act makes it unlawful to refuse reasonable accommodations in rules, policies or practices when a person with a disability needs them to use and enjoy a home (42 U.S.C. 3604(f)(3)(B); 24 CFR 100.204). The noise is tied to symptoms that treatment is now addressing, and the client wants to stay, so a written request, such as time for the medication adjustment before the lease is enforced, uses the client's right through the proper first channel (Hepworth et al.). It beats C because a complaint fits after a request is refused, and no request has been made yet.

**B** **Incorrect** (trap: Good but not best). Moving might end the complaints, but the client wants to keep this apartment and has a legal route to do so. Starting with a move gives up an entitlement before using it. A move becomes an option if the client later prefers it.

**C** **Incorrect** (trap: Skips assessment). A fair housing complaint is the right step if the landlord refuses a reasonable request. The landlord has not yet been asked, so filing now skips the direct request that may settle the matter and that a complaint would usually build on.

**D** **Incorrect** (trap: Misreads law or ethics). A request can describe the disability-related need and the change sought without naming a diagnosis or treatment details. Sending

the diagnosis shares more than the purpose requires; disclosure should be limited to what is needed (NASW Code 1.07(c)).

**Takeaway:** When disability symptoms threaten housing, help the client ask for a reasonable accommodation before moving or filing a complaint.

**Sources:** Fair Housing Act, 42 U.S.C. 3604(f)(3)(B); Fair Housing Act regulations, 24 CFR 100.204; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Advocacy and social action: case and cause advocacy; NASW Code of Ethics (2021), 1.07(c)

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.13 Advocating for needed services and resources · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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## 93. INT-1653 (rev 1) · Best answer: B

A parent of two arrives upset, saying the family's food assistance 'just got cut off out of nowhere.' The parent holds a letter from the benefits office but has not read it closely and asks the social worker to call the office director right away. What should the social worker do **FIRST**?

**A.** Call the office director to object to the cutoff

**B.** Review the notice together to find the stated reason

**C.** Help the parent submit a written fair hearing request today

**Why each option is right or wrong**

**A** **Incorrect** (trap: Skips assessment). Calling leadership may become useful later, but without the stated reason the social worker cannot make a specific case. Starting at the top also bypasses the frontline fix and the formal appeal route that may resolve it.

**B** **Correct.** Effective case advocacy starts with the facts: what action was taken, why, and what deadlines and appeal rights apply (Hepworth et al.). A notice of adverse action must state the reason and the right to a fair hearing (7 CFR 273.13(a)(2)), so reading it together takes minutes and shows whether the problem is a missing document, an error, or something only a hearing can fix. It beats C because a hearing may well be needed, but the notice may point to a quicker fix and will show the deadline and whether benefits can continue meanwhile.

**C** **Incorrect** (trap: Right action, wrong time). A fair hearing protects the family's rights and may be the right step. Filing before reading the notice risks missing a quicker fix, such as sending a missing pay stub, and leaves the request without the facts it needs.

**Takeaway:** Before advocating against a benefits decision, read the notice with the client to learn the reason, deadlines and appeal rights.

**Sources:** Supplemental Nutrition Assistance Program regulations, 7 CFR 273.13(a)(2); Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Advocacy and social action: case and cause advocacy

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### 94. INT-1117 (rev 1) · Best answer: C

In an Illness Management and Recovery group at a community mental health center, the social worker has been reading each module's handout aloud. A member with schizophrenia who reads at a basic level says the material 'goes right over my head' and has started missing sessions. What is the **MOST** helpful adjustment?

**A.** Assign the full handout as homework between sessions
**B.** Refer the member to individual therapy instead
**C.** Teach interactively with pictures and frequent review
**D.** Give a printed summary written at a lower reading level

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Reading the full handout alone at home asks more of the member's literacy, not less, and removes the support of the group. Between-session practice in IMR is short and tailored, not extra reading.

**B** **Incorrect** (trap: Hands off too soon). Individual work may suit some people, but the problem here is how the material is taught, not the group itself. Moving the member out first skips a teaching change that could keep the member engaged with peers.

**C** **Correct.** Illness Management and Recovery delivers psychoeducation through interactive teaching combined with motivational and cognitive-behavioral strategies, tailored to each person's reading ability, prior knowledge and goals, with review of earlier material and visual supports (Mueser et al., 2006). Reading a handout aloud is passive and depends on literacy, and the member's missed sessions show it is not working. Teaching with pictures, frequent review and links to the member's own recovery goals makes the content usable. It beats D because an easier summary helps, but it keeps learning passive and print-based for a member who is already disengaging.

**D** **Incorrect** (trap: Good but not best). Matching written material to reading level is a sound adaptation and can be part of the fix. On its own, it keeps learning passive and print-based, so it does less to re-engage a member who is already missing sessions.

**Takeaway:** Psychoeducation for people with serious mental illness works best when interactive, reviewed often, visually supported and tailored to the person.

**Sources:** Mueser, K. T., Meyer, P. S., Penn, D. L., Clancy, R., Clancy, D. M., & Salyers, M. P. (2006). The Illness Management and Recovery program: Rationale, development, and preliminary findings. Schizophrenia Bulletin, 32(Suppl. 1), S32-S43, teaching strategies: educational, motivational, cognitive-behavioral
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.14 Delivering psychoeducation · **Qualifier** MOST · **Cognitive** application · **Difficulty** 3 of 3

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### 95. INT-1118 (rev 1) · Best answer: A

After a young adult's first hospitalization for schizophrenia, the parents meet with the inpatient social worker. One parent asks, 'Did our strict parenting cause this?' What is the **BEST** response?

**A.** Explain that the illness comes from biological vulnerability plus stress
**B.** Explore the parent's guilt about past parenting decisions in detail
**C.** Recommend family therapy to change the household's parenting style

**Why each option is right or wrong**

**A** **Correct.** Psychoeducation answers families' questions with accurate, non-blaming information. The stress-vulnerability model holds that schizophrenia arises when a biological vulnerability meets stressors, not from parenting style (Zubin & Spring, 1977). Family psychoeducation programs take this non-blaming stance and then teach families how to support recovery (Dixon et al., 2000). The parent asked a direct question, so a clear answer relieves misplaced blame and opens the door to learning how the family can help. It beats B because exploring guilt is supportive, but leaving the question unanswered can deepen the belief that the parents caused the illness.

**B** **Incorrect** (trap: Good but not best). Guilt is common and worth acknowledging, but the parent asked a specific question that has an evidence-based answer. Exploring the guilt without correcting the mistaken belief can reinforce self-blame. Exploration fits after accurate information is given.

**C** **Incorrect** (trap: Wrong model or concept). Recommending therapy to change parenting implies the family caused the illness, echoing discredited theories that blamed parents. Families do benefit from support and skills, but through non-blaming family psychoeducation, not treatment framed around their faults.

**Takeaway:** When families ask whether they caused schizophrenia, explain the stress-vulnerability model in non-blaming terms, then teach ways to support recovery.

**Sources:** Zubin, J., & Spring, B. (1977). Vulnerability: A new view of schizophrenia. Journal of Abnormal Psychology, 86(2), 103-126, whole article; Dixon, L., Adams, C., & Lucksted, A. (2000). Update on family psychoeducation for schizophrenia. Schizophrenia Bulletin, 26(1), 5-20, whole article

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## 96. INT-1119 (rev 2) · Best answer: B

In an early psychosis program, a 22-year-old client who smokes cannabis daily asks whether it is 'really that bad.' The social worker first asks what the client has heard, then briefly shares research on heavy cannabis use and psychotic relapse. What should the social worker do **NEXT**?

**A.** Advise the client to cut back before the next visit

**B.** Ask what the client makes of this information

**C.** Ask whether the client is willing to cut back now

**D.** Give a handout that summarizes the research findings

**Why each option is right or wrong**

**A** **Incorrect** (trap: Too directive). Advice given before hearing the client's reaction tends to provoke push-back and turns a shared exploration into a lecture. Advice can be offered with permission once the client has weighed the information.

**B** **Correct.** Elicit-provide-elicit ends by asking for the person's reaction to the information just shared (Miller & Rollnick, 2023). The worker has already asked what the client knew and offered the research, so the next step is to learn how the client interprets it. The response shows whether the information landed, brings out doubts, and lets the client draw their own conclusions about cannabis, which supports

autonomy. It beats D because a handout can reinforce the message later, but handing it over now skips the client's reaction, which shapes everything that follows.

**C** **Incorrect** (trap: Misuses the technique). Asking about commitment has a place in MI, but jumping to it now skips the client's reaction to what was just shared and tilts the talk toward compliance. A yes-or-no question this early invites a quick 'no' or an empty 'yes.'

**D** **Incorrect** (trap: Right action, wrong time). A written summary can support memory and fits near the end of the conversation. Giving it now skips hearing how the client understands the information and what they make of it.

**Takeaway:** Close an information exchange by inviting the client's interpretation before any advice, so the conclusions remain the client's own.

**Sources:** Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, Exchanging information: elicit-provide-elicit

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## 97. INT-1120 (rev 1) · Best answer: A

A client has gone to the emergency department three times for a racing heart and chest tightness. Cardiac testing was normal each time, and the physician diagnosed panic disorder and referred the client to the social worker. The client asks, 'If my heart is fine, why does my body do this?' What should the social worker do **FIRST**?

**A.** Explain how the body's alarm response produces these sensations

**B.** Begin exercises that bring on a racing heart in session

**C.** Ask the physician to order further heart tests for reassurance

#### Why each option is right or wrong

**A** **Correct.** Cognitive-behavioral treatment for panic disorder begins with psychoeducation about the nature of anxiety and panic, including how the body's fight-or-flight response creates sensations like a racing heart (Craske & Barlow, 2022). The client's question invites exactly this, and understanding the cycle of sensation and fear reduces catastrophic thinking. It also lays the groundwork for later steps. It beats B because exercises that deliberately bring on body sensations are an effective later step, but starting them before the client understands the alarm response can feel frightening and confusing.

**B** **Incorrect** (trap: Right action, wrong time). Interoceptive exposure, which brings on feared body sensations on purpose, is a core part of panic treatment. It comes after psychoeducation, when the client understands why the sensations are harmless and agrees to the rationale.

**C** **Incorrect** (trap: Hands off too soon). Medical causes have been ruled out three times. More testing can reinforce the belief that something is physically wrong and feed reassurance seeking. New or different symptoms would justify another medical look.

**Takeaway:** In CBT for panic, start with psychoeducation on the body's alarm response before exposure to feared sensations.

**Sources:** Craske, M. G., & Barlow, D. H. (2022). Mastery of Your Anxiety and Panic: Therapist Guide (5th ed.). Oxford University Press, psychoeducation on the nature of anxiety and panic
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.14 Delivering psychoeducation · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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## 98. INT-1121 (rev 2) · Best answer: B

During a home visit, an adult child caring for a parent with major neurocognitive disorder says the parent asks the same question 20 times an hour. The social worker acknowledges how draining that sounds. The caregiver replies, 'I think it's on purpose, to get under my skin,' and says they have started snapping at the parent. What is the **MOST** appropriate response?

**A.** Suggest gently reminding the parent that the question was answered
**B.** Explain that memory loss keeps the parent from recalling asking
**C.** Reflect the caregiver's frustration and exhaustion

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misuses the technique). Pointing out that the question was already answered seems logical, but the parent cannot remember asking, so the reminder usually adds confusion and distress. Caregiver guidance advises against arguing or using logic.

**B** **Correct.** Psychoeducation corrects a misunderstanding that is driving harm. The worker has already acknowledged the strain, and the caregiver still believes the repetition is deliberate; that belief is fueling the snapping. Explaining that the disease damages memory, so the parent does not recall having asked, reframes the behavior as a symptom and leads to strategies such as answering calmly, responding to the worry behind the question, or posting a written answer (Alzheimer's Association). It beats C because the strain has already been acknowledged; reflecting it again leaves the 'on purpose' belief in place and keeps the conflict going.

**C** **Incorrect** (trap: Good but not best). Caregiver frustration is real and deserves empathy, but the social worker has just acknowledged it. Reflecting it again leaves the belief that the parent is doing this on purpose untouched, so the snapping is likely to continue. Reflection fits as the opening response before the misattribution is voiced.

**Takeaway:** When a caregiver misreads a dementia symptom as deliberate, explain the symptom and offer coping strategies; empathy alone leaves the misattribution.

**Sources:** Alzheimer's Association. Repetition (Caregiving: Stages and Behaviors), causes of repetition and caregiver tips
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.14 Delivering psychoeducation · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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### 99. INT-1190 (rev 1) · Best answer: C

A social worker leads an illness management group at a community mental health center for adults with schizophrenia. During a session on stress and symptoms, a member says, 'This is a waste of time. I'm not sick.' Earlier, the member said they want to keep their apartment and get a job. What is the **BEST** response?

**A.** Review the member's symptoms with the group to build insight
**B.** Note the member's view and continue with the planned material
**C.** Connect the skills to what the member wants for work and housing
**D.** Suggest individual therapy to work on accepting the diagnosis

**Why each option is right or wrong**

**A** **Incorrect** (trap: Too directive). Pressing symptom evidence in front of peers is likely to shame the member and harden their position. Lack of insight in schizophrenia often reflects the illness itself, so confrontation rarely produces agreement.

**B** **Incorrect** (trap: Good but not best). Acknowledging the member's view is a good start, and the group can move on. Without linking the content to something the member values, though, the session stays irrelevant to them and engagement fades.

**C** **Correct.** When a person does not see themselves as ill, arguing about the diagnosis usually deepens resistance. Amador's LEAP approach instead listens, empathizes, finds points of agreement and partners on the person's own goals, and illness management and recovery work ties information to personal recovery goals (Amador, 2000; SAMHSA IMR, 2009). This member wants an apartment and a job, so framing stress skills as tools for those goals keeps them engaged without a fight over labels. It beats B because continuing as planned is respectful but gives the member no reason to use the content.

**D** **Incorrect** (trap: Hands off too soon). Individual work may help some members, but sending the member away to accept the diagnosis treats agreement with the label as the goal and hands off a problem the group can address now.

**Takeaway:** When a client rejects a diagnosis, deliver psychoeducation through the client's own goals instead of debating the label.

**Sources:** Amador, X. (2000). I Am Not Sick, I Don't Need Help! Vida Press, LEAP: listen, empathize, agree, partner; SAMHSA (2009) Illness Management and Recovery Evidence-Based Practices KIT, Recovery goals as the frame for psychoeducation

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### 100. INT-1192 (rev 3) · Best answer: C

A client with bipolar I disorder, stable for 6 months after a manic episode, asks for help avoiding another hospitalization. The client's partner joins the session and asks for a complete list of symptoms to watch for each day. The client says the partner's constant questions about mood feel like being watched. Which psychoeducation approach is **MOST** helpful?

**A.** Give the partner a daily checklist of bipolar symptoms to watch for
**B.** Meet with the client alone to protect the client's privacy
**C.** Help the client name their own early signs and the partner's role
**D.** Teach the partner to watch for reduced sleep as the key warning sign

#### Why each option is right or wrong

**A** **Incorrect** (trap: Overrides the client's choice). A daily symptom checklist in the partner's hands makes the partner a monitor and increases the conflict the client described. Generic lists also miss the client's personal early signs, which are more useful for preventing relapse.

**B** **Incorrect** (trap: Good but not best). Privacy matters, but the client did not ask to exclude the partner, and involving family in relapse planning can help prevent episodes. Leaving the partner out loses an ally and leaves the conflict over monitoring unresolved.

**C** **Correct.** Bipolar psychoeducation centers on each person's own pattern of early warning signs that come before an episode, plus an agreed plan for responding (Colom & Vieta, 2006). Family-focused treatment brings relatives into that plan as allies with defined roles, which eases conflict over monitoring (Miklowitz, 2010). The client wants to prevent hospitalization but feels watched, so building the client's own list and deciding together what the partner will notice and do respects autonomy while using the partner's support. It beats A because a generic checklist hands control to the partner and intensifies the monitoring the client resents.

**D** **Incorrect** (trap: Good but not best). Reduced sleep is a common early sign of mania, so this sounds targeted. It still makes the partner the monitor and swaps one general sign for the client's own pattern; relapse plans work best when the client names personal warning signs and agrees on the partner's role.

**Takeaway:** In bipolar psychoeducation, build the client's personal early warning signs and a shared response plan that gives family an agreed role.

**Sources:** Colom, F., & Vieta, E. (2006). Psychoeducation Manual for Bipolar Disorder. Cambridge University Press, Early detection of new episodes; Miklowitz, D. J. (2010). Bipolar Disorder: A Family-Focused Treatment Approach (2nd ed.). Guilford Press, Relapse prevention plan with family

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.14 Delivering psychoeducation · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 2 of 3

### 101. INT-1520 (rev 1) · Best answer: D

Which treatment objective for a client with depression is written in measurable terms?

**A.** Client will feel less depressed and more hopeful about the future
**B.** Client will increase pleasant activities as often as possible
**C.** Client will gain insight into the causes of the low mood
**D.** Client will walk 20 minutes on 5 days each week by week 6

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Hope and mood matter, but 'feel less depressed' names no observable behavior, amount, or deadline. It reads as a broad goal that needs specific objectives under it.

**B** **Incorrect** (trap: Misuses the technique). This names a behavior, but 'as often as possible' sets no amount or time frame, so there is no way to tell when it has been met.

**C** **Incorrect** (trap: Good but not best). Insight is an internal state that cannot be observed or counted, and the objective has no time frame. It may be a process aim, but it is not a measurable objective.

**D** **Correct.** Measurable objectives name an observable behavior, a quantity, and a time frame, so client and worker can tell whether they were met (Hepworth et al.). Walking 20 minutes on 5 days a week by week 6 can be counted and checked. It beats B because B names a behavior, but 'as often as possible' sets no amount or deadline, so progress cannot be judged. Measurable objectives also make ongoing progress monitoring possible (Bloom, Fischer & Orme, 2009).

**Takeaway:** A measurable client objective states an observable behavior, how much or how often, and by when.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Negotiating goals and formulating a contract; Bloom, Fischer & Orme (2009). Evaluating Practice (6th ed.), specifying goals and objectives
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.15 Contracting and goal setting: partializing, measurable objectives · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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### 102. INT-1521 (rev 2) · Best answer: A

At a primary care clinic, a client recently diagnosed with type 2 diabetes also lost a job last month. Early in their first meeting, the client says, 'I'm supposed to change my diet, test my blood, exercise, and find work. I can't do any of it.' What should the social worker do **FIRST**?

**A.** Reflect how overwhelming the many new demands feel
**B.** Create a full plan that covers each of the four demands
**C.** Agree with the client on one small step to take this week

**Why each option is right or wrong**

**A** **Correct.** When a client feels swamped, an empathic reflection comes before problem solving: naming how overwhelming the demands feel shows understanding, lowers distress and opens the way to planning (Hepworth et al.). The client has just listed four new demands and said 'I can't do any of it,' so the first move is to show that the feeling has been heard. It beats C because partializing into one small step is the right next move (Reid & Epstein, 1972), but offered before the client feels understood it can sound like brushing the overwhelm aside.

**B** **Incorrect** (trap: Misuses the technique). Planning is the right kind of move, but a plan covering every demand at once overwhelms a client who already feels unable to start. Goals are taken one piece at a time.

**C** **Incorrect** (trap: Right action, wrong time). Partializing, choosing one concrete step for the week such as checking blood sugar each morning, is the likely next move and helps an overwhelmed client act (Reid & Epstein, 1972). Proposed before the overwhelm has been acknowledged, it can feel like being handed one more task.

**Takeaway:** When a client feels overwhelmed by many demands, reflect the feeling first; then partialize into one small, concrete step.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Ch. 5: empathic communication; Reid, W. J., & Epstein, L. (1972). Task-Centered Casework. Columbia University Press, target problems and tasks
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## 103. INT-1525 (rev 1) · Best answer: C

A client at a family service agency agreed to an 8-session task-centered contract focused on conflict with a teenage child. At session 5, the client arrives with an eviction notice and says it is all they can think about. The original goals are about half met. What is the social worker's **BEST** response?

**A.** Keep the original contract and plan to address housing afterward
**B.** Explore how the eviction connects to the conflict at home
**C.** Revise the contract with the client to add the housing problem

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Focus matters in time-limited work, but an eviction is an urgent basic need that the client now ranks first. Waiting until the end risks losing housing and the client's engagement. Holding the agreed focus fits when a new issue is minor or can wait.

**B** **Incorrect** (trap: Good but not best). The stem already shows what the client needs: help with an eviction that dominates their thinking. Linking it to the family conflict may be useful later, but exploring meaning first delays a decision the client has clearly signaled.

**C** **Correct.** A contract is a working agreement that is revisited when the client's priorities or circumstances change (Hepworth et al.). An eviction notice is an urgent basic-needs problem, and the client says it now fills their attention, so worker and client revise the target problems and time frame together. It beats A because holding to the old focus

ignores a pressing problem and the client's own ranking, which weakens engagement. Task-centered work expects target problems to be added or reordered when a new problem becomes the client's priority (Reid & Epstein, 1972).

**Takeaway:** Contracts are living agreements: when an urgent new problem becomes the client's priority, renegotiate goals and time frame together.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Negotiating goals and formulating a contract; Reid, W. J., & Epstein, L. (1972). Task-Centered Casework. Columbia University Press, target problems, contract and task implementation

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## 104. INT-1526 (rev 1) · Best answer: A

In a telehealth employee assistance session, a client says the main goal is for their partner to stop drinking. The assessment shows the client is losing sleep and phoning the partner's employer to excuse missed shifts. The social worker and client are now setting goals. Which goal is **MOST** appropriate?

**A.** Client will let the partner handle their own work absences for 4 weeks

**B.** Partner will attend a mutual-help meeting each week by the second month

**C.** Client will get the partner to agree to an assessment by week 4

**D.** Client will accept that the partner's drinking is outside their control

#### Why each option is right or wrong

**A** **Correct.** Goals should target changes the client can make, be stated in observable terms and include a time frame (Hepworth et al.). The client cannot control the partner's drinking but can stop managing its consequences. Letting the partner handle their own work absences for 4 weeks is observable, feasible and within the client's power. It beats C, which sounds like the client's action but succeeds only if the partner agrees, so the client could do everything right and still fail the goal.

**B** **Incorrect** (trap: Misuses the technique). This is measurable, but it describes the partner's behavior. The partner is not the client and has agreed to no goal, so the client has no way to make it happen.

**C** **Incorrect** (trap: Misuses the technique). This is phrased as the client's action, but success depends on the partner's choice. A goal the client cannot reach alone sets up failure. Inviting an assessment could still be one step within a plan.

**D** **Incorrect** (trap: Good but not best). Accepting the limits of control is a worthy aim, but it names an inner state with no observable behavior or time frame, so progress cannot be tracked. It works better as a theme behind a measurable goal.

**Takeaway:** Set goals on changes within the client's own control; a goal that depends on another person's behavior is not feasible.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Negotiating goals and formulating a contract

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### 105. INT-1636 (rev 2) · Best answer: D

In an integrated primary care clinic, a client with moderate depression says, 'My license is suspended, so I can't get to work. That has to come first.' The social worker believes the depression should be treated before anything else. What is the **BEST** way to set the first goal?

**A.** Make treating the depression the first goal of the contract
**B.** Ask the client to rank the problems in order of importance
**C.** Work on the license and the depression as equal goals
**D.** Start with the steps to reinstate the license

**Why each option is right or wrong**

**A** **Incorrect** (trap: Overrides the client's choice). Putting the social worker's priority first overrides the client's stated view and risks a contract the client does not own. Depression can be monitored and addressed as the work proceeds unless risk requires otherwise.

**B** **Incorrect** (trap: Right action, wrong time). Ranking problems helps when priorities are unclear. This client has already said what comes first and why, so asking again delays goal setting and may signal that the answer was not heard.

**C** **Incorrect** (trap: Good but not best). Including both problems sounds thorough, but with moderate depression, splitting effort across two goals can overwhelm. Partializing starts with one manageable, client-chosen target and adds others later.

**D** **Correct.** Goals should reflect what the client wants to change; starting where the client is builds motivation and commitment to the contract (Hepworth et al.). The client has said why the license matters most, since without it there is no way to get to work, and the problem is concrete and solvable. Early success on a client-chosen goal can also lift mood, and mood can be monitored as the work proceeds. It beats C because tackling both at once spreads limited energy in a client with depression; partializing means choosing one starting point, and the client has named it.

**Takeaway:** Set the first goal around the client's own priority unless safety dictates otherwise; client-owned goals build motivation and commitment.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Negotiating goals and formulating a contract
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.15 Contracting and goal setting: partializing, measurable objectives · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 106. INT-1637 (rev 1) · Best answer: A

By telehealth, a parent says the main goal is to stop yelling at the children, which happens most evenings. The parent and the social worker are writing a measurable objective for the next 6 weeks. Which objective follows goal-setting guidelines?

**A.** Use a calm voice for evening directions on 5 of 7 days
**B.** Stop yelling at the children completely within 6 weeks
**C.** Yell less often at the children over the next 6 weeks
**D.** Become a calmer, more patient parent by the end of 6 weeks

**Why each option is right or wrong**

**A** **Correct.** Well-formed objectives are explicit and measurable, feasible, and stated in positive terms: they name what the client will do rather than what the client will stop doing (Hepworth et al.). A calm voice for evening directions on 5 of 7 days gives the parent a behavior to practice and a count to track. It beats B because B can be measured but names only the absence of a behavior and sets a perfection target for a habit that occurs most evenings, which invites early failure.

**B** **Incorrect** (trap: Misuses the technique). This objective can be measured, but it tells the parent only what not to do and expects perfection within 6 weeks from a nearly nightly pattern. Positive, attainable targets are easier to practice and sustain.

**C** **Incorrect** (trap: Misuses the technique). 'Less often' gives no amount, so neither the parent nor the social worker can tell whether the objective was met. A baseline and a target number would make it measurable.

**D** **Incorrect** (trap: Misuses the technique). Calm and patient are worthy aims, but they describe traits rather than observable behaviors. Without a specified action and amount, progress cannot be tracked.

**Takeaway:** Write objectives as positive, observable behaviors with a measurable, attainable target, naming what the client will do rather than stop doing.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Guidelines for selecting and defining goals: stated in positive terms
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.15 Contracting and goal setting: partializing, measurable objectives · **Qualifier** none · **Cognitive** application · **Difficulty** 2 of 3

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## 107. INT-1527 (rev 2) · Best answer: B

At a senior center, a client caring for a spouse with major neurocognitive disorder is using structured problem solving to tackle exhaustion. Together they defined the problem and listed options, and the client chose an adult day program 2 days a week. The client has now used it for 3 weeks. What should the social worker do **NEXT**?

**A.** Brainstorm more respite options in case the program falls short
**B.** Review with the client how well the program is meeting the goal
**C.** Begin structured problem solving on another stressor

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Returning to alternatives is correct when the chosen solution has not worked. Here no one has checked the results yet, so brainstorming now skips verification and may signal that the plan is expected to fail.

**B** **Correct.** Structured problem solving ends with implementation and verification: once the chosen solution has been tried, client and worker check the results against the original goal (D'Zurilla & Nezu, 2007). Three weeks of day program use is enough to judge whether the caregiver's exhaustion is easing. It beats A because generating more options is the right move only if the review shows the solution is falling short; doing it now skips evaluation and may undercut a plan that is working.

**C** **Incorrect** (trap: Right action, wrong time). Taking on the next problem is part of structured problem solving, and other stressors may well need attention. Moving on

before checking whether the day program has eased the exhaustion skips the verification step, so neither the client nor the worker learns whether the chosen solution worked.

**Takeaway:** In structured problem solving, after a solution is put in place, evaluate its results against the goal before generating new options.

**Sources:** D'Zurilla, T. J., & Nezu, A. M. (2007). Problem-Solving Therapy: A Positive Approach to Clinical Intervention (3rd ed.). Springer, rational problem-solving skills

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## 108. INT-1528 (rev 3) · Best answer: A

A social worker leads a problem-solving skills group at a clubhouse for adults with serious mental illness. Members are suggesting ways to get to a new job site after a bus route ended. One member responds to each idea as it is offered, saying, 'That won't work.' What should the social worker do **FIRST**?

**A.** Ask members to list every idea before judging any of them

**B.** Invite the critical member to offer an idea of their own

**C.** Ask the member to explain what makes each idea unworkable

#### Why each option is right or wrong

**A** **Correct.** When generating alternatives, structured problem solving defers judgment, because criticism cuts the number and variety of options (D'Zurilla & Nezu, 2007). The member's running critique is shutting ideas down, so the worker first restates the ground rule: list everything, then evaluate. Skills training for serious mental illness teaches the same order (Bellack et al., 2004). It beats C because weighing each idea's drawbacks belongs to the next step, decision making, and doing it now ends idea generation early.

**B** **Incorrect** (trap: Good but not best). Drawing the critic into generating ideas is a sound engagement move and may help. It leaves the group without the ground rule, though, so the next suggestion can still be shot down; restating 'list first, judge later' protects every member's ideas.

**C** **Incorrect** (trap: Right action, wrong time). Examining drawbacks is part of the decision-making step that follows. Inviting it now rewards the critique and stops idea generation. The member's concerns become useful once the list is complete.

**Takeaway:** When generating solutions, list all ideas before evaluating any; early criticism narrows the options.

**Sources:** D'Zurilla, T. J., & Nezu, A. M. (2007). Problem-Solving Therapy: A Positive Approach to Clinical Intervention (3rd ed.). Springer, rational problem-solving skills; Bellack, Mueser, Gingerich & Agresta (2004). Social Skills Training for Schizophrenia (2nd ed.). Guilford Press, problem-solving skills

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### 109. INT-1529 (rev 1) · Best answer: D

At a reentry program, a social worker and client sign an agreement to work on the two problems the client ranked highest, over 8 sessions. Each session opens with a review of progress on those problems. Each session closes with the client choosing an action step, rehearsing it, and planning for obstacles. Which approach is the social worker **MOST** likely using?

**A.** Solution-focused brief therapy
**B.** Problem-solving therapy
**C.** Behavioral activation
**D.** Task-centered practice

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). SFBT is also brief, but it builds on exceptions, scaling and the miracle question to construct a preferred future. It does not contract around ranked problems or rehearse obstacle plans for between-session steps.

**B** **Incorrect** (trap: Wrong model or concept). Problem-solving therapy is structured too, but its core is training a general skill set: problem orientation, defining problems, and generating and weighing options. Ranked target problems, a fixed agreement and rehearsed steps point to the task-centered model.

**C** **Incorrect** (trap: Wrong model or concept). Behavioral activation schedules rewarding and valued activities to lift depressed mood. The stem describes no mood target or activity monitoring; it describes client-ranked problems worked through contracted steps.

**D** **Correct.** Task-centered practice is brief and time-limited, works on a few target problems the client selects and ranks, uses an explicit contract, and moves forward through tasks planned and rehearsed in session, with attention to obstacles (Reid & Epstein, 1972). Every feature in the stem matches. It beats B because problem-solving therapy also follows structured steps but centers on teaching a general problem-solving method, not on contracting around ranked target problems with between-session tasks (D'Zurilla & Nezu, 2007).

**Takeaway:** Task-centered practice: brief, contracted work on a few client-ranked target problems through rehearsed between-session tasks.

**Sources:** Reid, W. J., & Epstein, L. (1972). Task-Centered Casework. Columbia University Press, target problems, contract and task implementation; D'Zurilla, T. J., & Nezu, A. M. (2007). Problem-Solving Therapy: A Positive Approach to Clinical Intervention (3rd ed.). Springer, rational problem-solving skills
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.16 Structured problem-solving approaches · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 3 of 3

### 110. INT-1638 (rev 3) · Best answer: A

At a senior center, an older adult in problem-solving therapy chose to ask a neighbor for rides to medical visits. At the next session, the client sighs, reports that the neighbor said no, and says, 'This method just doesn't work for me.' What should the social worker do **FIRST**?

**A.** Acknowledge the client's disappointment about the refusal
**B.** Arrange agency transportation for the client's visits
**C.** Revisit the earlier list of options with the client

**Why each option is right or wrong**

**A** **Correct.** A setback often brings a negative problem orientation, the sense that problems cannot be solved and effort is pointless, which problem-solving therapy treats as a target in its own right (Nezu, Nezu & D'Zurilla, 2013). Acknowledging the client's disappointment first shows understanding and eases the discouragement that would otherwise undercut the next attempt. It beats C because returning to the alternatives list is the right next step, but going straight there can feel like brushing past the letdown and may confirm the client's view that the method ignores how they feel.

**B** **Incorrect** (trap: Good but not best). Agency transportation could belong on the client's list of options, but booking it for the client skips the client's own problem solving. The therapy aims to build skills the client can use when the next problem arises.

**C** **Incorrect** (trap: Right action, wrong time). Returning to the alternatives list and choosing another option is how problem-solving therapy handles a failed solution (D'Zurilla & Goldfried, 1971), and it is the next step here. Jumping to it before acknowledging the client's discouragement risks pressing on while the client is ready to give up on the method.

**Takeaway:** When a chosen solution fails, acknowledge the client's discouragement first; then return to the alternatives list and try another option.

**Sources:** D'Zurilla, T. J., & Goldfried, M. R. (1971). Problem solving and behavior modification. Journal of Abnormal Psychology, 78(1), 107-126, whole article; Nezu, A. M., Nezu, C. M., & D'Zurilla, T. J. (2013). Problem-Solving Therapy: A Treatment Manual. Springer Publishing, Planful problem solving: solution implementation and verification

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### 111. INT-0109 (rev 1) · Best answer: B

A client in a primary care clinic says, 'Drinking is the only thing that helps me sleep. But my liver tests are getting worse, and that scares me.' The client has not decided whether to cut back. Which response by the social worker is **MOST** appropriate?

**A.** For your liver's sake, it's time to find another way to sleep.
**B.** Drinking gets you to sleep, and your liver results worry you.
**C.** Let's set a date to cut back on drinking, and I can help.

**Why each option is right or wrong**

**A** **Incorrect** (trap: Too directive). Telling the client what to do is the righting reflex: well-meant advice that tends to draw out more arguments for drinking. Advice can come later, with permission, using elicit-provide-elicit.

**B** **Correct.** The client voices both a reason to keep drinking and a reason to change, which marks the contemplation stage (Prochaska & DiClemente, 1983). The stage-matched response explores ambivalence, and a double-sided reflection does this by acknowledging the sustain talk and pairing it with the client's own change talk (SAMHSA TIP 35, Ch. 3). Hearing both sides in their own words helps the client weigh them. It beats C because setting a start date is a preparation-stage task, and pushing a plan before the client decides tends to provoke more sustain talk.

**C** **Incorrect** (trap: Right action, wrong time). Setting a date fits a client who has decided to change, which is the preparation stage. This client is still undecided, so moving to planning skips the work of resolving ambivalence.

**Takeaway:** For a contemplative client, reflect both sides of the ambivalence; save advice and action plans for clients who have decided to change.

**Sources:** SAMHSA TIP 35 (2019), Ch. 3, Motivational Interviewing as a Counseling Style: double-sided reflections; Prochaska, J. O., & DiClemente, C. C. (1983). Stages and processes of self-change of smoking: Toward an integrative model of change. Journal of Consulting and Clinical Psychology, 51(3), 390-395, contemplation stage **Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.17 Intervening with addiction · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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### 112. INT-1100 (rev 1) · Best answer: B

A client seeks help because their partner drinks heavily most nights and refuses to discuss treatment. The client feels safe at home and wants the partner to get help. A relative suggests gathering the family for a surprise confrontation. What is the **BEST** approach for the social worker to offer?

**A.** Help the client plan a family confrontation with clear consequences
**B.** Coach the client to reward sober behavior and ask at calm moments
**C.** Refer the client to Al-Anon to focus on their own coping

#### Why each option is right or wrong

**A** **Incorrect** (trap: Too directive). A surprise confrontation can feel decisive, but many families never follow through with one, and it engaged far fewer drinkers than CRAFT in head-to-head research. Confrontation also risks anger and withdrawal, so it is a weaker route to the client's goal.

**B** **Correct.** Community Reinforcement and Family Training (CRAFT) teaches a concerned family member to reward sober behavior, let the natural consequences of drinking occur, communicate positively, and suggest treatment when the drinker is most receptive. The client wants the partner in treatment and feels safe at home, which CRAFT requires. In a randomized trial, CRAFT engaged about two-thirds of drinkers in treatment, far more than a confrontational intervention or Al-Anon facilitation (Miller, Meyers & Tonigan, 1999). It beats C because Al-Anon supports the client's own coping but rarely moves the drinker into care, which is the client's stated aim.

**C** **Incorrect** (trap: Good but not best). Al-Anon offers real support, and family members' well-being improved in research, but it is not designed to bring the drinker into treatment. It fits a client whose main goal is their own coping rather than the partner's engagement.

**Takeaway:** To help a family member engage a treatment-refusing drinker, CRAFT's positive reinforcement outperforms confrontation and Al-Anon facilitation.

**Sources:** Miller, W. R., Meyers, R. J., & Tonigan, J. S. (1999). Engaging the unmotivated in treatment for alcohol problems: A comparison of three strategies for intervention through family members. Journal of Consulting and Clinical Psychology, 67(5), 688-697, whole article
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.17 Intervening with addiction · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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## 113. INT-1101 (rev 1) · Best answer: A

An outpatient client stopped drinking 4 months ago. In session, the client reports having three beers at a cousin's wedding last weekend and none since. The client says, 'I blew it. I'm just someone who can't stay sober, so why keep trying?' What should the social worker do **FIRST**?

**A.** Help the client see the slip as a lapse to learn from
**B.** Recommend moving the client to a more intensive level of care
**C.** Review the wedding situation to plan for future high-risk events

#### Why each option is right or wrong

**A** **Correct.** In relapse prevention, the abstinence violation effect is the guilt and self-blame after a slip, when a person concludes they are a failure who cannot change. That belief can turn a single lapse into a full relapse (Marlatt & Gordon, 1985). The client's words, 'someone who can't stay sober,' show this effect, so the first task is to reframe the drinking as a lapse to learn from. It beats C because reviewing the high-risk situation is the right next step, but a client who has decided trying is pointless will not engage in planning until that belief eases.

**B** **Incorrect** (trap: Hands off too soon). One brief lapse after 4 months of abstinence, with no drinking since, does not show that outpatient care has failed. Stepping up the level of care would fit if drinking resumed and continued despite a revised plan.

**C** **Incorrect** (trap: Right action, wrong time). Analyzing the wedding as a high-risk situation and planning coping responses is a core relapse prevention step. It works best after the client's all-or-nothing self-blame eases; otherwise the client may see no point in planning.

**Takeaway:** After a slip, address the abstinence violation effect first: frame the lapse as a learning event, then plan for the high-risk situation.

**Sources:** Marlatt, G. A., & Gordon, J. R. (Eds.). (1985). Relapse Prevention: Maintenance Strategies in the Treatment of Addictive Behaviors. Guilford Press, abstinence violation effect; lapse versus relapse
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.17 Intervening with addiction · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 114. INT-1102 (rev 3) · Best answer: B

A client has taken buprenorphine for 8 months and is stable at work and home. In a telehealth session, the client mentions that people at a mutual-help meeting call medication 'not real recovery' and says, 'Maybe I should just stop it next week.' What should the social worker do **FIRST**?

**A.** Accept the plan and schedule weekly check-ins during the stop
**B.** Ask what the client heard and how they now see the medication
**C.** Suggest a mutual-help group that welcomes members on medication
**D.** Help the client meet with the prescriber before any change

#### Why each option is right or wrong

**A** **Incorrect** (trap: Ignores risk). Respecting choice matters, but the client has voiced a 'maybe,' not a settled decision. Simply monitoring an abrupt stop leaves out both the client's own reasons and the medical guidance any stop would need; check-ins fit once a change is planned with the prescriber.

**B** **Correct.** The client's 'maybe' after criticism at a meeting signals ambivalence, and the worker does not yet know what the client heard, believes or wants. Motivational interviewing meets ambivalence by exploring it, so the client can voice their own reasons for and against staying on medication (Miller & Rollnick, 2023). Asking first also shows what misinformation to address and keeps the choice with the client (NASW Code 1.02). It beats D because a prescriber visit is the right step if the client still intends to stop, but arranging it before hearing the client out treats a passing doubt as a decision; with a week to go, there is time to explore now and link to the prescriber next.

**C** **Incorrect** (trap: Right action, wrong time). A group that accepts medication-assisted recovery may ease the pressure and is a good later step. Suggesting it now assumes the meeting is the whole story before the worker knows what the client heard or what they want.

**D** **Incorrect** (trap: Right action, wrong time). Medication decisions belong with the prescriber, and stopping buprenorphine abruptly risks withdrawal and overdose once tolerance drops; most people who stop medication return to opioid use (SAMHSA TIP 63, 2021). A prescriber visit is therefore the next step if the client still plans to stop. Arranging it first skips learning what the client thinks and wants.

**Takeaway:** When a client on MOUD voices doubts about continuing, explore their reasons first; link to the prescriber before any planned change.

**Sources:** Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, Evoking: exploring ambivalence; SAMHSA TIP 63 (2021), Part 1: Introduction to Medications for Opioid Use Disorder Treatment; NASW Code of Ethics (2021), 1.02

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### 115. INT-1103 (rev 1) · Best answer: C

In a primary care clinic, a client who has drunk about a fifth of liquor daily for years says they will quit alone this weekend. Last year, the client had a seizure two days after stopping suddenly. The client asks the social worker for tips to get through the weekend. What should the social worker do **FIRST**?

**A.** Plan coping strategies and supports to use over the weekend
**B.** Explore what is motivating the client to quit drinking now
**C.** Arrange medically supervised withdrawal before the client stops

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). The client asked for coping tips, and they will help, but they do not address the medical danger of stopping suddenly after a prior withdrawal seizure. Coping planning fits alongside or after medically managed withdrawal.

**B** **Incorrect** (trap: Good but not best). The client has already decided to quit this weekend, so exploring motivation is not the priority. Exploring reasons fits a client who is unsure about change; here the urgent issue is a safe way to stop.

**C** **Correct.** Safety comes first. A past withdrawal seizure is a major warning sign for complicated alcohol withdrawal, so stopping heavy daily drinking without medical care could again cause seizures or delirium (ASAM, 2020). The client's readiness to quit is a strength; the task is to make quitting safe by linking the client to medically supervised withdrawal management. It beats A because coping strategies matter but cannot prevent a medical complication; they belong in the plan once withdrawal care is in place.

**Takeaway:** A history of withdrawal seizures means stopping heavy drinking needs medical withdrawal management first; coping plans come after safety.

**Sources:** ASAM (2020). The ASAM Clinical Practice Guideline on Alcohol Withdrawal Management. Journal of Addiction Medicine, 14(3S), 1-72, Assessment and level of care: risk factors for complicated withdrawal
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.17 Intervening with addiction · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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### 116. INT-1104 (rev 1) · Best answer: A

Which treatment has the **MOST** consistent research support for helping adults stop using cocaine or methamphetamine?

**A.** Contingency management plus community reinforcement
**B.** Cognitive-behavioral relapse prevention therapy
**C.** Twelve-step facilitation with peer sponsorship
**D.** Supportive-expressive psychodynamic therapy

**Why each option is right or wrong**

- **A** **Correct.** Contingency management gives tangible rewards, such as vouchers or prize draws, for drug-free urine samples; the community reinforcement approach builds rewarding non-using activities. In a network meta-analysis of psychosocial treatments for cocaine and amphetamine use, this combination was the only one that raised abstinence compared with usual treatment, and it outperformed CBT and 12-step approaches (De Crescenzo et al., 2018). With no FDA-approved medication for stimulant use disorders (SAMHSA, 2020), it is the leading option. It beats B because CBT is widely used and helpful, but it was less effective on abstinence at the end of treatment.
- **B** **Incorrect** (trap: Wrong model or concept). CBT relapse prevention teaches coping skills for high-risk situations and is widely used. In the network meta-analysis, though, contingency management plus community reinforcement produced more abstinence than CBT at the end of treatment.
- **C** **Incorrect** (trap: Wrong model or concept). Twelve-step approaches offer peer support and help many people, but in the network meta-analysis they produced less abstinence than contingency management plus community reinforcement.
- **D** **Incorrect** (trap: Wrong model or concept). Supportive-expressive psychodynamic therapy has been tested with cocaine use, but the community reinforcement approach outperformed it at longest follow-up. It is not the best-supported option for stimulant abstinence.

**Takeaway:** For stimulant use disorders, contingency management, best with community reinforcement, has the strongest abstinence evidence; no medication is FDA-approved.

**Sources:** De Crescenzo, F., Ciabattini, M., D'Alo, G. L., De Giorgi, R., Del Giovane, C., Cassar, C., et al. (2018). Comparative efficacy and acceptability of psychosocial interventions for individuals with cocaine and amphetamine addiction: A systematic review and network meta-analysis. PLoS Medicine, 15(12), e1002715, whole article; SAMHSA (2020). Treatment of Stimulant Use Disorders. Evidence-Based Resource Guide Series (PEP20-06-01-001), medications: none FDA-approved

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.17 Intervening with addiction · **Qualifier** MOST · **Cognitive** recall · **Difficulty** 2 of 3

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## 117. INT-1176 (rev 3) · Best answer: A

In an emergency department, a social worker meets with a 52-year-old patient who fell on icy steps after a party. A screening questionnaire shows 5 or 6 drinks on most weekend nights, and brief follow-up questions point to at-risk drinking rather than an alcohol use disorder. What should the social worker do **NEXT**?

- **A.** Discuss how alcohol relates to the injury and advise cutting down
- **B.** Give the patient a handout on low-risk drinking limits
- **C.** Complete a full biopsychosocial assessment before discussing alcohol

**Why each option is right or wrong**

**A** **Correct.** SBIRT pairs screening with a brief intervention matched to risk. For at-risk drinking without signs of an alcohol use disorder, the clinician gives personal feedback, advises cutting down and asks for the patient's view, often in one short talk (NIAAA, 2005). Screening and brief follow-up questions place this patient in the at-risk range, and the injury opens a teachable moment. It beats C because SBIRT acts on the screening result now; a long assessment delays the conversation and may never happen in a busy emergency department.

**B** **Incorrect** (trap: Good but not best). Written information on low-risk limits is a useful add-on, and busy departments often lean on it. On its own, though, it skips the short conversation, with personal feedback, clear advice and the patient's own view, that makes a brief intervention work.

**C** **Incorrect** (trap: Right action, wrong time). The brief assessment SBIRT calls for is already done. A full biopsychosocial assessment fits when a disorder or referral is likely; here it delays the conversation while the injury makes the link to drinking easy to see.

**Takeaway:** After screening shows risky drinking without signs of a disorder, deliver a brief intervention now; save specialty referral for likely disorders.

**Sources:** NIAAA (2005). Helping Patients Who Drink Too Much: A Clinician's Guide, Steps 2-3: assess for disorder, then advise and assist (at-risk drinking)
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.17 Intervening with addiction · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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## 118. INT-1187 (rev 1) · Best answer: A

A client in early recovery from alcohol use disorder starts taking a new route home from work 'to beat traffic.' The route passes a liquor store the client used to visit daily. The next week, the client stops there 'just to buy a soda' and drinks again that night. Which relapse prevention concept **MOST** likely describes the route change?

**A.** An apparently irrelevant decision
**B.** The abstinence violation effect
**C.** Positive outcome expectancies
**D.** A lapse

**Why each option is right or wrong**

**A** **Correct.** In Marlatt's relapse prevention model, apparently irrelevant decisions are small, harmless-looking choices that move a person step by step toward a high-risk situation while hiding the link to using (Marlatt & Gordon, 1985). Choosing a route past the old liquor store 'to beat traffic' fits that pattern and set up the later drink. It beats D because a lapse is the first return to use itself, the drinking that night, not the earlier choice that led there.

**B** **Incorrect** (trap: Wrong model or concept). The abstinence violation effect is the guilt and 'I've blown it' thinking that can follow a lapse and turn it into a full relapse. It describes a reaction after drinking, not the earlier route choice.

**C** **Incorrect** (trap: Wrong model or concept). Positive outcome expectancies are beliefs that drinking will bring good effects, such as relaxation or confidence. The vignette shows a chain of small choices, not a stated belief about what alcohol will do.

**D** **Incorrect** (trap: Wrong model or concept). A lapse is a single slip back to use, here the drinking that night. The question asks about the route change, the earlier choice that set up the high-risk situation.

**Takeaway:** Apparently irrelevant decisions are small choices that quietly lead toward high-risk situations; relapse prevention teaches clients to spot them early.

**Sources:** Marlatt, G. A., & Gordon, J. R. (Eds.). (1985). Relapse Prevention: Maintenance Strategies in the Treatment of Addictive Behaviors. Guilford Press, Apparently irrelevant decisions; lapse versus relapse
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.17 Intervening with addiction · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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## 119. INT-1191 (rev 1) · Best answer: B

A client in outpatient treatment for alcohol use disorder wants peer support between sessions. After attending two 12-step meetings, the client says, 'I'm an atheist. All the talk about a higher power made me feel like I didn't belong.' What is the **MOST** appropriate response?

**A.** Encourage the client to keep attending and define a higher power freely
**B.** Share secular mutual-help options, such as SMART Recovery
**C.** Suggest relying on counseling sessions alone for now

#### Why each option is right or wrong

**A** **Incorrect** (trap: Overrides the client's choice). Many nonreligious members do find a personal meaning for a higher power, and the idea can be mentioned. Pushing continued attendance after the client reported feeling excluded overrides their stated values and risks losing peer support altogether.

**B** **Correct.** Recovery occurs through many pathways, and supports should fit the person's values and culture (SAMHSA, 2012). Secular mutual-help groups such as SMART Recovery offer peer support without spiritual language. The client wants support between sessions but felt excluded by the spiritual framing, so offering alternatives keeps that goal alive. It beats A because reframing a higher power works for some atheists, but pressing it after the client said it made them feel they did not belong risks dropout from peer support entirely.

**C** **Incorrect** (trap: Good but not best). Counseling alone is an acceptable choice if the client prefers it, but this client asked for support between sessions. Dropping that goal discards a resource the client wants when other options exist.

**Takeaway:** Match mutual-help referrals to the client's values; recovery has many pathways, including secular peer support groups.

**Sources:** SAMHSA (2012) Working Definition of Recovery, Guiding principles: recovery occurs via many pathways
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.17 Intervening with addiction · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 1 of 3

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### 120. INT-1534 (rev 2) · Best answer: B

A case manager on a community mental health team makes a home visit to a client with schizophrenia. The client says the neighbors are poisoning the tap water. Six sealed water jugs are stacked by the door, and the refrigerator is empty. Which progress-note entry **BEST** documents the visit?

**A.** Client appeared paranoid about the water and hoarded jugs; refrigerator empty, likely off medications
**B.** Client said neighbors are poisoning the tap water; six jugs by door; refrigerator empty
**C.** Client's fear that neighbors use poison seemed worse than last month; self-care looked poor

#### Why each option is right or wrong

**A** **Incorrect** (trap: Skips assessment). This labels the client and guesses at a cause no one confirmed. Medication status should be checked and recorded as fact, not assumed. An interpretation belongs in a marked assessment section, tied to observations.

**B** **Correct.** Records should be accurate and specific enough to support continuity of care (NASW Code 3.04(a), 3.04(b)). The best entry records what the client said and what the worker saw, without labels or guesses, so the next reader can judge the situation and follow up on the empty refrigerator. It beats C because 'seemed worse' and 'looked poor' are impressions with no observable facts behind them; a later reader cannot tell what changed or how serious it is (Kagle & Kopels, 2008).

**C** **Incorrect** (trap: Misuses the technique). This is an impression without observable facts. A reader cannot tell what the client said, what the worker saw, or how serious the food situation is. Specific description is what makes a note useful.

**Takeaway:** Document observable facts and the client's own words; labels and guesses without supporting observations make records inaccurate and less useful.

**Sources:** NASW Code of Ethics (2021), 3.04(a)-(b); Kagle, J. D., & Kopels, S. (2008). Social Work Records (3rd ed.). Waveland Press, content and language of records
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.18 Case management: recording, documentation, case presentation · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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### 121. INT-1535 (rev 2) · Best answer: D

In a SOAP progress note, which section holds the social worker's clinical interpretation of what the client reported and what the worker observed?

**A.** Subjective
**B.** Objective
**C.** Plan
**D.** Assessment

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). The Subjective section records the client's own statements and reported experience, often in quotes. The worker's interpretation of those statements belongs in Assessment, so readers can separate data from judgment.

**B** **Incorrect** (trap: Wrong model or concept). The Objective section holds observable facts such as appearance, behavior, test scores and collateral information. Interpretation goes beyond what was observed, so it belongs in Assessment.

**C** **Incorrect** (trap: Wrong model or concept). The Plan section lists next steps such as referrals, tasks and the next appointment. It follows from the assessment but does not contain the interpretation itself.

**D** **Correct.** SOAP notes, which grew out of Weed's problem-oriented record, separate four kinds of content: Subjective (the client's report), Objective (observed data), Assessment (the professional's interpretation of those data) and Plan (next steps) (Weed, 1968). The worker's clinical interpretation therefore goes in Assessment. It beats A because the interpretation draws on what the client reported, but the Subjective section holds the client's statements themselves, not the worker's judgment about them. Keeping data and judgment apart shows which conclusions rest on which facts (Kagle & Kopels, 2008).

**Takeaway:** SOAP: Subjective is the client's report, Objective is observed data, Assessment is the worker's interpretation, Plan is next steps.

**Sources:** Weed, L. L. (1968). Medical records that guide and teach. New England Journal of Medicine, 278(11), 593-600, problem-oriented progress notes; Kagle, J. D., & Kopels, S. (2008). Social Work Records (3rd ed.). Waveland Press, content and language of records

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.18 Case management: recording, documentation, case presentation · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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## 122. INT-1536 (rev 1) · Best answer: C

A client with schizophrenia has been hospitalized 4 times in the past year and is staying in a shelter. The client misses most monthly office visits with an outpatient case manager and stops medication between admissions. The client says they want to stay out of the hospital. What is the **MOST** appropriate step for the case manager?

**A.** Increase to weekly office visits with a reminder call before each

**B.** Ask the psychiatrist to consider a long-acting injectable medication

**C.** Refer the client to an assertive community treatment team

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misuses the technique). More frequent office visits keep the same office-based model the client has not used. This pattern calls for outreach where the client lives. Weekly visits fit a client who attends but needs more support.

**B** **Incorrect** (trap: Good but not best). Raising medication options with the prescriber is reasonable, but it addresses only adherence, and the choice belongs to the prescriber. Housing instability and missed visits would remain. The question can be revisited within the ACT team's work.

**C** **Correct.** Assertive community treatment is designed for adults with serious mental illness who cycle through hospitals, face unstable housing and are not engaged by office-based services (SAMHSA ACT EBP KIT, 2008). A multidisciplinary team shares the caseload, delivers most services where the client lives and offers round-the-clock coverage. Four admissions, shelter living and missed visits meet these indications, and the client's goal of staying out of the hospital fits. It beats B because a medication

change addresses one piece; ACT brings medication support, housing help and outreach together.

**Takeaway:** Adults with serious mental illness who cycle through hospitals and miss office-based care need assertive community treatment's team-based outreach.

**Sources:** SAMHSA (2008) Assertive Community Treatment (ACT) Evidence-Based Practices KIT, Getting Started with Evidence-Based Practices: who benefits from ACT
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.18 Case management: recording, documentation, case presentation · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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## 123. INT-1538 (rev 1) · Best answer: B

During a home visit for a job placement program, a client chats about weekend plans and mentions that a sibling who lives in another state is undocumented. The client's service plan centers on finding warehouse work. Which way of handling the sibling's status in the case record is **BEST**?

**A.** Record the sibling's status to keep the social history complete
**B.** Leave the sibling's status out of the case record
**C.** Ask the client's permission before writing the status down

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Completeness is not the standard; relevance is. A relative's legal status has nothing to do with finding warehouse work, and recording it adds risk to a third party without improving services.

**B** **Correct.** Documentation should protect privacy and include only information directly relevant to the delivery of services (NASW Code 3.04(c)). A sibling's immigration status, mentioned in passing by a client whose plan centers on job placement, does not bear on the services, and recording it could expose a third party to harm if records are later released. It beats C because asking permission treats the question as one of consent, but relevance decides what goes into the record; even with permission, unrelated third-party details do not belong there.

**C** **Incorrect** (trap: Good but not best). Consent matters for disclosure, but it does not make unrelated information relevant. The record holds what services require, and this detail fails that test whether or not the client agrees.

**Takeaway:** Record only information relevant to services; consent does not make unrelated details, especially about third parties, belong in the record.

**Sources:** NASW Code of Ethics (2021), 3.04(c)
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.18 Case management: recording, documentation, case presentation · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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### 124. INT-1641 (rev 2) · Best answer: C

A social worker providing case management helped a client with schizoaffective disorder schedule a first primary care visit for high blood pressure. The appointment was last Tuesday. The social worker has not heard from the client or the clinic since. What should the social worker do **NEXT**?

**A.** Record the primary care linkage as complete
**B.** Begin the dental referral that is next on the plan
**C.** Check with the client about whether the visit happened
**D.** Ask the clinic to send the notes from the visit

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). The visit was scheduled, but no one knows whether it took place. Recording the goal as met would make the record inaccurate (NASW Code 3.04(a)) and could leave high blood pressure untreated.

**B** **Incorrect** (trap: Right action, wrong time). Moving on to the next service assumes the first linkage worked. Confirming the primary care connection comes first, especially with an untreated medical condition.

**C** **Correct.** Monitoring is a core case management function: the social worker monitors whether linked services are actually delivered or accessed in a timely way and still meet the client's needs (NASW Standards for Social Work Case Management, 2013, Standard 6). Asking the client directly can reveal barriers, such as transportation, cost or worry about the visit, which the worker can then address. It beats D because clinic notes, even with a release, show only whether a visit occurred, not why it may have failed or what the client thought of it.

**D** **Incorrect** (trap: Good but not best). With the client's written consent, clinic records can help coordination. But going to the clinic first bypasses the client and will not explain a missed visit or the client's experience of care.

**Takeaway:** After linking a client to a service, monitor by confirming with the client that the connection happened and addressing any barriers.

**Sources:** NASW Standards for Social Work Case Management (2013), Standard 6: Service Planning, Implementation, and Monitoring; NASW Code of Ethics (2021), 3.04(a)
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.18 Case management: recording, documentation, case presentation · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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### 125. INT-1642 (rev 2) · Best answer: B

A client with schizophrenia lives in a supportive housing program. The landlord calls the social worker and says the client yelled at neighbors in the hallway on several nights this week. Staff have not seen this happen. How should the social worker record the call?

**A.** Record that the client has been aggressive toward neighbors
**B.** Record the landlord's report and name the landlord as its source
**C.** Leave out the landlord's secondhand report

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). This turns an unverified secondhand report into a staff conclusion and uses a label in place of the described behavior. Later readers could treat aggression as an established fact.

**B** **Correct.** Records should be accurate and include only information directly relevant to the delivery of services (NASW Code 3.04(a), (c)). A landlord's report bears directly on housing stability, so it belongs in the record, attributed to its source and described in behavioral terms, so readers can tell what staff observed from what others reported (Kagle & Kopels, 2008). It beats C because omitting a report that could threaten the client's housing removes information the team needs to plan, such as talking with the client and working to prevent an eviction.

**C** **Incorrect** (trap: Good but not best). Information does not need to be witnessed by staff to be relevant. Omitting a landlord's complaint leaves the team unaware of a threat to the client's housing that needs follow-up.

**Takeaway:** Record relevant third-party reports in behavioral terms and attribute them to their source, so observation and secondhand information stay distinct.

**Sources:** NASW Code of Ethics (2021), 3.04(a), (c); Kagle, J. D., & Kopels, S. (2008). Social Work Records (3rd ed.). Waveland Press, Content of records: sources of information

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.18 Case management: recording, documentation, case presentation · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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## 126. INT-1643 (rev 1) · Best answer: A

A social worker recommends that a client with schizophrenia join an intensive outpatient program after a recent rise in hearing voices. The client declines because the program hours conflict with a valued job. After going over the likely benefits and dangers, the social worker judges that the client understands them. Which entry **BEST** documents this decision?

**A.** The recommendation, risks discussed and the client's reasons

**B.** That the client was noncompliant with the treatment plan

**C.** A signed statement releasing the agency from responsibility

**D.** That the client declined the referral

**Why each option is right or wrong**

**A** **Correct.** Documenting an informed refusal means recording what was recommended, the risks and benefits discussed, the client's understanding, and the client's own reasons. Such a record shows that the client's self-determination was respected (NASW Code 1.02) and gives sufficient documentation for continuity of care (3.04(b)). It beats D because a bare note that the client declined leaves out what the next worker, or a reviewer, needs to understand the decision and revisit it if symptoms worsen.

**B** **Incorrect** (trap: Misuses the technique). 'Noncompliant' is a judgmental label that misdescribes a capable client's informed choice. It can bias future providers and says nothing about the reasons or the risks discussed.

**C** **Incorrect** (trap: Misreads law or ethics). A release form does not replace clear documentation or remove professional duties. Asking for one can feel punitive and damage trust with a client making a legitimate choice.

**D** **Incorrect** (trap: Good but not best). This is accurate but too thin. Without the recommendation, the risks discussed and the client's reasons, later readers cannot tell whether the refusal was informed or know what to watch for.

**Takeaway:** Document informed refusal fully: the recommendation, the risks and benefits discussed, the client's understanding and the client's own reasons for declining.

**Sources:** NASW Code of Ethics (2021), 1.02; NASW Code of Ethics (2021), 3.04(b)
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.18 Case management: recording, documentation, case presentation · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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### 127. INT-1058 (rev 2) · Best answer: A

In a supportive housing program, two roommates have argued for weeks over the shared kitchen, and each now demands that the other move out. The social worker meets with both, sets ground rules for the talk, and has each describe the conflict. What should the social worker do **NEXT**?

**A.** Ask each roommate what they need from the kitchen
**B.** Propose a schedule that splits kitchen hours evenly
**C.** Ask each roommate to propose a fair solution

#### Why each option is right or wrong

**A** **Correct.** Principled negotiation separates positions, the demands people arrive with, from interests, the needs beneath them. 'Move out' is a position; needs such as cooking without waiting or finding a clean counter are interests, and interests open up solutions both people can accept (Fisher et al., 2011). With ground rules set and both sides heard, uncovering interests is the next step. It beats B because an even split is a compromise offered before anyone knows what each roommate needs, so it may satisfy neither; options come after interests are clear.

**B** **Incorrect** (trap: Right action, wrong time). Generating options is part of resolving the conflict, but proposing one now skips learning what each roommate needs. An even split may miss the real issue, such as cleanliness, and a plan from the worker may not be owned by either party.

**C** **Incorrect** (trap: Right action, wrong time). Inviting each roommate's ideas builds ownership, and generating options is a real later step. Asked for solutions now, though, each is likely to restate a position ('the other one moves out'). Options work better once the needs beneath those demands are on the table.

**Takeaway:** In resolving conflict, move the parties from positions to underlying interests before generating or judging options.

**Sources:** Fisher, R., Ury, W., & Patton, B. (2011). Getting to Yes: Negotiating Agreement Without Giving In (3rd ed.). Penguin Books, focus on interests, not positions
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.19 Resolving conflict · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 3 of 3

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### 128. INT-1059 (rev 1) · Best answer: D

In the Thomas-Kilmann model of conflict, which mode combines high assertiveness with high cooperativeness?

**A.** Compromising
**B.** Accommodating
**C.** Competing
**D.** Collaborating

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Compromising is intermediate on both assertiveness and cooperativeness. Each side gives up something to reach a quick middle ground, so neither party's concerns are fully met.

**B** **Incorrect** (trap: Wrong model or concept). Accommodating is cooperative but unassertive: one party sets aside its own concerns to satisfy the other. It can protect a relationship but leaves the accommodating party's needs unmet.

**C** **Incorrect** (trap: Wrong model or concept). Competing is assertive but uncooperative: one party pursues its own concerns at the other's expense. It can fit an emergency that needs a fast, firm decision.

**D** **Correct.** Thomas and Kilmann map conflict-handling modes on two axes: assertiveness, pursuing one's own concerns, and cooperativeness, pursuing the other party's concerns. Collaborating is high on both: the parties work together toward a solution that fully meets each side's concerns, often by digging into underlying interests (Thomas & Kilmann, 1974). It beats A because compromising sits in the middle of both axes and gives each side part of what it wants, rather than a solution that satisfies both fully.

**Takeaway:** Collaborating is high in both assertiveness and cooperativeness; compromising is moderate on both and splits the difference.

**Sources:** Thomas, K. W., & Kilmann, R. H. (1974). Thomas-Kilmann Conflict Mode Instrument. Xicom, five conflict-handling modes
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.19 Resolving conflict · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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### 129. INT-1060 (rev 1) · Best answer: B

At a drop-in center for adults experiencing homelessness, two clients begin shouting about a missing backpack. One stands inches from the other with fists clenched, and other clients have stopped to watch. What should the social worker do **FIRST**?

**A.** Bring both to a table to give their sides
**B.** Separate them calmly and speak with each apart
**C.** Warn both that more shouting will get them barred

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Hearing both sides is how the worker will settle the dispute, but it belongs after tempers cool. Bringing two aroused people face to face, with one already standing over the other, invites escalation.

**B** **Correct.** A conflict cannot be resolved while the parties are highly aroused, and safety comes before problem solving. Calmly creating physical space, moving away from the audience, and speaking with each person separately lowers the risk of a fight and lets emotions settle; respecting personal space and avoiding provocation are core de-escalation steps (Richmond et al., 2012). Once both are calmer, the worker can bring them together about the backpack. It beats A because a joint conversation is the right later step, but holding it now keeps the face-to-face standoff going.

**C** **Incorrect** (trap: Too directive). Clear limits may be needed later, but a threat delivered first, in front of onlookers, can feel humiliating and provoking. It does nothing to lower arousal and may push one client to act.

**Takeaway:** With parties in heated conflict, create space and calm each person separately before any joint problem solving.

**Sources:** Richmond, J. S., et al. (2012). Verbal de-escalation of the agitated patient: Consensus statement of the American Association for Emergency Psychiatry Project BETA De-escalation Workgroup. Western Journal of Emergency Medicine, 13(1), 17-25, ten domains of verbal de-escalation: respect personal space; do not be provocative

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.19 Resolving conflict · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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## 130. INT-1157 (rev 2) · Best answer: C

A social worker at a family services agency is helping separated co-parents settle a dispute over their child's weekday schedule. In the second joint session, one parent agrees to each proposal within seconds whenever the other parent's voice rises and keeps glancing at the door. After the session, that parent asks whether anything said privately would be shared with the other parent. What should the social worker do **FIRST**?

**A.** Ask both parents in the next joint session whether either feels unsafe

**B.** Switch to separate-room sessions with each parent from now on

**C.** Meet alone with that parent to screen for abuse and fear

#### Why each option is right or wrong

**A** **Incorrect** (trap: Ignores risk). Asking about safety is the right instinct, but not with the other parent in the room. A frightened parent is unlikely to answer honestly in front of the person they fear, and the question itself may raise the risk. Abuse screening is done privately.

**B** **Incorrect** (trap: Right action, wrong time). Separate sessions are a recognized safeguard when abuse is present, and they may well follow. Chosen before screening, they could leave a parent in danger and miss that the process should be suspended entirely.

**C** **Correct.** Family mediation standards call for screening for domestic abuse, including control and intimidation, and continuing to assess throughout the process (AFCC Model Standards, Standard X). Instant agreement whenever the other parent's voice rises, glances at the door, and a question about privacy suggest fear that could make any agreement unsafe and not truly voluntary. A private screening tells the social worker whether to adapt the process or stop it. It beats B because separate sessions are one possible safeguard, but choosing a safeguard before learning what is happening may leave danger unaddressed.

**Takeaway:** Signs of fear or intimidation in mediation call for private abuse screening before deciding how, or whether, joint problem solving continues.

**Sources:** AFCC, ABA & ACR (2000). Model Standards of Practice for Family and Divorce Mediation, Standard X: domestic abuse

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## 131. INT-1105 (rev 2) · Best answer: C

During a video session, the social worker glances several times at case records on a second monitor. The client stops mid-sentence and says, 'You keep looking off to the side. I guess I'm pretty boring.' What is the social worker's **BEST** response?

**A.** Ask what leads the client to see themselves as boring

**B.** Explain that the notes help track the client's goals

**C.** Acknowledge reading notes and set them aside

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). The comment may reflect a long-standing belief worth exploring, but the client is responding to something that really happened. Asking about the belief before owning the glances implies the client misread the situation. It becomes a useful step after the acknowledgment.

**B** **Incorrect** (trap: Good but not best). Explaining why the notes were open is partly honest and may reassure the client. It justifies the behavior, though, rather than owning its effect, and it leaves the distraction on screen. The client needs the worker's attention back, not a reason it was elsewhere.

**C** **Correct.** Congruence means the social worker's words match what the client can see. The client accurately noticed the glances and linked them to feeling boring, so the honest response is to own the behavior, say briefly what it was, and remove the distraction (Rogers, 1957; Hepworth et al.). This confirms the client's observation and restores full attention, which repairs the moment. It beats A because exploring the client's belief about being boring is valuable, but doing it first treats an accurate observation as the client's distortion; it fits once the worker has owned the glances.

**Takeaway:** When a client accurately notices the worker's distracting behavior, acknowledge it honestly and correct it before exploring the client's interpretation.

**Sources:** Rogers, C. R. (1957). The necessary and sufficient conditions of therapeutic personality change. Journal of Consulting Psychology, 21(2), 95-103, congruence (genuineness) as a core condition; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Ch. 5: authenticity

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### 132. INT-1106 (rev 1) · Best answer: B

On a stroke rehabilitation unit, a 68-year-old patient has expressive aphasia. The patient follows conversation and nods appropriately but struggles to find words. The social worker needs the patient's preferences for discharge. The patient's adult child offers to answer for the patient. What is the **MOST** appropriate way for the social worker to proceed?

**A.** Ask the adult child to describe the patient's preferences
**B.** Use short, simple questions and allow ample time to answer
**C.** Finish the patient's sentences when the patient pauses
**D.** Give the patient written questions to answer later

**Why each option is right or wrong**

**A** **Incorrect** (trap: Overrides the client's choice). Relatives often know a great deal, but letting the adult child speak for a patient who understands, and can communicate with support, takes the decision away from the patient. Family input fits as a supplement, with the patient's agreement.

**B** **Correct.** Aphasia affects language, not the right to decide. Because the patient understands speech, the social worker communicates directly: short, uncomplicated sentences, fewer distractions, acceptance of gestures or pointing, and plenty of time to respond (NIDCD). This keeps the patient at the center of a discharge decision that is theirs to make. It beats A because the adult child can add information with the patient's agreement, but answering for a patient who can express preferences, given time, sidelines the patient (NASW Code 1.02).

**C** **Incorrect** (trap: Misuses the technique). Completing sentences seems helpful but puts words in the patient's mouth and cuts off the extra time people with aphasia need. Guidance for families advises allowing time and accepting any form of communication instead.

**D** **Incorrect** (trap: Good but not best). Written questions help some patients, but expressive aphasia often affects writing as well as speech, and deferring the conversation delays the patient's input on discharge. Writing key words during the talk is a better use of print.

**Takeaway:** With aphasia, communicate directly using short sentences, extra time and any form of expression; don't let others speak for a patient who understands.

**Sources:** NIDCD (National Institute on Deafness and Other Communication Disorders). Aphasia (NIH fact sheet), tips for communicating with a person with aphasia; NASW Code of Ethics (2021), 1.02
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### 133. INT-1107 (rev 1) · Best answer: A

In an integrated primary care clinic, a client with diabetes and depression has just agreed to a plan for daily walks and a mood log. While agreeing, the client speaks softly, looks at the floor, and rubs the back of their neck. What should the social worker do **NEXT**?

**A.** Ask which part of the plan might be hardest to do
**B.** Ask the client why they seem unsure about the plan
**C.** Give the client a written copy of the agreed plan

**Why each option is right or wrong**

**A** **Correct.** The client's soft voice, lowered gaze and neck rubbing do not match the spoken agreement. When words and nonverbal cues conflict, the nonverbal message often carries doubts the client has not voiced. A question that assumes some difficulty makes it easy to name barriers without seeming to back out (Hepworth et al.). It beats B because 'why' questions tend to sound like demands for justification and can make clients defensive, so the doubts are less likely to surface.

**B** **Incorrect** (trap: Misuses the technique). This option notices the mixed message, but 'why' questions tend to put clients on the defensive and ask them to justify their feelings. An open question about what might get in the way invites the same information more safely.

**C** **Incorrect** (trap: Right action, wrong time). A written copy supports follow-through, but handing it over now treats the agreement as settled when the client's nonverbal cues suggest doubts. It fits once barriers have been discussed and the plan adjusted.

**Takeaway:** When nonverbal cues contradict a client's verbal agreement, invite possible barriers with an open, nonjudgmental question before finalizing the plan.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Physical attending and nonverbal communication; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Avoiding counterproductive communication patterns: 'why' and leading questions

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## 134. INT-1108 (rev 1) · Best answer: C

In Carl Rogers' person-centered approach, which term describes a helper whose outward responses fit what the helper is actually experiencing?

**A.** Unconditional positive regard
**B.** Accurate empathy
**C.** Congruence
**D.** Immediacy

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Unconditional positive regard is warm acceptance of the client as a person, without conditions of worth. It describes the helper's attitude toward the client, not whether the helper's expression fits their own inner experience.

**B** **Incorrect** (trap: Wrong model or concept). Accurate empathy is the helper's ability to sense the client's inner world and communicate that understanding. It centers on understanding the client, while the stem describes the helper being genuine about their own experience.

**C** **Correct.** Congruence, also called genuineness, is the helper's state of being integrated and authentic, so that what the helper communicates fits what the helper actually feels and thinks. Rogers (1957) named it one of the core conditions for therapeutic change, alongside unconditional positive regard and empathic understanding. It beats D because immediacy is a specific skill of discussing what is happening between client

and worker in the moment; congruence is the broader quality of being genuine in every response.

**D** **Incorrect** (trap: Wrong model or concept). Immediacy is a skill of discussing the here-and-now relationship between client and worker, such as tension in the session. It draws on genuineness, but it is a specific technique, not one of Rogers' core conditions.

**Takeaway:** Rogers' core conditions are congruence (genuineness), unconditional positive regard and empathic understanding, as perceived by the client.

**Sources:** Rogers, C. R. (1957). The necessary and sufficient conditions of therapeutic personality change. Journal of Consulting Psychology, 21(2), 95-103, core conditions: congruence

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## 135. INT-1178 (rev 2) · Best answer: C

During a first meeting at a community agency, a social worker leans forward in an open posture to show interest. Each time the social worker leans in, the client slides back in the chair, folds their arms, and gives shorter answers. How should the social worker respond?

**A.** Shift to less personal questions for now

**B.** Point out the folded arms and ask what they mean

**C.** Sit back and allow more personal space

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Lighter topics can ease a guarded client, but this client pulls back each time the worker leans in, not when a topic turns personal. Changing the questions misreads the cue and leaves the closeness that feels intrusive in place.

**B** **Incorrect** (trap: Too directive). Naming nonverbal behavior can be useful later in a relationship. In a first meeting it can feel like being watched and judged, and it asks the client to explain a reaction the worker can simply adjust to.

**C** **Correct.** Attending guidelines such as Egan's SOLER, which includes leaning toward the client, are meant to be used flexibly and adjusted to each client's comfort and culture (Egan, 2014). Here the client pulls back, closes up and shortens answers each time the worker leans in, a clear sign that the closeness feels intrusive. Matching the client's preferred distance shows respect without words. It beats B because spotlighting the client's body language in a first meeting can feel exposing and invites defensiveness before trust exists.

**Takeaway:** Attending behaviors serve the client's comfort; when a client withdraws from closeness, adjust posture and distance instead of persisting or interpreting.

**Sources:** Egan, G. (2014). The Skilled Helper (10th ed.). Brooks/Cole, Cengage Learning, Attending: SOLER as flexible guidelines

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### 136. INT-1183 (rev 1) · Best answer: B

Which term describes the vocal qualities that accompany spoken words, such as tone, pitch, pace and volume?

**A.** Kinesics
**B.** Paralanguage
**C.** Proxemics
**D.** Haptics

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Kinesics is the study of body movement, including gestures, posture and facial expression. It is the channel lost on a phone call; vocal qualities remain available there.

**B** **Correct.** Paralanguage, or vocal cues, covers how something is said rather than the words themselves: tone, pitch, rate, volume, pauses and sighs (Knapp, Hall & Horgan, 2014). It matters on the phone and in person, because a flat or rushed voice can contradict a client's words. It beats A because kinesics covers body movement, gesture, posture and facial expression, which are seen rather than heard.

**C** **Incorrect** (trap: Wrong model or concept). Proxemics concerns the use of space and interpersonal distance, such as how close people sit or stand. It says nothing about how the voice sounds.

**D** **Incorrect** (trap: Wrong model or concept). Haptics refers to touch as communication, such as a handshake or a hand on the shoulder. It is a separate nonverbal channel from the voice.

**Takeaway:** Nonverbal channels: paralanguage is the voice (tone, pace, volume), kinesics the body, proxemics space, haptics touch.

**Sources:** Knapp, M. L., Hall, J. A., & Horgan, T. G. (2014). Nonverbal Communication in Human Interaction (8th ed.). Wadsworth, Cengage Learning, Vocal cues; body movement; space; touch
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### 137. INT-1029 (rev 1) · Best answer: B

A probation officer sends a client to anger management after an arrest for shoving a partner. The client says the partner 'pushes my buttons.' The client also checks the partner's phone each night and controls all the household money. Coworkers describe the client as calm and easygoing. Which plan is **MOST** appropriate?

**A.** Enroll the client in a general anger management group
**B.** Refer the client to a partner-abuse intervention program
**C.** Offer joint sessions with the partner to improve communication

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). This matches the referral's wording, which makes it tempting. But the stem describes control aimed at the partner, not anger across settings, and anger-control framing can reinforce the client's view that the partner provokes the violence (Gondolf & Russell, 1986).

**B** **Correct.** Intimate partner violence is a pattern of coercive behavior used to gain power and control, not mainly a failure to regulate anger (Healey, Smith & O'Sullivan, 1998). Nightly phone checks, control of all the money, and calm behavior with coworkers point to control aimed at the partner. A partner-abuse intervention program targets those tactics and the client's accountability. It beats A because general anger management treats the violence as an anger problem that the partner 'provokes,' which fits the client's blaming account and leaves the controlling behavior untouched.

**C** **Incorrect** (trap: Ignores risk). Joint sessions can endanger a person who is being abused, because what they say may be punished later, and they imply shared responsibility for the violence (SAMHSA TIP 25). Conjoint work is not a first response to partner violence.

**Takeaway:** Coercive control and partner violence call for a partner-abuse intervention program, not general anger management or couples sessions.

**Sources:** Healey, K., Smith, C., & O'Sullivan, C. (1998). Batterer Intervention: Program Approaches and Criminal Justice Strategies. National Institute of Justice (NCJ 168638), Ch. 1, defining domestic violence; Ch. 2, couples counseling; Gondolf, E. W., & Russell, D. (1986). The case against anger control treatment programs for batterers. Response, 9(3), 2-5, whole article; SAMHSA TIP 25 (1997), Ch. 2: Survivors of Domestic Violence: An Overview (safety; cautions on couples therapy)

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### 138. INT-1030 (rev 1) · Best answer: C

A teacher refers a 10-year-old to the school social worker for anger management. The student has thrown books and torn up worksheets several times this month. The teacher's log shows that most outbursts happen when the class reads aloud, and the student's reading grades have dropped all year. What should the social worker do **FIRST**?

**A.** Enroll the student in a small anger management skills group
**B.** Teach the student a calm-down routine to use during reading
**C.** Request an evaluation of the student's reading skills
**D.** Set up a daily reward chart for calm behavior

#### Why each option is right or wrong

**A** **Incorrect** (trap: Skips assessment). A skills group assumes the problem is poor anger regulation. The pattern points to a reading difficulty that sets off the outbursts, so starting a group skips the assessment that would change the plan. A group may help once the cause is addressed.

**B** **Incorrect** (trap: Right action, wrong time). A calm-down routine is a reasonable support and could run alongside other steps. As the first step, though, it manages the outburst while the likely cause, struggling to read in front of classmates, goes unexamined.

**C** **Correct.** Anger work starts by identifying the events that trigger the anger (Reilly & Shopshire, 2019). The log ties the outbursts to reading aloud, and falling reading grades suggest an unmet learning need; the outbursts may help the student escape a task they cannot do in front of peers. Requesting a reading evaluation, which the school can initiate, targets the likely cause (IDEA, 34 CFR 300.301). It beats B because

a calm-down routine is useful and may be added, but alone it treats the outburst while the reading problem keeps producing it.

**D** **Incorrect** (trap: Good but not best). Rewards for calm days can shape behavior, but they don't address why reading time sets off the outbursts. A student who cannot do the task may keep failing to earn the reward, which adds frustration.

**Takeaway:** Before teaching anger control, look at what triggers the anger; an unmet need, such as a learning problem, may be driving it.

**Sources:** Reilly, P. M., & Shopshire, M. S. (2019). Anger Management for Substance Use Disorder and Mental Health Clients: A Cognitive-Behavioral Therapy Manual. SAMHSA (PEP19-02-01-001), Session 2: events and cues; Individuals with Disabilities Education Act regulations, 34 CFR 300.301(b)
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### 139. INT-1031 (rev 4) · Best answer: A

A client in an outpatient anger management group has started taking time-outs during arguments by walking out of the room. The client reports that the adult sibling they live with follows them, shouting, 'Don't walk away from me,' and the argument gets worse. The client says the sibling would be willing to talk about it at a calm time. How can the social worker **BEST** help the client handle these arguments?

**A.** Agree on a time-out signal and return plan with the sibling
**B.** Announce a return time to the sibling while walking out
**C.** Leave without a word to the sibling until fully calm

#### Why each option is right or wrong

**A** **Correct.** A formal time-out works best when it is agreed on in advance with the people involved: a signal that means 'I need a break,' how long the break will last, and a commitment to come back and finish or reschedule the talk (Reilly & Shopshire, 2019). The sibling reads the walk-out as being shut out, so they pursue and the argument escalates. Planning the time-out together removes that threat. It beats B because a return time announced mid-argument is a promise the sibling never agreed to, so the exit can still feel like being shut out; planning the signal and return together ahead of time removes that threat.

**B** **Incorrect** (trap: Misuses the technique). Naming a return time is better than a silent exit and moves toward a real time-out. Announced in the heat of the argument, though, it is a surprise the sibling has not agreed to, so the pursuit may continue. Time-outs work best when the signal and return are planned with the other person in advance.

**C** **Incorrect** (trap: Misuses the technique). Leaving can be part of a time-out, but an unexplained exit is what already sets off the sibling's pursuit. Without a signal and a plan to return, the break looks like shutting the sibling out.

**Takeaway:** Time-outs work best when agreed on in advance: a signal, a set break, and a commitment to return to the discussion.

**Sources:** Reilly, P. M., & Shopshire, M. S. (2019). Anger Management for Substance Use Disorder and Mental Health Clients: A Cognitive-Behavioral Therapy Manual. SAMHSA (PEP19-02-01-001), Session 3: anger control plans (formal and informal timeouts)
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### 140. INT-1137 (rev 1) · Best answer: B

A client asks an employee assistance social worker for help with anger. For 3 months, a coworker has made slurs about the client's background, and two other staff members have heard them. The client has not acted aggressively but lies awake furious and wants the remarks to stop. How should the social worker **BEST** help with the anger?

**A.** Teach slow breathing for the moments the coworker speaks
**B.** Help the client plan an assertive report through work channels
**C.** Help the client consider other reasons for the coworker's remarks

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Slow breathing is a sound arousal-control skill and could be added. On its own, it treats the client's reaction as the whole problem while repeated, witnessed slurs continue, and the client has said the goal is to make them stop.

**B** **Correct.** Anger can be a moral response to injustice, and anger management aims to express it assertively rather than aggressively, not to erase it (Reilly & Shopshire, 2019). The slurs are real, repeated, and witnessed, and the client wants them to stop, so the anger points to a problem that action can change. Planning an assertive report, with the witnesses as support, turns the anger into effective action. It beats A because breathing eases arousal in the moment but leaves the harassment, the source of the anger, untouched.

**C** **Incorrect** (trap: Misuses the technique). Looking for other explanations helps when anger rests on a hostile reading of an ambiguous event. These remarks are slurs that others also heard, so questioning the client's reading would invalidate an accurate perception.

**Takeaway:** When anger responds to real, ongoing mistreatment, help the client act on it assertively instead of only calming it or questioning the perception.

**Sources:** Reilly, P. M., & Shopshire, M. S. (2019). Anger Management for Substance Use Disorder and Mental Health Clients: A Cognitive-Behavioral Therapy Manual. SAMHSA (PEP19-02-01-001), anger as a normal emotion; Sessions 7 and 8: assertiveness training and the conflict resolution model
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### 141. INT-0004 (rev 1) · Best answer: B

In family sessions, whenever the parents try to set a curfew, their 14-year-old and one parent join forces against the other parent, and the discussion collapses. Which intervention is **MOST** consistent with structural family therapy?

**A.** Construct a three-generation genogram with the family
**B.** Coach the parents to decide the curfew together in session
**C.** Direct the parents to argue about the curfew each evening at home

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). A three-generation genogram is a Bowen technique for mapping multigenerational patterns. It can be informative, but it is assessment from a different model rather than a structural intervention on the coalition.

**B** **Correct.** Structural family therapy sees the problem as a structure in which a cross-generational coalition, here one parent allied with the adolescent, undercuts the parental subsystem. Coaching the parents to work out the curfew together in session, while blocking the adolescent from joining in, is an enactment with boundary making. It strengthens the parental hierarchy (Minuchin, 1974; Minuchin & Fishman, 1981). It beats A because a genogram belongs to Bowen family systems therapy, which tracks multigenerational patterns and differentiation rather than restructuring interactions in the room.

**C** **Incorrect** (trap: Wrong model or concept). Prescribing the symptom is a paradoxical directive from strategic family therapy, associated with Haley and the MRI group. It does not address the family structure that a structural therapist would target.

**Takeaway:** Structural family therapy uses enactment and boundary making to strengthen the parental subsystem and break cross-generational coalitions.

**Sources:** Minuchin, S. (1974). Families and Family Therapy. Harvard University Press, subsystems, boundaries and coalitions; Minuchin, S., & Fishman, H. C. (1981). Family Therapy Techniques. Harvard University Press, enactment; boundary making

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### 142. INT-0106 (rev 1) · Best answer: A

Parents seek help for their teen's school refusal. The social worker meets with the parents alone, stays calm while they argue, asks each parent what they think rather than feel about the conflict, and avoids siding with either one. Which family therapy model is the social worker **MOST** likely using?

**A.** Bowen family systems therapy
**B.** Structural family therapy
**C.** Strategic family therapy
**D.** Experiential family therapy

#### Why each option is right or wrong

**A** **Correct.** Bowen family systems therapy aims to raise differentiation of self, the ability to think clearly while staying emotionally connected (Bowen, 1978; Kerr & Bowen, 1988). The therapist stays calm and detriangulated, asks thinking-focused process questions, and often works with the parents without the symptomatic child, whose problem is seen as part of the parents' anxiety. Every cue fits. It beats B because a structural therapist would join the whole family and actively restructure interactions, sometimes taking sides on purpose, rather than stay neutral and reflective.

**B** **Incorrect** (trap: Wrong model or concept). Structural therapy works with the whole family in the room, using enactment, boundary making and unbalancing, in which the therapist may side with one member on purpose. Neutral, thinking-focused questions with the parents alone point elsewhere.

**C** **Incorrect** (trap: Wrong model or concept). Strategic therapy targets the sequence that maintains the symptom, often through directives or paradoxical tasks. This social worker gives no directive and focuses on how each parent thinks.

**D** **Incorrect** (trap: Wrong model or concept). Experiential therapy, such as Satir's, emphasizes emotional expression, congruent communication and the therapist's warmth. Asking for thoughts rather than feelings runs opposite to that emphasis.

**Takeaway:** A calm, detriangulated stance with thinking-focused process questions signals Bowen therapy; active restructuring signals structural; directives signal strategic.

**Sources:** Bowen, M. (1978). Family Therapy in Clinical Practice. Jason Aronson, differentiation of self; triangles; Kerr, M. E., & Bowen, M. (1988). Family Evaluation. W. W. Norton, therapist's role: detriangulation
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## 143. INT-1040 (rev 1) · Best answer: C

A family is referred for in-home therapy after a 15-year-old's school suspension. At the first visit, a parent sits with arms crossed and says, 'The last provider told us everything we were doing wrong.' The social worker uses structural family therapy. What should the social worker do **FIRST**?

**A.** Ask the family to act out a recent argument in session
**B.** Seat the parents together and the teen across the room
**C.** Learn the family's routines and language and follow their lead

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). Enactment, having the family show a problem interaction in the session, is the signature structural technique. It works once the social worker has joined the family; here it would come too early.

**B** **Incorrect** (trap: Right action, wrong time). Changing seating is a way to mark boundaries and hierarchy, a later structural move. Rearranging a guarded family at the first visit, before joining, risks looking like another provider taking over.

**C** **Correct.** In structural family therapy, joining comes first: the social worker accommodates to the family's style, tracks their story, and uses their language to build the alliance that later challenges require (Minuchin & Fishman, 1981). The parent's guarded stance and bad experience with a critical provider make joining essential before any restructuring. It beats A because enactment is a core structural technique, but asking a wary family to perform a conflict before the worker has standing with them is likely to deepen their defensiveness.

**Takeaway:** Join first: accommodate to and track a guarded family before challenging how it is organized.

**Sources:** Minuchin, S., & Fishman, H. C. (1981). Family Therapy Techniques. Harvard University Press, joining
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### 144. INT-1041 (rev 1) · Best answer: A

Parents of a 16-year-old report that the teen has stopped doing homework. Every evening they remind, lecture, and check the teen's work, and each week the teen does less. Which intervention is **MOST** consistent with the Mental Research Institute (MRI) brief therapy model?

**A.** Help the parents stop the nightly reminders and try something new
**B.** Build a three-generation genogram of the family's school achievement
**C.** Have the parents and teen enact a homework argument in session
**D.** Ask the teen when homework gets done without any reminders

**Why each option is right or wrong**

**A** **Correct.** The MRI model holds that problems persist because of the attempted solution: the more the family does what isn't working, the more the problem grows (Watzlawick, Weakland & Fisch, 1974). Nightly reminding, lecturing, and checking are followed by less homework, so the parents' solution has become part of the problem. Interrupting it and trying a different response is the MRI move. It beats D because asking about exceptions is a solution-focused technique; it looks for times the problem is absent rather than changing the sequence that keeps it going.

**B** **Incorrect** (trap: Wrong model or concept). A multigenerational genogram is a Bowen tool for tracing patterns across generations. MRI brief therapy focuses on the present interaction that keeps the problem going, not on family history.

**C** **Incorrect** (trap: Wrong model or concept). Enactment belongs to structural family therapy, which uses in-session interaction to restructure hierarchy and boundaries. MRI work targets the attempted solution itself rather than staging the conflict.

**D** **Incorrect** (trap: Wrong model or concept). Exception questions come from solution-focused brief therapy, which grew partly out of MRI work. They search for times the problem is absent instead of interrupting the parents' attempted solution.

**Takeaway:** MRI brief therapy targets the attempted solution: when what the family keeps doing maintains the problem, interrupt it and try something different.

**Sources:** Watzlawick, P., Weakland, J., & Fisch, R. (1974). Change: Principles of Problem Formation and Problem Resolution. W. W. Norton, attempted solutions; more of the same
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### 145. INT-1042 (rev 2) · Best answer: B

In a family session about a 12-year-old who refuses to go to school, the social worker asks the older sibling, 'When your grandparent and your parent argue about getting to school, who steps in first? Who seems most worried afterward?' Which technique is the social worker **MOST** likely using?

**A.** Process questioning
**B.** Circular questioning
**C.** Tracking
**D.** Relative influence questioning

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Bowen process questions invite a family member to reflect calmly on their own thinking and part in the pattern. Here the sibling is asked to report on and compare other members' reactions, which is the Milan approach.

**B** **Correct.** Circular questioning, from the Milan systemic team, asks one family member about relationships and differences among others, such as who reacts first or who is most upset (Selvini Palazzoli et al., 1980). Asking the sibling to describe the grandparent-parent disagreement and rank reactions brings out the family's relational patterns around the symptom. It beats A because Bowen process questions ask a person to think about their own reactions and role, to lower emotional reactivity, rather than to report on how others relate.

**C** **Incorrect** (trap: Wrong model or concept). Tracking is a structural joining skill: following the content of what the family says and encouraging them to go on. It does not ask one member to compare how others react.

**D** **Incorrect** (trap: Wrong model or concept). Relative influence questions, from narrative therapy, map how the problem affects people and how people affect the problem. These questions compare family members' reactions to one another instead.

**Takeaway:** Milan circular questions ask one member about differences and relationships among others, revealing the family's relational patterns.

**Sources:** Selvini Palazzoli, M., Boscolo, L., Cecchin, G., & Prata, G. (1980). Hypothesizing-circularity-neutrality: Three guidelines for the conductor of the session. Family Process, 19(1), 3-12

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## 146. INT-1043 (rev 1) · Best answer: B

During a family session about a teen's recent arrest, one parent speaks in a flat, even tone. The parent cites studies on teen brain growth and says feelings 'only cloud the issue.' According to Satir, which communication stance is this parent **MOST** likely showing?

**A.** Blaming
**B.** Super-reasonable
**C.** Placating
**D.** Irrelevant

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Blaming protects the self by attacking or finding fault with others. This parent is detached and impersonal, focusing on data rather than accusing the teen or anyone else.

**B** **Correct.** Satir described four stress stances, each discounting something: placating discounts the self, blaming discounts others, super-reasonable (called computing in her early work) discounts both self and others and keeps only facts and rules, and irrelevant discounts self, others, and context. Congruent communication is the goal (Satir, 1972). Citing research in a flat tone and dismissing feelings is the super-reasonable stance. It beats A because blaming involves fault-finding and attack, while this parent stays detached and impersonal.

**C** **Incorrect** (trap: Wrong model or concept). Placating gives up the self to keep others happy, through agreeing and apologizing. This parent is not seeking approval; they set feelings aside in favor of facts.

**D** **Incorrect** (trap: Wrong model or concept). The irrelevant stance distracts from the topic altogether by changing the subject or joking. This parent stays on topic but strips out feelings, which fits a different stance.

**Takeaway:** Satir's super-reasonable stance relies on facts and logic while discounting the feelings of self and others; congruence is the healthy goal.

**Sources:** Satir, V. (1972). Peoplemaking. Science and Behavior Books, communication stances under stress
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### 147. INT-1044 (rev 2) · Best answer: C

A 38-year-old client moved across the country 10 years ago and has had almost no contact with their family since. The client reports a series of intense, short romantic relationships and wants help with them. The social worker uses Bowen family systems therapy. Which intervention is **BEST**?

**A.** Invite the parents to a session to confront them about the past
**B.** Explore the pattern in the client's romantic relationships
**C.** Coach the client to plan brief visits and stay calm during them

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Confronting parents about the past invites the high emotion and blame that Bowen therapy works to lower. The model favors calm, one-to-one contact in which the client observes and manages their own reactions.

**B** **Incorrect** (trap: Wrong model or concept). Looking at the romantic pattern is a natural start, and it is what the client asked about. Bowen theory, though, places its source in unresolved attachment to the family of origin, so working only on current partners leaves the cutoff that drives the pattern untouched.

**C** **Correct.** Bowen theory treats emotional cutoff, managing unresolved family issues through distance, as a sign of low differentiation; the issues stay dormant and often resurface in other close relationships (Bowen, 1978). The client's intense, short relationships fit that pattern. Bowen-oriented work coaches the client to re-establish contact and practice staying calm and thoughtful, speaking from an I-position, when old reactions arise. It beats A because confrontation would heighten emotional reactivity, the opposite of the calm, differentiated contact the model seeks.

**Takeaway:** Bowen therapy views emotional cutoff as unresolved attachment; coach calm, differentiated reconnection rather than confrontation or continued distance.

**Sources:** Bowen, M. (1978). Family Therapy in Clinical Practice. Jason Aronson, emotional cutoff; differentiation of self
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### 148. INT-1045 (rev 2) · Best answer: C

On an inpatient unit, a 24-year-old patient with schizophrenia is in their second admission this year and will return home to live with their parents. The parents describe frequent criticism and say, 'We watch everything they do.' The patient agrees to family involvement. Which family intervention is **BEST** for reducing the risk of relapse?

**A.** Give the parents one education session before discharge
**B.** Refer the patient's parents to a family peer support group
**C.** Start months-long psychoeducation with the parents and patient
**D.** Coach the parents to examine patterns in their own families

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Brief education is better than none and often happens before discharge. But relapse reduction comes from multi-session programs that build skills over months, not from one meeting.

**B** **Incorrect** (trap: Good but not best). Family peer support groups ease caregiver burden and isolation and can complement treatment. They are not built to change the criticism and close monitoring that drive relapse, and they leave the patient out; structured psychoeducation with the patient included targets those patterns directly.

**C** **Correct.** Family psychoeducation for schizophrenia combines illness education, communication and problem-solving training, crisis planning, and support, usually over 9 months or more (Dixon, Adams & Lucksted, 2000). Meta-analyses show it cuts relapse by about half compared with usual care (McFarlane, 2016). Frequent criticism and intrusive monitoring are signs of high expressed emotion, which predicts relapse, and this patient has already been readmitted. It beats A because a single session can inform the parents but cannot build the skills that lower relapse over time.

**D** **Incorrect** (trap: Wrong model or concept). Examining multigenerational patterns is a Bowen approach. It does not target the current criticism and intrusiveness, or teach the skills that family psychoeducation uses to reduce relapse.

**Takeaway:** For schizophrenia with high expressed emotion at home, multi-month family psychoeducation reduces relapse more than brief education does.

**Sources:** McFarlane, W. R. (2016). Family interventions for schizophrenia and the psychoses: A review. Family Process, 55(3), 460-482, whole article; Dixon, L., Adams, C., & Lucksted, A. (2000). Update on family psychoeducation for schizophrenia. Schizophrenia Bulletin, 26(1), 5-20, whole article
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### 149. INT-1046 (rev 2) · Best answer: C

A 15-year-old on probation has two new arrests, skips school, and spends time with peers who steal cars. Six months of weekly clinic-based family therapy have not changed the pattern. The probation officer raises residential placement, and the family wants the teen to stay home. What should the social worker do **NEXT**?

**A.** Recommend placement in a residential treatment center
**B.** Add school-based mentoring alongside the current family therapy
**C.** Refer the family to a multisystemic therapy program

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Residential care may be needed if intensive community treatment fails or safety requires it. Moving there now skips a less restrictive, evidence-based option that keeps the teen at home, as the family wants.

**B** **Incorrect** (trap: Good but not best). Mentoring can reach the school and peer side of the problem while the teen stays home, so it is tempting. It is a piecemeal add-on, though, to clinic-based therapy that has not changed the pattern in six months; an integrated, intensive model works on family, peers and school together.

**C** **Correct.** Multisystemic therapy is an intensive, home- and community-based family treatment for serious antisocial behavior. It works through the caregivers to change the teen's ties to family, peers, and school (Henggeler et al., 2009). New arrests, truancy, and delinquent peers after six months of clinic-based therapy call for a more intensive approach that reaches all of those systems while the teen stays home. It beats A because residential placement is more restrictive and removes the teen from the settings where change must last; it fits if intensive home-based treatment fails.

**Takeaway:** When outpatient family therapy fails for serious delinquency, try intensive home-based multisystemic therapy before more restrictive residential placement.

**Sources:** Henggeler, S. W., Schoenwald, S. K., Borduin, C. M., Rowland, M. D., & Cunningham, P. B. (2009). Multisystemic Therapy for Antisocial Behavior in Children and Adolescents (2nd ed.). Guilford Press, MST principles and service delivery

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## 150. INT-1047 (rev 2) · Best answer: D

On a home visit, a single parent in treatment for depression reports returning to work and feeling better. During the session, the 12-year-old answers questions put to the parent and tells the younger siblings when to sit down. The parent looks to the 12-year-old before speaking. Using a structural approach, which response is **BEST**?

**A.** Thank the 12-year-old for helping and let them keep guiding the siblings

**B.** Arrange individual sessions to address the 12-year-old's stress

**C.** Pause family sessions until the parent's depression fully remits

**D.** Ask the parent to direct the siblings while the 12-year-old rests

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). Praising the 12-year-old for guiding the siblings reinforces the parentified role and the blurred hierarchy. In structural terms, the social worker would be joining the family's problem pattern rather than changing it.

**B** **Incorrect** (trap: Good but not best). Individual support for the 12-year-old may help later, but it treats a structural problem as the child's issue. The pattern lives in how the family is organized, which is best changed in family sessions.

**C** **Incorrect** (trap: Right action, wrong time). Waiting for full remission delays help the family needs now. The parent is working and feeling better, which suggests they can begin taking back the parental role with support.

**D** **Correct.** Structural family therapy sees parentification as a hierarchy problem: a child has taken over the parent's executive role. Boundary making in the session puts the parent back in charge and frees the child to be a sibling (Minuchin, 1974). The parent is in treatment, back at work, and feeling better, so they can take on that role now. It beats B because the 12-year-old's stress is real, but individual sessions leave the family structure that produces it unchanged; restructuring happens with the whole family present.

**Takeaway:** With a parentified child, structural work restores hierarchy: support the parent in the executive role and release the child from it.

**Sources:** Minuchin, S. (1974). Families and Family Therapy. Harvard University Press, subsystems, hierarchy, and boundary making

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### 151. INT-1050 (rev 2) · Best answer: C

At the first family session, the parents of a 16-year-old who is skipping school say, 'You're the expert. Just tell us what to do and fix our kid.' The social worker replies that only the family can decide what they want to change and how hard they will work at it. This response **MOST** likely reflects which concept?

**A.** Battle for structure
**B.** Restraining change
**C.** Battle for initiative
**D.** Positive connotation

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). The battle for structure is Whitaker's other early battle: the social worker sets the terms of therapy, such as who attends and when sessions occur (Napier & Whitaker, 1978). This response concerns who owns the motivation for change, not the ground rules of therapy.

**B** **Incorrect** (trap: Wrong model or concept). Restraining change is a strategic technique in which the worker warns the family to go slowly or names the dangers of improving, expecting them to push back. Here the worker does not discourage change; they place responsibility for it with the family.

**C** **Correct.** Whitaker's battle for initiative holds that the family, not the social worker, must own the motivation and energy for change; a therapist who supplies the drive leaves the family passive. Declining to 'fix our kid' and handing the question of what they want back to the parents is this stance (Nichols & Davis, 2020). It beats B because restraining change is a strategic paradox that cautions a family to go slow or warns of the risks of change. Here the worker is not discouraging change but insisting that the family lead it.

**D** **Incorrect** (trap: Wrong model or concept). Positive connotation is a Milan technique that gives a positive, system-serving meaning to every member's behavior, including the symptom. The worker here offers no new meaning for the school skipping; they decline to take over the family's role in changing it.

**Takeaway:** In Whitaker's model, the therapist wins the battle for structure but leaves the battle for initiative to the family, who must own the drive to change.

**Sources:** Nichols, M. P., & Davis, S. D. (2020). Family Therapy: Concepts and Methods (12th ed.). Pearson, symbolic-experiential family therapy: battle for structure and battle for initiative; Napier, A. Y., & Whitaker, C. A. (1978). The Family Crucible. Harper & Row, the battle for structure and the battle for initiative
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## 152. INT-1051 (rev 2) · Best answer: A

A 14-year-old with anorexia nervosa is discharged home after medical stabilization, with weekly weight checks by a pediatrician. The family begins family-based treatment with the social worker. In the second session, the parents say they want to let their teen choose and plan their own meals so that eating does not become a battle. Which response is **BEST**?

**A.** Coach the parents to plan and supervise every meal for now
**B.** Support the parents' plan to let the teen choose meals
**C.** Start individual sessions on the teen's need for control

#### Why each option is right or wrong

**A** **Correct.** In family-based treatment for adolescent anorexia nervosa, parents take charge of restoring weight in the first phase, because the illness, not the teen, is driving food choices. Control over eating returns to the adolescent in stages once weight is restored (Lock & Le Grange, 2013). The parents' wish to avoid conflict is understandable, so the social worker supports them while coaching them to stay in charge. It beats B because autonomy over meals is the right goal for a later phase; offered two sessions after discharge, it hands decisions back to the illness while weight is still low.

**B** **Incorrect** (trap: Right action, wrong time). Returning control of eating to the adolescent is a planned step in family-based treatment, but it comes after weight is largely restored. Two sessions after discharge, letting the teen plan meals risks renewed weight loss and medical danger.

**C** **Incorrect** (trap: Wrong model or concept). Individual insight-oriented work is an alternative model for some adolescents, but this family has begun family-based treatment, which keeps the parents central. Shifting the focus to the teen's need for control would sideline the parents when they are most needed.

**Takeaway:** In family-based treatment for adolescent anorexia, parents first take charge of meals and weight restoration; control returns to the teen as health recovers.

**Sources:** Lock, J., & Le Grange, D. (2013). Treatment Manual for Anorexia Nervosa: A Family-Based Approach (2nd ed.). Guilford Press, Phase 1: parents take charge of weight restoration
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### 153. INT-1052 (rev 1) · Best answer: B

A family in which three generations share a home seeks help for a 10-year-old who ignores household rules. By family tradition, the grandparent makes major household decisions. The parent works nights, and in session the parent and grandparent contradict each other's rules in front of the child. Using a structural approach, what is the **MOST** appropriate intervention?

**A.** Strengthen the parent's authority by limiting the grandparent's role
**B.** Help the parent and grandparent agree on rules as a team
**C.** Explore the child's feelings about the two sets of rules

#### Why each option is right or wrong

**A** **Incorrect** (trap: Overrides the client's choice). Reducing the grandparent's role imposes a nuclear-family model the family has not chosen. Structural work clarifies who leads, but it builds on the family's own arrangement, and here the grandparent's authority is part of it. This would fit only if the family itself wanted that shift.

**B** **Correct.** Structural family therapy aims for a clear, united executive subsystem, but who belongs in it depends on the family's own culture and arrangements (Minuchin, 1974). Here the grandparent's authority is a valued tradition, and the problem is that the two adults undercut each other. Building a parent-grandparent team restores a clear hierarchy without imposing an outside family form (McGoldrick et al., 2005). It beats A because textbook mapping often places parents alone at the top, but removing the grandparent's role would override the family's norms and likely cost the worker their trust.

**C** **Incorrect** (trap: Good but not best). The child's feelings matter, but the rule problem comes from the adults' split, not from the child. Exploring feelings alone leaves the contradiction in place; in structural terms, change comes when the executive subsystem works as a unit.

**Takeaway:** Structural work builds a united executive subsystem; define its members by the family's cultural arrangement, not by a nuclear-family template.

**Sources:** Minuchin, S. (1974). Families and Family Therapy. Harvard University Press, subsystems and hierarchy: the executive subsystem; McGoldrick, M., Giordano, J., & Garcia-Preto, N. (Eds.). (2005). Ethnicity and Family Therapy (3rd ed.). Guilford Press, overview: culture, ethnicity and family structure
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### 154. INT-1053 (rev 1) · Best answer: D

A parent has returned home after a year-long military deployment. In family sessions, the partner and two teens say everyone feels distant, but they struggle to put the feeling into words. The social worker uses Satir's experiential approach. Which intervention is **MOST** consistent with this approach?

**A.** Prescribe that the family keep its distance this week
**B.** Ask circular questions about who notices the distance first
**C.** Build a three-generation genogram of military service
**D.** Have members place one another to show closeness

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Prescribing the symptom is a strategic, paradoxical move. It does not reflect Satir's warm, growth-focused stance (Satir, 1972), and it offers no experiential method to help the family reach feelings they cannot yet name.

**B** **Incorrect** (trap: Wrong model or concept). Circular questions come from Milan systemic therapy. They can reveal relationship patterns, but they rely on talk about differences and are not the action-based method Satir used to bring feelings into the room.

**C** **Incorrect** (trap: Wrong model or concept). A genogram is the signature tool of Bowen theory, used to trace patterns across generations. It is a thinking-centered assessment, not an experiential exercise for feelings the family cannot yet put into words.

**D** **Correct.** Family sculpting is an experiential technique associated with Satir: members physically arrange one another in space to show closeness, distance and roles. It turns feelings the family cannot yet name into something everyone can see and talk about, which fits a family that feels distant but lacks words for it (Nichols & Davis, 2020). It beats B because circular questions are a Milan tool that works through spoken descriptions of differences, the very thing this family finds hard right now.

**Takeaway:** Satir's experiential work uses action methods such as sculpting to make unspoken feelings and family roles visible.

**Sources:** Nichols, M. P., & Davis, S. D. (2020). Family Therapy: Concepts and Methods (12th ed.). Pearson, experiential family therapy: family sculpting; Satir, V. (1972). Peoplemaking. Science and Behavior Books, the therapist's warmth and focus on growth

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## 155. INT-1141 (rev 1) · Best answer: A

During in-home structural family therapy for a 13-year-old's aggression, one parent asks to speak privately. The parent says the other parent controls the money, checks the parent's phone, and pushed the parent into a wall last week. The parent asks the social worker to keep family sessions going, fearing that stopping them would anger the other parent. What is the **BEST** response?

**A.** Connect the parent with a domestic violence advocate to plan safely

**B.** Continue family sessions as asked, keeping the focus on the teen

**C.** Stop family sessions and explain the reason to the family

**D.** Ask the other parent privately about the pushing

**Why each option is right or wrong**

**A** **Correct.** Family and couples therapy may be dangerous for a survivor of partner violence, and a provider who learns of abuse should refer the survivor to a domestic violence program (SAMHSA TIP 25, Ch. 2). The parent describes coercive control and a recent assault and fears retaliation if sessions change, so any change needs a careful safety plan. A domestic violence advocate can plan how and when to adjust the sessions without raising risk. It beats B because continuing joint work as if nothing changed can expose the parent to retaliation and leaves the danger unaddressed.

**B** **Incorrect** (trap: Ignores risk). Honoring the parent's request and the teen's needs is appealing, and abrupt changes can raise risk. Still, joint sessions with a controlling partner can lead to retaliation for what is said in them. Decisions about the sessions should come from a safety plan, not continue by default.

**C** **Incorrect** (trap: Misreads law or ethics). Stopping family sessions may be part of the eventual plan, but announcing the reason reveals the parent's private disclosure without consent and could provoke the retaliation the parent fears.

**D** **Incorrect** (trap: Ignores risk). Raising the pushing with the other parent tells the person using violence that the parent has disclosed it. That can increase danger and breaks the trust the parent placed in the social worker.

**Takeaway:** When partner abuse surfaces in family therapy, plan safety with a domestic violence specialist before changing or continuing joint sessions.

**Sources:** SAMHSA TIP 25 (1997), Ch. 2: couples and family therapy with survivors; referral to domestic violence programs

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### 156. INT-1142 (rev 1) · Best answer: D

A family therapy team meets with parents, a grandparent, and a 16-year-old who has stopped eating meals with the family. At the end of the session, the team says the teen's refusal keeps the parents united and the grandparent's frequent advice keeps everyone close. Each member's behavior is described as a sacrifice for the family's unity. Which technique is the team **MOST** likely using?

**A.** Prescribing the symptom
**B.** Unbalancing
**C.** Circular questioning
**D.** Positive connotation

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Symptom prescription is a paradoxical directive that tells the family to keep doing the problem behavior, used in strategic and Milan work. The team here describes the meaning of each person's behavior but gives no directive.

**B** **Incorrect** (trap: Wrong model or concept). Unbalancing is a structural move in which the worker temporarily sides with one member to shift the family's power arrangement (Minuchin & Fishman, 1981). The team treats all members alike rather than backing one.

**C** **Incorrect** (trap: Wrong model or concept). Circular questions ask members about differences and relationships, such as who reacts most when something happens. The team here makes a closing statement rather than asking questions.

**D** **Correct.** Positive connotation, a Milan systemic technique, gives a good motive to every family member's behavior, including the symptom, as serving the family's cohesion (Selvini Palazzoli et al., 1978). The team gives the teen's refusal and the grandparent's advice the same caring purpose, which avoids blaming anyone and joins the whole family. It beats A because prescribing the symptom directs the family to

continue or exaggerate the problem; this team explains each behavior's function but gives no instruction.

**Takeaway:** Milan positive connotation frames every member's behavior, symptom included, as serving family cohesion, without blame or directives.

**Sources:** Selvini Palazzoli, M., Boscolo, L., Cecchin, G., & Prata, G. (1978). Paradox and Counterparadox. Jason Aronson, positive connotation; Minuchin, S., & Fishman, H. C. (1981). Family Therapy Techniques. Harvard University Press, unbalancing

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### 157. INT-1150 (rev 3) · Best answer: B

Parents bring their 9-year-old to family therapy for tantrums. The family used a behavior chart consistently for 4 weeks with no change. In sessions, the child starts acting out whenever the parents begin to disagree about money, and the parents then stop arguing and work together to calm the child. Both parents say they 'haven't been a team lately.' What is the **BEST** intervention at this point?

**A.** Coach the parents to stay calm and united during tantrums

**B.** Meet with the parents as a couple about their conflict

**C.** Tell the family that the tantrums protect the parents from fighting

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Calm, united responses to tantrums are sound parenting advice. But these parents already unite whenever the child acts out; that is the pattern keeping the symptom going. More teamwork around the tantrums leaves the money conflict untouched and keeps the child in the go-between role.

**B** **Correct.** Structural family therapy holds that a symptom can serve the family's structure. Here the parents detour their conflict through the child: tantrums begin when money disagreements surface and end the argument by uniting the parents. A consistent 4-week chart trial failed, so this is not mainly a skills problem. Meeting with the parents as a couple strengthens the spousal subsystem and draws a boundary that frees the child from the conflict (Minuchin, 1974). It beats C because announcing the pattern to the whole family, child included, invites defensiveness; structural work changes the pattern by restructuring rather than interpreting it.

**C** **Incorrect** (trap: Too directive). The hypothesis may well be accurate, but announcing it to the whole family this early invites defensiveness and can leave the child feeling responsible for the parents' relationship. Structural therapists change the pattern by restructuring, not by interpreting it to the family.

**Takeaway:** When a child's symptom detours parental conflict, strengthen the parents' couple subsystem instead of intensifying child-focused behavior plans.

**Sources:** Minuchin, S. (1974). Families and Family Therapy. Harvard University Press, detouring; spousal subsystem and boundaries

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### 158. INT-1155 (rev 1) · Best answer: D

In a family session, one parent makes every decision. This parent talks over the other parent, who rarely speaks. For a few minutes, the social worker takes the quiet parent's side in a fight about bedtime. The goal is to shift who holds power in the family. Which structural technique is the social worker using?

**A.** Joining
**B.** Enactment
**C.** Boundary making
**D.** Unbalancing

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Joining is how the social worker builds rapport and accommodates to the family's style, usually early, without challenging the structure. Taking one parent's side against the other is a deliberate challenge to the structure.

**B** **Incorrect** (trap: Wrong model or concept). In an enactment, the social worker has family members carry out their problem interaction with each other in the session, then observes and redirects. Here the social worker, not the family, is the one acting.

**C** **Incorrect** (trap: Wrong model or concept). Boundary making changes closeness and access between members or subsystems, for example through seating changes or blocking interruptions. The move described is a temporary alliance with one member, not a boundary.

**D** **Correct.** In unbalancing, the structural therapist temporarily allies with one family member to change the hierarchy or distribution of power within a subsystem (Minuchin & Fishman, 1981). The social worker here sides with the quieter parent on purpose and for a limited time to disrupt the other parent's dominance. It beats C, boundary making, because boundary making regulates closeness and distance between members or subsystems, such as blocking interruptions, rather than siding with one member to shift power.

**Takeaway:** Unbalancing means temporarily siding with one family member to shift power; boundary making regulates closeness between members and subsystems.

**Sources:** Minuchin, S., & Fishman, H. C. (1981). Family Therapy Techniques. Harvard University Press, Unbalancing

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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### 159. INT-1160 (rev 1) · Best answer: C

A client asks for family therapy because of escalating conflict with their partner, who refuses to attend any sessions. The social worker practices from a Bowen family systems approach. The client wants to start now and says, 'I'm the one who keeps losing it.' What is the **BEST** way to proceed?

**A.** Wait to begin until the partner agrees to come to sessions
**B.** Refer the client for individual psychodynamic treatment
**C.** Coach the client alone to lower their own reactivity

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Some family approaches need the whole family present. Bowen work does not, and making the reluctant partner's attendance a condition leaves the motivated client without help.

**B** **Incorrect** (trap: Hands off too soon). An individual referral abandons the systems lens the client asked for and the social worker can provide. Bowen coaching with one member is a recognized family therapy format, so no hand-off is needed.

**C** **Correct.** Bowen theory holds that a change in one motivated member's differentiation can shift the whole system, so Bowenian therapists often work with one person (Bowen, 1978; Carter & Orfanidis, 1976). The client is ready, sees their own part, and the partner will not attend. Coaching the client to observe the pattern, manage emotional reactivity, and take calm I-positions is family systems work even with one person in the room. It beats A because waiting for the reluctant partner delays help the client wants and is not required by the model.

**Takeaway:** Bowen therapy can work through one motivated family member; raising that person's differentiation can change the whole system's pattern.

**Sources:** Bowen, M. (1978). Family Therapy in Clinical Practice. Jason Aronson, differentiation of self; therapy with one family member; Carter, E., & Orfanidis, M. M. (1976). Family therapy with one person and the family therapist's own family. In P. J. Guerin (Ed.), Family Therapy: Theory and Practice. Gardner Press, whole chapter
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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## 160. INT-1166 (rev 1) · Best answer: A

Which family therapy model focuses on hidden loyalties and on what each generation owes and is owed?

**A.** Contextual therapy
**B.** Bowenian therapy
**C.** Symbolic-experiential therapy
**D.** Milan systemic therapy

**Why each option is right or wrong**

**A** **Correct.** Contextual therapy, developed by Ivan Boszormenyi-Nagy, centers on relational ethics: invisible loyalties to family members and a multigenerational ledger of what each person owes and is owed (Boszormenyi-Nagy & Spark, 1973). Therapy aims to restore fairness and trust among members. It beats B, Bowenian therapy, because Bowen also looks across generations but focuses on differentiation of self and emotional transmission, not on loyalty, entitlement, and fairness.

**B** **Incorrect** (trap: Wrong model or concept). Bowen therapy is also multigenerational, using genograms and the multigenerational transmission process. Its core concept is differentiation of self, not loyalty and fairness.

**C** **Incorrect** (trap: Wrong model or concept). Whitaker's symbolic-experiential therapy emphasizes spontaneity, the battles for structure and initiative, and the therapist's use of self. It does not center on fairness between generations.

**D** **Incorrect** (trap: Wrong model or concept). Milan therapy uses hypothesizing, circular questions, neutrality, and positive connotation to change family patterns. Loyalty ledgers are not part of the model.

**Takeaway:** Contextual therapy addresses invisible loyalties and intergenerational fairness; Bowen therapy addresses differentiation and emotional transmission across generations.

**Sources:** Boszormenyi-Nagy, I., & Spark, G. M. (1973). Invisible Loyalties: Reciprocity in Intergenerational Family Therapy. Harper & Row, invisible loyalties; the family ledger

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.21 Family therapy models and interventions · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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## 161. INT-0005 (rev 1) · Best answer: C

An inpatient social worker is planning discharge for an adult with schizophrenia. The patient has had three admissions this year, each after missing the first outpatient appointment. The patient is stable and agrees to outpatient care. Which discharge step is **BEST**?

**A.** Give the patient a list of clinics to call after discharge

**B.** Extend the stay until the patient shows full insight

**C.** Schedule a clinic visit within 7 days and arrange a warm handoff

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). A resource list is better than nothing, but it leaves the patient to make contact alone, the very step that failed three times. This patient needs active linkage.

**B** **Incorrect** (trap: Overrides the client's choice). Keeping a stable, agreeable patient in the hospital is more restrictive than needed, and full insight is not a discharge requirement. The least restrictive setting plus strong linkage fits better.

**C** **Correct.** This patient's history shows that the gap between hospital and clinic is where care breaks down. Scheduling the first visit before discharge, within 7 days, and arranging a warm handoff closes that gap. Seven-day follow-up is a standard quality measure (NCQA HEDIS FUH), and the weeks right after discharge carry sharply raised suicide risk (Chung et al., 2017). It beats A because a list leaves the hardest step, making first contact, to the patient, which is exactly where past discharges failed.

**Takeaway:** For discharge, link actively: book the first follow-up within 7 days and hand off warmly, especially when past transitions failed.

**Sources:** National Committee for Quality Assurance. HEDIS measure: Follow-Up After Hospitalization for Mental Illness (FUH), 7-day follow-up rate; Chung, D. T., Ryan, C. J., Hadzi-Pavlovic, D., Singh, S. P., Stanton, C., & Large, M. M. (2017). Suicide rates after discharge from psychiatric facilities: A systematic review and meta-analysis. JAMA Psychiatry, 74(7), 694-702, whole article

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.22 Discharge planning, aftercare, wraparound, follow-up · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 162. INT-0110 (rev 1) · Best answer: A

A client with bipolar disorder was discharged from a psychiatric unit 3 days ago after a suicidal crisis. The client does not arrive for the first appointment at the outpatient clinic and has not called. What should the outpatient social worker do **FIRST**?

**A.** Phone the client today to check in and reschedule
**B.** Mail a letter offering a new appointment time
**C.** Close the referral under the clinic's no-show policy
**D.** Request a police welfare check at the client's home

**Why each option is right or wrong**

**A** **Correct.** The weeks after psychiatric discharge carry sharply raised suicide risk, especially after an admission for suicidal behavior (Chung et al., 2017). Care-transition guidance tells outpatient providers to contact a patient right away after a missed first appointment and to keep pursuing contact (National Action Alliance for Suicide Prevention, 2019). A same-day call lets the social worker reconnect, check on safety and rebook. It beats D because a welfare check is an escalation for imminent risk or for when direct outreach fails, and the clinic has not yet tried to reach the client.

**B** **Incorrect** (trap: Good but not best). A letter is slow and passive during the highest-risk period after discharge. It can be part of persistent outreach but should not replace prompt, direct contact.

**C** **Incorrect** (trap: Ignores risk). Applying a routine no-show policy to a client 3 days after a suicidal crisis ignores the danger of this period. Recently discharged clients need active outreach, not case closure.

**D** **Incorrect** (trap: Skips assessment). A police welfare check is appropriate when risk appears imminent or when the client and the client's safety contacts cannot be reached. As a first step, it escalates before the simplest contact has been tried.

**Takeaway:** After psychiatric discharge, treat a missed first appointment as a risk signal: reach out directly and promptly, escalating if contact fails.

**Sources:** National Action Alliance for Suicide Prevention (2019). Best Practices in Care Transitions for Individuals with Suicide Risk: Inpatient Care to Outpatient Care, outpatient provider actions after a missed first appointment; Chung, D. T., Ryan, C. J., Hadzi-Pavlovic, D., Singh, S. P., Stanton, C., & Large, M. M. (2017). Suicide rates after discharge from psychiatric facilities: A systematic review and meta-analysis. JAMA Psychiatry, 74(7), 694-702, whole article

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.22 Discharge planning, aftercare, wraparound, follow-up · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 163. INT-1539 (rev 2) · Best answer: A

On an inpatient psychiatric unit, a patient with bipolar disorder is set for discharge tomorrow after an overdose attempt. While reviewing aftercare plans, the patient says, 'I still have a month of pills at home. If I lose my job, I'll finish what I started.' The patient's employer has announced layoffs. When the social worker asks directly, the patient says this means taking all the pills to end their life. What should the social worker do **NEXT**?

**A.** Ask the treatment team to reassess discharge readiness today
**B.** Add removal of the stored pills to the discharge safety plan
**C.** Explore the patient's worries about losing the job

**Why each option is right or wrong**

**A** **Correct.** Any new suicidal ideation, especially at discharge, calls for reassessment, and discharge rests on the team's judgment that the patient is ready (SAMHSA SAFE-T). The social worker has already asked directly: the patient confirms a plan to overdose on pills at home, tied to a layoff that may come soon, after a recent attempt. Suicide risk is sharply elevated in the weeks after psychiatric discharge (Chung et al., 2017), so the team, including the psychiatrist, must reassess readiness today. It beats B because limiting access to means is essential but belongs in a plan built after the team decides whether discharge is still safe.

**B** **Incorrect** (trap: Right action, wrong time). Means restriction is a core part of any safety plan and will be needed (Stanley & Brown, 2012). Adding it now treats the discharge as settled, when the confirmed plan and intent call readiness itself into question.

**C** **Incorrect** (trap: Ignores risk). Working on the job worry can help later, within safety planning. But the patient has already confirmed a plan, the means and a likely trigger, so delaying the team's reassessment risks a discharge that no longer fits the risk. Exploring the stressor fits after the team has reviewed discharge.

**Takeaway:** A newly confirmed suicide plan with access to means before discharge reopens the discharge decision; inform the team before refining aftercare.

**Sources:** Chung, D. T., Ryan, C. J., Hadzi-Pavlovic, D., et al. (2017). Suicide rates after discharge from psychiatric facilities: a systematic review and meta-analysis. JAMA Psychiatry, 74(7), 694-702, whole article; Stanley, B., & Brown, G. K. (2012). Safety planning intervention. Cognitive and Behavioral Practice, 19(2), 256-264, making the environment safe; SAMHSA (2009). SAFE-T: Suicide Assessment Five-step Evaluation and Triage pocket card (SMA09-4432), Step 3: suicide inquiry; reassess as circumstances change and at inpatient discharge
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.22 Discharge planning, aftercare, wraparound, follow-up · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 3 of 3

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## 164. INT-1540 (rev 3) · Best answer: C

A 15-year-old has received wraparound services for 3 months to avoid residential placement. After a second school suspension, the school asks the team to recommend a residential program. The parent and youth want to keep working on the home-based plan. Which response by the wraparound facilitator is **MOST** consistent with wraparound principles?

**A.** Ask the school to hold its request until the plan's next review
**B.** Add a separate anger management program while the plan continues
**C.** Reconvene the team with the family to revise the plan

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Asking for time protects the home-based plan the family wants, so it is tempting. It waits for a scheduled review, though, instead of responding to the setback now; wraparound treats a suspension as a reason to bring the team, including the school, together promptly to revise strategies.

**B** **Incorrect** (trap: Good but not best). Anger management might become part of the plan, but adding it outside a team meeting bypasses family choice and joint planning. In wraparound, strategies are chosen together and fitted to this youth.

**C** **Correct.** Wraparound is unconditional: the team does not give up on or reject a family when things go wrong but treats setbacks as a reason to revise the plan, and family voice and choice carry the most weight (Bruns, Walker et al., 2008). The parent and youth want to continue, so the facilitator brings the team, including the family and school, back together to rework strategies after the suspension. It beats B because adding a service may help, but choosing it outside the team bypasses family voice and the joint, individualized planning wraparound requires.

**Takeaway:** In wraparound, setbacks lead the team and family to revise the plan rather than end home-based care or add services unilaterally.

**Sources:** Bruns, E. J., Walker, J. S., & NWI Advisory Group (2008). Ten principles of the wraparound process. The Resource Guide to Wraparound. National Wraparound Initiative, Principle 1: Family voice and choice; Principle 9: Unconditional

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.22 Discharge planning, aftercare, wraparound, follow-up · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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## 165. INT-1541 (rev 1) · Best answer: D

A 78-year-old patient is admitted to a hospital medical unit after a stroke that weakened one side of the body. The patient lives with a partner in a two-story house. The medical team has not set a discharge date. What should the social worker do **FIRST**?

**A.** Ask the physician for an expected discharge date to guide planning

**B.** Give the patient's partner a list of inpatient rehabilitation facilities

**C.** Request a home safety evaluation from occupational therapy

**D.** Meet with the patient and partner to assess needs and supports

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). A target date helps with timing, but planning should not wait for it. Starting the assessment now prevents rushed or unsafe arrangements when the date arrives.

**B** **Incorrect** (trap: Skips assessment). Rehabilitation may well be needed, but handing over a list assumes a placement before needs, preferences and options have been assessed. Facility choices come after the evaluation, with the patient's input.

**C** **Incorrect** (trap: Right action, wrong time). A home safety review may be needed, but it follows from a needs assessment and goes through the team's usual orders. The social worker first learns the patient's and partner's situation and goals.

**D** **Correct.** Discharge planning starts at admission: hospitals identify patients early who are likely to need help after discharge and complete a discharge planning evaluation promptly, so arrangements are ready in time and the patient and family take part (42 CFR 482.43(a)). New one-sided weakness and a two-story home signal likely needs, so the social worker first meets the patient and partner to assess function, caregiving capacity, home layout and preferences. It beats A because waiting for a date delays the assessment that the date and plan depend on.

**Takeaway:** Discharge planning begins at admission with an assessment of the patient's needs, supports and preferences, not when a discharge date is set.

**Sources:** Medicare Conditions of Participation: Discharge planning, 42 CFR 482.43, 42 CFR 482.43(a)
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.22 Discharge planning, aftercare, wraparound, follow-up · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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### 166. INT-1542 (rev 1) · Best answer: B

On a medical unit, a patient being treated for pneumonia insists on leaving today, before the planned discharge, to attend a parent's funeral in another city. The physician has explained the risks, the patient has decision-making capacity, and the patient still plans to leave. What is the **MOST** appropriate action by the social worker?

**A.** Explain that insurance may deny payment if the patient leaves early
**B.** Arrange follow-up care and prescriptions before the patient leaves
**C.** Explore whether the patient could delay the trip by one day

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). This is inaccurate: a study of discharges against medical advice found no insurer denied payment because the patient left early (Schaefer et al., 2012). Using a false financial threat to keep a capable patient is coercive and undermines trust.

**B** **Correct.** A patient with decision-making capacity may refuse further hospital care (NASW Code 1.02). Leaving against advice does not end the duty to plan: the team should still offer the plan that does the least harm, including follow-up appointments, medications and instructions (Alfandre, 2009). The patient has heard the risks and decided, so the social worker's job is to make the departure as safe as possible. It beats C because the funeral is fixed and the decision is made; pressing for delay serves the original plan, not the patient, and uses time needed for arrangements.

**C** **Incorrect** (trap: Good but not best). Problem solving around barriers to staying fits when the reason is flexible or the patient is unsure. Here the patient has heard the risks and decided because of a parent's funeral, so planning a safe exit is the better use of time.

**Takeaway:** When a capable patient leaves against medical advice, respect the choice and still arrange follow-up care, medications and instructions.

**Sources:** Alfandre, D. J. (2009). 'I'm going home': discharges against medical advice. Mayo Clinic Proceedings, 84(3), 255-260, whole article; Schaefer, G. R., Matus, H., Schumann, J. H., et al. (2012). Financial responsibility of hospitalized patients who left against medical advice: medical urban legend? Journal of General Internal Medicine, 27(7), 825-830, whole article; NASW Code of Ethics (2021), 1.02
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.22 Discharge planning, aftercare, wraparound, follow-up · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 167. INT-1644 (rev 1) · Best answer: C

A patient with schizophrenia, admitted after a suicide attempt, is ready for discharge. The team finds the patient no longer at imminent risk and able to make treatment decisions. The patient declines any outpatient appointment, saying the hospital stay was enough, and plans to return home. Which follow-up plan is **MOST** appropriate?

**A.** Delay discharge until the patient agrees to an appointment
**B.** Give a list of local clinics and close the case at discharge
**C.** Offer to send brief, caring check-in messages after discharge

#### Why each option is right or wrong

**A** **Incorrect** (trap: Overrides the client's choice). A patient who is no longer at imminent risk and can make treatment decisions may decline aftercare. Holding discharge to extract agreement is coercive and can damage trust the patient may need later.

**B** **Incorrect** (trap: Good but not best). A resource list respects the refusal and is part of any discharge, but closing the case ends all contact during the highest-risk weeks. A low-demand way to stay connected serves the patient better.

**C** **Correct.** The weeks after psychiatric discharge carry high suicide risk, and a capable patient may still decline treatment. Motto and Bostrom (2001) found that brief, nondemanding letters sent periodically to people who refused ongoing care after hospitalization for depression or suicidal behavior were associated with fewer suicides in the following two years. Offering caring contacts respects the patient's choice while keeping a connection through the riskiest period. It beats B because a clinic list and closure meet the minimum but leave no contact at all after a suicide attempt, when follow-up matters most.

**Takeaway:** When a capable patient declines aftercare after a suicidal crisis, offer brief caring contacts that keep a connection without making demands.

**Sources:** Motto, J. A., & Bostrom, A. G. (2001). A randomized controlled trial of postcrisis suicide prevention. Psychiatric Services, 52(6), 828-833, whole article
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.22 Discharge planning, aftercare, wraparound, follow-up · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 168. INT-1650 (rev 1) · Best answer: B

A client with schizophrenia was discharged from an inpatient unit 4 days ago to supported housing and missed the first clinic appointment yesterday. The social worker has called twice this afternoon with no answer. Housing staff report the client has not left the room in 2 days, has refused meals, and can be heard shouting when no one else is there. What should the social worker do **NEXT**?

**A.** Notify the outpatient prescriber about the missed appointment
**B.** Request a mobile crisis team visit to the client today
**C.** Ask housing staff to urge the client to call the clinic
**D.** Schedule a home visit with the client for later this week

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). The prescriber should hear about the missed visit and the staff report, and the social worker will share them. Notifying the prescriber does not, by itself, get anyone to the client today, and two days of refused meals and shouting at no one need same-day eyes on the client.

**B** **Correct.** The weeks after psychiatric discharge carry sharply raised risk, so failed outreach plus signs of decompensation calls for prompt in-person evaluation (Chung et al., 2017). Here the phone attempts have failed, and staff describe isolation, refused meals, and apparent responses to voices. A mobile crisis team can assess the client where they live today, a less restrictive response than police or an ambulance (SAMHSA, 2020). It beats C because asking a client who appears to be decompensating to make the call leaves the next step to the person least able to take it.

**C** **Incorrect** (trap: Ignores risk). Staff encouragement suits a stable client who simply forgot an appointment. This client is isolating, refusing food, and appears to be responding to voices. Waiting for that client to call puts the next step on the person least able to take it.

**D** **Incorrect** (trap: Ignores risk). An in-person check is the right kind of response, but later this week is too slow. Refused meals and signs of psychosis during the high-risk period after discharge call for evaluation today, not a routine visit.

**Takeaway:** When outreach after discharge fails and signs of decompensation appear, arrange same-day in-person crisis evaluation rather than more routine contact.

**Sources:** Chung, D. T., Ryan, C. J., Hadzi-Pavlovic, D., Singh, S. P., Stanton, C., & Large, M. M. (2017). Suicide rates after discharge from psychiatric facilities: A systematic review and meta-analysis. JAMA Psychiatry, 74(7), 694-702, whole article; SAMHSA (2020). National Guidelines for Behavioral Health Crisis Care: Best Practice Toolkit, Mobile crisis team services

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.22 Discharge planning, aftercare, wraparound, follow-up · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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## 169. INT-1651 (rev 2) · Best answer: C

Two days after discharge from a psychiatric unit, a client with bipolar disorder tells the social worker on a scheduled follow-up call that the pharmacy would not fill the new mood stabilizer. The insurer needs a prior authorization form from the prescriber. The client has called the insurer twice, wants to restart the medication, and says, 'I've been on the phone all day. I don't have the energy to keep calling.' What should the social worker do **FIRST**?

**A.** Move the client's outpatient appointment to an earlier date

**B.** Coach the client to ask the prescriber's office for the form

**C.** Call the prescriber's office today about the form

**D.** Ask how the client feels about taking a mood stabilizer

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Seeing the prescriber sooner may help and can be arranged, but the earliest opening may still be days away. The form can be handled today, so moving the visit first leaves the client without the medication in the meantime.

**B** **Incorrect** (trap: Good but not best). Supporting self-advocacy is usually a strong first move when a client has the energy and time allows. Here the client has made repeated calls and is running out of energy, and the barrier needs only a form from the prescriber. Coaching fits once the medication gap is closed.

**C** **Correct.** Follow-up after discharge exists to catch and remove barriers before they undo the plan; case management standards call for monitoring service delivery and acting when a plan stalls (NASW, 2013, Standard 6). The client wants the medication, has already called the insurer twice, and is running out of energy, while the weeks after discharge are a period of raised risk (Chung et al., 2017), so a lapse in the new mood stabilizer should not wait. Calling the prescriber's office with the client's agreement clears the block today. It beats B because coaching builds skills, but this client has done their part and is losing steam.

**D** **Incorrect** (trap: Good but not best). Exploring feelings about medication fits when a client is unsure about taking it. This client has said they want to restart, and the problem is a paperwork barrier. Asking about feelings now delays the practical fix.

**Takeaway:** When follow-up after discharge finds a barrier to treatment the client wants, remove it promptly rather than adding steps for an exhausted client.

**Sources:** NASW Standards for Social Work Case Management (2013), Standard 6: Service Planning, Implementation, and Monitoring; Chung, D. T., Ryan, C. J., Hadzi-Pavlovic, D., Singh, S. P., Stanton, C., & Large, M. M. (2017). Suicide rates after discharge from psychiatric facilities: A systematic review and meta-analysis. JAMA Psychiatry, 74(7), 694-702, whole article

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.22 Discharge planning, aftercare, wraparound, follow-up · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 170. INT-0006 (rev 1) · Best answer: B

In the third meeting of a closed anger management group, several members complain that the attendance rules are unfair and question the leader's competence. The social worker calmly restates the group agreements, but the complaints continue. What should the social worker do **NEXT**?

**A.** Restate the rules firmly and move on to the planned skill

**B.** Invite the group to discuss its reactions to the rules

**C.** Meet outside the group with the members who complain

#### Why each option is right or wrong

**A** **Incorrect** (trap: Too directive). Repeating a step that has already failed escalates a power struggle. Members may hear it as the leader avoiding their concerns, which deepens the challenge to the leader.

**B** **Correct.** Members challenging rules and testing the leader in early meetings is typical of the storming, or power and control, stage (Tuckman, 1965; Garland, Jones & Kolodny, 1965). Restating the rules has not settled it, so the next step is to open the conflict for discussion without defensiveness. Working through it builds trust and norms (Yalom & Leszcz, 2020). It beats C because private meetings move a group-level process out of the group, which can create subgroups and keep members from learning to handle conflict together.

**C** **Incorrect** (trap: Good but not best). Individual meetings may help with a member who keeps disrupting after the group has addressed it. As a first response, they pull a normal group process outside the group and can create subgroups.

**Takeaway:** Early challenges to rules and the leader signal the storming stage; address the conflict openly in the group without defensiveness.

**Sources:** Tuckman, B. W. (1965). Developmental sequence in small groups. Psychological Bulletin, 63(6), 384-399, storming stage; Yalom, I. D., & Leszcz, M. (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books, early group development: conflict; Garland, J. A., Jones, H. E., & Kolodny, R. L. (1965). A model for stages of development in social work groups. In S. Bernstein (Ed.), Explorations in Group Work. Boston University School of Social Work, power and control stage

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.23 Group work: managing process and building cohesion · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 171. INT-1061 (rev 1) · Best answer: B

In week 4 of a closed support group for adults with chronic pain, one member talks at length each session about medical appointments. Others have started checking their phones, and two arrived late last week. No one has interrupted the talkative member. What should the social worker do **FIRST**?

**A.** Meet privately with the talkative member after group
**B.** Ask the group how it has been sharing session time
**C.** Set a fixed time limit for each member's turn
**D.** Ask the member to save medical updates for later

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). Meeting privately is a useful second step if group-level work does not change the pattern. Done first, it singles out the member and misses the group's part in letting one person hold the floor.

**B** **Correct.** A monopolizing member is a group-level problem: the group allows it, often to avoid its own material. The leader first works through the group, inviting members to look at how time is shared, which brings in quieter members and spares the talkative one public blame (Yalom & Leszcz, 2020; SAMHSA TIP 41). It beats A because a private meeting is a sound step if the group-level approach fails, but used first it treats the problem as one person's and leaves the group's silence unaddressed.

**C** **Incorrect** (trap: Good but not best). A uniform time limit treats the symptom with a rule and skips the process the group needs to look at. It can feel punitive and rarely helps members understand why the pattern developed.

**D** **Incorrect** (trap: Too directive). Publicly redirecting the member's content may embarrass them and casts the issue as one person's fault. It also leaves the other members' silence and withdrawal unexplored.

**Takeaway:** Address a monopolizing member first at the group level, exploring how members share time, before meeting with the member privately.

**Sources:** Yalom, I. D., & Leszcz, M. (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books, problem group members: the monopolist; SAMHSA TIP 41 (2005), Ch. 6: Group Leadership, Concepts, and Techniques

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### 172. INT-1062 (rev 3) · Best answer: A

In a residential treatment group for adolescents, the social worker announces that a planned outing is canceled because of staffing. Members begin blaming one quiet member, saying the outing was canceled because of that member's 'drama,' and the comments grow harsher. Which response is **BEST**?

**A.** Stop the blaming and name the group's shared frustration
**B.** Explain to the group the staffing reasons for canceling
**C.** Coach the blamed member privately on coping with peers

**Why each option is right or wrong**

**A** **Correct.** Scapegoating is a group process: members displace shared anger onto one person, often anger meant for the leader. The leader protects the targeted member and makes the hidden feeling speakable, here frustration about a cancellation the worker announced (SAMHSA TIP 41). Doing both in the group stops the harm and gives members a safer outlet. It beats C because private coaching may help the member later, but it leaves the group free to keep blaming and treats a group dynamic as one teen's problem.

**B** **Incorrect** (trap: Good but not best). Explaining the staffing problem corrects the false cause and may help once things settle. By itself it leaves the harsh comments unchallenged and the shared frustration unspoken, so the group can keep displacing it onto the quiet member.

**C** **Incorrect** (trap: Good but not best). Supporting the targeted member one-on-one can be a helpful follow-up, but it leaves the harsh comments unchallenged in the room. The pattern belongs to the group and needs to be worked on there.

**Takeaway:** When a group scapegoats one member, protect that member and redirect the displaced feeling to its real source within the group.

**Sources:** SAMHSA TIP 41 (2005), Ch. 6: Group Leadership, Concepts, and Techniques (scapegoating)
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### 173. INT-1063 (rev 2) · Best answer: C

As an outpatient substance use group begins, a member arrives smelling of alcohol, slurring words, and says they drove to the session. The member asks to stay and talk about the relapse, and several members say they want to help. What should the social worker do **FIRST**?

**A.** Let the member stay and discuss the relapse with the group
**B.** Discharge the member from the program for drinking
**C.** Excuse the member from group and arrange a safe ride

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Discussing the relapse is valuable, and the group's wish to help is a strength. With the member intoxicated now, the talk will not be usable and may unsettle others. It fits the next session, when the member is sober.

**B** **Incorrect** (trap: Hands off too soon). A relapse is an expected part of recovery, not a reason to end treatment. Discharging the member removes support when it is most needed. Today's issue is taking part while intoxicated, not program eligibility.

**C** **Correct.** An intoxicated member cannot use group work, and processing with them present can unsettle members in early recovery. Guidance for substance use groups is to have the person leave that session and return next time, while making sure they do not drive (SAMHSA TIP 41). This member has said they drove, so a safe ride is part of the first step. It beats A because the relapse is important group material, but it is best worked on at the next session, when the member is sober and can use it.

**Takeaway:** A member who arrives intoxicated leaves that session safely, without driving, and the relapse is addressed once they are sober.

**Sources:** SAMHSA TIP 41 (2005), Ch. 6: Group Leadership, Concepts, and Techniques (intoxicated clients)
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### 174. INT-1064 (rev 1) · Best answer: A

In month 3 of an outpatient process group, a member often makes jokes when others share painful stories. Two members tell the member that the jokes leave them feeling brushed off. Over the next weeks, the member says a partner and coworkers have complained of the same habit, and starts responding seriously in group. Which therapeutic factor is **MOST** likely at work?

**A.** Interpersonal learning
**B.** Catharsis
**C.** Universality
**D.** Imitative behavior

#### Why each option is right or wrong

**A** **Correct.** Yalom describes interpersonal learning as discovering, through members' honest feedback in the here and now, how one's behavior affects others, then trying new behavior in the group. The group works as a social microcosm: the member hears how the jokes land, links the pattern to life outside, and changes in session (Yalom & Leszcz, 2020). It beats D because imitative behavior means modeling oneself on other members or the leader; here the change follows direct feedback about the member's own impact.

**B** **Incorrect** (trap: Wrong model or concept). Catharsis is the release of strong feeling that brings relief. The member's change came from hearing feedback and recognizing a pattern, not from expressing pent-up emotion.

**C** **Incorrect** (trap: Wrong model or concept). Universality is the relief of finding that others share one's problems. The member is not discovering shared struggles; they are hearing how their own behavior affects others and changing it.

**D** **Incorrect** (trap: Wrong model or concept). Imitative behavior means copying another member's or the leader's way of acting. Nothing suggests the member is modeling anyone; the change follows feedback about their own habit.

**Takeaway:** Interpersonal learning: honest member feedback in the here and now shows a person their impact, and the group becomes a place to try new behavior.

**Sources:** Yalom, I. D., & Leszcz, M. (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books, therapeutic factors: interpersonal learning; the group as social microcosm
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## 175. INT-1065 (rev 1) · Best answer: B

A social worker is screening applicants for a 12-week interpersonal process group at a community mental health center. One applicant with schizophrenia stopped taking medication a month ago, hears critical voices most of the day, and believes neighbors are watching through the walls. The applicant hopes the group will help them make friends. What is the **MOST** appropriate action?

**A.** Admit the applicant and pair them with a peer buddy
**B.** Defer group entry and link the applicant to psychiatric care
**C.** Explore what the applicant hopes to gain from the group

**Why each option is right or wrong**

**A** **Incorrect** (trap: Ignores risk). A peer buddy cannot make an intense process group workable for someone with active psychosis, and the group's feedback could worsen distress. A structured, supportive group could be considered later as symptoms settle.

**B** **Correct.** Screening matches members to a group they can use. Active psychosis is a standard reason to exclude someone from an interpersonal process group, because its intensity and feedback can overwhelm a person who cannot yet use them (Yalom & Leszcz, 2020). The stopped medication and constant voices also need prompt psychiatric attention, so the worker links the applicant to care and offers the group later. It beats C because the applicant's hope for friendship matters, but it does not change the fit decision now; exploring it fits once symptoms are stable.

**C** **Incorrect** (trap: Good but not best). Exploring goals is part of every screening interview, but this applicant's current symptoms already decide the fit question. The wish to make friends is worth carrying into psychiatric care and later group planning.

**Takeaway:** Pre-group screening defers people with active psychosis or acute crisis from process groups and links them to appropriate care first.

**Sources:** Yalom, I. D., & Leszcz, M. (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books, selection of clients: criteria for exclusion
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### 176. INT-1066 (rev 2) · Best answer: A

In a closed school group for 9th graders whose parents are divorcing, three members who share a lunch period start sitting together. During sessions, they trade glances and laugh quietly when others speak, and two members who used to share have gone quiet. Which response is **BEST**?

**A.** Name the pattern aloud and invite members to discuss it
**B.** Meet with the three members privately and ask them to stop
**C.** Wait a session to see whether the pattern fades

**Why each option is right or wrong**

**A** **Correct.** Subgroups form in most groups; the leader's task is to make covert alliances overt so the group can examine how they affect trust and participation (SAMHSA TIP 41). Naming the glances and laughter without blame, and asking how members experience them, lets the quieted members be heard and turns the pattern into group work. It beats B because a private talk may stop the behavior for a while, but it keeps the dynamic hidden, leaves the silenced members' reactions unaddressed, and can deepen the split between insiders and outsiders.

**B** **Incorrect** (trap: Good but not best). Talking privately with the three may quiet the behavior, but it keeps the dynamic secret from the group it affects. A private meeting fits if the subgroup's behavior turns harmful after it has been discussed openly.

**C** **Incorrect** (trap: Good but not best). Some subgroup behavior does fade, and waiting avoids overreacting to a new friendship. Here two members have already gone quiet, so the alliance is shaping participation now; leaving it unnamed lets the split harden in a closed group with a limited number of sessions.

**Takeaway:** Make covert subgroups overt: name the pattern in the group so members can examine its effect on trust and participation.

**Sources:** SAMHSA TIP 41 (2005), Ch. 6: Group Leadership, Concepts, and Techniques (subgroups)
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### 177. INT-1067 (rev 1) · Best answer: C

A closed 16-week grief group for older adults meets at a senior center. In week 12, the social worker reminded members that the group ends in 4 weeks. In week 14, two members miss without notice, and a member who had been sleeping well says the sleep problems are back. What should the social worker do **NEXT**?

**A.** Offer to extend the group by 4 more weeks
**B.** Refer the member with sleep problems for evaluation
**C.** Invite members to talk about the approaching last session

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). Extending the group avoids the feelings the ending stirs up and may teach members that endings can be dodged. For a grief group, a planned, well-processed ending is itself a chance to practice coping with loss.

**B** **Incorrect** (trap: Hands off too soon). Sleep changes in an older adult can warrant evaluation if they persist or come with other warning signs. Here they appear as the group nears its end, a common regression the leader should first address in the group.

**C** **Correct.** Members often react to a group's approaching end with absences, denial, or a return of old symptoms; these are expected termination reactions (Yalom & Leszcz, 2020). Having announced the ending, the leader's next task is to bring those reactions into the room, help members name feelings about another loss, review gains, and plan support after the group. It beats B because returning sleep problems could need evaluation if they persist, but in week 14 of a grief group they fit the pattern of reacting to the coming separation, which the group can address now.

**Takeaway:** Absences, denial and returning symptoms near a group's end are expected termination reactions; address them openly in the group.

**Sources:** Yalom, I. D., & Leszcz, M. (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books, termination of the group

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## 178. INT-1068 (rev 1) · Best answer: B

A social worker is asked to start a daily group on an acute inpatient psychiatric unit. Most patients stay 4 to 7 days, and new patients arrive every day. Which group format fits this unit?

**A.** Closed group meeting weekly for 8 sessions

**B.** Open group with each session complete in itself

**C.** Open group whose themes build week to week

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). A closed 8-week group needs stable members who attend every session (Toseland & Rivas, 2017). Patients here leave within a week, so most would miss the majority of sessions. Closed groups fit longer-stay or outpatient programs.

**B** **Correct.** With short stays and daily admissions, membership changes almost every meeting. Yalom's model for acute inpatient groups uses a single-session time frame: each meeting is self-contained, with its own goals, so every patient can benefit even after attending only once or twice (Yalom, 1983). It beats C because an open format is right for this unit, but themes that build across weeks assume a stable membership that rapid turnover rules out.

**C** **Incorrect** (trap: Misuses the technique). Open membership suits this unit, but linking themes from week to week assumes members carry continuity between sessions. With stays of 4 to 7 days, most patients would arrive mid-thread and leave before it resolves.

**Takeaway:** When group membership turns over rapidly, use an open group in which each session is complete in itself.

**Sources:** Yalom, I. D. (1983). Inpatient Group Psychotherapy. Basic Books, single-session time frame for acute inpatient groups; Toseland, R. W., & Rivas, R. F. (2017). An Introduction to Group Work Practice (8th ed.). Pearson, planning the group: open and closed membership
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### 179. INT-1069 (rev 1) · Best answer: A

In the working stage of a process group at a veterans' center, one member describes feeling like a stranger at home. Another member turns to the social worker and says, 'I think what they mean is they feel invisible to their family.' What is the **MOST** appropriate response?

**A.** Ask the member to say that directly to the other member
**B.** Agree and add the social worker's own reading of it
**C.** Thank the member and move on to the next person
**D.** Restate the speaker's feelings in the leader's own words

#### Why each option is right or wrong

**A** **Correct.** In the working stage the leader becomes less central and fosters direct member-to-member exchange, the main channel of here-and-now work and cohesion (Yalom & Leszcz, 2020). Asking the member to speak to the other member directly turns a comment about someone into a conversation with them. It beats D because reflecting feelings is a sound skill, but done by the leader here it pulls the exchange back through the leader at a stage when members should be relating to each other.

**B** **Incorrect** (trap: Good but not best). Adding the leader's own reading makes the leader the authority on the member's experience and keeps communication flowing through the leader. It also risks putting words in the speaker's mouth.

**C** **Incorrect** (trap: Good but not best). Moving on drops a moment of connection between two members. The working stage is when such exchanges carry the most therapeutic value, so the leader builds on them rather than passing over them.

**D** **Incorrect** (trap: Misuses the technique). Restating feelings is a core skill that fits early sessions, when the leader carries more of the work. In the working stage it routes the exchange through the leader instead of between members.

**Takeaway:** In the working stage, the leader steps back and redirects comments so members speak to each other directly.

**Sources:** Yalom, I. D., & Leszcz, M. (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books, the here-and-now; the therapist's role in the working group
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### 180. INT-1152 (rev 1) · Best answer: A

In a hospital support group for adults newly diagnosed with cancer, a member in the second week of chemotherapy listens as another member describes finishing treatment 5 years ago and returning to work. Afterward the newer member says, 'If they got through it, maybe I can too.' Which therapeutic factor is **MOST** likely at work for the newer member?

**A.** Instillation of hope
**B.** Universality
**C.** Imitative behavior
**D.** Imparting information

#### Why each option is right or wrong

**A** **Correct.** Yalom describes instillation of hope as the encouragement members gain from seeing others who have coped with, or improved from, the same problem (Yalom & Leszcz, 2020). The newer member's words look forward: a peer's survival and return to work make recovery seem possible. It beats B, universality, because universality is the relief of learning that others share one's problem or feelings. Here the member draws belief from another person's progress, not from shared suffering.

**B** **Incorrect** (trap: Wrong model or concept). Universality would fit if the member said they felt less alone or less abnormal on hearing others describe the same fears. The statement here is about a possible future, which marks hope.

**C** **Incorrect** (trap: Wrong model or concept). Imitative behavior means taking on another member's or the leader's way of acting or coping. The newer member expresses a belief about the future, not an adopted behavior.

**D** **Incorrect** (trap: Wrong model or concept). Imparting information covers teaching or advice about an illness or about coping. The survivor shared a personal story, and the newer member's response centers on possibility rather than new facts.

**Takeaway:** Instillation of hope comes from seeing peers who are further along improve; universality comes from discovering that others share the same problem.

**Sources:** Yalom & Leszcz (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books, Ch. 1, The therapeutic factors: instillation of hope; universality
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### 181. INT-1154 (rev 2) · Best answer: A

In week 5 of a middle school group for students who have a parent in prison, a member's grandparent calls the school social worker. At lunch, another group member told classmates about the member's parent's charges, which the member had shared in group. The member is upset and now refuses to return. What should the social worker do **FIRST**?

**A.** Meet with the member to hear the impact and plan next steps
**B.** Address the breach at the next session and restate group rules
**C.** Meet first with the member who shared the information

**Why each option is right or wrong**

**A** **Correct.** When a group confidence is broken, the leader first learns how the affected member was harmed and involves that member in deciding how the breach will be handled. This member is hurt and withdrawing, and discussing the breach in group without the member's input could expose the information again. Leaders seek members' agreement to keep confidences but cannot guarantee it (NASW Code 1.07(f)). It beats B because repairing group norms is the right next step, but it should follow the affected member's safety and wishes (Corey, Corey & Corey, 2018).

**B** **Incorrect** (trap: Right action, wrong time). Processing the breach with the group and restating confidentiality is necessary and will likely follow. Doing it before talking with the injured member risks reopening the disclosure without consent and losing a member who has not yet been heard.

**C** **Incorrect** (trap: Right action, wrong time). The member who disclosed will need a conversation about the breach and its consequences, and some leaders start there. Doing it first puts the harmed member second and risks decisions about the breach before that member has been heard or consulted.

**Takeaway:** After a group confidentiality breach, start with the affected member: learn the harm and plan together before processing the breach in group.

**Sources:** NASW Code of Ethics (2021), 1.07(f); Corey, M. S., Corey, G., & Corey, C. (2018). Groups: Process and Practice (10th ed.). Cengage, confidentiality in groups; handling breaches

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## 182. INT-1158 (rev 2) · Best answer: A

In a caregiver support group at a senior center, one member has attended 5 sessions, listens closely, and speaks only when asked a direct question. During a break, the member explains that in their family, talking about relatives' problems with outsiders brings shame, but hearing the others 'helps a lot.' What is the **MOST** appropriate response by the social worker?

**A.** Use go-around turns in which any member may pass

**B.** Invite the member to share one struggle at the next session

**C.** Suggest individual counseling as a better fit for the member

**Why each option is right or wrong**

**A** **Correct.** Group leaders respect silence, consider its cultural meaning, and invite participation without forcing disclosure (Corey, Corey & Corey, 2018). The member has explained that sharing family problems with outsiders brings shame, and says the group is helping. Structured rounds with a clear option to pass keep the door open without singling the member out, which honors the member's cultural values (NASW Cultural Competence Standard 4). It beats B because a direct invitation puts the member on the spot and pushes against the norm the member just described.

**B** **Incorrect** (trap: Too directive). A personal invitation can help a shy member who wants to speak but needs an opening. This member has described a cultural norm against such sharing, so being asked to disclose a struggle in the group risks shame and withdrawal.

**C** **Incorrect** (trap: Hands off too soon). The member is benefiting from listening and wants to stay. Moving the member to individual counseling treats a cultural style as a poor fit and removes a support the member values.

**Takeaway:** Respect culturally rooted silence in groups: offer low-pressure ways to take part, such as rounds with the option to pass, instead of pressing for disclosure.

**Sources:** Corey, M. S., Corey, G., & Corey, C. (2018). Groups: Process and Practice (10th ed.). Cengage, problem behaviors: silence and lack of participation; NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 4: Cross-Cultural Skills

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### 183. INT-1162 (rev 2) · Best answer: A

In week 4 of a 10-week hospice bereavement group, a member whose partner died 3 months ago stays after the session. In response to the social worker's questions, the member says they now drink most of a bottle of wine every night to sleep, have missed 6 workdays this month, and their adult child has asked them to cut back. What should the social worker do **NEXT**?

**A.** Help the member arrange a substance use evaluation and stay in group

**B.** Suggest a bedtime routine and check on the drinking next week

**C.** Explore further how the loss has affected the member's sleep

**D.** Ask the member to pause group until the drinking is under control

#### Why each option is right or wrong

**A** **Correct.** Social workers refer clients when another professional's expertise is needed to serve them fully (NASW Code 1.16(a)). Nightly heavy drinking, missed work, and family concern go beyond what a time-limited bereavement support group can address. Linking the member to a substance use evaluation while keeping them in the group adds needed care without removing a support. It beats B because a sleep routine and a check-in show care but leave a pattern that is already costing work and worrying family unassessed for another week.

**B** **Incorrect** (trap: Ignores risk). A bedtime routine is reasonable grief support, and checking back shows care. Nightly heavy drinking with missed work and family concern, though, calls for a professional substance use assessment now; waiting a week while the pattern continues under-reacts to the risk.

**C** **Incorrect** (trap: Ignores risk). Exploring grief and sleep fits a bereavement group, but the member has already described the drinking and its effects. More exploration postpones the step those facts call for while the health risk continues.

**D** **Incorrect** (trap: Hands off too soon). Making return depend on controlled drinking removes a support during a high-risk time and makes help conditional. Grief support and substance use care can run at the same time.

**Takeaway:** When a group member's needs exceed the group's scope, add a referral for specialized assessment while keeping the member's group support.

**Sources:** NASW Code of Ethics (2021), 1.16(a)
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### 184. INT-1165 (rev 1) · Best answer: B

In the first session of a closed outpatient group for adults coping with anxiety, introductions begin. One member starts describing childhood sexual abuse in vivid detail and is crying. Other members look at the floor, and one moves their chair back. What should the social worker do **FIRST**?

**A.** Allow the member to finish the account without interruption
**B.** Gently pause the member and suggest returning to it later
**C.** Talk privately with the member after the session ends

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Letting a member speak can feel respectful, and catharsis is a group factor. In a first session, before trust or norms exist, a detailed trauma account often leads to regret and early dropout and overwhelms other members.

**B** **Correct.** Group leaders protect members from disclosing more than the group's early trust can hold. A premature, intense disclosure in a first session can leave the member ashamed and push them, or others, to drop out (Yalom & Leszcz, 2020). The other members' withdrawal shows the group is not ready. Gently pausing the member, with warmth and a clear offer to return to it, protects both the member and the group (Corey, Corey & Corey, 2018). It beats A because letting the account continue honors the member's voice now at the cost of later shame and group fragmentation.

**C** **Incorrect** (trap: Right action, wrong time). A private check-in after the session is valuable and should follow. Waiting until then leaves the member exposed and the group overwhelmed for the rest of the disclosure.

**Takeaway:** In early group sessions, gently block premature intense disclosure to protect the member and the group, then return to it as trust develops.

**Sources:** Yalom & Leszcz (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books, self-disclosure; premature disclosure and early dropouts; Corey, M. S., Corey, G., & Corey, C. (2018). Groups: Process and Practice (10th ed.). Cengage, leadership skills: blocking
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### 185. INT-1550 (rev 1) · Best answer: C

A social worker at a rural aging services agency finds that several older adult clients miss medical appointments because the county has no public bus service. The county has several active congregations, a senior center and a hospital system. The agency director proposes buying a van and hiring a driver. What should the social worker do **FIRST**?

**A.** Write a grant proposal to purchase an agency van
**B.** Ask county commissioners to fund a public bus route
**C.** Inventory existing transportation options and possible partners
**D.** Arrange taxi vouchers for clients one appointment at a time

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). A grant for a van may become part of the plan, but writing it now skips learning what transportation already exists. Duplicating an existing service wastes scarce funds and can strain ties with groups that already give rides.

**B** **Incorrect** (trap: Right action, wrong time). Pressing for public transit is legitimate advocacy and may be a later or parallel step. It is slow and uncertain and will not help clients soon. A scan of current options also gives the request supporting data.

**C** **Correct.** Resource development starts by finding out what a community already has before building something new (Kretzmann & McKnight, 1993). Congregations, a senior center and a hospital system may already offer rides or could become partners, which makes any solution cheaper and longer lasting. It beats A because buying a van commits the agency to vehicle, driver and insurance costs before anyone knows whether existing options could meet the need. The inventory also gives any later grant or funding request the evidence it needs.

**D** **Incorrect** (trap: Good but not best). Vouchers solve individual trips and may help in the short term, but they treat a county-wide gap one case at a time. Building a lasting resource starts with understanding the options and partners already present.

**Takeaway:** Before creating a new service, map the community's existing resources and potential partners to avoid duplication and build on local assets.

**Sources:** Kretzmann, J. P., & McKnight, J. L. (1993). Building Communities from the Inside Out: A Path Toward Finding and Mobilizing a Community's Assets. ACTA Publications, asset mapping: inventory of individual, associational and institutional assets

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### 186. INT-1551 (rev 2) · Best answer: A

A hospital emergency department social worker refers patients treated for opioid overdose to a community clinic that offers medication for opioid use disorder. For 2 years, a clinic nurse personally scheduled each referred patient. Since that nurse left 3 months ago, 7 of 9 referred patients never received an intake. Clinic leaders say they want the partnership to work. What is the social worker's **BEST** response?

**A.** Develop a written referral agreement with clinic leadership
**B.** Find a new staff member at the clinic to schedule intakes
**C.** Phone the clinic to book each patient's intake before discharge

**Why each option is right or wrong**

**A** **Correct.** Service networks last when the links between organizations are built into structure rather than resting on one person; NASW's case management standards note that written interorganizational agreements can be useful (NASW Standard 8). The pathway collapsed when the nurse left, and clinic leaders are willing partners. A written agreement can name roles, a scheduling process and a feedback loop that survive staff turnover. It beats B because a new personal contact restores referrals only until that person leaves, repeating the same failure.

**B** **Incorrect** (trap: Good but not best). A new contact may restore referrals quickly and could be named in a formal agreement. Relying on another single relationship leaves the pathway as fragile as before, which is exactly what failed when the nurse left.

**C** **Incorrect** (trap: Good but not best). Booking intakes from the emergency department may restore referrals quickly and is a good stopgap. It moves the single point of failure from the clinic nurse to the social worker, though, so the pathway still rests on one person's effort rather than an agreed process between the two organizations.

**Takeaway:** Make interagency referral pathways durable through formal agreements with defined roles, not one staff member's personal relationships.

**Sources:** NASW Standards for Social Work Case Management (2013), Standard 8: Interdisciplinary and Interorganizational Collaboration

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## 187. INT-1552 (rev 1) · Best answer: B

A community mental health agency sees almost no clients from a recently resettled refugee community, although resettlement staff report high stress among its families. Translated flyers mailed last spring produced no calls. The community has an active mutual assistance association whose elected leaders families consult on many decisions. What should the social worker do **FIRST**?

**A.** Ask community leaders to hand out the translated flyers

**B.** Plan the outreach jointly with the community's leaders

**C.** Host an open house at the agency with interpreters

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Involving leaders is a step in the right direction, but it asks them only to deliver a message the agency wrote, and that message already failed. This fits after the content and approach have been shaped with the community.

**B** **Correct.** Building a service network with a community means treating its own organizations as partners in designing services, not only as audiences (NASW Cultural Competence Standard 5). The association is a trusted local asset, and the failed flyers suggest the message or the service may not fit. Co-planning lets leaders shape how help is described, where it is offered and who delivers it (Kretzmann & McKnight, 1993). It beats A because handing leaders a flyer that already failed uses their trust without using their knowledge.

**C** **Incorrect** (trap: Right action, wrong time). An open house is welcoming, but it asks families to come to an unfamiliar agency before any trust exists. It fits later, once community partners have helped plan and promote it.

**Takeaway:** Engage underserved communities through their trusted organizations as co-planners of outreach and services, not just as distribution channels.

**Sources:** NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 5: Service Delivery; Kretzmann, J. P., & McKnight, J. L. (1993). Building Communities from the Inside Out: A Path Toward Finding and Mobilizing a Community's Assets. ACTA Publications, asset mapping: inventory of individual, associational and institutional assets

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### 188. INT-1654 (rev 1) · Best answer: C

An 84-year-old client who lives alone in a rural area needs help with meals, rides to the store, and a daily check-in while recovering from a hip fracture. The regional aging agency's in-home program has an 8-month waitlist. The client has belonged to a local congregation for 50 years, and two neighbors already stop by now and then. The client wants to stay home. What is the social worker's **BEST** plan for the coming months?

**A.** Recommend a move to an assisted living facility

**B.** Add the client to the in-home program waitlist and check in monthly

**C.** Turn the client's informal contacts into a weekly helper schedule

#### Why each option is right or wrong

**A** **Incorrect** (trap: Overrides the client's choice). Assisted living could meet these needs, but the client wants to stay home and has supports that can be organized. Recommending a move overrides a capable client's preference before less restrictive options are tried.

**B** **Incorrect** (trap: Good but not best). Joining the waitlist is sensible and can proceed alongside other steps. On its own, it leaves an 84-year-old recovering from a hip fracture without meals, rides or daily check-ins for months.

**C** **Correct.** Building a client's service network includes organizing natural supports, not only formal programs; the strengths model treats the community, including congregations and neighbors, as a source of resources (Rapp & Goscha, 2012). Here the client has deep congregation ties, neighbors who already visit, and an 8-month gap before formal help, so turning informal contact into a planned schedule for meals, rides and daily check-ins meets the needs now and fits the wish to stay home. It beats B because joining the waitlist is worth doing, but by itself it leaves the client without daily help for months (NASW, 2013, Standard 6).

**Takeaway:** When formal services are delayed, organize the client's natural supports into a planned network while waiting for formal help.

**Sources:** Rapp, C. A., & Goscha, R. J. (2012). The Strengths Model: A Recovery-Oriented Approach to Mental Health Services (3rd ed.). Oxford University Press, principles of the strengths model; NASW Standards for Social Work Case Management (2013), Standard 6: Service Planning, Implementation, and Monitoring

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### 189. INT-1032 (rev 1) · Best answer: B

At a community health center, a client asks the social worker for help 'being more assertive' with their partner. The client says the partner checks their phone, gets 'scary' when they disagree, and last month broke the client's phone during an argument. The client wants to practice speaking up at home. What should the social worker do **FIRST**?

**A.** Role-play a calm I-statement for the next disagreement
**B.** Assess the danger posed by the partner and plan for safety
**C.** Suggest joint sessions so both partners can practice

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Role-playing I-statements is a standard assertiveness step and may come later. Done first, it prepares the client to confront a partner who already becomes intimidating, before anyone knows how dangerous that could be.

**B** **Correct.** When a client describes possible partner abuse, safety comes before skill-building (SAMHSA TIP 25). Monitoring, intimidation when the client disagrees, and breaking the client's phone are warning signs. Speaking up to a controlling partner can raise the risk of retaliation, and better communication cannot make the partner stop. Assessing danger and planning for safety lets the client decide how, and whether, to assert themselves. It beats A because assertiveness practice may still help, but only after the risk is understood and the plan accounts for it.

**C** **Incorrect** (trap: Ignores risk). Joint sessions can put a person at risk when there are signs of abuse, because what they say in session may be punished at home (SAMHSA TIP 25). Individual safety assessment comes first.

**Takeaway:** Before teaching assertiveness with a partner, screen for abuse and plan for safety; speaking up to an abusive partner can raise the danger.

**Sources:** SAMHSA TIP 25 (1997), Ch. 2: Survivors of Domestic Violence: An Overview (safety; cautions on couples therapy)
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### 190. INT-1033 (rev 2) · Best answer: A

A client tells the social worker that a cousin keeps pressing them to co-sign a car loan. Each time, the client explains why they can't, the cousin argues with every reason, and the client ends up saying they will 'think about it.' Which assertiveness technique is **BEST** for the social worker to teach?

**A.** Calmly repeat the same short refusal each time
**B.** Give a stronger reason the cousin cannot dispute
**C.** Offer to help the cousin find a different co-signer

**Why each option is right or wrong**

**A** **Correct.** The broken-record technique has the client calmly restate the same brief refusal, without new explanations, until the other person accepts it (Smith, 1975). The cousin argues with every reason, so each explanation opens a new debate that ends with the client giving in. Repeating one short 'No, I won't co-sign' gives the cousin nothing new to argue with while staying respectful. It beats B because a better reason is still a reason to argue with; the problem is the pattern of justifying, not the quality of the reasons.

**B** **Incorrect** (trap: Misuses the technique). Searching for an airtight reason keeps the client defending the decision. The cousin has already shown they will dispute any reason given, so this repeats the pattern that leads to giving in.

**C** **Incorrect** (trap: Good but not best). A workable compromise is a real assertiveness skill when both sides can give a little. Here the client simply wants to say no, and offering an alternative reopens the negotiation in which the client keeps giving in. A firm, repeated refusal fits this pressure better.

**Takeaway:** When someone argues with every reason, teach the broken record: calmly repeat one brief refusal without adding justifications.

**Sources:** Smith, M. J. (1975). When I Say No, I Feel Guilty. Dial Press, broken record
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## 191. INT-1034 (rev 3) · Best answer: C

A client at a family services agency wants to ask their parent-in-law, who lives with the family, to stop giving the client's 6-year-old candy at bedtime. When the social worker offers a handout with an I-statement script, the client frowns and says, 'That's not how we talk to elders in my family.' What should the social worker do **FIRST**?

**A.** Coach the client to deliver the handout script as written
**B.** Help the client plan a respectful request made in private
**C.** Ask how the client would usually raise such a concern at home

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). I-statements are a standard assertiveness tool, but pressing the script after the client has said it clashes with family norms imposes one cultural style of directness and makes success less likely.

**B** **Incorrect** (trap: Right action, wrong time). A private, respectful request is a common way to adapt assertiveness where elders are owed deference, and it may be where this conversation lands. Proposing it now assumes the cultural rule instead of asking; in some families the right route is through a spouse or another relative.

**C** **Correct.** Assertiveness means stating one's needs while respecting others, and its form should fit the client's cultural values, which the client knows best (NASW Cultural Competence Standard 4). The client has signaled that the script clashes with family expectations but has not said what would fit: a private talk, going through a spouse, or different wording. Asking how concerns are usually raised with elders lets the worker shape the skill around the client's own knowledge. It beats B because a private request

may be exactly right, but choosing it for the client guesses at a norm the client is ready to explain.

**Takeaway:** When a client signals that a skill clashes with cultural norms, ask how things are done in their family before adapting the skill.

**Sources:** NASW Standards and Indicators for Cultural Competence in Social Work Practice (2015), Standard 4: Cross-Cultural Skills

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## 192. INT-1138 (rev 2) · Best answer: A

A client with a lifelong pattern of giving in to others starts assertiveness training at a housing agency. In 3 days, the client meets with a landlord who is holding back a security deposit the client needs for a new apartment. Only one session fits before the meeting, and the client asks to use it to prepare. Which practice situation is **BEST** for that session?

**A.** The upcoming talk to get the money returned

**B.** A low-stakes request, such as returning a store item

**C.** A long-standing conflict with a relative

#### Why each option is right or wrong

**A** **Correct.** Assertiveness training rehearses the real situations a client faces (Lange & Jakubowski, 1976), and work starts where the client is: with the problem the client names as most pressing (Hepworth et al., 11th ed.). Only one session remains before a meeting that decides whether the client can afford the new apartment, so rehearsing that conversation gives the session its greatest value. It beats B because a graded start with easy situations is the usual approach when several sessions are available, but here it would spend the only session before the meeting on practice the client did not ask for.

**B** **Incorrect** (trap: Right action, wrong time). Starting with low-risk situations and building up is a common, sound way to teach assertiveness when several sessions are available, and it can resume after the meeting. With one session left and the client asking to prepare, spending it on easy situations would leave the client unrehearsed for the conversation that matters most.

**C** **Incorrect** (trap: Good but not best). A long-standing family conflict may matter to the client, but it carries heavy emotion, has no deadline, and is not what the client asked to work on. It fits later in training, once basic skills are stronger.

**Takeaway:** Rehearse the real situation a client faces soon; graded practice from easy situations fits when time allows several sessions.

**Sources:** Lange, A. J., & Jakubowski, P. (1976). Responsible Assertive Behavior: Cognitive/Behavioral Procedures for Trainers. Research Press, behavior rehearsal; Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, engagement: starting where the client is

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### 193. INT-1035 (rev 2) · Best answer: B

Before discharge from an inpatient unit, a client with social anxiety must call a landlord to ask about a lease. The client watches the social worker's confident manner and says, 'I could never sound as smooth as you.' How should the social worker **BEST** demonstrate the call?

**A.** Model the call smoothly and confidently from start to finish
**B.** Model the call with some hesitation and spoken coping self-talk
**C.** Describe each step aloud and let the client try the call first

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). A smooth demonstration shows the skill clearly and suits confident learners. This client has just said they could never sound that smooth, so a flawless model may widen the gap they feel.

**B** **Correct.** Observers who doubt their ability gain more from a coping model, who shows some struggle and works through it, than from a mastery model who performs flawlessly (Schunk & Hanson, 1989). The client's remark shows they see the worker as unlike them. Modeling a pause, a small stumble, and spoken self-talk such as 'Slow down, just ask the question' shows a path the client can picture following (Bandura, 1977). It beats A because a polished demonstration confirms the client's belief that the gap is too wide to cross.

**C** **Incorrect** (trap: Right action, wrong time). Talking through the steps helps, but sending an anxious client to try first skips the demonstration that builds confidence. Modeling usually comes before the client's first attempt.

**Takeaway:** For clients who doubt themselves, demonstrate as a coping model, showing some struggle and self-talk, rather than a flawless performance.

**Sources:** Schunk, D. H., & Hanson, A. R. (1989). Influence of peer-model attributes on children's beliefs and learning. Journal of Educational Psychology, 81(3), 431-434, coping versus mastery models; Bandura, A. (1977). Social Learning Theory. Prentice-Hall, observational learning: attention, retention, reproduction, motivation
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### 194. INT-1036 (rev 1) · Best answer: D

In a middle school group on handling peer pressure, a 13-year-old accurately lists and acts out the refusal steps the social worker modeled. Later, the student says that when an older cousin turned down a vape at a party, friends mocked the cousin for weeks. Since then, the student laughs along when offered a vape. Which part of observational learning is **MOST** likely blocking use of the skill?

**A.** Attention
**B.** Retention
**C.** Reproduction
**D.** Motivation

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Attention problems would show up as missing key parts of the model. The student watched closely enough to list every refusal step, so attention is working.

**B** **Incorrect** (trap: Wrong model or concept). Retention means storing the behavior in memory. The student can still list the steps accurately, which shows the skill has been retained.

**C** **Incorrect** (trap: Wrong model or concept). Reproduction is the ability to perform what was observed. The student acted out the steps in the group, so the gap lies in whether they choose to use the skill with peers.

**D** **Correct.** Bandura described four processes in observational learning: attention, retention, reproduction, and motivation (Bandura, 1977). A person can learn a behavior fully and still not use it if they expect it to be punished. Watching the cousin get mocked was vicarious punishment, which lowers the incentive to refuse. The student's accurate recall and performance show the skill was attended to, stored, and can be produced. It beats C because the student has already acted out the steps in the group, so the problem is the expected cost, not the ability.

**Takeaway:** Learning a modeled behavior differs from performing it; seeing a model punished lowers motivation even when the skill is learned.

**Sources:** Bandura, A. (1977). Social Learning Theory. Prentice-Hall, observational learning: attention, retention, reproduction, motivation

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### 195. INT-1037 (rev 2) · Best answer: C

In an outpatient substance use group, a member who is 3 weeks sober dreads a coworker's retirement party where alcohol will be served. When the social worker offers to demonstrate refusing a drink, the member says, 'Easy for you, you've never been in my shoes.' Another member, sober for 2 years, has handled many similar work events. Which approach to teaching drink refusal is **MOST** effective?

**A.** Have the social worker demonstrate the refusal anyway

**B.** Give the member a list of refusal phrases to rehearse

**C.** Invite the member in longer recovery to demonstrate it

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). A demonstration by the leader is a common, useful way to model refusal skills. Here the member has already discounted the worker as someone who has never faced this, which weakens the model's influence.

**B** **Incorrect** (trap: Good but not best). Refusal phrases supply useful words, but a list skips seeing the skill performed, including tone and body language. Modeling followed by rehearsal builds skill and confidence more than written phrases alone.

**C** **Correct.** People learn more readily from a model they see as similar to themselves, especially when they doubt their own ability (Bandura, 1977; Schunk & Hanson, 1989). The newer member has rejected the worker as unlike them, while a peer who has faced the same work events offers a credible, similar model. A peer demonstration also draws on imitative behavior, one of the therapeutic factors of groups (Yalom & Leszcz, 2020). It beats A because the worker's demonstration may be technically sound, but the member has just said it won't feel relevant.

**Takeaway:** Choose models the client sees as similar and credible; in groups, a peer who has coped with the same situation can model best.

**Sources:** Bandura, A. (1977). Social Learning Theory. Prentice-Hall, observational learning: attention, retention, reproduction, motivation; Schunk, D. H., & Hanson, A. R. (1989). Influence of peer-model attributes on children's beliefs and learning. Journal of Educational Psychology, 81(3), 431-434, coping versus mastery models; Yalom & Leszcz (2020). The Theory and Practice of Group Psychotherapy (6th ed.). Basic Books, therapeutic factors: imitative behavior

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### 196. INT-1139 (rev 1) · Best answer: C

In a parenting group for families working toward reunification, the social worker models giving a 4-year-old a calm, specific instruction. The next week, a parent accurately lists every step of the demonstration and says they want to use it. The parent reports that at visits the words come out rushed and harsh. What should the social worker do **NEXT**?

**A.** Model the instruction again, more slowly this time

**B.** Offer the parent a reward for using the skill at visits

**C.** Have the parent practice the instruction with coaching

**D.** Explain the reasons the calm instruction works

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). A slower demonstration helps when an observer missed or forgot steps. This parent recalls every step, so watching again adds little; the difficulty lies in performing the skill.

**B** **Incorrect** (trap: Wrong model or concept). Incentives target the motivation process, which matters when someone can perform a skill but chooses not to. This parent wants to use it and is trying, so a reward would not fix the rushed delivery.

**C** **Correct.** Bandura's model of observational learning has four processes: attention, retention, reproduction, and motivation (Bandura, 1977). The parent lists every step (retention) and wants to use the skill (motivation), but the words come out rushed and harsh, so the gap lies in turning the memory into skilled action. Guided practice with feedback builds that reproduction. It beats A because another demonstration feeds retention, which is already intact; the parent needs to perform the skill, not watch it again.

**D** **Incorrect** (trap: Good but not best). Explaining why a skill works supports attention and buy-in, which this parent already shows by recalling every step and wanting to use it. The gap is in performance, which needs practice.

**Takeaway:** If a client remembers a modeled skill and wants to use it but performs it poorly, provide guided practice to build reproduction.

**Sources:** Bandura, A. (1977). Social Learning Theory. Prentice-Hall, observational learning: attention, retention, reproduction, motivation

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### 197. INT-0108 (rev 1) · Best answer: D

During a couples session about household spending, one partner rolls their eyes and says in a mocking tone, 'Oh, here comes the financial genius again.' Which of Gottman's Four Horsemen does this behavior show?

**A.** Criticism
**B.** Defensiveness
**C.** Stonewalling
**D.** Contempt

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Criticism attacks the partner's character instead of naming a specific complaint, as in 'You're so careless with money.' The sarcasm and eye-rolling here go further, signaling scorn and superiority.

**B** **Incorrect** (trap: Wrong model or concept). Defensiveness is self-protection through excuses, counterattacks or playing the innocent victim in response to a complaint. This partner is mocking the other, not defending against a complaint.

**C** **Incorrect** (trap: Wrong model or concept). Stonewalling is withdrawing from the interaction by going silent, looking away or shutting down, often when flooded. This partner stays engaged, but scornfully.

**D** **Correct.** Contempt communicates disrespect and superiority, often through sarcasm, mockery, name-calling or eye-rolling (Gottman, 1994). The eye roll and the mocking label 'financial genius' are classic signs. Gottman found contempt to be the strongest single predictor of divorce, so it is a priority target in couples work, often countered by nurturing fondness and admiration (Gottman & Silver, 1999). It beats A because criticism attacks the partner's character with a complaint, such as 'You never think about money,' while contempt adds open scorn.

**Takeaway:** In Gottman's model, sarcasm, mockery and eye-rolling signal contempt, the strongest predictor of divorce; criticism attacks character without open scorn.

**Sources:** Gottman, J. M. (1994). What Predicts Divorce? The Relationship Between Marital Processes and Marital Outcomes. Lawrence Erlbaum, Four Horsemen: contempt; Gottman, J. M., & Silver, N. (1999). The Seven Principles for Making Marriage Work. Crown, Four Horsemen; nurturing fondness and admiration
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### 198. INT-1054 (rev 1) · Best answer: C

A couple asks for help with frequent arguments. In the individual screening session, one partner says the other checks their phone daily, controls all the money, and once blocked the door to keep them from leaving an argument. The client says they fear the partner's reaction to anything said in joint sessions but wants to 'work on the relationship.' What is the **BEST** response?

**A.** Begin joint sessions with firm ground rules for respectful talk
**B.** Explore the client's hopes for the relationship in more depth
**C.** Refer the client to separate services addressing relationship abuse

**Why each option is right or wrong**

**A** **Incorrect** (trap: Ignores risk). Ground rules cannot make joint sessions safe when one partner fears the other's reaction. With signs of coercive control, what the client says in session can lead to retaliation at home. Joint work may be considered only for low-level conflict in which both partners feel safe.

**B** **Incorrect** (trap: Good but not best). Exploring goals is normally part of screening, but this client has already described control, a blocked exit and fear. Those facts decide the treatment format now; the client's hopes can be explored further within individual services.

**C** **Correct.** Individual screening exists to find intimate partner violence before conjoint work begins. Phone monitoring, financial control, a blocked exit and fear of the partner's reaction point to coercive control, a pattern in which joint therapy can raise the client's risk after sessions. Individual, violence-informed services with safety planning protect the client, and the worker does not reveal the disclosure to the other partner (SAMHSA TIP 25). It beats B because the client's hopes matter, but the facts already establish the danger, and more exploration only delays protection.

**Takeaway:** When screening reveals coercive control or fear of a partner, replace conjoint therapy with individual, violence-informed services and safety planning.

**Sources:** SAMHSA TIP 25 (1997), Ch. 2: Survivors of Domestic Violence: An Overview (safety; cautions on couples therapy)
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## 199. INT-1055 (rev 1) · Best answer: A

In emotionally focused couples therapy, during which stage does the social worker help partners see their pattern of pursuing and withdrawing as a shared enemy rather than either partner's fault?

**A.** Cycle de-escalation
**B.** Restructuring interactions
**C.** Consolidation

**Why each option is right or wrong**

**A** **Correct.** Emotionally focused therapy moves through three stages. In the first, cycle de-escalation, the worker tracks the negative interaction pattern, reaches the attachment fears beneath it, and frames the pattern itself as the problem both partners face. This lowers blame and makes safer contact possible (Johnson, 2019). It beats B because restructuring comes next: once the pattern is named and calmer, the withdrawn partner re-engages and the pursuing partner softens, creating new bonding events.

**B** **Incorrect** (trap: Wrong model or concept). Restructuring is the second stage. It builds on a calmer, already-named pattern by helping the withdrawer re-engage and the pursuer soften. Framing the pattern as a shared enemy is the groundwork laid before this stage begins.

**C** **Incorrect** (trap: Wrong model or concept). Consolidation is the final stage, when partners apply their new ways of relating to old problems and build a shared story of how they changed. It comes well after the pursue-withdraw pattern is first named.

**Takeaway:** EFT stage one de-escalates the negative cycle by framing it as the couple's common enemy; restructuring and consolidation follow.

**Sources:** Johnson, S. M. (2019). The Practice of Emotionally Focused Couple Therapy: Creating Connection (3rd ed.). Routledge, Stage 1: cycle de-escalation

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## 200. INT-1056 (rev 1) · Best answer: B

During a video session, one partner raises a complaint about household chores. The other partner goes silent, looks away from the camera, and says, 'My heart is pounding. I can't do this right now.' The social worker uses the Gottman Method. What should the social worker do **FIRST**?

**A.** Ask the quiet partner to share their view now

**B.** Call a break of at least 20 minutes to self-soothe

**C.** Teach both partners a structured listening exercise

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misuses the technique). Pressing a flooded partner to respond usually deepens the shutdown or sparks a defensive reply. Their view matters, and it is best asked for after a self-soothing break, when they can think and listen again.

**B** **Correct.** Gottman describes flooding: physical arousal so high that a partner shuts down, the state behind stonewalling. The antidote is a break of at least 20 minutes spent self-soothing, because the body needs that long to calm, followed by a return to the topic (Gottman & Silver, 1999). A pounding heart and 'I can't do this' signal flooding. It beats C because listening skills are useful for this couple, but a flooded partner cannot take in new skills; they help once both partners are calm.

**C** **Incorrect** (trap: Right action, wrong time). Structured listening can help this couple talk about chores, but it is a skill to build once arousal drops. Teaching it to a partner who is flooded right now is unlikely to work.

**Takeaway:** When a partner is flooded, pause for at least 20 minutes of self-soothing before continuing any discussion or skill work.

**Sources:** Gottman, J. M., & Silver, N. (1999). The Seven Principles for Making Marriage Work. Crown, flooding and stonewalling; self-soothing breaks

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### 201. INT-1057 (rev 1) · Best answer: C

At intake for couples therapy, one partner says they have nearly decided to divorce and came only because the other partner pleaded. The other partner wants to save the marriage and asks to start work on communication right away. Both agree to a few sessions. Which approach is **BEST**?

**A.** Begin couples therapy focused on communication skills
**B.** Refer the partner leaning toward divorce for individual therapy
**C.** Help both partners reach clarity about their next step

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Communication training is the right work once both partners commit to improving the relationship. Here one partner has nearly decided to leave, so skills work would rest on a goal only one partner shares and is likely to stall.

**B** **Incorrect** (trap: Hands off too soon). Sending the leaning-out partner elsewhere hands off the couple's central question instead of addressing it. Both partners agreed to a few sessions together, so the work can begin with the couple.

**C** **Correct.** Couples therapy needs both partners willing to work on the relationship. When one partner leans toward divorce and the other toward repair, this mixed agenda tends to stall standard couples work. Discernment counseling responds by helping the couple reach clarity and confidence about their next step, using individual conversations with each partner, before any decision to begin couples therapy (Doherty et al., 2016). It beats A because the hopeful partner's request is understandable, but skills training assumes a shared commitment that the facts show is missing.

**Takeaway:** When one partner leans toward divorce and the other toward repair, clarify the relationship's direction before beginning couples therapy.

**Sources:** Doherty, W. J., Harris, S. M., & Wilde, J. L. (2016). Discernment counseling for "mixed-agenda" couples. Journal of Marital and Family Therapy, 42(2), 246-255, whole article
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.27 Couples treatment approaches · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 202. INT-1153 (rev 3) · Best answer: C

A couple seeks help for frequent arguments about parenting. In separate screening interviews, each partner describes one argument 8 months ago in which both pushed each other, and each takes responsibility for their part. Both say they feel safe at home, make money and social decisions together, and want to stay together. What is the **MOST** appropriate plan?

**A.** Refer each partner to a separate violence intervention program first
**B.** Begin couples sessions once both sign a no-violence agreement, with no further screening
**C.** Begin couples sessions that include periodic individual safety check-ins

**Why each option is right or wrong**

**A** **Incorrect** (trap: Hands off too soon). Separate services are the right path when screening shows coercive control, fear, or serious violence, because joint sessions could then endanger a victim. These interviews describe one mutual, low-level incident without those features, so this plan withholds appropriate couples care.

**B** **Incorrect** (trap: Misuses the technique). Couples work fits this couple, and a no-violence agreement can be part of it. A signed promise is not a safety assessment, though. After any physical incident, safety is monitored privately throughout conjoint treatment, because a partner may not voice fear in front of the other.

**C** **Correct.** Johnson (2008) distinguishes situational couple violence, which grows out of conflict without a pattern of control, from coercive, controlling violence. Conjoint treatment may be considered for low-level situational violence when both partners feel safe and accept responsibility, with continued individual safety screening (Stith, McCollum & Rosen, 2011). The separate interviews show one mutual push, shared decisions, no fear, and a shared wish to stay together. It beats A because nothing here shows the coercive control or fear that would make joint sessions unsafe.

**Takeaway:** Mutual, isolated, low-level partner violence without fear can permit joint sessions, provided safety is rechecked privately throughout.

**Sources:** Johnson, M. P. (2008). A Typology of Domestic Violence: Intimate Terrorism, Violent Resistance, and Situational Couple Violence. Northeastern University Press, situational couple violence versus intimate terrorism; Stith, S. M., McCollum, E. E., & Rosen, K. H. (2011). Couples Therapy for Domestic Violence: Finding Safe Solutions. American Psychological Association, assessment and screening for inclusion in conjoint treatment

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.27 Couples treatment approaches · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 203. INT-1164 (rev 2) · Best answer: C

During a video couples session, one partner opens a talk about chores by saying, 'You never do anything around here unless I nag you. You only think about yourself.' The other partner sits back and calmly starts to explain their week. The social worker uses the Gottman Method. What is the **BEST** intervention?

**A.** Ask the listening partner to repeat back what they heard

**B.** Suggest a 20-minute break so both partners can calm down

**C.** Coach the speaker to restate the complaint as a feeling and need

**D.** Explore how chores were divided in each partner's childhood home

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Reflective listening helps once a complaint is stated without blame. Having the partner mirror a character attack keeps the criticism at the center and invites defensiveness; restating comes first.

**B** **Incorrect** (trap: Misuses the technique). A self-soothing break of about 20 minutes is the Gottman response to flooding. The listener is calm and engaged, so stopping now interrupts a moment the social worker can use.

**C** **Correct.** Gottman names criticism, an attack on a partner's character, as one of the Four Horsemen. Its antidote is a gentle start-up, in which the speaker describes their own feeling and states a positive need without blame (Gottman & Silver, 2015). 'You

never' and 'you only think about yourself' attack character instead of naming a specific complaint, and there is no mockery or name-calling (marks of contempt). This is criticism, so coaching the speaker to restate the complaint changes the pattern at its source. It beats A because having the partner repeat back an attack rehearses the criticism instead of changing it.

**D** **Incorrect** (trap: Wrong model or concept). Family-of-origin patterns are a Bowen-style focus and may matter later. The Gottman approach first changes how the couple talks to each other in the moment.

**Takeaway:** In the Gottman Method, the antidote to criticism is a gentle start-up: describe your own feeling and state a positive need, without blame.

**Sources:** Gottman, J. M., & Silver, N. (2015). The Seven Principles for Making Marriage Work (rev. ed.). Harmony, Four Horsemen; soften startup

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.27 Couples treatment approaches · **Qualifier** BEST · **Cognitive** application · **Difficulty** 3 of 3

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### 204. INT-0113 (rev 1) · Best answer: B

A client plans to ask a supervisor at work for a schedule change to care for a relative. The client says they freeze whenever they speak with people in authority. Which approach is **BEST** for helping the client prepare during the session?

**A.** Write a script for the client to read to the supervisor

**B.** Rehearse the request with the social worker acting as supervisor

**C.** Offer to call the supervisor and explain the client's needs

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). A script can help a client organize thoughts and may be a useful first draft. Without rehearsal, though, the client still meets the freezing response the first time they speak with the supervisor.

**B** **Correct.** Role play lets a client practice a hard conversation in a safe setting, get feedback and try again until the skill feels manageable. Successful practice is a mastery experience, the strongest source of self-efficacy (Bandura, 1977). Because this client freezes with authority figures, rehearsing with the social worker in the supervisor's role targets the exact moment that goes wrong. It beats A because a written script supplies words but no practice delivering them under pressure, which is where the client struggles.

**C** **Incorrect** (trap: Boundary slip or collusion). Calling for the client does a task the client can learn to do and may reinforce the belief that they cannot handle authority figures. Advocacy fits when a client is unable to act for themselves.

**Takeaway:** When a client fears a specific conversation, role-play rehearsal builds skill and self-efficacy better than scripts or acting on the client's behalf.

**Sources:** Bandura, A. (1977). Self-efficacy: Toward a unifying theory of behavioral change. Psychological Review, 84(2), 191-215, performance accomplishments as a source of efficacy

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### 205. INT-1038 (rev 1) · Best answer: A

In a role-play, which technique has the client portray the other person in a conflict while the social worker portrays the client?

**A.** Role reversal
**B.** Doubling
**C.** Mirroring
**D.** Empty chair

#### Why each option is right or wrong

**A** **Correct.** In role reversal, the client takes the part of the other person, such as a parent or partner, while someone else plays the client. Speaking from the other person's position helps the client understand that person's view and hear their own words from the outside (Blatner, 1996). It beats C because in the mirror technique the client steps out of the scene to watch someone portray them, but never takes on the other person's role.

**B** **Incorrect** (trap: Wrong model or concept). In doubling, someone stands beside the client and voices feelings or thoughts the client may be holding back. The client keeps their own role throughout.

**C** **Incorrect** (trap: Wrong model or concept). In mirroring, the client watches from outside the scene while another person portrays them, to gain perspective. The client does not play the other person, which is the defining feature here.

**D** **Incorrect** (trap: Wrong model or concept). In the empty-chair technique, the client speaks to an imagined person in an empty seat. No one else plays the client, and the other person is imagined rather than portrayed by the social worker.

**Takeaway:** Role reversal: the client plays the other person while someone plays the client, building empathy and perspective on the conflict.

**Sources:** Blatner, A. (1996). Acting-In: Practical Applications of Psychodramatic Methods (3rd ed.). Springer Publishing, role reversal, doubling, and mirror techniques

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.28 Using role-play in sessions · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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### 206. INT-1039 (rev 2) · Best answer: A

In a residential program, a 15-year-old practices asking staff for an extra phone call instead of demanding one. In the first role-play, the teen uses a calm voice and says 'please,' but stands very close and talks over the staff member's reply. What should the social worker do **NEXT**?

**A.** Praise what worked, coach one change, and run it again
**B.** Model the whole request again with the right distance
**C.** Assign the teen to try the request with staff later today

**Why each option is right or wrong**

**A** **Correct.** In skills training, feedback after a role-play starts with specific praise for what the client did well, adds one or two concrete suggestions, and is followed by another role-play so the client can try the change right away (Bellack et al., 2004). The teen's calm voice and 'please' are real gains worth naming; standing close and interrupting are later targets, taken one at a time. It beats C because real-world practice is the final step, after repeated rehearsal in session has made the new behavior more reliable.

**B** **Incorrect** (trap: Good but not best). Repeating the demonstration helps a learner who missed the skill entirely. This teen already shows the calm voice and 'please', so a full re-demonstration skips the step that shapes the teen's own attempt: naming those gains, coaching one specific change and rehearsing it right away.

**C** **Incorrect** (trap: Right action, wrong time). Practice outside the session is the goal, but assigning it after one partly successful rehearsal comes too early. More role-plays with feedback come first, so the teen practices the improved version.

**Takeaway:** After a role-play, praise specific strengths, give one or two concrete suggestions, and rehearse again before assigning real-life practice.

**Sources:** Bellack, A. S., Mueser, K. T., Gingerich, S., & Agresta, J. (2004). Social Skills Training for Schizophrenia: A Step-by-Step Guide (2nd ed.). Guilford Press, steps of social skills training: feedback and repeated role-play
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.28 Using role-play in sessions · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 3 of 3

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## 207. INT-1140 (rev 2) · Best answer: B

A client with schizophrenia in a supported employment program has had five job interviews without an offer. The client says the interviews 'went fine' and cannot say what might have gone wrong. What should the social worker do **FIRST** to help the client improve?

**A.** Teach a standard set of tips for answering interview questions
**B.** Ask the client to role-play an interview while the worker observes
**C.** Help the client request feedback from the interviewers

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Teaching interview skills will likely be part of the plan, but choosing tips before seeing how the client interviews guesses at the problem. Assessment through observation comes first.

**B** **Correct.** Social skills training starts with assessment, and a role-played situation lets the worker see the actual behaviors, such as eye contact, voice, and length of answers, that self-report misses (Bellack et al., 2004). The client says the interviews went fine and cannot name a problem, so asking more questions is unlikely to reveal the gaps. Observing a practice interview pinpoints which skills to teach. It beats A because generic tips may target skills the client already has and miss the ones costing offers.

**C** **Incorrect** (trap: Good but not best). Employer feedback can be informative, so this is a reasonable thought. Many interviewers decline to give it, though, and waiting for replies delays help, while the worker can see the client's interview behavior directly in a practice interview today.

**Takeaway:** When a client cannot describe what goes wrong in a social situation, use role-play to observe and assess skills before teaching.

**Sources:** Bellack, A. S., Mueser, K. T., Gingerich, S., & Agresta, J. (2004). Social Skills Training for Schizophrenia: A Step-by-Step Guide (2nd ed.). Guilford Press, Ch. 3: Assessment of social skills
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.28 Using role-play in sessions · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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## 208. INT-1109 (rev 1) · Best answer: B

In a jail reentry program, a client says, 'Things got messed up with my probation officer last week.' The social worker responds, 'When you say things got messed up, what happened?' Which listening skill is the social worker using?

**A.** Paraphrase
**B.** Clarification
**C.** Interpretation

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). A paraphrase restates the content of what the client said in fresh words. The worker here does not restate anything; the worker asks the client to explain a vague phrase, which is clarification.

**B** **Correct.** Clarification asks the client to make a vague statement specific so the social worker understands it accurately. 'Messed up' could mean a missed meeting, an argument or a violation, so the worker checks what the phrase means before responding (Hepworth et al.). It beats A because a paraphrase would restate the content, such as 'Something went wrong with your probation officer,' without asking for the missing detail.

**C** **Incorrect** (trap: Wrong model or concept). Interpretation offers a new meaning or pattern, such as linking the problem to the client's feelings about authority. The worker adds no new meaning; the response only seeks the client's own account of what happened.

**Takeaway:** Clarification checks the meaning of vague words; paraphrase restates content; interpretation adds a new meaning.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Verbal following, exploring, and focusing skills: seeking concreteness
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.3 Active listening and observation skills · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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## 209. INT-1110 (rev 2) · Best answer: D

In a first session at a community mental health center, an adult client talks rapidly about eviction worries, a recent breakup, poor sleep, and conflict with a sibling, jumping between topics. Twenty minutes remain in the session. What is the **BEST** next move to begin setting priorities?

**A.** Focus the rest of the session on the eviction threat
**B.** Reflect how overwhelmed the client seems to feel right now
**C.** Ask closed questions to cover each problem in turn
**D.** Summarize the concerns and ask which one matters most

**Why each option is right or wrong**

**A** **Incorrect** (trap: Too directive). Housing threats are serious, and the client may well choose this first. But the stem shows no emergency, and the worker deciding the focus alone skips the client's view of what is most pressing. A summary lets the client choose.

**B** **Incorrect** (trap: Good but not best). Reflecting the feeling of being overwhelmed is a sound empathic response and could come just before the summary. On its own, it does not organize the concerns or move toward priorities, which the remaining time calls for.

**C** **Incorrect** (trap: Misuses the technique). Closed questions gather facts but would turn the session into a checklist and use up the time without the client choosing a focus. They fit later, when a chosen problem needs specific detail.

**D** **Correct.** A summary ties scattered statements together, shows the client they were heard, and creates a natural transition to the next task (Hepworth et al.). With four concerns raised rapidly and limited time left, summarizing them and asking the client where to start moves toward shared priorities while keeping the client in charge of the agenda (NASW Code 1.02). It beats A because eviction may turn out to be the client's top concern, but the worker choosing it without asking takes the decision from the client and may miss what feels most urgent to them.

**Takeaway:** When a client raises many concerns at once, summarize them and let the client choose where to start; this shows listening and sets shared priorities.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Verbal following skills: summarizing responses; NASW Code of Ethics (2021), 1.02
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.3 Active listening and observation skills · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 210. INT-1111 (rev 2) · Best answer: A

At an adult day program, a 79-year-old client says a new sleep medicine 'is working fine.' During a 30-minute meeting with the social worker, the client's head drops forward twice and the client loses track of questions. What should the social worker do **FIRST**?

**A.** Share what was observed and ask about daytime drowsiness
**B.** Contact the prescriber about a possible medication side effect
**C.** Accept the client's report and continue the meeting
**D.** Suggest the client take the medicine earlier in the evening

**Why each option is right or wrong**

**A** **Correct.** Observation is part of active listening: what the social worker sees can add to or contradict what the client says. The report that the medicine 'is working fine' conflicts with visible drowsiness and lost focus. Naming the observation tentatively and asking about daytime sleepiness lets the client explain and gathers details the prescriber will need, such as timing and any falls (Hepworth et al.). It beats B because contacting the prescriber is likely the next step, but doing it first skips the client's own account and their agreement to share it.

**B** **Incorrect** (trap: Right action, wrong time). Possible medication side effects in an older adult should reach the prescriber, and this will likely follow. First, the social worker

checks the observation with the client, gathers details, and seeks the client's agreement to share them.

**C** **Incorrect** (trap: Ignores risk). Taking the report at face value ignores direct evidence of drowsiness in an older adult, which can raise the risk of falls and confusion. Observations that contradict self-report call for a gentle follow-up question.

**D** **Incorrect** (trap: Outside the social work role). Changing when a medication is taken is a prescribing decision. Social workers bring concerns to the prescriber; they do not adjust medication timing or doses, however well meant.

**Takeaway:** When observed behavior contradicts a client's report, share the observation tentatively and ask about it before acting on it.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Assessment: sources of information, including direct observation of nonverbal behavior
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.3 Active listening and observation skills · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 211. INT-1112 (rev 1) · Best answer: C

In a senior housing building, a resident tells the social worker, 'My kids keep pushing me to move to assisted living. I've lived in this apartment for 40 years.' Which response **BEST** shows accurate reflective listening?

**A.** 'Your children are worried and just want you to be safe.'
**B.** 'Why don't you want to consider assisted living?'
**C.** 'Leaving this home would mean losing a big part of your life.'
**D.** 'Many people your age feel that same way about moving.'

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). The children's concern may be real, but restating it moves the focus from the resident to the family and can sound like the worker agrees with them. Their worries can be discussed later, once the resident feels heard.

**B** **Incorrect** (trap: Misuses the technique). A 'why' question asks the resident to defend a position and tends to raise defensiveness. It gathers reasons but shows no understanding of what the apartment means. A reflection invites the same information more safely.

**C** **Correct.** A reflection captures the feeling and meaning beneath the client's words and offers it back for the client to confirm or correct (Miller & Rollnick, 2023). The resident's mention of 40 years in the apartment signals attachment and loss, not just a housing question. Reflecting that meaning shows the worker understood what the move would cost and invites the resident to say more. It beats A because voicing the children's worries shifts attention away from the resident's experience and can sound like taking the family's side.

**D** **Incorrect** (trap: Good but not best). Normalizing can reduce shame, but offered first it brushes past this resident's particular attachment to the home and can feel dismissive. It works better after the resident's own feelings have been reflected and understood.

**Takeaway:** Reflective listening returns the feeling and meaning behind the client's words; avoid 'why' questions, taking sides, or early normalizing.

**Sources:** Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, Engaging: reflective listening
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.3 Active listening and observation skills · **Qualifier** BEST · **Cognitive** application · **Difficulty** 1 of 3

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## 212. INT-1180 (rev 1) · Best answer: B

An 80-year-old client comes to a senior center for help with a benefits renewal form that is due tomorrow. For 20 minutes the client has talked about past jobs and old friends, and the form is still blank. The client seems to enjoy the visit and has no other appointments today. Which response is **MOST** helpful?

**A.** Keep listening warmly and let the client set the pace of the visit
**B.** Briefly reflect the stories' meaning, then return to the form
**C.** Explain that the form comes first and the stories can wait

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Following the client's lead fits open-ended sessions and may be what the client enjoys most. Here the client came for a form due tomorrow, and letting the visit drift risks a lapse in benefits the client is counting on.

**B** **Correct.** Focusing brings a conversation back from tangents to the task the client came for, and it works best when the worker first shows the client was heard (Hepworth et al.). A brief reflection honors what reminiscing means to this client, and steering back to the form protects benefits that are due tomorrow. It beats A because following the client's lead is usually right, but here open-ended listening lets an urgent goal the client chose slip away; active listening includes judging when to refocus.

**C** **Incorrect** (trap: Misuses the technique). Returning to the task is right, but doing it without first acknowledging the stories can feel dismissive to an older adult who values the conversation, and it may make the rest of the visit feel rushed and impersonal.

**Takeaway:** When tangents threaten an urgent client goal, briefly reflect what was shared and then refocus; listening includes knowing when to redirect.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Focusing skills
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.3 Active listening and observation skills · **Qualifier** MOST · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 213. INT-1185 (rev 1) · Best answer: D

During a video session from home, a 67-year-old client who lives alone suddenly begins slurring words. The social worker sees that one side of the client's mouth droops and that the client cannot lift their right arm to adjust the camera. When asked how they feel, the client says, 'I'm just tired. I'll lie down.' What should the social worker do **FIRST**?

**A.** Ask about the client's recent sleep and medication changes
**B.** Call the client's emergency contact to check on them
**C.** Suggest the client rest and reconnect in one hour
**D.** Request an ambulance to the client's home

**Why each option is right or wrong**

**A** **Incorrect** (trap: Ignores risk). Gathering history is useful for most changes in a client's condition. Here the observed signs already warrant emergency care, and every minute spent on questions delays treatment that works best when given quickly.

**B** **Incorrect** (trap: Right action, wrong time). Reaching the emergency contact is a sensible follow-up once emergency services are on the way. Relying on a relative first wastes minutes that matter in stroke care, and the contact may be slow or unreachable.

**C** **Incorrect** (trap: Ignores risk). A client's wishes usually guide practice, but these signs point to a medical emergency that can also cloud judgment. Waiting an hour could cost the window for effective stroke treatment.

**D** **Correct.** Sudden facial droop, one-sided arm weakness and slurred speech are stroke warning signs, and the American Stroke Association advises calling 911 immediately, even if symptoms fade. The worker has observed these signs directly and the client has already answered, so more questions only delay care; the client's reassurance does not change the emergency. It beats B because an emergency contact may be slow or unreachable, and calling them is a useful step after emergency services are dispatched.

**Takeaway:** Observed signs of a medical emergency outweigh a client's reassurance; call emergency services first, then notify supports.

**Sources:** American Stroke Association (2026). Stroke Symptoms: B.E. F.A.S.T., Warning signs; call 911 immediately

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.3 Active listening and observation skills · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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## 214. INT-1113 (rev 3) · Best answer: C

A client in weekly outpatient therapy has begun calling the agency 4 to 6 times a day to talk through daily conflicts. The calls started 2 weeks ago, after months with no contact between sessions. They are not about emergencies, and front desk staff are struggling to keep up. What should the social worker do **FIRST**?

**A.** Return each call promptly to protect the working relationship

**B.** Set a scheduled check-in time and crisis plan with the client

**C.** Ask the client what has changed for them recently

**Why each option is right or wrong**

**A** **Incorrect** (trap: Boundary slip or collusion). Returning every call feels supportive but teaches the client that unlimited contact is available and can build dependence. It also sets up a pattern the worker cannot sustain, which risks a more painful rupture later (Reamer, 2020).

**B** **Incorrect** (trap: Right action, wrong time). Clear, predictable limits on contact, a set check-in plus crisis resources, protect both the client and the work (Linehan, 1993), and they will likely be needed. Setting them before learning why the calls suddenly began skips assessment and could miss a new stressor or worsening symptoms.

**C** **Correct.** Assessment comes before intervention. A sudden change, frequent calls after months without contact between sessions, signals that something has shifted, such as a new stressor, rising symptoms or a problem the client has not yet named (Hepworth et

al.). Exploring the calls with the client shows what they provide and whether risk has changed, and lets the two design limits that still meet the need. It beats B because a check-in schedule is the likely next step, but setting it before understanding the calls could miss a worsening problem and feel like rejection.

**Takeaway:** When a client's contact pattern suddenly changes, explore what prompted it before setting limits; then agree on a predictable check-in plan.

**Sources:** Hepworth et al. Direct Social Work Practice (11th ed.). Cengage, Assessment: exploring and understanding problems and strengths; Linehan, M. M. (1993). Cognitive-Behavioral Treatment of Borderline Personality Disorder. Guilford Press, observing limits procedures; Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, managing boundaries; dependence and availability

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.4 Setting limits and boundaries with clients · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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## 215. INT-1114 (rev 1) · Best answer: C

In a supportive housing program, a client with schizoaffective disorder invites the social worker to a public gallery showing of the client's paintings. Showing work in public was a recovery goal the two set a year ago. The event is an evening opening at a community center. What is the **MOST** appropriate response?

**A.** Decline, explaining that attending would create a dual relationship

**B.** Ask the client to bring photos of the paintings to a session

**C.** Attend briefly after agreeing on boundaries with the client

**D.** Attend and join the client's celebration dinner afterward

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). The Code warns against dual relationships that risk exploitation or harm, not against every contact outside the office (NASW Code 1.06(c)). A short, planned visit to a public event tied to a treatment goal is a boundary crossing, not a second relationship.

**B** **Incorrect** (trap: Good but not best). Seeing the paintings in session is warm and safe, and it may suit a worker who cannot attend. Here, though, the public showing itself was the goal, and redirecting to photos misses a chance to support it.

**C** **Correct.** Not every boundary crossing is harmful. A brief, planned crossing that serves a client's treatment goal can be appropriate when the worker weighs the risks, keeps the role clear and documents the reason (Reamer, 2020). Attending a public showing that marks a recovery goal the two set together supports the client's progress and community ties. Agreeing in advance on how long the worker will stay and how they will greet each other keeps the role clear. It beats B because viewing photos in session is safe but misses a milestone the client specifically invited the worker to witness.

**D** **Incorrect** (trap: Boundary slip or collusion). Joining a private celebration moves from a brief, purposeful appearance into a social role. That blurs the professional relationship and can confuse the client about what the relationship is.

**Takeaway:** A brief, planned boundary crossing that serves a client's goal, such as attending a public milestone, can be appropriate; avoid social roles.

**Sources:** Reamer, F. G. (2020). Boundary Issues and Dual Relationships in the Human Services (3rd ed.). Columbia University Press, boundary crossings versus boundary violations; NASW Code of Ethics (2021), 1.06(c)
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## 216. INT-1115 (rev 1) · Best answer: A

A client often runs late, and the social worker has been holding firm on ending sessions on time. With 2 minutes left today, the client says their partner shoved them last night, and they are going home after the session. Another client is waiting. What is the **BEST** action?

**A.** Briefly extend the session to check the client's safety tonight
**B.** End on time and make this the first topic next session
**C.** Give a hotline number and end the session on time

#### Why each option is right or wrong

**A** **Correct.** Limits on session time are useful, but safety comes before structure. A disclosure of recent physical violence by a partner the client is returning to tonight calls for a brief check of immediate danger and a basic safety plan before the client leaves (Davies, Lyon & Monti-Catania, 1998). A short, purposeful extension is a boundary crossing that serves the client, not a violation (Gutheil & Gabbard, 1993). It beats C because a hotline number is a helpful resource, but it does not tell the worker whether the client is in danger tonight.

**B** **Incorrect** (trap: Ignores risk). Holding the time limit protects the structure of the work and is usually right with chronic lateness. Here, a new disclosure of violence, with the client returning home tonight, makes waiting a week unsafe.

**C** **Incorrect** (trap: Good but not best). Hotline information is a useful part of safety planning, but handing it over without asking about tonight's danger leaves the risk unknown. It works best as part of a brief safety check, not in place of one.

**Takeaway:** Hold session-time limits, but make a brief, purposeful exception when a client discloses current danger at the end of a session.

**Sources:** Gutheil, T. G., & Gabbard, G. O. (1993). The concept of boundaries in clinical practice: Theoretical and risk-management dimensions. American Journal of Psychiatry, 150(2), 188-196, boundary crossings versus violations; time as a boundary dimension; Davies, J., Lyon, E., & Monti-Catania, D. (1998). Safety Planning with Battered Women: Complex Lives/Difficult Choices. Sage, safety planning
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## 217. INT-1116 (rev 1) · Best answer: B

During home-based services, a client has made comments about the social worker's body. At the last visit, the social worker clearly stated that such comments must stop. Today the client repeats them and briefly stands between the social worker and the door. What should the social worker do **FIRST**?

**A.** Restate the limit firmly and continue with the visit
**B.** End the visit and review safety with the supervisor
**C.** Explore what the comments mean to the client

**Why each option is right or wrong**

**A** **Incorrect** (trap: Ignores risk). Restating the limit was the right step the first time. The comments have continued and now include blocking the door, so another warning while staying in the home does not address the risk.

**B** **Correct.** Worker safety and a clear response to repeated harassment come first. The limit was already stated and the behavior has escalated to blocking the exit, so the social worker ends the visit and brings the incident to the supervisor for reporting and a plan, such as office-based or paired visits (NASW Guidelines for Social Worker Safety, 2013). Ending this visit is not ending services, and the plan protects the client's care as well (NASW Code 1.17(b)). It beats A because repeating a limit that has already failed, while the client controls the exit, leaves the worker at risk.

**C** **Incorrect** (trap: Right action, wrong time). Understanding what drives the comments may matter clinically, but exploring now, in the home with the exit blocked, puts the worker at risk and treats harassment as ordinary material. It can be addressed later in a safer setting.

**Takeaway:** When a stated limit fails and behavior escalates on a home visit, leave and involve the supervisor in a safety plan; don't abandon services.

**Sources:** NASW (2013). Guidelines for Social Worker Safety in the Workplace, Standard 6: risk assessment for field visits; Standard 9: post-incident reporting and response; NASW Code of Ethics (2021), 1.17(b)
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## 218. INT-1181 (rev 1) · Best answer: C

In Landreth's ACT model for setting limits in child-centered play therapy, what does the social worker do **FIRST**?

**A.** State the limit on the behavior
**B.** Offer an acceptable alternative
**C.** Acknowledge the feeling behind the behavior
**D.** Name the consequence if it continues

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Communicating the limit is the second step. Leading with it treats the behavior as the whole story and often escalates a child whose feelings have not yet been recognized.

**B** **Incorrect** (trap: Right action, wrong time). Targeting an alternative is the third step, giving the child an acceptable way to express the feeling. Offered first, it can sound like a distraction rather than a response to what the child feels.

**C** **Correct.** Landreth's ACT sequence begins by acknowledging the child's feeling or wish ('You're really mad at me'), then communicating the limit ('but I'm not for hitting'), then targeting an acceptable alternative ('you can hit the bop bag'). Starting with the feeling tells the child the emotion is accepted even though the act is not (Landreth, 2012). It beats A because stating the limit first is the most common error; it skips the empathy that helps the child accept the limit.

**D** **Incorrect** (trap: Wrong model or concept). Consequences enter only if a child keeps breaking a limit after the ACT steps, usually framed as a choice the child can make. Naming one first makes the response punitive and skips the model's three steps.

**Takeaway:** Limit setting with children: acknowledge the feeling, communicate the limit, then target an acceptable alternative.

**Sources:** Landreth, G. L. (2012). Play Therapy: The Art of the Relationship (3rd ed.). Routledge, Therapeutic limit setting: the ACT model

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## 219. INT-1188 (rev 2) · Best answer: B

A client with a trauma history yelled profanities at the front desk staff after a long wait. In the next session, the client explained that crowded waiting rooms feel unsafe, and the clinic began offering a quiet room for waiting. This week, after a 5-minute delay in the quiet room, the client swore at a receptionist again. What should the social worker do **NEXT**?

**A.** Explore again what waiting brings up for the client

**B.** Set clear conduct expectations and state what happens next time

**C.** Arrange for the client to skip the waiting area entirely

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). Exploring the trigger was the right first response and was done after the first incident, leading to the quiet room. Repeating it without addressing the behavior signals that mistreating staff carries no expectation.

**B** **Correct.** Trauma-informed care pairs understanding with clear, predictable expectations, because physical and psychological safety must hold for staff as well as clients (SAMHSA, 2014). The clinic has already explored the trigger and made an accommodation, yet the client swore at staff again while waiting in the quiet room. Calmly stating the expected conduct and the specific consequence of another incident, while affirming the client's place in services, is the limit the situation now needs. It beats C because a further accommodation responds to the trigger again but leaves the behavior toward staff unaddressed.

**C** **Incorrect** (trap: Good but not best). More accommodation can be part of the plan, but on its own it treats the outburst only as a scheduling problem. Staff safety and a clear expectation for the client's conduct still need to be stated.

**Takeaway:** Trauma-informed limits combine understanding with clear expectations; after an accommodation fails, state the limit rather than only accommodating again.

**Sources:** SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Key principles: safety; trustworthiness and transparency

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### 220. INT-1071 (rev 2) · Best answer: C

A crisis stabilization program offers up to 6 sessions. A client seen after a sudden job loss and panic attacks is, by session 5, sleeping again, applying for jobs, and using a coping plan. The client now asks to work on feelings of worthlessness that go back to childhood. What is the **MOST** appropriate step?

**A.** Extend crisis sessions to address the childhood pattern
**B.** Close the case now that functioning has returned
**C.** Refer for ongoing therapy and set a follow-up contact

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Long-standing feelings of worthlessness call for ongoing therapy, not crisis sessions stretched past their purpose. Extending would also delay the client's link to a provider who can offer the longer work.

**B** **Incorrect** (trap: Good but not best). Functioning has returned, but closing without a referral ignores the client's request for help with a long-standing problem. Crisis work ends with linkage and follow-up, not simply discharge.

**C** **Correct.** Crisis intervention is brief and present-focused: it aims to restore at least pre-crisis functioning, then link the person to longer-term help for deeper issues and plan follow-up (Roberts, 2005). This client is sleeping, job hunting and coping, so the crisis goal is met, and the new request is long-standing work. It beats A because the client's wish is valid, but stretching a crisis service into open-ended therapy blurs its purpose; a referral gives the childhood pattern the time it needs, and follow-up closes the crisis episode (Roberts & Ottens, 2005).

**Takeaway:** Crisis intervention restores functioning, then links the client to longer-term services for underlying issues, with follow-up.

**Sources:** Roberts, A. R. (Ed.). (2005). Crisis Intervention Handbook: Assessment, Treatment, and Research (3rd ed.). Oxford University Press, goals of crisis intervention; referral after stabilization; Roberts, A. R., & Ottens, A. J. (2005). The seven-stage crisis intervention model: A road map to goal attainment, problem solving, and crisis resolution. Brief Treatment and Crisis Intervention, 5(4), 329-339, Stage 7: follow-up
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### 221. INT-1075 (rev 1) · Best answer: C

A crisis line social worker takes a call from an adult who is crying and talking fast. The worker asks about suicide and violence; the caller has no such thoughts, has no weapons, and is staying at a friend's apartment. The caller has now calmed, says they feel heard, and wants help sorting out 'everything falling apart.' What should the social worker do **NEXT**?

**A.** Help the caller brainstorm options for the coming week
**B.** Teach the caller a slow-breathing exercise to settle down
**C.** Ask what happened today that prompted reaching out
**D.** Work out a written step-by-step plan for tonight

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Generating alternatives is a later stage of the Roberts model. Without first defining the main problem and what set off the crisis, the options will be scattered and may miss what matters most to the caller.

**B** **Incorrect** (trap: Good but not best). Calming skills help when a person is too aroused to think, and they fit early in a call. This caller has already settled and feels heard, so stopping for a breathing exercise delays the problem-focused work the caller is asking for.

**C** **Correct.** Roberts' seven-stage model moves from lethality assessment and rapid rapport to identifying the major problems, starting with the precipitating event: what happened that pushed the person past their usual coping (Roberts & Ottens, 2005). Safety has been checked and the caller feels heard, so the first two stages are done. 'Everything falling apart' is too vague to act on; naming today's trigger focuses the work. It beats A because brainstorming alternatives is stage 5 and needs a defined problem first; options generated now would target a blur.

**D** **Incorrect** (trap: Right action, wrong time). A concrete action plan is the right step after the problem is defined and options are explored. Building it now skips identifying what triggered the crisis, so the plan may not address the real problem.

**Takeaway:** In Roberts' model, once safety is assessed and contact is made, define the major problems, starting with the precipitating event, before generating options.

**Sources:** Roberts, A. R., & Ottens, A. J. (2005). The seven-stage crisis intervention model: A road map to goal attainment, problem solving, and crisis resolution. Brief Treatment and Crisis Intervention, 5(4), 329-339, Stage 3: identify major problems, including the precipitating event

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## 222. INT-1076 (rev 2) · Best answer: A

Two hours after an apartment fire, a parent sits in a hospital emergency department while nurses treat the parent's two children for minor injuries. The parent keeps repeating, 'I don't know what to do,' stares at the floor, and cannot answer when asked whom to call. What is the **BEST** approach for the social worker?

**A.** Give brief, concrete directions for the immediate next steps

**B.** Reflect the parent's feelings and wait for the parent to choose

**C.** Complete a full psychosocial history for the record

**Why each option is right or wrong**

**A** **Correct.** In crisis work, the worker's level of directiveness matches the client's level of functioning: nondirective for a mobile client, collaborative for a partly mobile one, and directive for one who is immobilized (James & Gilliland, 2017). This parent cannot answer a simple question and keeps repeating that they don't know what to do, which are signs of immobility. Brief, concrete steps restore a sense of order. It beats B because reflection respects autonomy but leaves an immobilized person stuck; choices can return as functioning improves.

**B** **Incorrect** (trap: Misuses the technique). Reflection and waiting fit a client who can think through choices. A parent who cannot say whom to call is immobilized right now,

and waiting prolongs the disorganization. As the parent steadies, the worker can shift to a more collaborative stance.

**C** **Incorrect** (trap: Right action, wrong time). A full history matters later, but it asks a disorganized parent to answer many questions at the moment they can answer none. Stabilization and practical next steps come first; detailed history can follow once functioning returns.

**Takeaway:** Match crisis directiveness to functioning: the more immobilized the client, the more directive and concrete the worker, stepping back as coping returns.

**Sources:** James, R. K., & Gilliland, B. E. (2017). Crisis Intervention Strategies (8th ed.). Cengage, assessing client mobility; directive, collaborative and nondirective action continuum
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### 223. INT-1077 (rev 1) · Best answer: B

A high school social worker completes a suicide risk assessment with a 16-year-old who reported fleeting thoughts that 'it would be easier not to wake up.' The thoughts come at night when the student worries about grades, and the student names a parent and a coach as supports. An assistant principal asks the worker to have the student sign a no-harm agreement before returning to class. What is the **MOST** appropriate step?

**A.** Have the student sign the agreement the administrator requested
**B.** Build a written safety plan with the student and family
**C.** Send the student back to class and check in next week

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). The request sounds protective, but a signed promise not to self-harm has no evidence of preventing suicide and gives no legal protection. It can also give staff false reassurance. A safety plan gives the student skills and contacts instead.

**B** **Correct.** A safety plan is a written, collaborative list of warning signs, coping strategies, people and places for distraction, people to ask for help, crisis contacts and steps to make the environment safer (Stanley & Brown, 2012). It gives the student and family concrete actions for the nighttime worries described. It beats A because no-harm agreements lack evidence that they prevent suicide and do not protect the worker legally; critics recommend replacing them with a commitment to treatment and safety planning (Rudd et al., 2006).

**C** **Incorrect** (trap: Ignores risk). The thoughts are low-level, but the student still needs a plan for the worried nights when they occur. Sending the student back without coping steps or family involvement leaves the risk unaddressed until next week.

**Takeaway:** After a risk assessment, build a collaborative written safety plan; no-harm contracts lack evidence and are not a substitute.

**Sources:** Stanley, B., & Brown, G. K. (2012). Safety planning intervention. Cognitive and Behavioral Practice, 19(2), 256-264, safety plan components; Rudd, M. D., Mandrusiak, M., & Joiner, T. E. (2006). The case against no-suicide contracts: The commitment to treatment statement as a practice alternative. Journal of Clinical Psychology, 62(2), 243-251, whole article

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## 224. INT-1078 (rev 1) · Best answer: D

A crisis that arises when a normal, expected life change, such as becoming a parent or retiring, overwhelms a person's usual coping is which type?

- **A.** Situational crisis
- **B.** Existential crisis
- **C.** Environmental crisis
- **D.** Developmental crisis

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Situational crises follow sudden, uncommon events outside a person's control, such as an assault, a crash or a layoff. Becoming a parent or retiring is an expected life change, so the crisis comes from the transition, not an unexpected event.

**B** **Incorrect** (trap: Wrong model or concept). Existential crises center on questions of purpose, meaning, freedom or responsibility, such as feeling that one's life has had no impact. A crisis set off by a new role at a normal life stage is the hallmark of a developmental crisis instead.

**C** **Incorrect** (trap: Wrong model or concept). Environmental (ecosystemic) crises are set off by natural or human-caused disasters that affect many people, such as floods or wildfires. An expected personal life change is not a community-wide event.

**D** **Correct.** A developmental crisis arises from a normal, expected life transition, such as the birth of a child, starting college, a midlife change or retirement, when the new demands outstrip usual coping (James & Gilliland, 2017). The defining feature is that the trigger is a normal life stage that most people pass through, and the disorganization comes from the demands of the new role. It beats A because a situational crisis follows a sudden, unusual event, such as an accident or a job loss; an expected life change, however overwhelming, is developmental.

**Takeaway:** Developmental crises stem from expected life transitions; situational ones from sudden, unusual events; existential from meaning; ecosystemic from disasters.

**Sources:** James, R. K., & Gilliland, B. E. (2017). Crisis Intervention Strategies (8th ed.). Cengage, types of crisis: developmental, situational, existential, ecosystemic

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### 225. INT-1079 (rev 1) · Best answer: A

On a text-based crisis line, a person writes that they swallowed a full bottle of a prescription sleep medication 30 minutes ago and now feel 'calm and ready.' Earlier in the chat they gave their home address. When the social worker raises getting help, the person replies, 'Please don't send anyone.' What should the social worker do **FIRST**?

**A.** Send emergency medical services to the person now
**B.** Ask what happened today that led to taking the pills
**C.** Work with the person on a safety plan for tonight

**Why each option is right or wrong**

**A** **Correct.** Lethality and imminent danger come first in crisis intervention (Roberts & Ottens, 2005), and a person who has already swallowed a full bottle of medication is in a medical emergency. Social workers may limit self-determination when a client's actions pose a serious, foreseeable and imminent risk to self (NASW Code 1.02). The worker has an address and can send help while continuing to text. It beats B because understanding the trigger matters, but every minute spent exploring delays treatment for a possibly fatal overdose; the conversation can continue while help is on the way.

**B** **Incorrect** (trap: Right action, wrong time). Exploring the precipitating event is part of crisis work once immediate danger is handled. Here an overdose is already in progress, and asking about the day's events delays the medical help the person needs now.

**C** **Incorrect** (trap: Ignores risk). Safety planning helps prevent a future attempt. It cannot reverse pills already swallowed, and the 'calm' the person describes may reflect the medication or a decision already made. Medical care comes first; a safety plan can follow recovery.

**Takeaway:** When a person has already acted on a suicide attempt, emergency medical help comes before exploring causes or planning, even over objection.

**Sources:** Roberts, A. R., & Ottens, A. J. (2005). The seven-stage crisis intervention model: A road map to goal attainment, problem solving, and crisis resolution. Brief Treatment and Crisis Intervention, 5(4), 329-339, Stage 1: lethality and imminent danger assessment; NASW Code of Ethics (2021), 1.02
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### 226. INT-1080 (rev 1) · Best answer: B

At a public benefits office, a client whose food assistance was denied is pacing near the front desk, shouting that staff 'never help anyone.' The client's hands are empty, and other people in the lobby have moved away. The social worker approaches. What should the social worker do **FIRST**?

**A.** Tell the client security will be called unless they sit down
**B.** Speak calmly from a safe distance and offer a quieter room
**C.** Ask the client to explain why the application was denied
**D.** Ask security staff to escort the client out of the building

**Why each option is right or wrong**

**A** **Incorrect** (trap: Too directive). Clear limits are part of de-escalation, but leading with a threat is provocative and tends to raise agitation. Limits fit later, if calm contact and choices have not worked or behavior becomes unsafe.

**B** **Correct.** Verbal de-escalation starts by respecting personal space, establishing calm verbal contact and offering choices, before problem solving or limit setting (Richmond et al., 2012). The client is agitated but holding nothing, so the worker can approach and lower the intensity. A quieter room also removes the audience. It beats C because the denial will need review, but a person this aroused cannot take in details; calming comes before problem solving.

**C** **Incorrect** (trap: Right action, wrong time). Reviewing the denial is the eventual goal, but problem solving requires a calmer state. Asking for detailed reasons while the client is shouting is likely to feed the anger rather than resolve it.

**D** **Incorrect** (trap: Skips assessment). Removal is appropriate if the client threatens or harms someone. Here the client is loud and upset but is not threatening anyone, so calling security skips less restrictive steps and may provoke a confrontation.

**Takeaway:** With an agitated client, start with calm verbal contact, personal space and choices; problem solving and firm limits come after arousal drops.

**Sources:** Richmond, J. S., et al. (2012). Verbal de-escalation of the agitated patient: Consensus statement of the American Association for Emergency Psychiatry Project BETA De-escalation Workgroup. Western Journal of Emergency Medicine, 13(1), 17-25, domains: respect personal space; establish verbal contact; offer choices

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### 227. INT-1081 (rev 1) · Best answer: C

Late at night, an 8-year-old waits at a child welfare office after being removed from home. The required investigative interview is already done. The child is crying and keeps asking, 'Where am I going to sleep? Who's going to feed my dog?' What would be **MOST** helpful for the social worker to do?

**A.** Teach the child a slow-breathing exercise to calm down

**B.** Promise the child they will go home within a few days

**C.** Tell the child plainly what will happen tonight, step by step

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Brief calming skills are part of stabilization and may help later tonight. This child, however, is asking specific questions; teaching a skill instead of answering them leaves the main source of fear in place.

**B** **Incorrect** (trap: Misreads law or ethics). A promise to return home soon may calm the child for a moment, but the worker cannot know the outcome. If the promise fails, trust is damaged when the child most needs reliable adults. Honest, simple information is safer.

**C** **Correct.** Psychological First Aid stresses safety, comfort and simple, accurate information about what will happen next, which restores some predictability (Brymer et al., 2006). This child's distress centers on concrete unknowns: where to sleep and who will care for the dog. Answering those plainly, including what the worker can find out about the pet, targets the actual source of fear. It beats A because calming skills are useful, but a breathing exercise leaves the child's urgent questions unanswered, and the worry returns when the exercise ends.

**Takeaway:** For a child in acute crisis, give simple, honest information about immediate next steps; uncertainty is often the core fear.

**Sources:** Brymer, M., et al. (2006). Psychological First Aid: Field Operations Guide (2nd ed.). National Child Traumatic Stress Network and National Center for PTSD, Core Action 2: Safety and Comfort (providing information; attending to children)
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## 228. INT-1151 (rev 1) · Best answer: C

Ten days after a fatal crane accident, a social worker on an employer's crisis response team meets again with an employee who saw it happen. The employee sleeps about 2 hours a night, eats little, and has not been able to drive near the site. The employee says the breathing exercises, walks, and talks with their partner that they planned together last week have not helped. What should the social worker do **NEXT**?

**A.** Review the coping plan again and meet next week
**B.** Ask the employee to go over the accident in more detail
**C.** Link the employee to a mental health provider for assessment
**D.** Encourage a gradual return to the site alongside a coworker

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Reviewing coping and following up fit earlier in crisis work, and that step was done last week. Repeating a plan the employee says has not worked delays the evaluation that persistent impairment calls for.

**B** **Incorrect** (trap: Misuses the technique). A detailed retelling resembles single-session debriefing, which has not been shown to prevent PTSD and is not part of crisis stabilization. The employee needs further evaluation, not more recounting.

**C** **Correct.** Crisis intervention ends by checking whether functioning is returning and referring when it is not; the last step of Everly's SAFER-R model is recovery or referral (Everly, 1996). Ten days on, the employee is barely sleeping or eating, is avoiding the site, and the coping plan built last week has not helped. Those facts call for evaluation by a mental health professional who can assess for an acute stress reaction and offer treatment. It beats A because repeating a plan that has already failed delays care while impairment continues.

**D** **Incorrect** (trap: Right action, wrong time). Gradual return to avoided places can be part of later trauma treatment planned by a qualified clinician. Pushing it now, while the employee is barely sleeping or eating, comes before the evaluation that should happen first.

**Takeaway:** When crisis intervention has not restored functioning, the final step is referral for mental health evaluation, not repeating stabilization.

**Sources:** Everly, G. S., Jr. (1996). A rapid crisis intervention technique for law enforcement: The SAFER(R) model. In J. T. Reese & R. Solomon (Eds.), Organizational Issues in Law Enforcement. Federal Bureau of Investigation, SAFER-R final step: recovery or referral
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### 229. INT-1156 (rev 2) · Best answer: B

On the morning their home was condemned, a client arrives at a community crisis center too shaken to speak. An hour later, the client is calm, has listed what must happen today, and mentions usually handling the family's bills and appointments. The client then asks the social worker to 'just make all the calls.' What is the **BEST** response?

**A.** Make the calls for the client while the client rests
**B.** Plan which calls the client will make, with help as needed
**C.** Refer the client to a housing agency for case management

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Taking over tasks fits a client who cannot function, as this client could not an hour ago. Now that the client is calm and capable, doing everything undermines the coping that crisis work aims to restore.

**B** **Correct.** In crisis work, directiveness matches functioning: directive while a client is immobilized, more collaborative as coping returns (James & Gilliland, 2017). Building and carrying out an action plan with the client restores a sense of mastery (Roberts & Ottens, 2005). This client was shaken an hour ago but is now calm, has listed the day's tasks, and usually manages the family's affairs. Planning calls the client makes, with the social worker stepping in where needed, builds on those strengths. It beats A because taking over now treats a recovering client as helpless and can deepen dependence.

**C** **Incorrect** (trap: Hands off too soon). Housing case management may help later. Handing off now, when the client needs a plan for today and is ready to act, moves the work away from the crisis contact without addressing the immediate tasks.

**Takeaway:** As a person in crisis regains composure, hand tasks back through a shared action plan instead of doing things for them.

**Sources:** Roberts, A. R., & Ottens, A. J. (2005). The seven-stage crisis intervention model: A road map to goal attainment, problem solving, and crisis resolution. Brief Treatment and Crisis Intervention, 5(4), 329-339, Stage 6: developing and formulating an action plan; James, R. K., & Gilliland, B. E. (2017). Crisis Intervention Strategies (8th ed.). Cengage, assessing client mobility; directive, collaborative and nondirective action continuum
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.5 Crisis intervention and crisis treatment approaches · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 1 of 3

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### 230. INT-1159 (rev 1) · Best answer: B

Who studied survivors of the Cocoanut Grove nightclub fire and described acute grief as a normal reaction that runs its course?

**A.** Gerald Caplan
**B.** Erich Lindemann
**C.** Elisabeth Kubler-Ross
**D.** Naomi Golan

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Caplan worked alongside Lindemann and is credited with defining crisis as a temporary state of disequilibrium when usual coping fails, and with preventive psychiatry. The fire study itself was Lindemann's.

**B** **Correct.** Erich Lindemann studied survivors and relatives of those who died in the 1942 Cocoanut Grove fire in Boston. He described acute grief as a normal syndrome with a predictable course that brief help could support, and this work became a foundation of crisis theory (Lindemann, 1944). It beats A, Gerald Caplan, because Caplan built on Lindemann's work to define crisis as disequilibrium when usual coping fails; he did not conduct the Cocoanut Grove study.

**C** **Incorrect** (trap: Wrong model or concept). Kubler-Ross described reactions to dying, drawn from interviews with terminally ill patients in the 1960s. Her work did not come from the Cocoanut Grove survivors.

**D** **Incorrect** (trap: Wrong model or concept). Golan developed crisis concepts such as the hazardous event and the precipitating factor, decades after the fire. She did not study its survivors.

**Takeaway:** Lindemann's Cocoanut Grove study framed acute grief as normal and time-limited; Caplan later defined crisis as disequilibrium when coping fails.

**Sources:** Lindemann, E. (1944). Symptomatology and management of acute grief. American Journal of Psychiatry, 101(2), 141-148, whole article

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### 231. INT-1163 (rev 1) · Best answer: D

A high school student died by suicide over the weekend. The family has confirmed the cause of death and agreed that it may be shared. The principal asks the school social worker how to inform students on Monday morning. Which approach is **MOST** helpful?

**A.** Read a brief statement over the school intercom
**B.** Hold an assembly where staff share facts and resources
**C.** Say a student died and decline to discuss the cause
**D.** Have trained staff share the news in homeroom-size groups

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misuses the technique). An intercom announcement reaches everyone at once but leaves students alone with the news and gives staff no way to see reactions or answer questions. Postvention guidance advises against it.

**B** **Incorrect** (trap: Ignores risk). An assembly shares accurate facts, but in a large crowd staff cannot watch individual reactions, and the event can magnify the death. Guidance favors smaller settings for this reason.

**C** **Incorrect** (trap: Good but not best). Withholding a confirmed cause that the family agreed to share tends to fuel rumors and misses a chance to talk about suicide safely and point students to help. Discretion fits when a family asks for privacy.

**D** **Correct.** School postvention guidance recommends telling students in small groups, such as homerooms, where trained staff can share accurate facts, answer questions, and notice students who are struggling (American Foundation for Suicide Prevention & SPRC, 2011). Small groups support grief and help identify close friends and other vulnerable students for follow-up, which lowers contagion risk. It beats B because an assembly gives the same facts to a large crowd, where staff cannot watch individual reactions and the event can magnify the death.

**Takeaway:** After a student suicide, share facts in small groups led by trained staff, avoid assemblies, and identify vulnerable students for follow-up.

**Sources:** American Foundation for Suicide Prevention & Suicide Prevention Resource Center (2011). After a Suicide: A Toolkit for Schools, Crisis response: notifying students; identifying students at risk
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### 232. INT-1167 (rev 2) · Best answer: C

Hours after a shooting at a warehouse, a social worker is providing psychological first aid at a family assistance center. An employee who hid during the shooting is trembling but answers questions clearly. The employee says their phone died and their family does not yet know they are alive. What is the **BEST** response?

**A.** Give the employee information on common stress reactions
**B.** Guide the employee through a brief breathing exercise
**C.** Help the employee contact loved ones right away

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Information about common stress reactions is a core part of psychological first aid and will help. Offered now, it comes ahead of the employee's own stated priority; PFA meets survivors' immediate needs first, and reaching family is this employee's most pressing one.

**B** **Incorrect** (trap: Good but not best). Breathing exercises help survivors who are overwhelmed or disoriented. This employee is coherent and has named a pressing practical need, so meeting that need comes before calming techniques.

**C** **Correct.** Psychological first aid responds to survivors' current needs and priorities, and one of its core actions is connecting survivors with primary support persons as soon as possible (Brymer et al., 2006). The employee is oriented, names a clear need, and the family is waiting for news. Helping make that contact meets the stated need and brings support. It beats B because stabilization techniques are for survivors who are disoriented or overwhelmed; this employee is trembling but thinking clearly.

**Takeaway:** Psychological first aid follows the survivor's immediate priorities, such as reaching loved ones, and does not require recounting the traumatic event.

**Sources:** Brymer, M., Jacobs, A., Layne, C., et al. (2006). Psychological First Aid: Field Operations Guide (2nd ed.). National Child Traumatic Stress Network and National Center for PTSD, Core Action 6: Connection with Social Supports
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### 233. INT-1017 (rev 2) · Best answer: A

In a residential treatment program, a 15-year-old comes out of a phone call with a parent shaking, panting, and saying they want to punch the wall. The teen has been learning DBT skills. What should the social worker help the teen do **FIRST**?

**A.** Splash cold water on their face and slow their breathing
**B.** Check the facts about what the parent said on the call
**C.** Talk through what upset them during the call

**Why each option is right or wrong**

**A** **Correct.** When emotional arousal is extreme, DBT teaches skills that change body chemistry quickly, such as cold water on the face and paced breathing, before any skill that depends on clear thinking (Linehan, 2015). The teen is shaking and breathing rapidly, with an urge to hit something, so lowering physical arousal comes first. It beats B because checking the facts is a valuable emotion-regulation skill, but it relies on reasoning that is hard to reach at peak arousal; it fits once the teen has calmed.

**B** **Incorrect** (trap: Right action, wrong time). Checking the facts helps a client test whether an emotion fits the situation. It is the right skill once arousal drops; at this intensity, the teen is unlikely to think it through.

**C** **Incorrect** (trap: Right action, wrong time). Talking through the call can help the teen make sense of it and feel heard, and it fits later. At this level of arousal, though, discussing the upsetting conversation tends to keep the emotion high; the body needs to calm first.

**Takeaway:** At peak emotional arousal, lower the body's arousal first with fast physical skills; cognitive skills work once the client is calmer.

**Sources:** Linehan, M. M. (2015). DBT Skills Training Manual (2nd ed.). Guilford Press, distress tolerance: TIP skills
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.6 Teaching emotion-regulation skills · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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## 234. INT-1018 (rev 2) · Best answer: B

Through an employee assistance program, a client meets with a social worker by video. The client says they handle anger in staff meetings by clamping down and showing nothing, but later get headaches and snap at their children. Which emotion-regulation skill is **BEST** for the social worker to teach?

**A.** Vent the anger fully at home to release the pressure
**B.** Reframe triggering events before anger builds
**C.** Practice muscle relaxation each evening after work

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Venting seems to release pressure, but research on catharsis shows that expressing anger this way tends to keep it going (Bushman, 2002). It would also land on the children, which is already part of the problem.

**B** **Correct.** Emotion regulation research separates strategies used early, such as cognitive reappraisal, from strategies used once an emotion is under way, such as suppression (Gross, 1998). Reappraisal reduces felt distress, while habitual suppression hides expression but leaves inner arousal and is linked to poorer well-being and relationships (Gross & John, 2003). The client's headaches and irritability at home fit the costs of suppression. It beats C because evening relaxation eases tension afterward but leaves the suppression pattern in place.

**C** **Incorrect** (trap: Good but not best). Progressive muscle relaxation is a sound skill and may ease the headaches. It works after the fact, though, and leaves in place the suppression that builds the arousal and spills over at home.

**Takeaway:** Cognitive reappraisal changes an emotion's impact early; habitual suppression hides expression but keeps arousal and carries physical and relational costs.

**Sources:** Gross, J. J. (1998). The emerging field of emotion regulation: An integrative review. Review of General Psychology, 2(3), 271-299, antecedent-focused versus response-focused regulation; Gross, J. J., & John, O. P. (2003). Individual differences in two emotion regulation processes: Implications for affect, relationships, and well-being. Journal of Personality and Social Psychology, 85(2), 348-362, whole article; Bushman, B. J. (2002). Does venting anger feed or extinguish the flame? Catharsis, rumination, distraction, anger, and aggressive responding. Personality and Social Psychology Bulletin, 28(6), 724-731, whole article

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## 235. INT-1019 (rev 2) · Best answer: C

A 6-year-old in a new foster home has daily meltdowns. When the child welfare social worker asks how the child felt before a meltdown, the child shrugs or says 'bad.' The foster parent wants help so the child can calm down. What should the social worker do **FIRST** to build the child's regulation skills?

**A.** Ask the child to tell the story of leaving home

**B.** Set up a sticker chart that rewards calm days

**C.** Teach the child to name feelings using a faces chart

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Telling the story of a painful separation belongs to later trauma-processing work, after the child has coping and regulation skills. Asking for it first could raise distress in a child already struggling to stay regulated.

**B** **Incorrect** (trap: Good but not best). Rewards can reinforce calm behavior and may help alongside skills teaching. Without a way to notice and name feelings, though, the child has no tool for earning the reward, and repeated failure may add frustration.

**C** **Correct.** Emotion regulation builds on emotional awareness: children first learn to recognize and name feelings, then link them to body cues and calming steps. Trauma-focused CBT, for example, teaches feelings identification among its early coping-skills components, well before any trauma narrative (Cohen, Mannarino & Deblinger, 2017). A child who can only say 'bad' needs that vocabulary first. It beats B because a sticker chart rewards calm days but gives the child no way to notice rising feelings and respond.

**Takeaway:** A child who cannot name feelings needs feelings identification first; regulation strategies build on it, and trauma processing comes much later.

**Sources:** Cohen, J. A., Mannarino, A. P., & Deblinger, E. (2017). Treating Trauma and Traumatic Grief in Children and Adolescents (2nd ed.). Guilford Press, affective modulation skills

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### 236. INT-1020 (rev 1) · Best answer: C

A client in a DBT skills group feels intense shame and wants to stay home after stumbling over words in a work presentation. With the social worker, the client has already checked the facts and concluded the shame does not fit, since no personal values were violated. Which skill is **BEST** to teach next?

**A.** Radical acceptance
**B.** Interpersonal effectiveness
**C.** Opposite action
**D.** Self-soothing

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Radical acceptance means fully accepting a reality that cannot be changed. It belongs to distress tolerance, and here the client faces not an unchangeable reality but an emotion that does not fit the facts, with an urge to avoid.

**B** **Incorrect** (trap: Wrong model or concept). Interpersonal effectiveness skills help a client ask for something or say no while keeping relationships and self-respect. The client's problem is shame and an urge to withdraw, not a request to make of someone.

**C** **Correct.** In DBT emotion regulation, when checking the facts shows an emotion does not fit the situation, the next step is opposite action: acting against the emotion's urge (Linehan, 2015). Shame's urge is to hide, so the opposite is to show up, such as returning to work and making eye contact with colleagues. It beats A because radical acceptance is a distress-tolerance skill for facts that cannot be changed; here the problem is an emotion that does not fit the facts and an urge that can be acted against.

**D** **Incorrect** (trap: Wrong model or concept). Self-soothing uses the senses to get through a painful moment. It may ease distress but leaves the urge to withdraw unchallenged, so avoidance and shame can grow.

**Takeaway:** When an emotion doesn't fit the facts, DBT's opposite action means acting against its urge; radical acceptance is for unchangeable realities.

**Sources:** Linehan, M. M. (2015). DBT Skills Training Manual (2nd ed.). Guilford Press, emotion regulation: check the facts and opposite action

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### 237. INT-1133 (rev 2) · Best answer: B

Which DBT skills help a client feel less easily upset by caring for sleep, food, exercise, and illness?

**A.** Opposite action
**B.** PLEASE skills
**C.** TIP skills
**D.** Accumulating positive emotions

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Opposite action is an emotion regulation skill, but it changes an emotion that does not fit the facts by acting against its urge. It does not target sleep, eating, or illness.

**B** **Correct.** PLEASE is the DBT emotion regulation set for reducing vulnerability to emotion mind: treat physical illness, balance eating, avoid mood-altering substances, balance sleep, and get exercise (Linehan, 2015). A body that is tired, hungry, ill, or intoxicated reacts to triggers faster and more intensely. It beats D because accumulating positive emotions, the neighboring ABC skill, lowers vulnerability by building pleasant experiences rather than by caring for the body; and TIP skills (temperature, intense exercise, paced breathing, paired muscle relaxation) are distress tolerance tools for bringing down acute arousal in a crisis, not routine habits that lower day-to-day vulnerability.

**C** **Incorrect** (trap: Wrong model or concept). TIP skills include intense exercise, which makes them tempting, but they are a distress tolerance set for quickly lowering extreme arousal in a crisis (temperature, intense exercise, paced breathing, paired muscle relaxation), not a daily self-care routine.

**D** **Incorrect** (trap: Wrong model or concept). Accumulating positive emotions sits in the same emotion regulation set for reducing vulnerability (the ABC of ABC PLEASE), so it is a close neighbor. It works by scheduling pleasant events and building a life worth living, not by caring for sleep, eating, exercise, and illness.

**Takeaway:** DBT's PLEASE skills reduce emotional vulnerability by caring for the body: illness, eating, substances, sleep, and exercise.

**Sources:** Linehan, M. M. (2015). DBT Skills Training Manual (2nd ed.). Guilford Press, Emotion regulation: reducing vulnerability (PLEASE skills)
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## 238. INT-1134 (rev 1) · Best answer: A

A 15-year-old in a school-based DBT program uses emotion regulation skills well in sessions. Diary cards show that the teen's worst outbursts follow times when a parent says things like 'You're overreacting, just get over it.' The parent attends sessions and wants to help. What is the **MOST** appropriate next step?

**A.** Coach the parent in validating the teen's feelings
**B.** Add home distress tolerance practice the parent can prompt
**C.** Refer the teen to a residential DBT program

**Why each option is right or wrong**

**A** **Correct.** DBT's biosocial theory holds that emotional sensitivity is intensified by an invalidating environment (Linehan, 1993), so adolescent DBT teaches caregivers validation alongside the teen (Rathus & Miller, 2015). The diary cards tie the worst outbursts to the parent's dismissive comments, and the parent is willing to help. Coaching the parent to acknowledge what the teen feels changes the trigger itself. It beats B because more skill practice asks the teen to cope with invalidation that can be reduced at its source.

**B** **Incorrect** (trap: Good but not best). Extra practice is useful, and the teen already uses skills well in session. It leaves the main trigger, the parent's dismissive comments, in place, so outbursts at home are likely to continue.

**C** **Incorrect** (trap: Hands off too soon). Residential care is for youth who cannot be kept safe or treated in outpatient care. Nothing here suggests that level of need, and the problem has a clear trigger at home that outpatient work can address.

**Takeaway:** When invalidation at home drives a teen's dysregulation, teach caregivers validation rather than only adding skills for the teen.

**Sources:** Linehan, M. M. (1993). Cognitive-Behavioral Treatment of Borderline Personality Disorder. Guilford Press, biosocial theory: invalidating environment; Rathus, J. H., & Miller, A. L. (2015). DBT Skills Manual for Adolescents. Guilford Press, Walking the Middle Path: validation

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### 239. INT-0002 (rev 1) · Best answer: C

A client in psychodynamic treatment arrives late and apologizes repeatedly, saying the social worker must be angry. The social worker has shown no displeasure. The client adds that a parent used to explode over small mistakes. What is the **BEST** response within a psychodynamic approach?

**A.** Reassure the client that being late is fine

**B.** Assign a thought record to test the belief about anger

**C.** Explore how this fear may echo the client's past relationships

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Reassurance is kind and may lower anxiety, but it bypasses the transference. In psychodynamic work, the client's expectation of anger is information to explore rather than something to smooth over.

**B** **Incorrect** (trap: Wrong model or concept). A thought record is a cognitive behavioral technique for testing beliefs. It may help in CBT, but it does not fit the psychodynamic approach named in the stem.

**C** **Correct.** The client expects the social worker to react the way an explosive parent did, though nothing in the session supports that. This is transference: feelings and expectations from early relationships placed onto the social worker. A psychodynamic approach uses it by gently exploring the link, which builds insight into a pattern that likely appears in other relationships (Greenson, 1967). It beats A because reassurance may calm the moment but closes off the material the approach is designed to work with.

**Takeaway:** When a client reacts to the social worker as to an early caregiver (transference), psychodynamic work explores the link to build insight.

**Sources:** Greenson, R. R. (1967). The Technique and Practice of Psychoanalysis. International Universities Press, transference

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### 240. INT-0105 (rev 1) · Best answer: B

A client changes the subject each time a sibling's death comes up. The social worker points out the pattern and suggests that avoiding the topic may protect the client from guilt carried since childhood. Which treatment approach is the social worker **MOST** likely using?

**A.** Cognitive behavioral therapy
**B.** Psychodynamic therapy
**C.** Eye movement desensitization and reprocessing

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). CBT might target guilt by examining the thoughts behind it, such as self-blame, and weighing the evidence. It focuses on present thinking, not on interpreting in-session avoidance as a defense with childhood roots.

**B** **Correct.** Psychodynamic therapy explores how clients avoid distressing feelings and links present patterns to earlier experience (Shedler, 2010). Pointing out the avoidance and suggesting it guards against long-held guilt is an interpretation of a defense, a core psychodynamic technique aimed at insight. It beats A because cognitive behavioral therapy would identify the guilt-related thoughts and test their accuracy in the present, rather than interpret avoidance in session as a defense rooted in childhood.

**C** **Incorrect** (trap: Wrong model or concept). EMDR processes distressing memories while the client attends to bilateral stimulation, such as guided eye movements. Nothing in this stem describes that kind of structured memory processing.

**Takeaway:** Interpreting a client's avoidance as a defense linked to past experience is a psychodynamic technique aimed at insight.

**Sources:** Shedler, J. (2010). The efficacy of psychodynamic psychotherapy. American Psychologist, 65(2), 98-109, distinctive features of psychodynamic technique
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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### 241. INT-1000 (rev 1) · Best answer: B

A retired client has been in weekly psychodynamic therapy exploring lifelong conflicts with authority figures. The client's spouse died suddenly 2 weeks ago. The client now arrives disheveled, has been skipping meals, and says they cannot think clearly enough to deal with the funeral bills. What is the **BEST** way for the social worker to adjust the approach?

**A.** Interpret how the loss revives early conflicts with authority
**B.** Shift to ego-supportive work on daily coping and tasks
**C.** Refer the client to a grief support group instead

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Interpretation is a core ego-modifying technique and fits the client's earlier work. Right now, though, the client's coping is overwhelmed, and stirring up old conflicts could deepen the disorganization. Insight-oriented work can resume once daily functioning is steadier.

**B** **Correct.** Ego psychology distinguishes ego-supportive from ego-modifying work. When crisis weakens a client's ego functioning, the social worker shifts to supportive methods: bolstering coping, helping with concrete tasks, and lending structure and judgment (Goldstein, 1995). Disorganization, skipped meals, and trouble handling bills after a sudden death show that the client's coping is overwhelmed. It beats A because interpreting old conflicts asks a strained ego to tolerate more anxiety now; insight work can resume once the client stabilizes.

**C** **Incorrect** (trap: Hands off too soon). A grief group may help later as an added support. Replacing individual therapy with a group now removes the established relationship at the moment the client most needs steady, individualized help with basic functioning.

**Takeaway:** When crisis overwhelms ego functioning, psychodynamic work shifts from insight-oriented interpretation to ego-supportive help with coping and daily tasks.

**Sources:** Goldstein, E. G. (1995). Ego Psychology and Social Work Practice (2nd ed.). Free Press, ego-supportive and ego-modifying intervention

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## 242. INT-1001 (rev 2) · Best answer: C

A client in treatment informed by self psychology often describes successes at work and brightens when the social worker responds with interest. In one session, the social worker glances at a phone message while the client is speaking. The client falls silent, then says nothing they do really matters. What is the **MOST** appropriate response within this approach?

**A.** Explore what at work has left the client feeling unimportant

**B.** Reassure the client that their work successes are impressive

**C.** Acknowledge the lapse in attention and how it affected the client

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Exploring the feeling is sound in many approaches, but this option looks for its source at work. The client deflated right after the worker's glance at the phone, and self psychology attends first to that in-session empathic break. Exploring elsewhere leaves the rupture unnamed and unrepaired.

**B** **Incorrect** (trap: Good but not best). Mirroring achievements is part of self psychology, but praise now steps over what just happened. The client withdrew right after the worker's lapse, and that rupture needs to be recognized before affirmation can land.

**C** **Correct.** Self psychology holds that clients build a cohesive self through selfobject experiences such as mirroring, and that the worker's empathic failures are inevitable. What heals is recognizing and repairing them, which over time strengthens the client's own self-functions (Kohut, 1971; Kohut, 1984). The client deflated right after the worker's glance at a phone. Naming the lapse and its effect repairs the bond. It beats B because praise alone skips the rupture and leaves the client's experience of being dropped unspoken, and it beats A because exploring the work setting misses the in-session trigger.

**Takeaway:** In self psychology, recognizing and repairing the worker's empathic failures builds self-cohesion; praise alone does not repair a rupture.

**Sources:** Kohut, H. (1971). The Analysis of the Self. International Universities Press, mirror transference; Kohut, H. (1984). How Does Analysis Cure? University of Chicago Press, empathic failure and its repair
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 243. INT-1002 (rev 1) · Best answer: A

A client with a history of chaotic early caregiving often arrives furious and accuses the social worker of not caring. The social worker keeps the same session time and room, responds calmly without retaliating, and reliably resumes the work the following week. Which psychodynamic concept is the social worker's stance **MOST** likely drawing on?

**A.** Holding environment
**B.** Transitional object
**C.** Mirroring transference
**D.** Working through

#### Why each option is right or wrong

**A** **Correct.** Winnicott described the holding environment: the reliable, attuned care a good-enough caregiver provides, which the therapeutic frame can recreate (Winnicott, 1965). A steady schedule, a consistent room, and a calm, nonretaliating response to rage give the client a relationship that survives strong feelings, which chaotic early care did not provide. It beats D because working through is the client's repeated processing of insights over time, not the worker's reliable stance.

**B** **Incorrect** (trap: Wrong model or concept). A transitional object, such as a toddler's blanket, helps a child bridge the move from dependence toward separateness. It is also Winnicott's idea, but it names something the client uses, not the worker's reliable frame.

**C** **Incorrect** (trap: Wrong model or concept). A mirroring transference, from Kohut's self psychology, is the client's need to be affirmed and admired by the worker (Kohut, 1971). Here the client is attacking, and the worker's steadiness, not affirmation, is the point.

**D** **Incorrect** (trap: Wrong model or concept). Working through is the gradual, repeated processing of insights as they show up across situations. It describes the client's ongoing work after interpretation, not the consistency and survival the worker offers.

**Takeaway:** Winnicott's holding environment: a reliable frame and a nonretaliating response let the client experience a relationship that survives strong feelings.

**Sources:** Winnicott, D. W. (1965). The Maturational Processes and the Facilitating Environment. Hogarth Press, holding environment; Kohut, H. (1971). The Analysis of the Self. International Universities Press, mirror transference
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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### 244. INT-1003 (rev 1) · Best answer: D

A social worker and client agree to 16 sessions. Early on, they identify a recurring pattern: the client longs for closeness, expects rejection, and pulls away first. The social worker actively keeps sessions on this pattern, tracing it with the client's partner, with coworkers, and with the social worker. Which approach is the social worker **MOST** likely using?

**A.** Interpersonal psychotherapy
**B.** Task-centered practice
**C.** Solution-focused therapy
**D.** Time-limited dynamic psychotherapy

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Interpersonal psychotherapy is also time-limited and relationship-centered, but it links symptoms to a current problem area such as grief, a role dispute, or a role transition. It does not trace the pattern into the relationship with the worker.

**B** **Incorrect** (trap: Wrong model or concept). Task-centered practice is short-term and structured, but it works on client-chosen target problems through tasks done between sessions. It does not explore a recurring relational pattern or how it appears with the worker.

**C** **Incorrect** (trap: Wrong model or concept). Solution-focused work is brief and future-focused, building on exceptions and small steps. It avoids analyzing problem patterns, which is the core activity in this stem.

**D** **Correct.** Time-limited dynamic psychotherapy, a brief psychodynamic model, sets a session limit, keeps an active focus on one recurring maladaptive relational pattern, and attends to how that pattern shows up with the worker (Levenson, 2017). Here the pattern is traced with the partner, coworkers, and the social worker, which brings in the transference. It beats A because interpersonal psychotherapy is also time-limited and relational, but it targets a current problem area, such as a role dispute or transition, rather than the relationship with the worker.

**Takeaway:** Time-limited dynamic psychotherapy, a brief psychodynamic model, focuses on one recurring relational pattern and examines it in the relationship with the worker.

**Sources:** Levenson, H. (2017). Brief Dynamic Therapy (2nd ed.). American Psychological Association, common features of brief dynamic therapy

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### 245. INT-1004 (rev 1) · Best answer: B

A client in a hospital's outpatient cardiac rehabilitation program has medical clearance to exercise but avoids the treadmill. In a CBT session, the social worker helps the client identify the thought 'If I push myself, my heart will stop,' and the client rates belief in it at 90%. What should the social worker do **NEXT**?

**A.** Remind the client that the cardiologist approved the exercise plan
**B.** Use guided questions to test how accurate the prediction is
**C.** Begin graded treadmill sessions starting at a slow speed
**D.** Explore early experiences that shaped the client's fear of illness

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). The cardiologist's approval is true and relevant, but reassurance rarely shifts a belief held at 90%. CBT helps the client examine the thought directly rather than relying on the authority of the worker or the doctor.

**B** **Correct.** In cognitive therapy, once an automatic thought is identified and rated, the next step is to evaluate it through Socratic questioning: what supports it, what does not, and what a more balanced view would be (Beck, 2021). The client holds a catastrophic prediction at 90%, so testing it together comes next. It beats C because graded activity works best once the client has examined the prediction and agreed to test it; starting on the treadmill first skips the cognitive step.

**C** **Incorrect** (trap: Right action, wrong time). Graded activity is a good later step, often set up as a behavioral experiment to test the prediction. Starting it before the thought is examined skips the collaborative evaluation that helps the client agree to try.

**D** **Incorrect** (trap: Wrong model or concept). Tracing the early roots of a fear belongs to psychodynamic work. CBT stays with the present thought and the evidence for and against it.

**Takeaway:** In CBT, after identifying and rating an automatic thought, evaluate it with guided Socratic questions before moving to behavioral tests.

**Sources:** Beck, J. S. (2021). Cognitive Behavior Therapy: Basics and Beyond (3rd ed.). Guilford Press, evaluating automatic thoughts
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 3 of 3

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### 246. INT-1005 (rev 1) · Best answer: A

A client with PTSD after a car crash meets with a social worker trained in cognitive processing therapy (CPT) but not in EMDR. After hearing about both treatments, the client declines CPT, saying that writing about the crash feels unbearable, and asks for EMDR. Another practice in the area offers EMDR and accepts the client's insurance. What is the **MOST** appropriate action?

**A.** Refer the client elsewhere for EMDR
**B.** Begin EMDR after studying a published protocol manual
**C.** Encourage the client to try a few CPT sessions first

**Why each option is right or wrong**

**A** **Correct.** Social workers practice within their competence and refer when another professional's specialized skills are needed to serve the client fully (NASW Code 1.04(a), 1.16(a)). EMDR, like CPT, is a recommended trauma-focused therapy for PTSD (VA/DoD, 2023). The client made an informed choice after hearing both options, and a trained provider is available nearby. It beats C because pressing the treatment the worker happens to offer overrides an informed, reasonable preference and risks dropout.

**B** **Incorrect** (trap: Outside the social work role). EMDR requires formal training with consultation; reading a manual does not make the worker competent. Using an approach new to the worker without training, consultation, and supervision conflicts with the competence standard (NASW Code 1.04(b)).

**C** **Incorrect** (trap: Overrides the client's choice). CPT is equally well supported, and the worker may describe its benefits. Pushing it after the client has declined disregards an informed choice. Encouragement would fit if the client were undecided or no EMDR provider were available.

**Takeaway:** When a client makes an informed choice of an evidence-based treatment outside the worker's training, refer to a trained provider.

**Sources:** NASW Code of Ethics (2021), 1.04(a)-(b); NASW Code of Ethics (2021), 1.16(a); VA/DoD Clinical Practice Guideline for the Management of Posttraumatic Stress Disorder and Acute Stress Disorder (2023), recommended trauma-focused psychotherapies

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## 247. INT-1006 (rev 3) · Best answer: C

A patient on an inpatient psychiatric unit is in a third admission this year for self-harm and has a diagnosis of borderline personality disorder. Between admissions, the patient attended weekly supportive therapy with a provider untrained in DBT. A standard outpatient DBT program has an opening. What is the **BEST** discharge plan?

**A.** Resume weekly supportive therapy with an updated safety plan

**B.** Step the patient down to partial hospital, then supportive therapy

**C.** Refer the patient for comprehensive DBT

**D.** Add DBT skills worksheets to the current supportive therapy

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). An updated safety plan belongs in any discharge, but returning to the same treatment that has not prevented three admissions repeats a failing plan when a stronger, evidence-based option is open.

**B** **Incorrect** (trap: Good but not best). A partial hospital program can ease the step down from inpatient care, and many teams consider it after repeated admissions. It is time-limited, though, and it returns the patient to the same supportive therapy that has not prevented three admissions, while an evidence-based program for recurrent self-harm is open now.

**C** **Correct.** Comprehensive DBT combines individual therapy, a skills group, between-session coaching, and a therapist consultation team. It was developed for chronic self-harm and has the strongest evidence for borderline personality disorder (Linehan, 1993). Three admissions despite supportive therapy show the current plan is not enough, and a full program is available. It beats D because worksheets added to untrained supportive therapy give only one piece of DBT, without the coaching, structure, and team that make it effective.

**D** **Incorrect** (trap: Good but not best). Skills practice helps, but worksheets delivered by a provider untrained in DBT lack individual DBT therapy, coaching, and the consultation team. Partial DBT is a fallback only when a full program is unavailable.

**Takeaway:** For recurrent self-harm with borderline personality disorder, refer to comprehensive DBT when available rather than repeating a failing plan.

**Sources:** Linehan, M. M. (1993). Cognitive-Behavioral Treatment of Borderline Personality Disorder. Guilford Press, modes of comprehensive DBT; target hierarchy
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 248. INT-1008 (rev 1) · Best answer: C

In an integrated primary care clinic, a client with type 2 diabetes tells the social worker, 'I bought walking shoes over the weekend and went around the block twice this week.' In motivational interviewing, which type of change talk is this?

**A.** Commitment
**B.** Ability
**C.** Taking steps
**D.** Desire

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Commitment language states an intention or promise about the future, as in 'I'm going to start walking.' This client is describing actions already completed, not a pledge.

**B** **Incorrect** (trap: Wrong model or concept). Ability language expresses that change is possible, such as 'I could probably walk after dinner.' This client reports having done it, not being able to.

**C** **Correct.** Motivational interviewing sorts change talk into preparatory types (desire, ability, reasons, need) and mobilizing types (commitment, activation, taking steps) (SAMHSA TIP 35, 2019). Buying shoes and walking twice describe actions the client has already taken toward change, which is taking steps. It beats A because commitment language states future intent, such as 'I will walk every day,' while this client reports what they have already done.

**D** **Incorrect** (trap: Wrong model or concept). Desire language expresses wanting change, such as 'I wish I were more active.' The statement here reports behavior already carried out, not a wish.

**Takeaway:** Change talk describing actions already taken toward change is taking steps; commitment language states future intent.

**Sources:** SAMHSA TIP 35 (2019), Ch. 3: Motivational Interviewing as a Counseling Style
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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### 249. INT-1009 (rev 2) · Best answer: B

A hospital social worker meets with a patient admitted for heart failure who keeps adding salt to meals brought from home. The patient asks, 'Why does salt even matter for my heart?' Which response is **MOST** consistent with motivational interviewing?

**A.** Explain how salt causes fluid buildup and strains the heart
**B.** Ask what the patient already knows about salt and the heart
**C.** Reflect that the diet limits seem frustrating to the patient

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Accurate information can help, and MI allows giving it. Leading with a full explanation, though, skips the first step of asking what the patient knows, which makes the information feel imposed and invites pushback.

**B** **Correct.** When a client asks for information, motivational interviewing uses elicit-provide-elicit: first ask what the person already knows or wants to know, then offer information, then ask what they make of it (SAMHSA TIP 35, 2019). Starting with a question respects the patient's expertise and lets the worker fill real gaps. It beats A because delivering the explanation first, however accurate, casts the worker as the expert and often prompts the patient to defend current habits.

**C** **Incorrect** (trap: Good but not best). Reflections are central to MI, but the patient asked a direct question. Reflecting a feeling the patient has not expressed sidesteps the request; elicit-provide-elicit answers it while keeping the patient engaged.

**Takeaway:** When a client asks for information, MI uses elicit-provide-elicit: ask what they know, offer information, then ask their reaction.

**Sources:** SAMHSA TIP 35 (2019), Ch. 3: Motivational Interviewing as a Counseling Style
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** MOST · **Cognitive** application · **Difficulty** 3 of 3

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### 250. INT-1010 (rev 1) · Best answer: A

In EMDR therapy, the social worker explains what to expect and teaches a calm-place exercise. This happens before any target memory is chosen. Which phase is this?

**A.** Preparation
**B.** Assessment
**C.** Installation
**D.** Closure

**Why each option is right or wrong**

**A** **Correct.** EMDR has eight phases: history taking, preparation, assessment, desensitization, installation, body scan, closure, and reevaluation (Shapiro, 2018). Preparation builds the alliance, explains what to expect, and equips the client with calming resources, such as a calm-place exercise, before any reprocessing begins. It beats D because closure also uses calming skills, but it comes at the end of a session in which reprocessing has occurred, to return the client to balance.

**B** **Incorrect** (trap: Wrong model or concept). Assessment, phase 3, comes after preparation. The client selects a target memory and rates its image, negative belief, preferred belief, and level of distress, setting up reprocessing.

**C** **Incorrect** (trap: Wrong model or concept). Installation, phase 5, follows desensitization. It strengthens the client's chosen positive belief about themselves once distress about the memory has dropped.

**D** **Incorrect** (trap: Wrong model or concept). Closure, phase 7, ends each reprocessing session by returning the client to calm, often with the same skills. The question describes groundwork laid before any reprocessing, which is preparation.

**Takeaway:** EMDR preparation (phase 2) explains the process and builds calming resources before any trauma memory is reprocessed.

**Sources:** Shapiro, F. (2018). Eye Movement Desensitization and Reprocessing (EMDR) Therapy: Basic Principles, Protocols, and Procedures (3rd ed.). Guilford Press, the eight phases
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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## 251. INT-1011 (rev 1) · Best answer: D

A group program was first built at a medical center for patients with chronic pain and illness. It uses 8 weekly classes of body scan, sitting meditation, and gentle yoga. What is it called?

**A.** Mindfulness-based cognitive therapy
**B.** DBT skills training group
**C.** Acceptance and commitment therapy
**D.** Mindfulness-based stress reduction

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Mindfulness-based cognitive therapy uses an 8-week group much like MBSR, but it was designed for people with recurrent depression and adds cognitive therapy elements aimed at preventing relapse (Segal, Williams & Teasdale, 2013).

**B** **Incorrect** (trap: Wrong model or concept). DBT skills groups teach mindfulness as one of four modules, alongside distress tolerance, emotion regulation, and interpersonal effectiveness. They were developed for chronic self-harm, not chronic pain.

**C** **Incorrect** (trap: Wrong model or concept). ACT builds psychological flexibility through acceptance, defusion, and values-based action. It is usually delivered as individual therapy and is not a fixed 8-week meditation and yoga curriculum.

**D** **Correct.** Mindfulness-based stress reduction (MBSR) was developed by Jon Kabat-Zinn at the University of Massachusetts Medical Center for medical patients, including people with chronic pain (Kabat-Zinn, 1982). The 8-week group teaches the body scan, sitting meditation, and mindful movement. It beats A because mindfulness-based cognitive therapy borrows MBSR's 8-week format but was built later to prevent depressive relapse and adds cognitive therapy elements.

**Takeaway:** MBSR is Kabat-Zinn's 8-week group for medical patients; MBCT adapts it, adding cognitive elements, to prevent depressive relapse.

**Sources:** Kabat-Zinn, J. (1982). An outpatient program in behavioral medicine for chronic pain patients based on the practice of mindfulness meditation. General Hospital Psychiatry, 4(1), 33-47, whole article; Segal, Z. V., Williams, J. M. G., & Teasdale, J. D. (2013). Mindfulness-Based Cognitive Therapy for Depression (2nd ed.). Guilford Press, development of MBCT from MBSR

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## 252. INT-1013 (rev 1) · Best answer: C

A client with social anxiety in ACT-based treatment says, 'I'm boring, and everyone at work can tell.' The client has been skipping team lunches because of this. Which response is **MOST** consistent with acceptance and commitment therapy?

**A.** Ask the client to list evidence for and against being boring

**B.** Point out qualities that coworkers likely value in the client

**C.** Invite the client to observe the thought as passing words

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Examining evidence is cognitive restructuring from CBT, which disputes the accuracy of a thought. ACT does not argue with thought content; it changes how the client relates to the thought.

**B** **Incorrect** (trap: Good but not best). Reassurance may feel supportive, but it tries to talk the client out of the thought. It neither builds distance from the thought nor moves the client toward valued action, and the thought is likely to return.

**C** **Correct.** ACT aims to change a client's relationship to thoughts rather than their content. Cognitive defusion helps the client step back and see a thought as words passing through the mind, not a fact to obey (Hayes, Strosahl & Wilson, 2012). For example, the client might say, 'I'm having the thought that I'm boring.' Seeing the thought this way loosens its grip, so the client can act on values, such as joining lunch, while the thought is present. It beats A because weighing evidence aims to change the thought's content, a CBT restructuring move rather than an ACT one.

**Takeaway:** ACT's cognitive defusion changes how a client relates to a thought, treating it as a passing thought rather than disputing its accuracy.

**Sources:** Hayes, S. C., Strosahl, K. D., & Wilson, K. G. (2012). Acceptance and Commitment Therapy: The Process and Practice of Mindful Change (2nd ed.). Guilford Press, cognitive defusion

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### 253. INT-1014 (rev 1) · Best answer: A

A client staying in an emergency shelter meets with a social worker who uses solution-focused brief therapy. Today the client says the week was awful, nothing improved, and the job search went nowhere. Which question is **MOST** appropriate for the social worker to ask?

**A.** 'How have you kept things from getting even worse?'
**B.** 'If a miracle happened tonight, what would be different?'
**C.** 'What do you think is keeping you stuck right now?'

**Why each option is right or wrong**

**A** **Correct.** Solution-focused brief therapy uses coping questions when a client reports that nothing has improved. Asking how the client has kept things from getting worse draws out the strengths and effort the client is already using, even in a hard week (de Shazer et al., 2007). It beats B because the miracle question invites the client to picture a preferred future, which can feel out of reach to someone describing a week of setbacks; the coping question meets the client where they are.

**B** **Incorrect** (trap: Misuses the technique). The miracle question is a core SFBT technique for describing goals. Asked right after the client reports a terrible week, it can sound dismissive. It fits better once the client feels heard and can picture change.

**C** **Incorrect** (trap: Wrong model or concept). Asking what keeps the client stuck invites analysis of the problem's causes. SFBT deliberately avoids problem analysis and builds on what the client is already doing that works.

**Takeaway:** In SFBT, when a client reports no progress, coping questions uncover the strengths that kept things from getting worse.

**Sources:** de Shazer, S., Dolan, Y., Korman, H., Trepper, T., McCollum, E., & Berg, I. K. (2007). More Than Miracles: The State of the Art of Solution-Focused Brief Therapy. Haworth Press, coping questions
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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### 254. INT-1015 (rev 2) · Best answer: B

After being stuck in an elevator, a client now avoids medical appointments on upper floors. Using systematic desensitization, the social worker and client have built a ranked list of elevator situations, and the client has learned progressive muscle relaxation. What should the social worker do **NEXT**?

**A.** Have the client ride to the top floor right away
**B.** Pair relaxation with imagining the least distressing item
**C.** Pair relaxation with imagining the most distressing item

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Going straight to the most feared situation is flooding, a different exposure method. It can work, but it abandons the graded hierarchy and is not systematic desensitization.

**B** **Correct.** Systematic desensitization counters anxiety with an incompatible relaxed state, which Wolpe called reciprocal inhibition (Wolpe, 1958). With the hierarchy built and relaxation learned, the next step is to pair relaxation with the lowest-ranked item,

moving up only when that item no longer provokes anxiety. It beats A because riding straight to the top is flooding, an intense, immediate exposure that skips the gradual hierarchy the client agreed to.

**C** **Incorrect** (trap: Misuses the technique). Pairing imagery with relaxation is the right technique, but systematic desensitization starts at the bottom of the ranked list. The most feared scene would bring on anxiety strong enough to overwhelm relaxation, so the client moves up only after each lower item no longer provokes anxiety.

**Takeaway:** Systematic desensitization: build a hierarchy, teach relaxation, then pair relaxation with items from least to most feared.

**Sources:** Wolpe, J. (1958). Psychotherapy by Reciprocal Inhibition. Stanford University Press, systematic desensitization

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## 255. INT-1016 (rev 1) · Best answer: C

Two months after moving into assisted living, a client with depression stays in their room most of the day. The client says, 'I'll join the activities once I feel like myself again.' The social worker is using cognitive behavioral therapy. Which intervention is **BEST** to start with?

**A.** Use thought records to track the client's negative thoughts

**B.** Explore the client's grief about the move to assisted living

**C.** Schedule one brief, enjoyable activity for each day

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). Thought records are central to CBT and will likely help later. For a client this withdrawn, written cognitive work early on can feel overwhelming; activity scheduling builds momentum first.

**B** **Incorrect** (trap: Good but not best). Grief about the move may be part of the picture and deserves attention. Within CBT, though, waiting for feelings to change before acting is the pattern to break, and activity scheduling addresses it directly.

**C** **Correct.** In CBT for depression, behavioral activation often comes first for clients who are withdrawn and inactive. Scheduling small, rewarding activities lifts mood, and motivation tends to follow action rather than come before it (Beck, 2021). The client's plan to wait until feeling better keeps the cycle of withdrawal going. It beats A because thought records take energy and concentration that a markedly inactive client may lack; cognitive work is added as functioning improves.

**Takeaway:** For a withdrawn, depressed client waiting to feel motivated, CBT starts with behavioral activation because action comes before motivation.

**Sources:** Beck, J. S. (2021). Cognitive Behavior Therapy: Basics and Beyond (3rd ed.). Guilford Press, behavioral activation

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### 256. INT-1125 (rev 1) · Best answer: B

In the fourth month of psychodynamic therapy, a client makes a joke each time the conversation turns to a parent's recent cancer diagnosis, then changes the subject. The client has never remarked on this pattern. What should the social worker do **FIRST**?

**A.** Suggest that the joking guards against fear of losing the parent
**B.** Point out that the jokes appear whenever the illness comes up
**C.** Ask the client to stop the jokes and stay with the feeling

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). This interpretation may well be accurate, and it is where the work is heading. Offered now, before the client has noticed the joking at all, it is likely to feel like an accusation and be dismissed. Interpret once the client can see the pattern.

**B** **Correct.** In psychodynamic technique, a defense is first brought to the client's attention (confrontation and clarification) before its unconscious meaning is interpreted (Greenson, 1967). The client has never remarked on the joking, so the pattern is not yet something the client can observe. Pointing out when it happens invites the client to notice it and strengthens the observing part of the ego. It beats A because an interpretation offered before the client sees the defense tends to be rejected or intellectualized.

**C** **Incorrect** (trap: Too directive). Telling the client to drop the humor attacks the defense head-on instead of understanding it. Psychodynamic work treats a defense as meaningful material to explore, not a behavior to stop on command, and pressing can increase resistance.

**Takeaway:** Bring a defense to the client's attention before interpreting its unconscious meaning; interpretation works best once the client can observe the pattern.

**Sources:** Greenson, R. R. (1967). The Technique and Practice of Psychoanalysis. International Universities Press, analysis of resistance: confrontation, clarification, interpretation
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 257. INT-1126 (rev 1) · Best answer: C

A young adult in weekly video sessions describes finishing a first art exhibit, then pauses and looks eagerly at the social worker for a reaction. In earlier sessions, the client seemed deflated when accomplishments drew little response. The social worker uses a self psychology approach. Which response is **MOST** consistent with this approach?

**A.** Point out that the client seems to need approval to feel worthwhile
**B.** Keep a neutral stance so the wish for praise can be analyzed
**C.** Respond warmly to the pride the client is showing

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Naming the wish for approval as a problem treats the need as pathology to confront. That stance fits Kernberg's interpretive approach to narcissism (Kernberg, 1975), not Kohut's, and it is likely to land as criticism with a client who already deflates easily.

**B** **Incorrect** (trap: Wrong model or concept). Withholding a response so the wish can be analyzed reflects classical neutrality and abstinence. Self psychology instead lets this kind of transference develop, and a flat response here would repeat the deflation the client has shown before.

**C** **Correct.** Kohut's self psychology treats the need to have one's accomplishments received with delight, a mirroring selfobject need, as a normal developmental need rather than a defect (Kohut, 1971). The client is looking to the worker for that response and has deflated when it was missing. Warm, attuned interest in the client's pride meets the need and supports a more stable sense of self over time. It beats A because labeling the need for approval as a problem is a confrontive stance that this model avoids and that the client would likely hear as shaming.

**Takeaway:** In self psychology, meet a client's mirroring needs with attuned, warm responsiveness rather than confronting them or withholding a response.

**Sources:** Kohut, H. (1971). The Analysis of the Self. International Universities Press, mirror transference; Kernberg, O. F. (1975). Borderline Conditions and Pathological Narcissism. Jason Aronson, pathological narcissism: interpretation of grandiosity

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

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### 258. INT-1127 (rev 2) · Best answer: A

Early in psychodynamic therapy, a client realized that they keep choosing partners who dismiss them, much as a parent did. Over the next 8 months, the client spots the same expectation with a boss, a close friend, and the social worker. Each time, the client feels less upset and tries a new response. Which process is **MOST** likely taking place?

**A.** Working through
**B.** Repetition compulsion
**C.** Corrective emotional experience
**D.** Abreaction

#### Why each option is right or wrong

**A** **Correct.** Working through is the gradual, repeated process of applying an insight across many situations until it changes feeling and behavior (Freud, 1914). The client gained the insight months ago and now recognizes the same expectation with a boss, a friend, and the social worker, with less distress and new responses each time. It beats B because repetition compulsion means reenacting the old pattern without awareness; this client is noticing the pattern and acting differently, which shows it loosening.

**B** **Incorrect** (trap: Wrong model or concept). Repetition compulsion is the unconscious pull to recreate early painful relationships. The history of choosing dismissive partners fits it, but the client is now recognizing the expectation and responding differently, which shows the pattern changing rather than repeating.

**C** **Incorrect** (trap: Wrong model or concept). A corrective emotional experience occurs when the therapist responds differently from the original figure, so the client relives the old conflict with a new ending (Alexander & French, 1946). The stem centers on the client's own repeated recognition across many relationships, not on a particular response from the social worker.

**D** **Incorrect** (trap: Wrong model or concept). Abreaction is the emotional release that comes from reliving a repressed memory, usually in one intense moment. The client here changes gradually over months, with feelings becoming milder, not through a sudden discharge of emotion.

**Takeaway:** Insight alone rarely changes a pattern; working through applies it repeatedly across situations until feelings and behavior shift.

**Sources:** Freud, S. (1914). Remembering, repeating and working-through. In Standard Edition of the Complete Psychological Works of Sigmund Freud, Vol. 12. Hogarth Press, working through; Alexander, F., & French, T. M. (1946). Psychoanalytic Therapy: Principles and Application. Ronald Press, corrective emotional experience
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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## 259. INT-1128 (rev 3) · Best answer: A

After 8 months of psychodynamic therapy for relationship problems, a client starts having panic attacks and now avoids driving on highways. The client asks to focus on the panic. When the social worker explores the request, the client says the attacks feel urgent and are hurting the client's job. The social worker is trained in CBT for panic disorder. What is the **BEST** response?

**A.** Shift to treating the panic with interoceptive exposure
**B.** Explore how the panic connects to the relationship themes
**C.** Add a CBT panic workbook while continuing the current sessions

#### Why each option is right or wrong

**A** **Correct.** Evidence-based practice joins research evidence, clinical expertise, and client preference. CBT with interoceptive exposure is a well-supported first-line treatment for panic disorder (Craske & Barlow, 2022). The client wants the panic addressed, avoidance is spreading to work, and the social worker has the training, so shifting the focus by agreement to this CBT method fits all three. It beats B because the worker has already explored the request; continuing to search for relational meaning delays an effective treatment while the avoidance grows.

**B** **Incorrect** (trap: Good but not best). Linking symptoms to relationship themes is standard psychodynamic work, and it was right to explore what the request means. That step is done, and the client has said the attacks feel urgent. More interpretation now postpones a targeted, effective treatment.

**C** **Incorrect** (trap: Good but not best). CBT-based self-help can reduce panic and keeps the existing work going, so this is tempting. But the client wants the panic treated now, avoidance is spreading to work, and the worker is trained to deliver the treatment directly; a workbook beside sessions that stay on relationship themes addresses the urgent problem only in part.

**Takeaway:** When a new disorder emerges in therapy, offer an evidence-based treatment the client wants and the worker is trained to provide.

**Sources:** Craske, M. G., & Barlow, D. H. (2022). Mastery of Your Anxiety and Panic: Therapist Guide (5th ed.). Oxford University Press, treatment overview; interoceptive exposure
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 260. INT-1129 (rev 1) · Best answer: C

A client in standard outpatient DBT reports at the start of an individual session that they cut their forearm 2 days ago after an argument. The social worker checks the wound, which needed no medical care, and confirms the client has no current urge or intent to self-harm. What should the social worker do **NEXT**?

**A.** Teach a new distress tolerance skill to use after arguments
**B.** Turn to the argument and explore what made it so painful
**C.** Do a chain analysis of the events that led to the cutting

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Teaching a skill is part of the response, but in DBT it follows the chain analysis, which shows where in the sequence a skill would help. Choosing one now skips the analysis that makes the skill a good fit.

**B** **Incorrect** (trap: Misuses the technique). The argument is one link in the chain, but an open talk about feelings misses the structured, step-by-step review of vulnerabilities, links, and consequences that DBT uses to understand and change a target behavior.

**C** **Correct.** In DBT, self-harm since the last session is the top target in individual therapy, and the standard response is a behavioral chain analysis: a moment-by-moment review of vulnerabilities, the prompting event, thoughts, feelings, urges, the behavior, and its consequences (Linehan, 1993). Current safety has been checked, so the chain analysis comes next. It shows the links where skills could replace cutting. It beats A because picking a skill before the chain is mapped guesses at which link needs to change; solutions follow the analysis.

**Takeaway:** In DBT, review self-harm since the last session with a detailed chain analysis, then match skills to the links it reveals.

**Sources:** Linehan, M. M. (1993). Cognitive-Behavioral Treatment of Borderline Personality Disorder. Guilford Press, behavioral analysis: chain analysis; target hierarchy
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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### 261. INT-1130 (rev 1) · Best answer: D

In the third week of an 8-week MBSR group, a member with PTSD grips the chair and breathes rapidly during the body scan. Afterward, the member says the exercise brought back the assault and the room felt far away. The member wants to stay in the group. The social worker is trained in MBSR. What is the **BEST** response?

**A.** Encourage the member to keep attention on body sensations as they arise
**B.** Help the member process the assault memory in a private session
**C.** Arrange individual trauma therapy in place of the group sessions
**D.** Offer options such as eyes open and an outside focus during practice

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Turning toward sensations is the standard MBSR instruction, and it helps many members. For a member who is dissociating, more inward body focus can deepen the flashback. Trauma-sensitive practice shifts attention to steadier anchors first.

**B** **Incorrect** (trap: Good but not best). Processing the memory is trauma treatment, not part of a mindfulness group, and it does not address what the member should do during the next body scan. Memory work belongs in a planned trauma therapy if the member chooses one.

**C** **Incorrect** (trap: Hands off too soon). A trauma therapy referral can be offered as an addition if the member wants it. Making it a replacement overrides the member's wish to stay and skips simple changes that often make practice tolerable.

**D** **Correct.** Trauma-sensitive mindfulness keeps practice within the survivor's window of tolerance by offering choices, such as eyes open, shorter periods, or anchoring attention on sounds or the feet instead of inner sensations (Treleaven, 2018). The gripping, rapid breathing, and sense of distance signal hyperarousal and dissociation, yet the member wants to continue. Adjusting the practice honors that choice, a core trauma-informed principle (SAMHSA, 2014), while lowering risk. It beats C because removing the member from a valued group is not needed when simple changes can make practice safe.

**Takeaway:** When mindfulness practice triggers trauma symptoms, adapt it with choices and external anchors before removing the client from it.

**Sources:** Treleaven, D. A. (2018). Trauma-Sensitive Mindfulness: Practices for Safe and Transformative Healing. W. W. Norton, window of tolerance; shifting attention to stabilizing anchors; SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Six key principles: empowerment, voice and choice
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 262. INT-1131 (rev 1) · Best answer: B

A 78-year-old hospital patient recovering from a fall has agreed to talk with the social worker about safety at home. When the social worker mentions grab bars and a shower chair, the patient snaps, 'You sound just like my kids. Everyone is telling me what to do.' Which response is **MOST** appropriate for a social worker using motivational interviewing?

**A.** Explain the fall risks again in simpler, clearer terms
**B.** Emphasize that changes at home are the patient's decision
**C.** Ask what it would take for the patient to accept the changes

#### Why each option is right or wrong

**A** **Incorrect** (trap: Too directive). Repeating the risks is the righting reflex: the urge to persuade. When a patient already feels lectured, more information tends to strengthen the pushback. Information works better later, offered with permission.

**B** **Correct.** MI separates sustain talk about a behavior from discord, which is friction in the relationship itself. The patient's complaint about being told what to do signals discord, and MI responds by changing direction, often by emphasizing personal choice (TIP 35, Ch. 3). Affirming that decisions about the home belong to the patient eases the pressure and keeps the conversation open. It beats C because asking what it would take still pushes toward the worker's goal while the patient feels pushed.

**C** **Incorrect** (trap: Misuses the technique). Evoking the patient's own reasons is a core MI skill, but this question presses toward acceptance while the relationship is strained.

Discord calls for easing pressure first; evoking can resume once the patient feels respected.

**Takeaway:** When a client signals discord with the worker, ease pressure by emphasizing autonomy before trying to evoke change.

**Sources:** SAMHSA TIP 35 (2019), Ch. 3: Motivational Interviewing as a Counseling Style (discord; emphasizing autonomy)
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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### 263. INT-1132 (rev 1) · Best answer: C

Which PTSD treatment has the client write an impact statement and then examine 'stuck points,' such as self-blame for the trauma?

- **A.** Prolonged exposure
- **B.** EMDR
- **C.** Cognitive processing therapy
- **D.** Trauma-focused CBT

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Prolonged exposure reduces PTSD by having the client repeatedly recount the trauma memory and approach safe but avoided reminders. It works mainly through exposure, not through an impact statement and stuck-point work.

**B** **Incorrect** (trap: Wrong model or concept). EMDR pairs recall of trauma images with bilateral stimulation, such as eye movements, across eight phases. It does not use a written impact statement or structured work on stuck points.

**C** **Correct.** Cognitive processing therapy (CPT) is a structured, usually 12-session cognitive treatment for PTSD. The client writes an impact statement about why the trauma happened and how it changed beliefs about self, others, and the world, and the work then targets stuck points, such as self-blame, that keep recovery from moving forward (Resick, Monson & Chard, 2017). It beats A because prolonged exposure relies on repeated exposure to memories and reminders rather than on examining written beliefs.

**D** **Incorrect** (trap: Wrong model or concept). TF-CBT is designed for children and adolescents with their caregivers, building coping skills before a trauma narrative. It is not the protocol organized around an impact statement and stuck points.

**Takeaway:** Cognitive processing therapy targets trauma-related stuck points, such as self-blame, starting from a written impact statement.

**Sources:** Resick, P. A., Monson, C. M., & Chard, K. M. (2017). Cognitive Processing Therapy for PTSD: A Comprehensive Manual. Guilford Press, impact statement; stuck points
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.7 Evidence-based treatments: CBT, DBT, MBSR, MI, EMDR · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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### 264. INT-1021 (rev 1) · Best answer: A

After a layoff, a client at a community job center tells the social worker they lie awake panicking about a utility shutoff notice due in 10 days. The client has not yet contacted the utility or applied for bill assistance. Which coping skill is **BEST** for the social worker to teach?

**A.** Break the problem into concrete steps to act on
**B.** Practice slow breathing when panic rises at night
**C.** Write about the worries in a journal before bed

**Why each option is right or wrong**

**A** **Correct.** Coping works best when it fits the stressor. When a stressor can be changed, problem-focused coping (defining the problem, generating options, and acting) reduces the threat itself; emotion-focused coping suits stressors that cannot be changed (Lazarus & Folkman, 1984). The shutoff is controllable, with 10 days left and untried options such as calling the utility and applying for help, so structured problem solving fits (D'Zurilla & Nezu, 2007). It beats B because breathing calms the nights but leaves the shutoff unaddressed, so the panic will return.

**B** **Incorrect** (trap: Good but not best). Slow breathing is a useful skill for nighttime panic and could be added. Used alone, it soothes the feeling while the controllable problem driving it moves closer to its deadline.

**C** **Incorrect** (trap: Good but not best). Journaling can reduce rumination and help sleep. Like breathing, it manages the emotion but leaves an actionable, time-limited problem untouched.

**Takeaway:** Match coping to the stressor: problem-focused coping for changeable problems, emotion-focused coping for stressors that cannot be changed.

**Sources:** Lazarus, R. S., & Folkman, S. (1984). Stress, Appraisal, and Coping. Springer, problem-focused and emotion-focused coping; D'Zurilla, T. J., & Nezu, A. M. (2007). Problem-Solving Therapy: A Positive Approach to Clinical Intervention (3rd ed.). Springer, problem-solving steps
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.8 Teaching clients coping and self-care skills · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 265. INT-1022 (rev 2) · Best answer: B

A client with schizophrenia is moving into a supported apartment and wants to learn to cook simple meals safely. The social worker has explained why each step matters, such as turning off the stove. What should the social worker do **NEXT**?

**A.** Give the client a written checklist to follow at home
**B.** Demonstrate cooking a meal while the client watches
**C.** Have the client try cooking a meal with coaching

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). A written checklist is a helpful memory aid during home practice. Given now, it replaces seeing and rehearsing the skill, which is how clients with schizophrenia tend to learn most reliably.

**B** **Correct.** Skills training follows a set sequence: explain the rationale, model the skill, have the client rehearse it with feedback, then assign practice in real settings (Bellack et al., 2004). With the rationale covered, the social worker demonstrates the skill so the

client sees each step done correctly. It beats C because coached rehearsal comes after modeling; asking the client to perform before seeing each step done correctly skips the demonstration that rehearsal builds on.

**C** **Incorrect** (trap: Right action, wrong time). Coached rehearsal with feedback is the step after modeling. Asking the client to cook before seeing the skill demonstrated skips that step, so mistakes with safety steps such as the stove are more likely and harder to correct.

**Takeaway:** Teach self-care skills in sequence: explain the rationale, model, rehearse with feedback, then assign independent practice.

**Sources:** Bellack, A. S., Mueser, K. T., Gingerich, S., & Agresta, J. (2004). Social Skills Training for Schizophrenia: A Step-by-Step Guide (2nd ed.). Guilford Press, steps of skills training

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## 266. INT-1025 (rev 1) · Best answer: B

At a senior center, a 74-year-old client tells the social worker that most nights they lie awake in bed for an hour or more, watching TV and checking the news on a tablet until sleep comes. A physician found no medical or medication cause. Which self-care skill is **BEST** for the social worker to teach?

**A.** Swap the TV and tablet for reading in bed until drowsy

**B.** Get up when unable to sleep and return when sleepy

**C.** Go to bed earlier to allow more total time for sleep

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misuses the technique). Reading is calmer than screens, so this feels like progress. Stimulus control, though, asks that the bed be kept for sleep (and sex) only. Reading in bed still pairs the bed with being awake.

**B** **Correct.** Stimulus control is a recommended behavioral treatment for chronic insomnia: go to bed only when sleepy, get out of bed when unable to sleep, and keep the bed for sleep rather than screens or reading (Edinger et al., 2021). Lying awake with the TV and a tablet has taught this client that bed is a place to be awake. Getting up and returning only when sleepy rebuilds the link between bed and sleep. It beats A because reading in bed is gentler than screens but still pairs the bed with wakefulness.

**C** **Incorrect** (trap: Misuses the technique). More time in bed usually means more time lying awake, which deepens insomnia. Stimulus control asks people to go to bed only when sleepy, not earlier in the hope of banking extra hours.

**Takeaway:** For chronic insomnia, teach stimulus control: go to bed only when sleepy, keep the bed for sleep, and get up when unable to sleep.

**Sources:** Edinger, J. D., et al. (2021). Behavioral and psychological treatments for chronic insomnia disorder in adults: an American Academy of Sleep Medicine clinical practice guideline. Journal of Clinical Sleep Medicine, 17(2), 255-262, Recommendation 3: stimulus control

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.8 Teaching clients coping and self-care skills · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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### 267. INT-1135 (rev 2) · Best answer: B

A 74-year-old caring at home for a spouse with Parkinson's disease has worked with the social worker for 6 weeks on relaxation, sleep habits, and short daily breaks. The spouse now needs help turning and toileting several times a night, and the caregiver sleeps about 3 hours. The caregiver says, 'I can't keep this up, but letting strangers in feels like breaking my promise to care for my spouse myself.' What should the social worker do **FIRST**?

**A.** Teach a new set of relaxation exercises for the caregiver
**B.** Explore the caregiver's feelings about accepting outside help
**C.** Suggest the caregiver ask a physician about a sleep aid
**D.** Help arrange overnight in-home respite so the caregiver can sleep

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Relaxation skills were taught and practiced for 6 weeks. The barrier now is hands-on care through the night that leaves little time to sleep, which more coping techniques cannot fix.

**B** **Correct.** Exploring ambivalence comes before arranging help the client is unsure about. The caregiver both sees the need ('I can't keep this up') and feels that outside help breaks a promise, so a respite referral made now may be refused or cancelled. Talking through what help would mean, including the spouse's wishes and what kind of help might feel acceptable, lets the caregiver work through the conflict and choose the support (Miller & Rollnick, 2023). It beats D because overnight respite is the likely next step, but arranging it before hearing the caregiver's reservations sidesteps an unresolved choice.

**C** **Incorrect** (trap: Good but not best). A physician visit for the caregiver's own health is reasonable, but a sleep aid does not solve being woken repeatedly for care tasks and could make it harder to respond safely at night.

**D** **Incorrect** (trap: Right action, wrong time). Overnight respite fits the gap: 6 weeks of coping strategies cannot replace sleep lost to nightly care, and respite is a core caregiver support service (Older Americans Act, National Family Caregiver Support Program). It is the likely next step, but the caregiver has just voiced a conflict about outside help, and arranging it before exploring that risks a service the caregiver will not use.

**Takeaway:** When an overloaded caregiver is torn about outside help, explore the reservations first; then arrange respite the caregiver can accept.

**Sources:** Miller & Rollnick (2023). Motivational Interviewing (4th ed.). Guilford Press, Ambivalence: exploring both sides; Older Americans Act, National Family Caregiver Support Program, 42 U.S.C. 3030s-1(b)
**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.8 Teaching clients coping and self-care skills · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

### 268. INT-1026 (rev 3) · Best answer: A

An employee assistance program refers a 52-year-old client for stress management by video. The client describes long hours and a new manager. The client also mentions chest tightness when climbing stairs at work that eases after resting, and has not seen a physician in 5 years. The client asks to learn relaxation skills. What is the social worker's **BEST** response?

**A.** Urge the client to get a prompt medical evaluation
**B.** Teach slow breathing to use when the tightness starts
**C.** Ask the client to log when the tightness occurs this week

**Why each option is right or wrong**

**A** **Correct.** When physical symptoms could signal a medical problem, a medical evaluation comes before stress-management techniques. Chest tightness brought on by exertion and relieved by rest is a typical angina pattern and needs a physician's assessment (American Heart Association), and heart and lung conditions can produce symptoms that look like stress or anxiety (DSM-5-TR). Social workers don't diagnose medical conditions; they route the client to medical care. It beats C because a symptom log sounds careful but spends a week watching a possible heart problem instead of having it checked; a log can support the medical visit, not replace it. Symptoms at rest or worsening call for emergency care.

**B** **Incorrect** (trap: Ignores risk). Breathing skills may help later, but teaching them for this symptom treats possible chest pain as stress and could delay care for a heart problem. Relaxation training fits once medical causes have been evaluated.

**C** **Incorrect** (trap: Ignores risk). Tracking symptoms is a reasonable assessment habit, and a log could give a physician useful detail. Here it delays care: chest tightness brought on by exertion needs a physician's evaluation now, and a week of monitoring leaves a possible cardiac problem unchecked.

**Takeaway:** Before teaching stress-management skills, send possible medical symptoms, such as chest pain on exertion, for medical evaluation; social workers don't diagnose them.

**Sources:** DSM-5-TR (APA, 2022), Anxiety Disorders: Anxiety Disorder Due to Another Medical Condition; American Heart Association (2025). Angina (Chest Pain). heart.org, last reviewed Jan 30, 2025, Quick Facts; stable versus unstable angina

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.9 Managing stress: client techniques · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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### 269. INT-1027 (rev 1) · Best answer: C

In Meichenbaum's stress inoculation training, during which phase does the client practice coping skills under graded imagined and real-life stressors?

**A.** Conceptualization
**B.** Skills acquisition and rehearsal
**C.** Application and follow-through

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Conceptualization is the first phase. The client and social worker build a shared understanding of how stress works for this client, and coping skills have not yet been taught or tested.

**B** **Incorrect** (trap: Wrong model or concept). Skills are learned and rehearsed in this phase, so the word 'practice' makes it tempting. Exposure to graded stressors, imagined and then real, comes in the next phase, after the skills are in place.

**C** **Correct.** Stress inoculation training has three phases. Conceptualization builds a shared understanding of stress and the client's reactions. Skills acquisition and rehearsal teaches and practices coping skills such as relaxation and self-instruction. Application and follow-through has the client use those skills against graded stressors, first in imagery and role-play, then in real life, with relapse prevention (Meichenbaum, 1985). It beats B because the second phase rehearses skills within the session before they are tested against stress.

**Takeaway:** Stress inoculation moves from understanding stress, to learning and rehearsing coping skills, to applying them under graded stressors.

**Sources:** Meichenbaum, D. (1985). Stress Inoculation Training. Pergamon Press, three phases of stress inoculation training

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.9 Managing stress: client techniques · **Qualifier** none · **Cognitive** recall · **Difficulty** 3 of 3

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## 270. INT-1028 (rev 1) · Best answer: A

In a hospital ICU waiting room, a patient's adult child tells the social worker they have not slept in 2 days. They are pacing, their hands are shaking, and they say, 'I can't think straight.' The medical team will meet with the family in half an hour to discuss treatment choices. What should the social worker do **FIRST**?

**A.** Guide a few minutes of slow, paced breathing

**B.** Help write down questions for the medical team

**C.** Explore their fears about the parent's prognosis

#### Why each option is right or wrong

**A** **Correct.** When acute stress is high enough to block clear thinking, a brief calming technique comes before problem solving. Slow, even breathing is quick and portable, and Psychological First Aid uses it to help distressed people steady themselves (Brymer et al., 2006). The pacing, shaking, and 'I can't think straight' show arousal that will undercut the family meeting. It beats B because preparing questions is the right next step and will go better once the person is calmer; started now, it asks a flooded mind to plan. Thirty minutes leaves time for both.

**B** **Incorrect** (trap: Right action, wrong time). Writing down questions is useful problem-focused coping before a stressful meeting, and it should follow. Right now the person says they can't think straight, so planning will work better after a few minutes of calming.

**C** **Incorrect** (trap: Good but not best). Exploring fears has value later, but opening the hardest topic now may raise arousal further with the meeting 30 minutes away. Brief calming and practical preparation fit this moment better.

**Takeaway:** When acute stress floods thinking, use a brief calming technique first, then move to problem-focused steps such as preparing questions.

**Sources:** Brymer, M., et al. (2006). Psychological First Aid: Field Operations Guide (2nd ed.). National Child Traumatic Stress Network and National Center for PTSD, Stabilization and Information on Coping: breathing technique

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.9 Managing stress: client techniques · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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## 271. INT-1136 (rev 1) · Best answer: C

A graduate student tells a campus social worker that worries about loans and grades pop up all day, breaking concentration during study time and lectures. The student has tried to push the worries out of mind, but they come back stronger. Which stress management technique is **BEST** to teach?

**A.** Shift to a quick task whenever worries start

**B.** Talk worries over with a friend right away

**C.** Postpone worries to a set daily worry period

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Brief distraction can help in a single stressful moment. Used every time a worry starts, it lets each worry interrupt study and never gives the student a set time to deal with real problems such as loans.

**B** **Incorrect** (trap: Good but not best). Support from friends eases stress, but talking each worry over the moment it comes up interrupts study as much as the worry does and can turn into constant reassurance seeking.

**C** **Correct.** Worry postponement, a stimulus control technique, has the client notice a worry, set it aside, and save it for a fixed daily period in one place (Borkovec et al., 1983). It narrows the times and cues linked to worrying, freeing study and lecture time while the worries still get attention later. The student's attempts to suppress the worries backfired, as suppression often does (Wegner et al., 1987). It beats A because distracting each time still lets every worry interrupt and leaves no planned time to address real concerns.

**Takeaway:** For worry that intrudes all day, postponing it to a set daily worry period works better than suppression or constant distraction.

**Sources:** Borkovec, T. D., Wilkinson, L., Folensbee, R., & Lerman, C. (1983). Stimulus control applications to the treatment of worry. Behaviour Research and Therapy, 21(3), 247-251, whole article; Wegner, D. M., Schneider, D. J., Carter, S. R., & White, T. L. (1987). Paradoxical effects of thought suppression. Journal of Personality and Social Psychology, 53(1), 5-13, whole article

**Area** III Intervention and Practice · **Sub-area** III.B Intervention Methods and Techniques · **Topic** III.B.9 Managing stress: client techniques · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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# III.C Practice Evaluation and Research

### 272. INT-0007 (rev 1) · Best answer: D

A social worker records a client's daily anxiety ratings for 2 weeks, then begins cognitive behavioral therapy. Ratings fall steadily over the next 6 weeks. During the same period, the client starts a new job they enjoy. Which conclusion is **BEST** supported by these data?

**A.** The cognitive therapy caused the drop in the client's anxiety
**B.** The new job, rather than therapy, caused the change
**C.** The data are unusable because the baseline was too short
**D.** Anxiety improved, but the design cannot isolate the cause

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). AB designs cannot establish causation. With a new job starting during the intervention phase, history is a plausible rival explanation, so a causal claim about therapy goes beyond the data.

**B** **Incorrect** (trap: Wrong model or concept). This makes the same error in the other direction. The design can no more show that the job caused the change than it can show that therapy did.

**C** **Incorrect** (trap: Wrong model or concept). Fourteen daily ratings give a reasonable baseline, since several stable points are generally enough. The data are usable; the limit is causal inference, not baseline length.

**D** **Correct.** This is an AB single-system design: a baseline phase followed by an intervention phase. It can show that change followed the start of the intervention, but it cannot rule out other explanations such as history, here a new job starting at the same time (Bloom, Fischer & Orme, 2009). The supported conclusion is that anxiety improved and the cause is uncertain. It beats A because claiming therapy caused the change ignores the competing event, a threat to internal validity that an AB design cannot control.

**Takeaway:** An AB design can show change after intervention but cannot prove the intervention caused it; outside events remain rival explanations.

**Sources:** Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, AB design; threats to internal validity
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.1 Evaluating progress and effectiveness of intervention and service plans · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 273. INT-0114 (rev 1) · Best answer: C

A school social worker plans to use a token reward system to reduce a 7-year-old's classroom outbursts. The social worker wants data showing whether the system works. What should the social worker do **FIRST**?

**A.** Start the token system and count outbursts weekly
**B.** Ask the teacher to rate progress at the end of the term
**C.** Count the outbursts daily for a period before starting

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Tracking during the intervention is part of the design, but starting the token system first skips the baseline. Without pre-intervention data, the social worker cannot tell whether outbursts changed. Skipping a baseline is reserved for crises in which delay is unsafe.

**B** **Incorrect** (trap: Good but not best). A single end-of-term rating is subjective and gives no pattern over time or comparison with how things were before. Teacher input is useful as repeated, specific counts rather than one summary rating.

**C** **Correct.** Single-system evaluation begins with a baseline phase: repeated measurement of the target behavior before intervention, so later change can be compared with the starting pattern (Bloom, Fischer & Orme, 2009). Counting outbursts daily for a period, ideally until the pattern is stable, provides that comparison. It beats A because starting the intervention first leaves no record of how often outbursts happened before, so later counts have nothing to be measured against.

**Takeaway:** To evaluate an intervention with one client, measure the target behavior repeatedly before starting (baseline), then keep measuring.

**Sources:** Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, baseline phase; AB design

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## 274. INT-1567 (rev 1) · Best answer: B

An outpatient social worker gives a client with depression the PHQ-9 at each session of cognitive behavioral therapy. After 8 sessions, the totals remain in the moderate range with no downward trend. The client says the sessions are helpful and would like to keep going the same way. What should the social worker do **FIRST**?

**A.** Continue the current approach because the client finds it helpful

**B.** Go over the results with the client and revise the plan

**C.** Ask a psychiatric prescriber to evaluate the client for medication

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). The client's satisfaction and the alliance matter, but flat scores after 8 sessions show the approach is not reducing symptoms. Continuing without change risks prolonging the depression.

**B** **Correct.** Routine outcome monitoring is meant to change care when a client is not on track; feedback on stalled progress reduces deterioration (Lambert & Shimokawa, 2011). Eight sessions without change is a signal to act. Sharing the graph with the client and revising the plan together respects the client's view while taking the data seriously (Bloom, Fischer & Orme, 2009). It beats A because liking the sessions is not the same as improving, and continuing unchanged ignores what the measure shows.

**C** **Incorrect** (trap: Right action, wrong time). A medication evaluation may well become part of a revised plan. Choosing it before reviewing the data with the client skips the joint review that should shape what changes.

**Takeaway:** When progress data stay flat, review them with the client and revise the plan together, even if the client likes the sessions.

**Sources:** Lambert, M. J., & Shimokawa, K. (2011). Collecting client feedback. Psychotherapy, 48(1), 72-79, whole article; Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, single-system designs: baselines, AB, withdrawal and multiple-baseline designs
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## 275. INT-1568 (rev 1) · Best answer: C

A home-based case management program for older adults serves clients with very different aims. One wants to attend a senior center weekly, another to pay bills on time, and a third to keep medical appointments. The director wants one evaluation method that tracks each client's own targets and yields scores that can be compared across clients. Which method is **BEST**?

- **A.** Self-anchored rating scale
- **B.** Geriatric Depression Scale
- **C.** Goal Attainment Scaling
- **D.** Behavioral frequency count

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). A self-anchored scale lets each client define the end points of a rating, which suits individual tracking. It does not combine several goals into a standardized score, so comparing clients is difficult.

**B** **Incorrect** (trap: Wrong model or concept). The Geriatric Depression Scale is a standardized screen for one construct, depression. It does not measure progress on attendance, bill paying or appointments.

**C** **Correct.** Goal Attainment Scaling sets individualized goals for each client, defines expected outcome levels for each goal from -2 to +2, and converts results to a standard score that can be compared across clients (Kiresuk & Sherman, 1968). It fits a program whose clients pursue very different aims. It beats A because a self-anchored scale is individualized but tracks one dimension at a time and offers no standard way to combine goals or compare clients.

**D** **Incorrect** (trap: Wrong model or concept). Counting behaviors works well for one observable target, such as visits attended. Counts of different behaviors across clients cannot be combined or compared on a common metric.

**Takeaway:** Goal Attainment Scaling measures progress on individualized goals and converts it to a standard score comparable across clients.

**Sources:** Kiresuk, T. J., & Sherman, R. E. (1968). Goal attainment scaling: A general method for evaluating comprehensive community mental health programs. Community Mental Health Journal, 4(6), 443-453, whole article
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### 276. INT-1569 (rev 2) · Best answer: B

A residential treatment social worker plans a point system to reduce a 15-year-old's verbal aggression on the living unit, in the classroom and during family visits. The team wants evidence that the point system, rather than outside events, produces any change. Staff will not remove the point system once it starts working. Which evaluation design is **MOST** appropriate?

**A.** Reversal (ABAB) design
**B.** Multiple-baseline design
**C.** AB design

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Withdrawing and then reinstating the intervention gives strong causal evidence, but it requires removing a working program. The team will not do that, so this design does not fit.

**B** **Correct.** A multiple-baseline design starts the intervention at staggered times across behaviors, settings or people. If aggression drops in each place only when the point system begins there, outside events become an unlikely explanation, and no withdrawal is needed (Bloom, Fischer & Orme, 2009). The three locations suit this design, and staff refuse to remove a working intervention. It beats A because a reversal design gets its causal evidence by withdrawing the intervention, which the team will not do.

**C** **Incorrect** (trap: Wrong model or concept). An AB design shows whether change followed the intervention but cannot rule out outside events, such as a staff change or family news, so it does not meet the team's need for causal evidence.

**Takeaway:** To show an intervention caused change without withdrawing it, stagger its start across settings, behaviors or people in a multiple-baseline design.

**Sources:** Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, single-system designs: baselines, AB, withdrawal and multiple-baseline designs
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### 277. INT-1570 (rev 1) · Best answer: A

A social worker providing telehealth services plans to evaluate exposure work for a client with social anxiety. During a 2-week baseline, the client records how many conversations they start each day. The counts rise steadily across the 14 days, from 1 a day to 5 a day. The client is not in crisis. What should the social worker do **NEXT**?

**A.** Keep collecting daily counts until they level off
**B.** Begin the planned exposure sessions on schedule
**C.** Replace the baseline counts with a standardized anxiety scale

**Why each option is right or wrong**

**A** **Correct.** Change during an intervention can be credited to it only if the baseline is stable. When baseline data already trend in the desired direction, the usual step is to extend the baseline until it levels off, or to reconsider whether intervention is needed (Bloom, Fischer & Orme, 2009). The client is improving without exposure and is not in crisis, so waiting costs little. It beats B because starting now would make further gains impossible to separate from the existing upward trend.

**B** **Incorrect** (trap: Misuses the technique). Keeping to the schedule seems efficient, but an improving baseline means later gains could simply continue the trend. Starting now is reasonable when delay would harm the client, which is not the case here.

**C** **Incorrect** (trap: Misuses the technique). A standardized scale can add useful data, but switching measures does not solve the problem of a trending baseline and breaks the continuity needed to compare phases.

**Takeaway:** Begin an intervention phase only after a stable baseline; if the baseline trends toward improvement, extend it or reconsider the need.

**Sources:** Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, single-system designs: baselines, AB, withdrawal and multiple-baseline designs **Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.1 Evaluating progress and effectiveness of intervention and service plans · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 3 of 3

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## 278. INT-1571 (rev 2) · Best answer: C

A client with alcohol use disorder attends a weekly outpatient program. Four weeks ago, after drinking rose, the social worker and client revised the plan to add motivational interviewing and a second weekly session. Since then, drinking days have increased from 2 to 5 per week, the client has missed 3 workdays, and the client says, 'I want to stop, but I can't do it on this schedule.' Which action is **BEST**?

**A.** Explore the client's ambivalence about drinking once more

**B.** Add a peer recovery coach to the current outpatient plan

**C.** Refer the client for a higher level of care assessment

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Exploring ambivalence suits a client unsure about change. This client has already done motivational work, wants to stop and is asking for more help, so the barrier is the intensity of care, not motivation.

**B** **Incorrect** (trap: Good but not best). Peer recovery support is valuable and may belong in the next plan. Adding it at the same level of care, though, is another small adjustment right after the last one failed; with drinking more than doubled and the client asking for more help, the intensity of care needs a formal reassessment.

**C** **Correct.** Progress monitoring exists to guide decisions: when data show worsening despite a revised plan, care should move toward a more intensive level, guided by the ASAM transition and continued-service criteria (ASAM Criteria, 4th ed.). Drinking has more than doubled, work is affected, and the client asks for more support. Referral fits the Code's direction to refer when a client is not making reasonable progress and other

services are needed (NASW Code 1.16(a)). It beats B because another add-on at the same level repeats the approach that just failed while drinking keeps rising.

**Takeaway:** When monitoring shows worsening despite a revised plan, act on the data by referring for a more intensive level of care.

**Sources:** American Society of Addiction Medicine (2023). The ASAM Criteria (4th ed.), transition and continued service criteria; NASW Code of Ethics (2021), 1.16(a)
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### 279. INT-1575 (rev 1) · Best answer: C

A 14-year-old in a residential treatment program has been banging their head against walls several times a week. Staff incident reports from the past 6 weeks record the date and time of each episode. The team is ready to start a new behavior plan and wants to judge whether it works. Which approach is **BEST**?

**A.** Count episodes for 2 more weeks before starting the plan
**B.** Start the plan and log episodes in new incident reports
**C.** Use the past 6 weeks of incident reports as the baseline
**D.** Plan to pause the behavior plan later to confirm its effect

#### Why each option is right or wrong

**A** **Incorrect** (trap: Ignores risk). Collecting a fresh baseline before intervening is the usual first step when a problem is not urgent. Here self-injury is happening several times a week and dated records already exist, so a 2-week delay adds risk without adding needed information.

**B** **Incorrect** (trap: Good but not best). Tracking during treatment is useful and is the fallback when no earlier data exist. Ignoring 6 weeks of dated records, though, gives up the before-and-after comparison the team needs to judge whether the plan works.

**C** **Correct.** When reliable records of the target behavior already exist, the baseline can be reconstructed from them so that treatment starts at once (Di Noia & Tripodi, 2008). Dated incident reports covering 6 weeks give repeated measures of head banging, and self-injury makes any delay hard to justify. This beats B because starting without using the records leaves no pre-plan level to compare against, so the team could not tell whether episodes actually dropped.

**D** **Incorrect** (trap: Ignores risk). Withdrawing a plan to see whether head banging returns can strengthen causal claims, but it would expose the youth to renewed self-injury. With a behavior this harmful, designs that avoid withdrawal are preferred (Bloom, Fischer & Orme, 2009).

**Takeaway:** When reliable records of the target problem already exist, use them as a reconstructed baseline instead of delaying urgent treatment.

**Sources:** Di Noia, J., & Tripodi, T. (2008). Single-Case Design for Clinical Social Workers (2nd ed.). NASW Press, baselines: retrospective reconstruction from archival data; Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, single-system designs: withdrawal designs and ethical limits

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### 280. INT-1576 (rev 1) · Best answer: B

An adult client with panic disorder has logged panic attacks daily since intake. Over 3 baseline weeks the count held steady at about 6 per week. During 6 weeks of exposure-based treatment, it has fallen steadily to 2 per week. The client read about a newer therapy online and asks about switching to it. Which response is **BEST**?

- **A.** Agree to switch to the newer therapy as the client requests
- **B.** Review the progress data together and discuss continuing
- **C.** Blend the newer therapy's techniques into current sessions

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). The client does make the final choice, but an informed choice needs the data. Switching without reviewing a clear downward trend risks abandoning a treatment that is working. Agreeing would be reasonable if the client still prefers the change after seeing the results.

**B** **Correct.** Practice evaluation data are meant to guide decisions. A stable baseline followed by a steady drop in level and trend is the pattern that supports continuing the current intervention (Bloom, Fischer & Orme, 2009). Showing the client the data lets the client make an informed choice, which respects self-determination (NASW Code 1.02). This beats C because adding new techniques while the current plan is working changes two things at once and could dilute an approach that is clearly helping.

**C** **Incorrect** (trap: Misuses the technique). Mixing in new techniques mid-treatment changes two things at once. If progress continues or stalls, no one can tell which element was responsible, and the extra material may water down an approach that is already reducing panic attacks.

**Takeaway:** When progress data show clear improvement, share them with the client and support continuing what works before adopting something new.

**Sources:** Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, visual analysis of single-system data: level and trend; NASW Code of Ethics (2021), 1.02

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### 281. INT-1661 (rev 1) · Best answer: B

A single-system design sets a goal for one behavior. The goal then moves in steps, and each new level starts only after the client meets the current one. What is this design called?

**A.** Multiple-baseline design
**B.** Changing-criterion design
**C.** ABC design
**D.** ABAB reversal design

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). A multiple-baseline design starts the same intervention at staggered times across behaviors, settings or clients. It does not raise or lower a goal for one behavior in steps.

**B** **Correct.** A changing-criterion design sets a performance level and then shifts it step by step once the client meets each level, such as lowering a daily cigarette limit from 20 to 15 to 10. When behavior tracks each new level closely, that pattern supports the intervention's effect without withdrawing it (Kazdin, 2011). It beats C because an ABC design brings in a different intervention in each phase, while this design keeps one intervention and moves the goal.

**C** **Incorrect** (trap: Wrong model or concept). An ABC design follows the baseline with one intervention (B) and then a different one (C). The phases differ by treatment, not by a stepped goal for the same treatment.

**D** **Incorrect** (trap: Wrong model or concept). An ABAB design removes and then restores the intervention to show that behavior shifts with it. It tests effect by withdrawal, not by step-by-step goals.

**Takeaway:** A changing-criterion design shows effect when behavior follows each stepwise target while the same intervention continues.

**Sources:** Kazdin, A. E. (2011). Single-Case Research Designs: Methods for Clinical and Applied Settings (2nd ed.). Oxford University Press, changing-criterion designs

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### 282. INT-1662 (rev 2) · Best answer: A

A social worker is treating a 15-year-old for anxiety about attending school and tracks the teen's daily anxiety ratings on a 0-10 scale. Over 3 weeks the ratings fall from 8 to 2. In this session, the teen mentions that a parent recently arranged online classes so the teen can study at home most days. What should the social worker do **NEXT**?

**A.** Examine whether staying home accounts for the lower ratings
**B.** Praise the progress and continue the current treatment plan
**C.** Add a second anxiety scale to confirm the lower ratings

**Why each option is right or wrong**

**A** **Correct.** Progress data mean little until the social worker checks what produced the change; an outside event during the intervention is a classic rival explanation in an AB comparison (Bloom, Fischer & Orme, 2009). Anxiety ratings are expected to fall when a feared situation is simply avoided, and avoidance keeps school-related anxiety going (Kearney & Albano, 2007). The online classes began as the ratings dropped, so the next step is to compare the ratings with actual school attendance and discuss the pattern with the teen and parent. It beats B because carrying on as if treatment produced the drop would quietly reward the avoidance.

**B** **Incorrect** (trap: Good but not best). Recognizing progress is good practice, and continuing a plan that seems to work is usually sensible. Here the drop coincides with staying home, so the ratings may show relief from avoidance rather than less anxiety about attending school.

**C** **Incorrect** (trap: Misuses the technique). A second anxiety scale would measure the same feeling under the same avoidant conditions and would likely agree. What is missing is a measure of the behavior that matters, school attendance, not another rating of anxiety.

**Takeaway:** Before concluding a client improved, check whether the measured change reflects progress on the goal or a side effect such as avoidance.

**Sources:** Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, AB design; threats to internal validity; Kearney, C. A., & Albano, A. M. (2007). When Children Refuse School: A Cognitive-Behavioral Therapy Approach, Therapist Guide (2nd ed.). Oxford University Press, functions of school refusal behavior

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## 283. INT-1663 (rev 1) · Best answer: D

A client with depression has completed the PHQ-9 at the start of each session for 10 weeks. The social worker records the scores in the chart but has not discussed them with the client. The client now asks to stop, saying, 'I fill it out and never hear about it again.' What is the social worker's **BEST** response?

**A.** Stop the measure and rely on discussion in sessions

**B.** Explain why the scores matter and ask the client to continue

**C.** Switch to completing the measure once a month

**D.** Begin reviewing a graph of the scores with the client

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Clients may decline a measure, and that choice would be respected. Here the objection is that the results are never used, which the social worker can fix. Dropping the measure loses the clearest record of progress before the fix is tried.

**B** **Incorrect** (trap: Good but not best). Explaining the purpose is part of good monitoring, but the client's point is that nothing visible happens with the scores. More explanation without changing how they are used leaves the complaint unanswered.

**C** **Incorrect** (trap: Misuses the technique). Less frequent measurement lowers the burden but leaves the scores unused and makes changes harder to spot. The fix the client points to is using the results together, not filling out the form less often.

**D** **Correct.** Routine outcome monitoring helps when results are fed back and used; feedback reduces deterioration and improves outcomes, especially for clients who are off track (Lambert et al., 2018). The client's complaint names the problem: the scores go into the chart and nowhere else. Reviewing the trend together each session turns the measure into shared information for judging progress and adjusting the plan (Bloom, Fischer & Orme, 2009), and the client can still decline after seeing it used. It beats A because stopping would discard 10 weeks of data when the objection is to unused scores, not to measurement itself.

**Takeaway:** Progress measures help when results are shared and used with the client; unused scores turn monitoring into a burden.

**Sources:** Lambert, M. J., Whipple, J. L., & Kleinstäuber, M. (2018). Collecting and delivering progress feedback: A meta-analysis of routine outcome monitoring. Psychotherapy, 55(4), 520-537, whole article; Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, visual analysis of single-system data: level and trend

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### 284. INT-1664 (rev 3) · Best answer: A

A social worker reviews the service plan of a 79-year-old client who lives alone and receives home-delivered meals to address weight loss. After 2 months, the client has lost another 8 pounds. The client says they eat every meal, and the home aide's log confirms the meals are finished. What should the social worker do **NEXT**?

**A.** Ask the client's doctor to check promptly for a medical cause

**B.** Ask a dietitian to review the calorie content of the meals

**C.** Continue the current plan and reweigh next month

#### Why each option is right or wrong

**A** **Correct.** Evaluating a service plan means asking whether the services worked, and why not when they did not (NASW, 2013, Standard 6). Unintentional weight loss in an older adult warrants medical evaluation, because causes range from cancer to medication effects (Gaddey & Holder, 2021). The meals are confirmed eaten, so the plan's assumption that intake was the problem no longer explains the loss. It beats B because reviewing the meals' calories still treats intake as the problem, which the data now make unlikely, and could delay finding a medical cause.

**B** **Incorrect** (trap: Skips assessment). A dietitian's review is sensible when intake might be too low, and it may help later. The meals are confirmed eaten, though, so continued loss points beyond the menu to a possible medical cause, and reviewing calories first could delay that evaluation.

**C** **Incorrect** (trap: Ignores risk). Waiting another month can be reasonable when a plan is new and the trend unclear. Two months of continued loss despite confirmed eating is a clear signal, and delay risks missing a treatable illness.

**Takeaway:** When monitoring shows a fully delivered service plan is not working, reassess the cause, including medical evaluation, rather than adding more service.

**Sources:** Gaddey, H. L., & Holder, K. K. (2021). Unintentional weight loss in older adults. American Family Physician, 104(1), 34-40, whole article; NASW Standards for Social Work Case Management (2013), Standard 6: Service Planning, Implementation, and Monitoring

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## 285. INT-1587 (rev 1) · Best answer: A

A workforce program for young adults is writing objectives for a new grant. The team needs one objective that states the change participants should achieve. Which objective is **BEST**?

- **A.** By month 12, 60% of participants will keep a job for 90 days
- **B.** Staff will hold 40 job-readiness workshops during the grant year
- **C.** Participants will gain the confidence they need to succeed at work
- **D.** The program will help young adults become self-sufficient

#### Why each option is right or wrong

**A** **Correct.** An outcome objective states a measurable change in participants, with a target and a deadline, following the SMART guide: specific, measurable, achievable, relevant and time-bound (Doran, 1981; Kettner, Moroney & Martin, 2022). This option names who, what change, how much and by when. It beats B because B is a process objective: it counts staff activity, an output, rather than a change in participants.

**B** **Incorrect** (trap: Wrong model or concept). This is a well-written process objective, specific and time-bound, but it counts workshops delivered by staff. It describes program activity, not the change participants should achieve.

**C** **Incorrect** (trap: Misuses the technique). Confidence matters, but this statement names no measure, target or deadline, so no one could tell whether it was met. It could be rewritten with a confidence scale and a target date.

**D** **Incorrect** (trap: Wrong model or concept). This is a goal: a broad statement of intended direction. Goals guide a program, but objectives must state measurable results that show progress toward the goal.

**Takeaway:** A well-written outcome objective names the participant change, how much, how it is measured and by when.

**Sources:** Doran, G. T. (1981). There's a S.M.A.R.T. way to write management's goals and objectives. Management Review, 70(11), 35-36, whole article; Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 7: Organizational Mission and Program Goals, Objectives, and Activities

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### 286. INT-1588 (rev 2) · Best answer: C

An after-school program serves 40 middle school students in a city of 300,000. Its activities are tutoring, mentoring and family outreach. The new board chair wants the program's main objective to be a 50% drop in citywide juvenile arrests within 1 year. What is the **MOST** appropriate response from the program's social worker?

**A.** Keep the arrest objective but extend the deadline to 5 years
**B.** Report students served and hours of tutoring and mentoring
**C.** Propose student outcomes that its tutoring and mentoring can change

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). A longer timeline sounds more realistic, but 40 students cannot measurably change arrests across a city of 300,000, and any change could not be traced to the program. Citywide arrests fit as a long-term aim, not as the program's main objective.

**B** **Incorrect** (trap: Wrong model or concept). Counts of students and hours of tutoring and mentoring are outputs. They document activity and belong in reports, but they do not show whether students changed, so they cannot replace outcome objectives.

**C** **Correct.** Objectives should be achievable and logically linked to program activities. A logic model connects activities to short- and intermediate-term outcomes the program can influence, such as grades, school attendance or family engagement, with broader community change as a long-term aim (W.K. Kellogg Foundation, 2004; Kettner, Moroney & Martin, 2022). A 40-student program cannot plausibly halve citywide arrests in a year. This beats A because more time does not make citywide arrests something this program can change or be credited with, so results would mislead.

**Takeaway:** Set objectives the program can plausibly achieve and attribute to its activities; keep broad community change as a long-term goal.

**Sources:** W.K. Kellogg Foundation (2004). Logic Model Development Guide, Ch. 1: outcomes and impact; Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 7: Organizational Mission and Program Goals, Objectives, and Activities
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### 287. INT-1589 (rev 1) · Best answer: A

A middle school program sets this objective: 'By June, participating students will miss 25% fewer school days than last year.' To measure progress, staff plan to give students a survey asking how much they enjoy the program. Which indicator would **MOST** directly measure progress on the objective?

**A.** Official school attendance records for each participant
**B.** A student survey asking how often they miss school
**C.** The number of program sessions each student attends

**Why each option is right or wrong**

**A** **Correct.** An indicator must measure the exact change an objective names. For days missed, official attendance records are the most direct and objective source (Kettner, Moroney & Martin, 2022; Royse, Thyer & Padgett, 2016). The planned enjoyment survey measures satisfaction, not attendance. This beats B because self-reported absences are prone to memory errors and the wish to look good, while records already capture the exact information.

**B** **Incorrect** (trap: Misuses the technique). Asking students how often they miss school targets the right behavior, but self-report is open to recall errors and social desirability. Official records already hold the exact information needed.

**C** **Incorrect** (trap: Wrong model or concept). Session attendance measures participation in the program, an output. It does not show whether students are missing fewer school days, which is the change the objective names.

**Takeaway:** Choose an indicator that measures exactly the change an objective names, preferring direct records over opinions or participation counts.

**Sources:** Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 10: Performance Measurement, Monitoring, and Program Evaluation; Royse, D., Thyer, B. A., & Padgett, D. K. (2016). Program Evaluation: An Introduction to an Evidence-Based Approach (6th ed.). Cengage, Ch. 12: Selecting the Best Evaluation Measure for Your Project
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## 288. INT-1678 (rev 1) · Best answer: B

A reentry program is writing outcome objectives for a new grant. A draft reads: 'Within 12 months, increase stable housing among participants by 30%.' Staff have no figures on how many people like these participants are stably housed now. What should the social worker do **FIRST**?

**A.** Lower the target to 10% so it is achievable
**B.** Find the current housing rate for comparable participants
**C.** Reword it to promise better housing stability for participants

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Adjusting the target may be right once the starting point is known. Picking 10% now is as arbitrary as 30%, because the program still has nothing to measure change against.

**B** **Correct.** An outcome objective needs a measurable criterion, and a percentage change means little without a starting point. Getting baseline data on housing among similar participants lets staff set a target that is realistic and lets the evaluation show whether change happened. It beats A because lowering the number is still a guess; a 10% target could be trivial or out of reach until the current rate is known (Kettner, Moroney & Martin, 2022; Doran, 1981).

**C** **Incorrect** (trap: Misuses the technique). This drops the measurable criterion, turning the objective into a broad goal. Goals can be general, but objectives state what will change, by how much and by when, so progress can be measured.

**Takeaway:** Set an objective's numeric target from baseline data; without a starting point, a promised percentage change is a guess.

**Sources:** Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 7: Organizational Mission and Program Goals, Objectives, and Activities; Doran, G. T. (1981). There's a S.M.A.R.T. way to write management's goals and objectives. Management Review, 70(11), 35-36, whole article
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.2 Setting program objectives and outcomes · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 1 of 3

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## 289. INT-1590 (rev 2) · Best answer: B

A supported employment program's new contract pays the agency only for clients still employed 6 months after enrollment. Within a quarter, the social worker who manages the program sees intake staff steering applicants with criminal records or recent hospitalizations to a waitlist. Placement rates have risen. Intake staff completed eligibility training last month and say they fear losing the contract. What action is **BEST**?

**A.** Collect another quarter of intake data to confirm the pattern
**B.** Raise the pattern with leaders and seek fairer performance measures
**C.** Remind intake staff of the eligibility rules at a staff meeting

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). More data can firm up a pattern, but staff have already said why they are steering applicants, and each added quarter turns away more of the people the program exists to serve. Waiting would make sense only if the pattern were unclear.

**B** **Correct.** Evaluation and payment measures shape services: rewarding only easy successes can lead a program to enroll the clients most likely to succeed and turn away those with the greatest need. The fix has to reach the incentive, so the manager brings the data to leaders and advocates for measures that reward progress or adjust for client difficulty (NASW Code 3.07(b), 3.09(b)). This beats C because staff were trained on the rules last month and are responding to the contract; a reminder leaves that pressure in place.

**C** **Incorrect** (trap: Good but not best). A reminder treats the problem as a knowledge gap. Staff were trained on the rules last month and are reacting to the payment structure, so the screening is likely to continue unless the incentive itself changes.

**Takeaway:** When performance measures push staff to exclude harder-to-serve clients, address the measure itself through agency and funder channels.

**Sources:** NASW Code of Ethics (2021), 3.07(b); NASW Code of Ethics (2021), 3.09(b)
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.3 Program evaluation and its effect on services · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 290. INT-1591 (rev 2) · Best answer: C

A year-end evaluation of a home-based depression program shows strong overall results, and the board will vote next week on expanding it unchanged to three counties. When the social worker breaks the data down by age, clients 65 and older showed no improvement and dropped out at twice the rate of younger clients. What is the **MOST** appropriate action?

**A.** Support the board's plan, since overall results are strong
**B.** Hold the age results until a follow-up study confirms them
**C.** Present the age findings to the board before the vote

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Strong overall averages can hide groups a program does not help. Expanding unchanged would carry the same gap for older clients into three more counties.

**B** **Incorrect** (trap: Misreads law or ethics). Confirming a surprising subgroup result is wise, but the board votes next week. Holding the data back lets the expansion go forward on incomplete information, when the findings could be shared as preliminary with a plan to look into them.

**C** **Correct.** Evaluation findings should be reported accurately, and leaving out a known subgroup failure would misrepresent them (NASW Code 5.02(o)); and an evaluation earns its value when its results shape program decisions (Patton, 2008). An overall average here hides a group the program is failing. Sharing the age findings before the vote lets the board consider adapting the model for older clients. This beats B because waiting for a follow-up study lets a major decision rest on information the social worker already knows is incomplete.

**Takeaway:** Report subgroup findings that change the picture to decision makers before they act, so evaluation actually informs services.

**Sources:** NASW Code of Ethics (2021), 5.02(o); Patton, M. Q. (2008). Utilization-Focused Evaluation (4th ed.). SAGE, intended use by intended users
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.3 Program evaluation and its effect on services · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 291. INT-1592 (rev 2) · Best answer: A

An agency director announces that an outside firm will evaluate the family support program this spring. Several case managers tell the program's coordinator, a social worker, that they fear the results will be used to cut positions, and some say they may not cooperate. Planning has not begun. What should the coordinator do **FIRST**?

**A.** Involve staff in shaping the evaluation questions and uses
**B.** Assure staff that the evaluation results will stay confidential
**C.** Start data collection and address concerns as they come up

**Why each option is right or wrong**

**A** **Correct.** Utilization-focused evaluation holds that findings are used when the people who will use them help frame the questions and plan how results will be used from the start (Patton, 2008). Engaging case managers early lowers fear, improves cooperation and data quality, and makes it more likely the evaluation improves services. This beats C because collecting data over staff objections invites poor cooperation and distorted data, and concerns are easier to settle before methods are fixed.

**B** **Incorrect** (trap: Good but not best). The coordinator does not control how an outside firm or the director will use the results, so this promise may not hold. It also leaves the real worry, how findings will be used, unaddressed.

**C** **Incorrect** (trap: Right action, wrong time). Data collection is the right step later. Starting it before staff concerns are addressed invites poor cooperation and skewed data, which weakens the evaluation the program needs.

**Takeaway:** Engage the people who deliver and use a program in planning its evaluation; early involvement improves cooperation and use of findings.

**Sources:** Patton, M. Q. (2008). Utilization-Focused Evaluation (4th ed.). SAGE, Ch. 3: Fostering Intended Use by Intended Users: The Personal Factor

**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.3 Program evaluation and its effect on services · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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## 292. INT-1679 (rev 1) · Best answer: D

A youth shelter invites line staff to help design an evaluation of its case management. While drafting measures, months before data collection begins, staff start defining what a 'stable exit' means, discussing outcomes in team meetings and changing how they set goals with youth. Which type of evaluation use does this **MOST** likely show?

**A.** Instrumental use
**B.** Symbolic use
**C.** Conceptual use
**D.** Process use

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Instrumental use is acting directly on findings, such as changing a program because results showed a problem. Data collection has not started, so the changes cannot be a response to findings.

**B** **Incorrect** (trap: Wrong model or concept). Symbolic use is invoking an evaluation to legitimize decisions already made or to signal accountability. Staff here are changing their own practice in substance, not using the evaluation for appearances.

**C** **Incorrect** (trap: Wrong model or concept). Conceptual use is a change in how people understand a program that comes from evaluation findings, even without a specific decision. It fits the shift in thinking but not the timing: the shift came from designing measures, before any data.

**D** **Correct.** Process use refers to changes in thinking and behavior that come from taking part in an evaluation, as distinct from use of its findings (Patton, 2008). Staff are clarifying outcomes and changing goal setting while drafting measures, months before any data exist. It beats C because conceptual use is a shift in understanding produced

by evaluation results; here the change comes from participation itself. Patton treats process use as a benefit evaluators can plan for by involving staff.

**Takeaway:** Changes that come from taking part in an evaluation, before findings exist, are process use; acting on findings is instrumental use.

**Sources:** Patton, M. Q. (2008). Utilization-Focused Evaluation (4th ed.). SAGE, Ch. 5: Intended Process Uses: Impacts of Evaluative Thinking and Experiences

**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.3 Program evaluation and its effect on services · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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## 293. INT-1680 (rev 1) · Best answer: A

To meet a new funder requirement, a community mental health program added a 45-minute questionnaire battery to every intake. Over the next quarter, missed second appointments doubled. Exit calls with clients who left point to the long first visit spent on paperwork. The funder requires data on 3 outcomes. Which action is **BEST**?

**A.** Trim the battery to brief versions of the funder's measures

**B.** Pause the funder's battery until attendance recovers

**C.** Survey clients who dropped out about the intake visit

#### Why each option is right or wrong

**A** **Correct.** Evaluation should support services, and measurement burden that drives clients away harms both care and the data. The funder needs 3 outcomes, yet the battery takes 45 minutes and exit calls tie dropout to it. Brief measures that cover the required outcomes keep accountability while restoring a client-centered first visit. It beats B because stopping measurement breaks the funding agreement and leaves the program unable to see whether clients improve; the fix is proportion, not abandonment (Royse, Thyer & Padgett, 2016; NASW Code 5.02(a)).

**B** **Incorrect** (trap: Good but not best). Pausing measurement removes the burden but breaks the funder requirement and leaves the program without outcome data. Suspension makes sense only if no brief, valid option exists; here 3 outcomes can be measured quickly.

**C** **Incorrect** (trap: Right action, wrong time). Asking dropouts about their experience is right when the cause of dropout is unknown. Exit calls have already linked it to the long paperwork, so another survey adds burden and delays the fix.

**Takeaway:** When evaluation demands harm service delivery, streamline to measures that answer the required questions rather than dropping measurement.

**Sources:** Royse, D., Thyer, B. A., & Padgett, D. K. (2016). Program Evaluation: An Introduction to an Evidence-Based Approach (6th ed.). Cengage, Ch. 12: Selecting the Best Evaluation Measure for Your Project; NASW Code of Ethics (2021), 5.02(a)

**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.3 Program evaluation and its effect on services · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 294. INT-0008 (rev 1) · Best answer: A

Which type of evaluation examines whether a program is being delivered as designed while the program is still under way?

**A.** Formative evaluation
**B.** Summative evaluation
**C.** Needs assessment
**D.** Cost-benefit analysis

#### Why each option is right or wrong

**A** **Correct.** Formative, or process, evaluation takes place during implementation. It checks fidelity, meaning whether services reach the intended people in the intended way, so the program can be improved while it runs (Royse, Thyer & Padgett, 2016). Both cues in the question, delivery as designed and timing during operation, point to it. It beats B because summative evaluation comes at the end and judges whether the program achieved its outcomes, not how it is being carried out.

**B** **Incorrect** (trap: Wrong model or concept). Summative, or outcome, evaluation judges results after a program cycle, such as whether goals were met. It does not monitor delivery while the program runs.

**C** **Incorrect** (trap: Wrong model or concept). A needs assessment comes before a program is designed, to identify gaps and the population's needs. It does not examine how an existing program is being delivered.

**D** **Incorrect** (trap: Wrong model or concept). Cost-benefit analysis compares a program's costs and benefits in monetary terms. It answers a question about economic value rather than whether delivery matches the design.

**Takeaway:** Formative (process) evaluation monitors delivery during a program; summative evaluation judges results at the end; needs assessment comes first.

**Sources:** Royse, D., Thyer, B. A., & Padgett, D. K. (2016). Program Evaluation: An Introduction to an Evidence-Based Approach (6th ed.). Cengage, formative and process evaluation
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.4 Agency program evaluation: needs assessment, formative/summative, cost analyses · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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### 295. INT-0115 (rev 1) · Best answer: A

At the end of a 3-year grant, an agency analyzes whether participants in its supportive housing program stayed housed and made fewer emergency room visits. The board will use the results to decide whether to continue the program. Which type of evaluation is the agency conducting?

**A.** Summative evaluation
**B.** Formative evaluation
**C.** Needs assessment

**Why each option is right or wrong**

**A** **Correct.** Summative, or outcome, evaluation takes place at the end of a program or cycle and judges whether it achieved its intended outcomes, often to inform decisions about continuing or funding it (Royse, Thyer & Padgett, 2016). The timing (end of the grant), the focus on outcomes (housing stability and emergency visits) and the purpose (a continuation decision) all point to it. It beats B because formative evaluation happens while a program runs, to check delivery and improve it, rather than to judge final results.

**B** **Incorrect** (trap: Wrong model or concept). Formative, or process, evaluation is done during implementation to monitor fidelity and improve delivery. This agency is judging results at the end of the grant to make a continuation decision.

**C** **Incorrect** (trap: Wrong model or concept). A needs assessment comes before a program is designed, to identify problems and gaps in a population. It does not measure the results of a program that has already run.

**Takeaway:** Summative evaluation judges outcomes at the end to inform decisions; formative evaluation improves delivery midstream; needs assessment precedes design.

**Sources:** Royse, D., Thyer, B. A., & Padgett, D. K. (2016). Program Evaluation: An Introduction to an Evidence-Based Approach (6th ed.). Cengage, summative and outcome evaluation
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.4 Agency program evaluation: needs assessment, formative/summative, cost analyses · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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## 296. INT-1583 (rev 2) · Best answer: C

A family services agency is expanding into a new county and wants to learn which services residents lack. Staff meet one at a time with a pastor, two school principals, a clinic physician and the police chief to hear what problems they see in the community. Which needs assessment method is the agency using?

**A.** Community forum
**B.** Focus group
**C.** Key informant approach

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). A community forum is an open public meeting where any resident can speak. It reaches residents directly but can be dominated by the most vocal attendees. Here only selected leaders were consulted.

**B** **Incorrect** (trap: Wrong model or concept). A focus group brings a small group together for a guided discussion. These leaders were met one at a time, which does not fit a group format.

**C** **Correct.** The key informant approach collects views from people whose roles give them broad knowledge of a community's problems, such as clergy, school leaders, health providers and police (Rossi, Lipsey & Henry, 2019; Royse, Thyer & Padgett, 2016). It is quick and inexpensive, but it reflects the informants' perspectives, which may differ from residents' own. This beats B because a focus group gathers several people for a guided group discussion, while these leaders were consulted individually.

**Takeaway:** Key informant needs assessments draw on community leaders' views quickly and cheaply but may miss residents' own perspectives.

**Sources:** Rossi, P. H., Lipsey, M. W., & Henry, G. T. (2019). Evaluation: A Systematic Approach (8th ed.). SAGE, Ch. 2: Social Problems and Assessing the Need for a Program; Royse, D., Thyer, B. A., & Padgett, D. K. (2016). Program Evaluation: An Introduction to an Evidence-Based Approach (6th ed.). Cengage, Ch. 3: Needs Assessment
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.4 Agency program evaluation: needs assessment, formative/summative, cost analyses · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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## 297. INT-1584 (rev 1) · Best answer: A

A board member of a rural senior center returns from a conference excited about a meal-delivery program that succeeded in a large city. The board asks the center's social worker to begin planning to launch the same program next year. What should the social worker do **FIRST**?

**A.** Gather data on unmet needs among local older adults
**B.** Write a grant proposal to fund a local version
**C.** Pilot the program in one town and track its use

**Why each option is right or wrong**

**A** **Correct.** A needs assessment comes before program design: it shows whether a problem exists locally, how large it is and what is already available (Kettner, Moroney & Martin, 2022). Success in a large city does not show that rural older adults share the same gaps, which might involve transportation, isolation or existing meal services. This beats C because a pilot tests the delivery of a program already chosen, and running one before knowing local needs risks building the wrong service.

**B** **Incorrect** (trap: Skips assessment). Funders expect proposals to document local need. Writing one before gathering that information skips the step that justifies the program and shapes its design.

**C** **Incorrect** (trap: Right action, wrong time). Piloting is a sound way to test delivery once a program is chosen. Doing it before assessing local needs puts resources into a model that may not fit the county.

**Takeaway:** Assess the local population's needs before designing or piloting a program, even one that has succeeded elsewhere.

**Sources:** Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 4-5: Needs Assessment
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.4 Agency program evaluation: needs assessment, formative/summative, cost analyses · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 298. INT-1585 (rev 1) · Best answer: C

A youth mentoring program runs at four sites, and its funder asks for an outcome study within the year. Site reports show that two sites hold weekly mentor meetings as designed. The other two meet about once a month and have dropped mentor training. The program director asks the social worker to plan the evaluation. What should the social worker do **FIRST**?

**A.** Start measuring youth outcomes at every site now
**B.** Compare outcomes between high- and low-fidelity sites
**C.** Examine and correct how each site delivers the model
**D.** Ask the funder to drop the outcome requirement

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Measuring outcomes is the right step once delivery is consistent. Starting now would blend sites that deliver the model with sites that do not, so poor results could not be traced to the model or to its delivery.

**B** **Incorrect** (trap: Misuses the technique). Comparing sites seems to use the variation, but the sites differ in staff, youth and neighborhoods as well as fidelity. The comparison was not designed and cannot isolate the effect of delivering the model well.

**C** **Correct.** A process or formative evaluation checks whether a program is delivered as designed, and an outcome study of a poorly implemented program cannot tell implementation failure from a weak model (Rossi, Lipsey & Henry, 2019). Two sites have dropped core parts of the model, so documenting delivery and correcting the gaps comes first, with the outcome study to follow within the year. This beats B because a site comparison confounds fidelity with many other differences between sites.

**D** **Incorrect** (trap: Good but not best). Funder requirements can sometimes be renegotiated, but an outcome study is appropriate once delivery is consistent. Asking to drop it gives up accountability instead of putting the steps in order.

**Takeaway:** Confirm a program is delivered as designed before judging its outcomes; otherwise poor results cannot be traced to the model or the delivery.

**Sources:** Rossi, P. H., Lipsey, M. W., & Henry, G. T. (2019). Evaluation: A Systematic Approach (8th ed.). SAGE, Ch. 4: Assessing Program Process and Implementation
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.4 Agency program evaluation: needs assessment, formative/summative, cost analyses · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 299. INT-1586 (rev 1) · Best answer: B

A county commissioner asks a jail diversion program to show whether the money it saves the county exceeds what it costs. The program director assigns the task to a social worker whose evaluation training covers outcome tracking but not converting outcomes into dollar values. Which action is **BEST**?

**A.** Report cost-effectiveness as cost per person diverted
**B.** Consult an evaluator experienced in cost-benefit analysis
**C.** Decline the task because cost studies are outside social work

**Why each option is right or wrong**

- **A** **Incorrect** (trap: Wrong model or concept). Cost per person diverted is a cost-effectiveness measure. It is useful for comparing programs with the same outcome, but it does not put the outcome in dollars, so it cannot show whether savings exceed costs.
- **B** **Correct.** Asking whether savings exceed costs calls for cost-benefit analysis, which expresses both costs and outcomes, such as jail days avoided, in dollars (Rossi, Lipsey & Henry, 2019). Assigning dollar values takes specific skills, and social workers should take on tasks based on existing competence or the intent to gain it (NASW Code 4.01(a)). Consultation lets the work be done well. This beats A because cost per person diverted is a different analysis that leaves the commissioner's question unanswered.
- **C** **Incorrect** (trap: Hands off too soon). Evaluating programs is part of social work practice (NASW Code 5.02(a)). Declining leaves the county's question unanswered when consultation would let the social worker complete the task competently.

**Takeaway:** Questions about whether savings exceed costs require cost-benefit analysis; when it exceeds your skills, get expert consultation instead of substituting another measure.

**Sources:** Rossi, P. H., Lipsey, M. W., & Henry, G. T. (2019). Evaluation: A Systematic Approach (8th ed.). SAGE, Ch. 10: Assessing the Economic Efficiency of Programs; NASW Code of Ethics (2021), 4.01(a); NASW Code of Ethics (2021), 5.02(a)

**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.4 Agency program evaluation: needs assessment, formative/summative, cost analyses · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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## 300. INT-1675 (rev 1) · Best answer: B

Which needs assessment approach uses public data, such as census and health figures, to compare needs across neighborhoods?

- **A.** Key informant approach
- **B.** Social indicators approach
- **C.** Community forum approach
- **D.** Rates-under-treatment approach

**Why each option is right or wrong**

- **A** **Incorrect** (trap: Wrong model or concept). Key informant work gathers the views of people who know the community well, such as clergy, principals or physicians, usually through interviews. The approach described relies on public statistics rather than the judgments of informed individuals.
- **B** **Correct.** The social indicators approach estimates need from existing public data, such as census, housing and health statistics, and compares areas or tracks change over time. Its strengths are low cost and broad coverage; its weakness is that the figures are indirect measures of need, so they are often paired with surveys or forums. It beats D because rates under treatment count people already receiving services, which misses residents whose needs are unmet (Royse, Thyer & Padgett, 2016; Kettner, Moroney & Martin, 2022).

**C** **Incorrect** (trap: Wrong model or concept). A community forum invites residents to an open meeting to describe needs in their own words. It captures lived experience but tends to draw the most vocal residents. The approach described analyzes existing data instead of holding a meeting.

**D** **Incorrect** (trap: Wrong model or concept). Rates under treatment use service statistics, such as caseloads and waitlists, to infer need. It is tempting because it also relies on existing numbers, but those come from agency records of people already served, not population-wide public statistics.

**Takeaway:** Needs assessment from existing public statistics compared across areas is the social indicators approach; counts of people served are rates under treatment.

**Sources:** Royse, D., Thyer, B. A., & Padgett, D. K. (2016). Program Evaluation: An Introduction to an Evidence-Based Approach (6th ed.). Cengage, Ch. 3: Needs Assessment; Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 4-5: Needs Assessment

**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.4 Agency program evaluation: needs assessment, formative/summative, cost analyses · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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### 301. INT-1676 (rev 1) · Best answer: C

A county parenting program is 5 months into its first year. Process data show that parents who speak Spanish come to the first session but rarely come back. Exit calls reveal that sessions are held in English with English handouts. The funder's summative outcome evaluation is scheduled for month 12. Which action is **BEST**?

**A.** Keep the model unchanged until the outcome evaluation is done

**B.** Interview the Spanish-speaking parents about why they stop attending

**C.** Add Spanish-language sessions and track retention by language

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Holding a model steady can protect the comparison in a tightly controlled trial, but this is a first-year program whose process data exist to improve delivery. Waiting 7 months leaves a whole group of parents without usable services; the summative report can describe the change.

**B** **Incorrect** (trap: Right action, wrong time). Asking parents why they leave is the right step when the reason is unknown. Exit calls have already shown that language is the barrier, so another round of interviews delays a fix the data already support.

**C** **Correct.** Formative evaluation exists to improve a program while it is running (Scriven, 1967). Process data have identified a delivery barrier and exit calls explain it, so the program should change delivery now and keep monitoring whether the change works. It beats A because freezing a component that is failing a group of families trades their access for a cleaner year-end comparison; the summative evaluation can account for a documented mid-course change. Fixing implementation problems first also makes the later outcome findings more meaningful (Rossi, Lipsey & Henry, 2019).

**Takeaway:** Use formative findings to correct delivery during implementation; don't hold a failing component unchanged to protect a later summative evaluation.

**Sources:** Scriven, M. (1967). The methodology of evaluation. In R. W. Tyler, R. M. Gagne, & M. Scriven (Eds.), Perspectives of Curriculum Evaluation (pp. 39-83). Rand McNally, formative versus summative roles of evaluation; Rossi, P. H., Lipsey, M. W., & Henry, G. T. (2019). Evaluation: A Systematic Approach (8th ed.). SAGE, Ch. 4: Assessing Program Process and Implementation

**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.4 Agency program evaluation: needs assessment, formative/summative, cost analyses · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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## 302. INT-1677 (rev 1) · Best answer: A

An agency compares the cost per youth served of two mentoring models. One model uses paid mentors. The other relies on volunteer mentors and meets in space a church donates. The draft analysis counts budgeted expenses alone, so the volunteer model looks far cheaper. How should the social worker revise the cost analysis?

**A.** Add a market value for unpaid mentor hours and the room

**B.** Keep budgeted expenses, since unpaid mentors cost the agency nothing

**C.** Convert both models' outcomes into dollar benefits

#### Why each option is right or wrong

**A** **Correct.** A sound cost analysis counts every resource a program uses, including volunteer labor and donated goods or space, valued at what they would cost to obtain (their opportunity cost). Leaving them out makes the volunteer model look cheaper than it is and misleads anyone who might replicate it where donations are unavailable. It beats B because the question is the real resource cost of each model, not the agency's cash outlay (Rossi, Lipsey & Henry, 2019; Levin et al., 2018).

**B** **Incorrect** (trap: Misuses the technique). Cash expenses matter for the agency's own budget, but a comparison of program models must include every resource consumed. Excluding volunteer hours and donated space understates the volunteer model's true cost and misleads replication decisions.

**C** **Incorrect** (trap: Wrong model or concept). Putting outcomes in dollars turns the study into a cost-benefit analysis, which answers a different question. The flaw here is on the cost side: missing resources. A cost-benefit analysis with the same omission would be just as biased.

**Takeaway:** Cost analyses value every resource used, including volunteer time and donated space, not just budgeted cash expenses.

**Sources:** Rossi, P. H., Lipsey, M. W., & Henry, G. T. (2019). Evaluation: A Systematic Approach (8th ed.). SAGE, Ch. 10: Assessing the Economic Efficiency of Programs; Levin, H. M., McEwan, P. J., Belfield, C., Bowden, A. B., & Shand, R. (2018). Economic Evaluation in Education: Cost-Effectiveness and Benefit-Cost Analysis (3rd ed.). SAGE, ingredients method: volunteer time and donated resources

**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.4 Agency program evaluation: needs assessment, formative/summative, cost analyses · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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### 303. INT-0116 (rev 2) · Best answer: B

A risk scale is completed at intake. Two years later, clients who scored high at intake have returned to the hospital far more often than clients who scored low. Which measurement property does this show?

**A.** Concurrent validity
**B.** Predictive validity
**C.** Test-retest reliability
**D.** Content validity

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Concurrent validity is also criterion validity, but the criterion is measured at about the same time as the scale. Forecasting a later outcome is what distinguishes predictive validity.

**B** **Correct.** Predictive validity is a form of criterion validity in which scores on a measure are checked against a criterion that occurs later (Bloom, Fischer & Orme, 2009). Here the criterion, rehospitalization, happens during the 2 years after the scale is completed, and high intake scores line up with it. It beats A because concurrent validity compares scores with a criterion measured at about the same time, such as current hospital status, rather than with a later outcome.

**C** **Incorrect** (trap: Wrong model or concept). Test-retest reliability checks whether the same people get similar scores when they complete a measure twice. The 2-year span here is a follow-up period for an outcome, not a second administration of the scale.

**D** **Incorrect** (trap: Wrong model or concept). Content validity is the extent to which a measure's items cover the full range of the concept, usually judged by experts. It says nothing about whether scores forecast outcomes.

**Takeaway:** Criterion validity is predictive when scores forecast a later outcome and concurrent when they match a criterion measured at the same time.

**Sources:** Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, criterion validity: predictive and concurrent
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** none · **Cognitive** recall · **Difficulty** 3 of 3

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### 304. INT-1577 (rev 1) · Best answer: D

Two direct support staff at a group home count one resident's aggressive episodes during the same evening shifts. On the same evenings, one records 2 episodes and the other records 7. When asked, one counts only hitting, while the other also counts shouting and slamming doors. Which step is **BEST** for getting usable data?

**A.** Average the two staff members' counts for each evening
**B.** Use the counts from the staff member with more experience
**C.** Switch to a standardized aggression rating scale
**D.** Write a shared definition with examples and train both staff

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Averaging blends two different measurements into a number that matches neither. It hides the disagreement instead of fixing its cause, so the data would still not reflect one consistent behavior.

**B** **Incorrect** (trap: Good but not best). Experience does not make one observer's definition correct. Dropping the other count leaves the team relying on a single unchecked observer, with no way to confirm the data are consistent.

**C** **Incorrect** (trap: Good but not best). A standardized scale can help, but staff would still need shared meanings and training to rate consistently. Switching tools also gives up behavior-specific counts that suit this plan.

**D** **Correct.** Interrater reliability depends on a clear operational definition of the target behavior and on training observers to apply it the same way (Bloom, Fischer & Orme, 2009). The gap here comes from the two staff counting different behaviors, so a written definition with examples, plus joint practice, addresses the cause. Agreement can then be checked. This beats C because a new instrument does not fix observers who apply different meanings unless they are trained together.

**Takeaway:** When observers disagree, define the target behavior precisely and train observers together before trusting the data.

**Sources:** Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, reliability: interobserver reliability and operational definitions
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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## 305. INT-1578 (rev 2) · Best answer: A

A parenting program screens 400 parents with a stress scale and offers places only to those in the top 10%. Because of a staffing delay, these parents are rescored 3 weeks later, before any sessions begin. Their scores have fallen noticeably as a group. Parents with moderate scores who were rescored at the same time changed little. What **MOST** likely explains the drop?

**A.** Statistical regression toward the mean
**B.** A testing effect from completing the scale twice
**C.** History, such as an outside event during the wait

**Why each option is right or wrong**

**A** **Correct.** When people are chosen because their scores are extreme, their later scores tend to move toward the average even with no intervention, because part of an extreme score reflects temporary factors and measurement error (Campbell & Stanley, 1963). Only the highest scorers changed, while moderate scorers rescored on the same schedule did not. This beats B because a testing effect from taking the scale twice would also appear in the moderate group, and it did not.

**B** **Incorrect** (trap: Wrong model or concept). A testing effect happens when taking a measure changes later scores. It would show up in both groups, since both completed the scale twice on the same schedule, yet the parents with moderate scores barely changed.

**C** **Incorrect** (trap: Wrong model or concept). History means outside events between measurements. An event during the same 3 weeks would likely affect both groups' scores, not only the parents selected for the highest stress.

**Takeaway:** Groups chosen for extreme scores tend to score closer to average on retest, even without any intervention.

**Sources:** Campbell, D. T., & Stanley, J. C. (1963). Experimental and Quasi-Experimental Designs for Research. Rand McNally, threats to internal validity: statistical regression

**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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## 306. INT-1579 (rev 1) · Best answer: B

A community clinic begins serving many adults who recently resettled from Afghanistan. Staff want to track depression with a scale developed and normed with English-speaking U.S. adults, and a staff member has translated it into Dari. Early scores look low, even for clients who describe poor sleep, hopelessness and withdrawal. What is the **MOST** appropriate response?

**A.** Compare the clients' scores with the scale's published norms

**B.** Seek a version with validity evidence for this population

**C.** Have a second translator check the Dari wording

**D.** Stop using standardized measures with this group

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misuses the technique). Published norms describe the group the scale was built with. Comparing these clients with English-speaking U.S. norms assumes the scale works the same way across language and culture, which has not been shown.

**B** **Correct.** Validity belongs to the interpretation of scores for a particular population and language, so a translated scale needs evidence that it measures the same construct in the new group (AERA, APA & NCME, 2014). Low scores alongside clear signs of distress suggest the tool may miss how depression is expressed in this group, a known cultural issue (DSM-5-TR). This beats C because an accurate translation preserves the words but not necessarily their meaning, the relevance of items, or the norms.

**C** **Incorrect** (trap: Misuses the technique). Checking translation accuracy is a useful step. It confirms the wording, but it does not show that the items capture how distress is expressed in this community or that the scores mean the same thing.

**D** **Incorrect** (trap: Good but not best). Dropping measurement gives up useful tracking. A measure with validity evidence for this group, used alongside clinical interviews, serves clients better than no measure at all.

**Takeaway:** A scale's validity is population-specific; before relying on scores from a new cultural or language group, seek evidence it works for them.

**Sources:** AERA, APA & NCME (2014). Standards for Educational and Psychological Testing. American Educational Research Association, Ch. 3: Fairness in Testing; DSM-5-TR (APA, 2022), Culture and Psychiatric Diagnosis: Cultural Concepts of Distress

**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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### 307. INT-1580 (rev 1) · Best answer: C

An agency director shares a study of a new parenting program that randomly assigned 3,000 families. Program families scored 1 point lower than controls on a 100-point child stress scale, and the difference was statistically significant. The director proposes replacing the agency's current parenting program with the new one. How should the social worker respond?

**A.** Support the change, since the result was statistically significant
**B.** Ask for the study to be repeated with a larger sample
**C.** Ask whether a change that small matters in practice

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Statistical significance means the difference is probably not due to chance. With 3,000 families, even a trivial difference can reach significance, so it is not by itself a reason to replace an existing program.

**B** **Incorrect** (trap: Wrong model or concept). A larger sample makes small differences more likely to be significant but does not make them any bigger. The concern here is the size of the effect, not whether it is real.

**C** **Correct.** Statistical significance says a difference is unlikely to be chance; it says nothing about how large or useful the difference is. With very large samples, tiny differences become significant. Clinical or practical significance asks whether the change makes a real difference in people's lives (Jacobson & Truax, 1991; Rossi, Lipsey & Henry, 2019). A 1-point change on a 100-point scale is unlikely to matter, and switching programs has real costs. This beats A because significance alone does not show the program is worth adopting.

**Takeaway:** Statistical significance shows a difference is probably real, not that it is large enough to matter; check effect size before changing practice.

**Sources:** Jacobson, N. S., & Truax, P. (1991). Clinical significance: A statistical approach to defining meaningful change in psychotherapy research. Journal of Consulting and Clinical Psychology, 59(1), 12-19, whole article; Rossi, P. H., Lipsey, M. W., & Henry, G. T. (2019). Evaluation: A Systematic Approach (8th ed.). SAGE, Ch. 9: Detecting, Interpreting, and Exploring Program Effects
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** none · **Cognitive** application · **Difficulty** 1 of 3

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### 308. INT-1581 (rev 2) · Best answer: A

Which type of validity is supported when a panel of experts judges that a scale's items represent every part of the construct it is meant to measure?

**A.** Content validity
**B.** Face validity
**C.** Construct validity
**D.** Concurrent validity

**Why each option is right or wrong**

**A** **Correct.** Content validity, which the current testing standards call evidence based on test content, concerns whether a measure's items adequately sample the whole domain of the construct, and it is usually judged systematically by experts in the area (AERA, APA & NCME, 2014; Rubin & Babbie, 2017). The decisive detail is that experts checked whether every part of the construct is covered. This beats B because face validity is only a surface judgment that the items look relevant, often made by test takers or laypeople, and it does not examine coverage.

**B** **Incorrect** (trap: Wrong model or concept). Face validity means a measure looks like it measures what it claims to, on the surface. It is an informal impression and does not involve experts checking that all parts of the construct are covered.

**C** **Incorrect** (trap: Wrong model or concept). Construct validity is shown when scores relate to other measures as theory predicts, correlating with similar measures and not with unrelated ones (Rubin & Babbie, 2017). It rests on patterns in data, not on expert review of items.

**D** **Incorrect** (trap: Wrong model or concept). Concurrent validity is shown when scores agree with a criterion measured at the same time, such as an established diagnostic interview. No outside criterion is involved here.

**Takeaway:** Content validity asks whether items cover the whole construct, judged by experts; face validity asks only whether items look relevant.

**Sources:** AERA, APA & NCME (2014). Standards for Educational and Psychological Testing. American Educational Research Association, Ch. 1: Validity, evidence based on test content; Rubin, A., & Babbie, E. R. (2017). Research Methods for Social Work (9th ed.). Cengage, Measurement: types of validity (face, content, criterion-related, construct)

**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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## 309. INT-1582 (rev 1) · Best answer: B

At the end of each final session, outpatient social workers hand their clients a satisfaction survey with the worker's name printed at the top. Each worker waits while the client fills it out and then collects it. Nearly every rating is 'very satisfied.' Which change would **MOST** improve the accuracy of the results?

**A.** Add more rating items to the survey for detailed feedback

**B.** Have staff outside the client's care collect anonymous surveys

**C.** Remove the worker's name but keep workers collecting it

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). More items can add detail, but they do not remove the pressure clients feel when rating their own worker face to face. Biased answers to more questions are still biased.

**B** **Correct.** Client satisfaction surveys tend to run very positive, and the bias grows when the person being rated hands out and collects the form (Royse, Thyer & Padgett, 2016). Making responses anonymous and having someone outside the client's care collect them reduces the pull to please the worker, and it protects the confidentiality of evaluation data (NASW Code 5.02(m)). This beats C because removing the name still

leaves clients answering under their worker's eye and handing the form back to that worker.

**C** **Incorrect** (trap: Good but not best). Removing the name helps a little, but the worker still waits and collects the form. Clients may still give favorable ratings to avoid seeming ungrateful to the person who helped them.

**Takeaway:** Reduce social desirability bias in client feedback by making responses anonymous and having people not involved in care collect them.

**Sources:** Royse, D., Thyer, B. A., & Padgett, D. K. (2016). Program Evaluation: An Introduction to an Evidence-Based Approach (6th ed.). Cengage, Ch. 7: Client Satisfaction; NASW Code of Ethics (2021), 5.02(m)
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3

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### 310. INT-1665 (rev 1) · Best answer: C

An agency evaluates a 12-week program for adults with alcohol use disorder by comparing intake and final scores. Of 60 clients who started, 25 left before week 12, most of them clients with the most severe drinking at intake. Final scores among those who finished are much better than the group's intake scores. Which threat to internal validity **MOST** likely affects this conclusion?

**A.** History
**B.** Maturation
**C.** Attrition
**D.** Selection

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). History refers to outside events during the program, such as a new law or a community crisis, that could explain change. Nothing here points to an outside event; the problem is who remained to be measured.

**B** **Incorrect** (trap: Wrong model or concept). Maturation refers to natural change within people over time, such as growth or spontaneous recovery. It may play some role over 12 weeks, but it does not explain why losing the most severe clients makes final scores look better.

**C** **Correct.** Attrition, sometimes called experimental mortality, is the loss of participants before the final measure; it threatens internal validity when those who leave differ from those who stay (Shadish, Cook & Campbell, 2002). Here 25 of 60 left, mostly those with the most severe drinking, so the final group was healthier from the start, and comparing its scores with the whole group's intake scores builds in apparent improvement. It beats D because selection concerns differences in who enters the groups being compared, while this bias comes from who left one group during the program.

**D** **Incorrect** (trap: Wrong model or concept). Selection threatens validity when the groups being compared differ from the start, as with nonrandom assignment. This was one group measured twice; the bias comes from losing participants during the program, not from how groups were formed.

**Takeaway:** When participants who leave a study differ from those who stay, attrition can make a program look more effective than it is.

**Sources:** Shadish, W. R., Cook, T. D., & Campbell, D. T. (2002). Experimental and Quasi-Experimental Designs for Generalized Causal Inference. Houghton Mifflin, threats to internal validity: attrition
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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### 311. INT-1666 (rev 1) · Best answer: B

An agency can offer a new caregiver support group to only 20 people at a time, and 60 eligible caregivers are waiting. Burden scores are in the moderate range for nearly all of them, and caregivers in crisis are routed to urgent services. The director wants to learn whether the group reduces burden and proposes starting with the 20 highest scorers. What should the social worker **BEST** recommend?

**A.** Start with the 20 highest scorers and compare their pre and post scores
**B.** Set start order by lottery and compare early and later groups
**C.** Start in list order and measure each group before and after

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Starting with the highest scorers looks need-based, but extreme scorers tend to score closer to average when retested even without help, and they differ from later groups. Both make the group look more effective than it is. Ranking makes sense when needs differ sharply, which they do not here.

**B** **Correct.** When a program cannot serve everyone at once, setting start order by lottery is fair, because each person has an equal chance, and it creates a randomized comparison: caregivers still waiting serve as the control for those in the group (Shadish, Cook & Campbell, 2002). With burden similar across the list and crises handled elsewhere, little is lost by not ranking. It beats C because list order is also fair, but measuring participants only before and after cannot separate the group's effect from time, outside events, or other help caregivers receive.

**C** **Incorrect** (trap: Good but not best). Serving people in list order is fair, but a before-and-after measure of each group has no concurrent comparison. Any improvement could reflect time passing or outside events, so the agency would not know whether the group itself helped.

**Takeaway:** When demand exceeds capacity, a lottery for start order is fair and creates a randomized comparison group for evaluation.

**Sources:** Shadish, W. R., Cook, T. D., & Campbell, D. T. (2002). Experimental and Quasi-Experimental Designs for Generalized Causal Inference. Houghton Mifflin, randomized experiments: random assignment when demand exceeds supply
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 312. INT-1667 (rev 2) · Best answer: A

A new scale is given to a group. Its items correlate strongly with one another. Which property does this result support?

**A.** Internal consistency reliability
**B.** Test-retest reliability
**C.** Construct validity

**Why each option is right or wrong**

**A** **Correct.** Internal consistency reliability asks whether a scale's items measure the same thing, judged by how strongly they correlate with one another on a single administration, usually summarized with Cronbach's alpha (Bloom, Fischer & Orme, 2009). It beats C because items that hang together show the scale is reliable, not that it measures the intended concept; validity needs other evidence, such as expected relationships with other measures, and a scale can be reliable without being valid.

**B** **Incorrect** (trap: Wrong model or concept). Test-retest reliability correlates scores from two administrations of the same scale to show that scores stay stable over time. Correlations among the items describe how the items relate to each other, not stability across time.

**C** **Incorrect** (trap: Wrong model or concept). Construct validity asks whether a scale measures the intended concept, shown by expected relationships with other measures. Strong item correlations show reliability, which is necessary but not enough for validity.

**Takeaway:** Strong item intercorrelation on one administration shows internal consistency; it supports reliability, not validity.

**Sources:** Bloom, M., Fischer, J., & Orme, J. G. (2009). Evaluating Practice: Guidelines for the Accountable Professional (6th ed.). Pearson, reliability: internal consistency and test-retest
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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### 313. INT-1668 (rev 1) · Best answer: C

A community mental health agency wants to learn how satisfied its 900 active clients are. Last year it surveyed clients who attended the annual picnic, and about 150 responded. A board member suggests repeating the picnic survey with more volunteers to collect more forms. Which change would **MOST** improve how well the results represent all active clients?

**A.** Make the picnic survey anonymous so answers are more candid
**B.** Double the number of picnic surveys collected
**C.** Survey a random sample drawn from the full client list

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Anonymity helps clients answer honestly and is worth keeping. It addresses response bias, not who is asked. A survey limited to picnic attendees still misses clients who do not attend, so the results would still not represent the agency.

**B** **Incorrect** (trap: Misuses the technique). More responses shrink random error but do not fix a biased sampling frame. Doubling the picnic sample gives a more precise estimate of picnic attendees' views, not of all active clients.

**C** **Correct.** A sample represents a population when every member has a known chance of selection; probability sampling from the full client list removes the bias of surveying only those who attend an event (Rubin & Babbie, 2017). Picnic attendees likely differ from other clients in engagement, mobility, or satisfaction itself. A random sample of all 900, with follow-up to raise response, gives results that generalize. It beats A because anonymity reduces pressure to answer favorably, a different bias, but still leaves out every client who skips the picnic.

**Takeaway:** Representativeness depends on how a sample is drawn, not its size; draw a probability sample from the whole population.

**Sources:** Rubin, A., & Babbie, E. R. (2017). Research Methods for Social Work (9th ed.). Cengage, sampling: probability sampling and sampling bias
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.5 Research design, data collection/analysis, reliability, validity · **Qualifier** MOST · **Cognitive** application · **Difficulty** 1 of 3

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## 314. INT-1593 (rev 2) · Best answer: C

During an external licensing review, a state reviewer found that 30% of sampled charts at a community mental health agency lacked treatment plan updates within the required period. The agency's quality coordinator, a social worker, has since fixed the cited charts and trained staff on the requirement. What should the coordinator do **NEXT**?

**A.** Require supervisor sign-off on each chart's plan update
**B.** Repeat the staff training at the next staff meeting
**C.** Audit a new sample of charts over the next few months

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). A sign-off step may strengthen the process and could become part of the fix. Adding it now assumes the cause without checking, and it adds work for every chart; a follow-up audit of new charts first shows whether the training worked and where any gap remains.

**B** **Incorrect** (trap: Skips assessment). Repeating training assumes knowledge is still the problem. Without data on current charts, the coordinator cannot tell whether the first training worked or whether a workflow barrier remains.

**C** **Correct.** Quality improvement does not end when cited records are fixed; a change must be monitored to confirm the improvement holds (Langley et al., 2009). A follow-up audit of newly written charts shows whether training changed practice and whether documentation is now timely (NASW Code 3.04(b)). This beats A because a new sign-off layer adds work without showing whether practice has changed; checking new charts tests the fix before more controls are added.

**Takeaway:** After correcting audit findings, monitor current performance with follow-up audits to confirm the fix is sustained.

**Sources:** Langley, G. J., Moen, R. D., Nolan, K. M., Nolan, T. W., Norman, C. L., & Provost, L. P. (2009). The Improvement Guide (2nd ed.). Jossey-Bass, Ch. 8: Implementing a Change; NASW Code of Ethics (2021), 3.04(b)
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.6 Quality assurance: external program reviews and audits · **Qualifier** NEXT · **Cognitive** application · **Difficulty** 2 of 3

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## 315. INT-1594 (rev 1) · Best answer: D

A social worker doing the quarterly internal chart audit at a counseling agency reviews 10 sampled charts for one colleague. Nine document sessions on dates when the electronic check-in system shows the client never arrived, and all nine were billed. Agency policy names a compliance officer to receive billing concerns. What should the social worker do **NEXT**?

**A.** Ask the colleague to explain the mismatched dates
**B.** Expand the review before reporting to the compliance officer
**C.** Correct the billing for the nine sessions
**D.** Report the findings to the compliance officer

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). Talking first with a colleague fits when that is likely to be productive and no formal process applies (NASW Code 2.10(c)). Here the social worker is acting as auditor, the evidence shows a pattern, and a private talk could prompt changes to records before review.

**B** **Incorrect** (trap: Right action, wrong time). A wider review may well be needed, but the compliance officer should set its scope and method. Expanding it first, on the auditor's own initiative, delays reporting a strong pattern the audit has already found.

**C** **Incorrect** (trap: Outside the social work role). Correcting claims is a billing and compliance task that follows investigation. Changing them now, before the compliance officer reviews the evidence, could alter the record.

**D** **Correct.** Billing must reflect services actually provided, and social workers may not take part in or condone fraud (NASW Code 3.05, 4.04). An auditor's findings go through the agency's established channel, which here is the compliance officer (NASW Code 2.10(b); HHS-OIG, 2023). Nine of 10 billed sessions contradicted by check-in data form a pattern, not a stray error. This beats A because informal resolution suits ordinary collegial concerns, while the audit role and policy route billing concerns to compliance.

**Takeaway:** Audit findings that suggest billing for services not provided go promptly through the designated compliance channel, not informal resolution.

**Sources:** NASW Code of Ethics (2021), 3.05; NASW Code of Ethics (2021), 4.04; NASW Code of Ethics (2021), 2.10(b); NASW Code of Ethics (2021), 2.10(c); HHS Office of Inspector General (2023). General Compliance Program Guidance, compliance officer; reporting and responding to potential noncompliance
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.6 Quality assurance: external program reviews and audits · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 316. INT-1595 (rev 1) · Best answer: B

Which external review checks a whole organization against published standards and grants it a recognized status for a set period?

**A.** Utilization review
**B.** Accreditation survey
**C.** Peer review
**D.** Financial audit

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Utilization review examines whether specific services, such as hospital admissions and lengths of stay, are medically necessary and efficiently used (42 CFR 482.30). It looks at cases, not at a whole organization against published standards.

**B** **Correct.** Accreditation is an external review in which a body such as CARF assesses whether an organization's programs and services meet defined standards of quality through a consultative peer-review process, and grants accreditation for a set term (CARF International). The decisive features are whole-organization review, published standards and a time-limited status. This beats D because a financial audit examines fiscal statements and controls, not whether services meet practice standards.

**C** **Incorrect** (trap: Wrong model or concept). Peer review has professionals examine colleagues' work, often a sample of cases, to judge the quality of practice. It does not grant an organization a recognized status for a fixed period.

**D** **Incorrect** (trap: Wrong model or concept). A financial audit examines an organization's financial statements and internal controls. It checks fiscal accuracy rather than whether policies and services meet practice standards.

**Takeaway:** Accreditation is an external review against published standards that grants recognized status for a set term; utilization review checks medical necessity.

**Sources:** CARF International. What is accreditation? (web page), carf.org/accreditation, accessed 2026.; Medicare Conditions of Participation for Hospitals, 42 CFR 482.30 (utilization review)
**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.6 Quality assurance: external program reviews and audits · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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### 317. INT-1681 (rev 2) · Best answer: C

An accreditation survey of an outpatient behavioral health clinic results in three cited deficiencies. The director asks the quality manager to draft a plan of correction and to decide which deficiency to correct before the others. Which finding should the plan address **FIRST**?

**A.** Client signatures missing on 40% of treatment plans
**B.** Consent forms omitting current confidentiality limits
**C.** Six positive suicide screens lacking reassessment

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Unsigned plans are the most frequent finding and need correction, since client participation in planning matters. Frequency alone does not set priority, though; a safety gap affecting fewer clients comes first.

**B** **Incorrect** (trap: Good but not best). Consent forms that omit current confidentiality limits affect every new client's informed consent, and the fix is quick, so it should follow soon. The gap is real but carries less immediate danger than six clients at known suicide risk going unreassessed.

**C** **Correct.** Corrective action after an external review is prioritized by potential for harm, not by how often a deficiency appears (Reamer, 2015). Six clients who screened positive for suicide risk have no documented reassessment, so the clinic must first reach and reassess them and then fix the process that let the gap occur; accreditors treat suicide risk reduction as a national patient safety goal (The Joint Commission, NPSG.15.01.01). It beats A because unsigned plans are more widespread but pose a documentation and engagement problem rather than a threat to life.

**Takeaway:** Address external review findings in order of potential client harm; a safety gap outranks more frequent documentation deficiencies.

**Sources:** Reamer, F. G. (2015). Risk Management in Social Work: Preventing Professional Malpractice, Liability, and Disciplinary Action. Columbia University Press, Ch. 1: Professional Risk Management: An Overview; The Joint Commission (2018). R3 Report Issue 18: National Patient Safety Goal for Suicide Prevention, NPSG.15.01.01 (hospitals and behavioral health care organizations)

**Area** III Intervention and Practice · **Sub-area** III.C Practice Evaluation and Research · **Topic** III.C.6 Quality assurance: external program reviews and audits · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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# III.D Supervision and Administration

### 318. INT-1605 (rev 2) · Best answer: A

An associate social worker sees an adult client by telehealth. The client, angry after losing a custody hearing, says the former partner 'will regret this' and mentions keeping a handgun at home. When asked directly, the client says the comment meant an appeal and denies intent to harm anyone. The associate remains unsure whether protective action is needed, and supervision is scheduled in 6 days. What should the associate do **NEXT**?

**A.** Contact the supervisor today to review the risk together

**B.** Bring detailed notes to the scheduled supervision

**C.** Revisit intent and firearm access at the next session

#### Why each option is right or wrong

**A** **Correct.** Part of the supervisee's role is disclosing risk situations promptly and getting direction when unsure of a protective threshold, rather than waiting for a routine meeting. Supervisors share responsibility for the services their supervisees provide (NASW & ASWB supervision standards, 2013), so the supervisor may need to act. A threat-like statement, a firearm and unresolved doubt call for same-day consultation, a core risk-management step (Reamer, 2015). It beats C because the direct questions have already been asked; another round days later delays a decision the associate should not make alone.

**B** **Incorrect** (trap: Ignores risk). Waiting 6 days leaves a possible threat with firearm access unreviewed. Bringing notes to routine supervision fits stable cases and questions that can safely wait.

**C** **Incorrect** (trap: Right action, wrong time). Discussing intent and firearm storage again can be useful, but the associate has already asked directly and is still unsure. The decision about protective action needs the supervisor's input now, not at a later session.

**Takeaway:** A supervisee unsure whether a client's statement requires protective action contacts the supervisor the same day instead of waiting for scheduled supervision.

**Sources:** NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 3 (Legal and Regulatory Issues): supervisors share responsibility for services to clients; Reamer, F. G. (2015). Risk Management in Social Work: Preventing Professional Malpractice, Liability, and Disciplinary Action. Columbia University Press, Ch. 6: Consultation, Referral, Documentation, and Records

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## 319. INT-1606 (rev 2) · Best answer: B

A newly hired associate social worker at a community mental health center is drafting goals for the supervision contract. The associate is skilled in individual cognitive behavioral work but has little experience with family sessions, which make up a third of the assigned caseload. The supervisor will also complete the associate's licensure evaluations. Which approach to the goals is **BEST**?

**A.** Let the supervisor set goals from the first licensure evaluation

**B.** Make family session work a priority learning goal

**C.** Ask that the family cases go to more experienced colleagues

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). Evaluation results can sharpen goals later, and the supervisor's view matters. Waiting for the first formal evaluation, though, leaves months of family sessions without targeted supervision when the gap is already known; supervisees help set their own learning goals from the start.

**B** **Correct.** Educational supervision is individualized to the supervisee's learning needs (Kadushin & Harkness, 2014), and the supervision contract should set written goals with responsibilities and time frames (NASW & ASWB supervision standards, 2013). The associate already knows the gap, and family sessions are a third of the caseload. Naming it now directs supervision where clients need it, even though the same supervisor will evaluate the associate. It beats A because the need is already clear, and delay leaves family clients served without focused support.

**C** **Incorrect** (trap: Hands off too soon). Social workers may take on work they intend to become competent in (NASW Code 4.01(a)). Asking to shed a third of the caseload avoids the learning that supervision is meant to support.

**Takeaway:** Supervisees self-assess and put known skill gaps into the supervision contract as priority learning goals, even when the supervisor also evaluates them.

**Sources:** Kadushin & Harkness (2014). Supervision in Social Work (5th ed.). Columbia University Press, Ch. 4: Educational Supervision; NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 2 (Conduct of Supervision): Contracting for Supervision; NASW Code of Ethics (2021), 4.01(a)

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### 320. INT-1607 (rev 1) · Best answer: C

An associate social worker at a family counseling agency records sessions, with client consent, for review in supervision. The supervisor asks the associate to bring one recording from this week. Which recording would be **MOST** useful for the associate's learning?

**A.** A session in which a new technique worked well
**B.** The most recent session, whatever happened in it
**C.** A session that felt stuck or went poorly

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Reviewing a success can reinforce a skill, but supervisees who bring only their best work hide the areas where supervision would help most.

**B** **Incorrect** (trap: Misuses the technique). A random recent session may be informative, but it leaves learning to chance. The supervisee's role is to choose material that targets current learning needs.

**C** **Correct.** Supervisees get the most from supervision when they bring the work they find hardest, including errors and stuck points, rather than work that shows them at their best (Kadushin & Harkness, 2014). Choosing where help is needed is part of self-assessment. A session that felt stuck shows exactly what the supervisor can help with. It beats A because a session that went well mainly confirms a skill already in place; it has some value, but less for learning.

**Takeaway:** Supervisees learn most by bringing the work they find hardest, such as sessions where they felt stuck, not their most polished work.

**Sources:** Kadushin & Harkness (2014). Supervision in Social Work (5th ed.). Columbia University Press, Ch. 5: Principles and Problems in Implementing Educational Supervision
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### 321. INT-1608 (rev 1) · Best answer: A

A hospital social worker covering two medical units falls 3 days behind on discharge assessments and progress notes. Two patients on the list have unsafe home situations, and several others need only routine paperwork. The social worker has been staying late, which has not closed the gap. What is the **MOST** appropriate action?

**A.** Rank the caseload by risk and review it with the supervisor
**B.** Keep working late until every assessment is current
**C.** Shorten notes on routine cases to free up time
**D.** Ask a colleague on another unit to take the two complex patients

**Why each option is right or wrong**

**A** **Correct.** Supervisees are responsible for recognizing when workload exceeds capacity and bringing it to supervision with their own view of priorities. Workload and case assignment fall within the supervisor's administrative function (Kadushin & Harkness, 2014). Staying late has failed, and two patients face unsafe discharges, so the worker needs help deciding what comes first and possibly redistributing cases. It beats D because an informal hand-off bypasses the person responsible for assignments and may leave the riskiest patients with someone unfamiliar with them.

**B** **Incorrect** (trap: Good but not best). Extra hours have already failed to close the gap, and continued overwork raises the risk of errors and burnout. Persistence makes sense for a brief backlog that is shrinking.

**C** **Incorrect** (trap: Misreads law or ethics). Records should be sufficient and timely enough to support continuity of care (NASW Code 3.04(b)). Cutting notes trades a workload problem for a documentation problem and does nothing for the unsafe discharges.

**D** **Incorrect** (trap: Hands off too soon). Colleagues may be able to help, but reassigning the two highest-risk patients informally bypasses the supervisor who controls assignments. The colleague may lack context, and the coverage gap stays unresolved.

**Takeaway:** When workload exceeds capacity, supervisees rank cases by risk and take the problem to supervision rather than overworking, cutting records or handing off informally.

**Sources:** Kadushin & Harkness (2014). Supervision in Social Work (5th ed.). Columbia University Press, Ch. 2: Administrative Supervision; NASW Code of Ethics (2021), 3.04(b)
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### 322. INT-1609 (rev 2) · Best answer: D

An associate social worker at a substance use treatment program has two supervisors. The program manager provides administrative supervision, and an outside licensed social worker provides clinical supervision for licensure. For one client, the manager directs weekly drug screens, while the clinical supervisor advises pausing screens to rebuild trust. Neither supervisor knows of the other's direction. What should the associate do **FIRST**?

**A.** Follow the program manager, who represents the employer
**B.** Follow the clinical supervisor, who oversees licensure hours
**C.** Seek an outside consultant's opinion on drug screening
**D.** Ask both supervisors to resolve the conflict together

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). When no agreement exists, the employment supervisor may have the final say, so the screens may end up following the manager. Complying before either supervisor knows of the conflict skips that step and leaves the clinical supervisor unaware of a change in the client's care.

**B** **Incorrect** (trap: Good but not best). The clinical supervisor's advice carries weight, but the program manager has legitimate authority over program procedures. Setting aside the manager's direction without discussion bypasses the agreed channels.

**C** **Incorrect** (trap: Right action, wrong time). Outside consultation can help if the supervisors cannot agree. As a first step it adds a third voice before the two people responsible for the associate's work have tried to settle it.

**D** **Correct.** When a supervisee has more than one supervisor, best practice is an agreement that defines each role, information sharing and how conflicts will be resolved (NASW & ASWB supervision standards, 2013, Dual Supervision and Conflict Resolution). Neither supervisor knows of the conflict, so the associate's first step is to bring it to both so they can settle it through that process, rather than choosing a side

alone. It beats A because, although the employment supervisor may have the final say when no agreement exists, that call belongs to the supervisors once both know of the conflict; silent compliance leaves the clinical supervisor unaware.

**Takeaway:** When two supervisors give conflicting directions, the supervisee brings the conflict to both to resolve rather than choosing one silently.

**Sources:** NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 1 (Context in Supervision): Dual Supervision and Conflict Resolution
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### 323. INT-1685 (rev 2) · Best answer: A

An associate social worker at a hospital realizes that a patient's psychosocial assessment, sent an hour ago by secure email, went to the wrong family's case manager at a nursing facility. The associate's supervisor is on site today, and weekly supervision is in 4 days. What should the associate do **FIRST**?

**A.** Inform the supervisor of the error now
**B.** Raise the error with the supervisor at the scheduled session
**C.** Ask the recipient to delete the message and confirm by reply

#### Why each option is right or wrong

**A** **Correct.** Supervision is a shared responsibility, and when a supervisee makes a mistake, the supervisor helps repair the harm and prevent a repeat (NASW & ASWB, 2013). A misdirected assessment is a privacy breach the organization must mitigate (HIPAA, 45 CFR 164.530(f)), so the supervisor needs to know now, while mitigation can still work. It beats C because contacting the recipient is one mitigation step that the agency's breach procedure should direct and record; with the supervisor on site, telling them first takes minutes and lets that step happen promptly and correctly.

**B** **Incorrect** (trap: Right action, wrong time). Supervision is the right place to review the error and learn from it, but 4 days is too long. Mitigation is time-sensitive, and the organization's breach procedure depends on prompt notice.

**C** **Incorrect** (trap: Right action, wrong time). Requesting deletion is a reasonable mitigation step, and the supervisor or privacy officer may direct it within minutes. Taken first on the associate's own, it bypasses the agency's breach procedure, leaves no record and may delay the timely notice the patient is owed under agency policy (NASW Code 1.07(n)).

**Takeaway:** Supervisees disclose errors to the supervisor promptly, especially time-sensitive ones, so repair follows agency procedure rather than private fixes.

**Sources:** NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 4 (Ethical Issues): Ethical Decision-Making; HIPAA Privacy Rule, 45 CFR 164.530(f); NASW Code of Ethics (2021), 1.07(n)
**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.1 Supervisee's role: learning needs, self-assessment, prioritizing · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 324. INT-1686 (rev 1) · Best answer: C

A child welfare associate is completing a self-assessment before a 6-month evaluation. The associate feels confident about court reports, but the supervisor returned three of the last five reports for missing safety details. The agency form asks for a rating and an example for each skill. Which approach to the self-assessment is **MOST** useful?

**A.** Rate court reports a strength, based on the associate's confidence
**B.** Give every skill a middle rating to avoid seeming overconfident
**C.** Rate court reports a growth area, noting the specific feedback

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misuses the technique). Confidence matters, but self-ratings that ignore concrete feedback signal a gap in self-awareness. Most recent reports came back for revision, so calling them a strength would steer supervision away from a real need.

**B** **Incorrect** (trap: Misuses the technique). Uniform middle ratings look modest but give the supervisor no usable information about strengths or needs. Self-assessment should distinguish among skills using evidence.

**C** **Correct.** Self-assessment is part of the supervisee's role, and it is useful only when grounded in evidence rather than feelings. Three of five reports came back for missing safety details, so an honest rating names this a growth area and ties it to the feedback, which lets supervision target it (Kadushin & Harkness, 2014; NASW & ASWB, 2013). It beats A because confidence is not evidence; rating the skill a strength despite returned reports suggests limited insight and delays help with a safety-related task.

**Takeaway:** Base self-assessment for supervision on concrete feedback and work samples, not on confidence, so learning goals target real needs.

**Sources:** Kadushin & Harkness (2014). Supervision in Social Work (5th ed.). Columbia University Press, Ch. 5: Principles and Problems in Implementing Educational Supervision; NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Overview of Supervision (shared responsibility)
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### 325. INT-0117 (rev 2) · Best answer: C

A social worker practicing under clinical supervision for licensure presents a case at a monthly peer consultation group. The group recommends a different treatment approach from the one the clinical supervisor directed. What should the social worker do **FIRST**?

**A.** Switch to the approach the consultation group recommended
**B.** Continue the supervisor's plan without raising the group's view
**C.** Discuss the group's recommendation with the supervisor

**Why each option is right or wrong**

**A** **Incorrect** (trap: Misreads law or ethics). Peer consultation offers advice, not direction, and carries no authority over a supervised case. Changing the treatment without the supervisor bypasses the person who shares responsibility and liability for the work.

**B** **Incorrect** (trap: Good but not best). Following the supervisor's plan respects the supervisor's authority, but staying silent keeps potentially useful clinical input from the person accountable for the case. Supervisees are expected to raise questions openly.

**C** **Correct.** Supervision carries authority and shared accountability: the supervisor directs the supervisee's work and can be held directly and vicariously liable for it (NASW & ASWB, 2013, Standard 3). Consultation is advisory; the consultant has no authority over the case, and the person consulting stays responsible for decisions (Caplan, 1970). The peer group's idea may be valuable, so the social worker brings it to the supervisor, where the decision belongs. It beats B because silently following the plan withholds clinically useful input from the person accountable for the case.

**Takeaway:** Consultation advises; supervision directs and shares accountability. A supervisee brings outside consultation to the supervisor rather than acting on it alone.

**Sources:** NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 3 (Legal and Regulatory Issues): Liability; Caplan, G. (1970). The Theory and Practice of Mental Health Consultation. Basic Books, definition of consultation: consultee retains responsibility

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### 326. INT-1600 (rev 1) · Best answer: B

A child welfare supervisor meets with a caseworker each week. In one meeting, the supervisor reviews the caseworker's cases and moves two families to a coworker who has fewer. The supervisor also checks that court reports will be filed on time. Which function of supervision is the supervisor **MOST** likely carrying out?

**A.** Educational

**B.** Administrative

**C.** Supportive

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). The educational function builds knowledge and skill, such as teaching how to write a strong court report. Checking whether reports will be filed on time is quality control of the work, not teaching.

**B** **Correct.** Kadushin's administrative function covers assigning work, balancing workloads, and monitoring policy compliance and quality (Kadushin & Harkness, 2014; NASW & ASWB supervision standards, 2013). Reassigning families to even out caseloads and checking court reports against agency deadlines are all administrative tasks. It beats C because the supervisor acts on assignments and deadlines without attending to the caseworker's stress or morale; the cases move to manage the unit's work, not as emotional support. It beats A because no knowledge or skill is being taught.

**C** **Incorrect** (trap: Wrong model or concept). The supportive function addresses morale, stress and burnout. Lighter caseloads can ease stress, but this supervisor is managing assignments and deadlines rather than responding to the caseworker's feelings, which points to administrative work.

**Takeaway:** Assigning work, balancing caseloads and monitoring compliance are administrative supervision; teaching skills is educational; tending morale and stress is supportive.

**Sources:** Kadushin & Harkness (2014). Supervision in Social Work (5th ed.). Columbia University Press, Ch. 2: Administrative Supervision; NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Overview of Supervision: Administrative

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### 327. INT-1601 (rev 1) · Best answer: D

A nursing home's charge nurse asks a social work consultant for help with a resident who refuses meals. The consultant learns the nurse has managed similar residents well, but this resident reminds the nurse of a parent, and the nurse now pleads and argues at mealtimes. The consultant works on the nurse's reactions to this resident. Which type of consultation is this **MOST** likely?

**A.** Client-centered case consultation
**B.** Program-centered administrative consultation
**C.** Consultee-centered administrative consultation
**D.** Consultee-centered case consultation

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Client-centered case consultation centers on the client: the consultant assesses the resident and recommends a care plan, and the consultee's own functioning is not the focus. It would fit if the nurse simply lacked information about meal refusal.

**B** **Incorrect** (trap: Wrong model or concept). Program-centered administrative consultation helps plan or run a program, such as designing a facility-wide nutrition protocol. This request concerns one resident and one staff member's handling of that resident.

**C** **Incorrect** (trap: Wrong model or concept). Consultee-centered administrative consultation addresses an administrator's difficulties with organizational or program tasks. The nurse's difficulty here involves the care of one resident, not a program or management task.

**D** **Correct.** Caplan (1970) classified consultation by its focus. In consultee-centered case consultation, the consultant addresses the consultee's difficulty with a particular case, such as a loss of objectivity, rather than the client's problem itself. The nurse has the needed skill, but a personal resemblance is distorting the work, and the consultant targets that reaction. It beats A because client-centered case consultation would center on assessing the resident and advising a plan for the resident, which is not what this consultant is doing.

**Takeaway:** When a consultant works on the consultee's difficulty with a case, such as lost objectivity, it is consultee-centered case consultation.

**Sources:** Caplan, G. (1970). The Theory and Practice of Mental Health Consultation. Basic Books, Types of mental health consultation

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## 328. INT-1602 (rev 1) · Best answer: A

An integrated primary care clinic hires 5 new associate social workers at once. All of them must learn the clinic's brief behavioral health protocol, and the one licensed supervisor has limited hours. Each associate already has a weekly individual meeting for case-specific risk questions. Which added format is **MOST** appropriate for teaching the protocol?

**A.** Group supervision for the associates as a cohort

**B.** A peer consultation group the associates run themselves

**C.** Live supervision of each associate's patient sessions

#### Why each option is right or wrong

**A** **Correct.** Group supervision is efficient when several supervisees share a learning need, and it adds peer learning and mutual support while a qualified supervisor guides the content (Kadushin & Harkness, 2014). All 5 associates must learn the same protocol, the supervisor's hours are limited, and individual meetings already cover case-specific risk. It beats C because live observation of every associate would use far more supervisor time to teach the same material; live supervision fits better for refining one person's skills once the protocol has been learned.

**B** **Incorrect** (trap: Good but not best). Peer consultation has no member with supervisory authority or expertise in the protocol. Five newcomers teaching each other a model none of them knows risks spreading errors. The Code points workers to colleagues with demonstrated expertise (NASW Code 2.05(b)); peer groups suit experienced practitioners.

**C** **Incorrect** (trap: Good but not best). Live supervision gives in-the-moment feedback and is valuable for skill refinement. Doing it for each of 5 associates would consume more of the supervisor's limited hours than one shared group session covering the same protocol.

**Takeaway:** When several supervisees share one learning need and supervisor time is scarce, supervisor-led group supervision teaches efficiently and adds peer learning.

**Sources:** Kadushin & Harkness (2014). Supervision in Social Work (5th ed.). Columbia University Press, Ch. 9: Group Supervision; NASW Code of Ethics (2021), 2.05(b)

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### 329. INT-1603 (rev 1) · Best answer: C

A social worker with 6 years of independent practice in adult outpatient therapy transfers to a pediatric oncology unit. In supervision, the social worker asks for scripts for explaining procedures to children and wants every family meeting plan reviewed. With adult caregivers, the social worker works confidently and needs little input. Which supervisory approach is **MOST** appropriate?

**A.** Keep the autonomy-focused approach that suits the worker's experience
**B.** Explore with the worker why confidence has dropped since the move
**C.** Give more structure and direction on the child-focused work
**D.** Give close direction across the worker's whole caseload

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Years of experience do not transfer automatically to a new population. A hands-off approach ignores the worker's clear requests for help with children. It does fit the adult caregiver work.

**B** **Incorrect** (trap: Good but not best). The stem already shows why the worker wants help: child-focused work is new. Exploring the drop in confidence adds little and delays the structure the worker needs now. Exploration fits when a change in functioning has no clear cause.

**C** **Correct.** Stoltenberg's Integrated Developmental Model holds that development is domain-specific: a clinician can work autonomously in one area and function like a beginner in another, and supervision should match the level in each domain (Stoltenberg & McNeill, 2010). This worker needs structure for child-focused work but not for adult caregiver work. It beats D, which treats the worker as a beginner overall and undercuts autonomy where the worker is competent, and A, which treats experience as one global level.

**D** **Incorrect** (trap: Misuses the technique). Close direction on every case treats the worker as a beginner across the board. That can erode motivation and autonomy in the adult caregiver work, where the worker already functions independently.

**Takeaway:** Supervisee development is domain-specific: give more structure in new areas while keeping autonomy where the supervisee is already competent.

**Sources:** Stoltenberg, C. D., & McNeill, B. W. (2010). IDM Supervision: An Integrative Developmental Model for Supervising Counselors and Therapists (3rd ed.). Routledge, Domains of development; supervision environments by level

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### 330. INT-1682 (rev 1) · Best answer: A

A first-year school social worker co-leads a social skills group for 8-year-olds. Sessions keep unraveling when two members begin taunting each other, and the social worker freezes. Discussing session notes in weekly supervision has produced sound plans that fall apart in the moment. Which supervision format is **BEST** suited to this need?

**A.** Live supervision with in-session coaching
**B.** Review of session recordings afterward
**C.** Peer consultation with other school staff
**D.** Group supervision with case presentations

**Why each option is right or wrong**

**A** **Correct.** Live supervision lets the supervisor observe a session directly, through a one-way mirror or by video, and coach the worker as events unfold, for example with a bug-in-the-ear device or a brief consultation break (Bernard & Goodyear, 2019). This worker's plans are sound but collapse in the moment, so feedback has to arrive during the session. It beats B because recordings show accurately what happened but still deliver feedback after the moment has passed, the same gap note-based supervision has.

**B** **Incorrect** (trap: Wrong model or concept). Recordings improve on self-report by showing what actually happened, and they help when the problem is unclear. The difficulty here is applying sound plans in real time, which after-the-fact review cannot coach.

**C** **Incorrect** (trap: Wrong model or concept). Peer consultation offers ideas and support but carries no supervisory role and still happens outside the session. It would not give a beginning worker real-time guidance.

**D** **Incorrect** (trap: Wrong model or concept). Group supervision builds shared learning and normalizes struggles, but it relies on case presentations after the fact. It suits a need several workers share better than one worker's in-the-moment difficulty.

**Takeaway:** When a supervisee's plans are sound but break down during sessions, live supervision provides feedback in real time.

**Sources:** Bernard, J. M., & Goodyear, R. K. (2019). Fundamentals of Clinical Supervision (6th ed.). Pearson, Live supervision

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### 331. INT-1683 (rev 1) · Best answer: B

In supervision, an associate describes rushing to end sessions whenever clients cry. The supervisor asks what the associate notices in themself at those moments and how those reactions shape the work, keeping the focus on the associate's practice rather than personal history. In Bernard's discrimination model, which supervisor role does this **MOST** likely show?

**A.** Teacher role
**B.** Counselor role
**C.** Consultant role

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). In the teacher role, the supervisor instructs, models or gives direct feedback on skills, such as demonstrating how to stay with a crying client. The supervisor here invites reflection on internal reactions instead of instructing.

**B** **Correct.** Bernard's discrimination model has the supervisor shift among three roles, teacher, counselor and consultant, depending on what the supervisee needs. In the counselor role, the supervisor helps the supervisee examine internal reactions that affect the work, while keeping supervision distinct from therapy. Asking what the associate notices internally when clients cry fits this role. It beats C because the consultant role is a collegial exchange in which the supervisee leads and the supervisor offers options, not an exploration of the supervisee's reactions (Bernard & Goodyear, 2019).

**C** **Incorrect** (trap: Wrong model or concept). The consultant role treats the supervisee as a colleague who sets the agenda while the supervisor offers resources or alternatives. It suits experienced workers solving a case problem, not the examination of a personal reaction.

**Takeaway:** In Bernard's discrimination model, the counselor role addresses the supervisee's own reactions that affect practice, without becoming therapy.

**Sources:** Bernard, J. M., & Goodyear, R. K. (2019). Fundamentals of Clinical Supervision (6th ed.). Pearson, Discrimination model: teacher, counselor and consultant roles

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## 332. INT-1684 (rev 2) · Best answer: C

To cut costs, a nonprofit's director proposes ending individual supervision for 4 associate social workers and replacing it with a monthly peer consultation group made up of the associates. The associates carry caseloads with frequent suicide risk. The director asks the clinical manager for a recommendation. What is the **MOST** appropriate recommendation?

**A.** Adopt the peer group, since associates can advise one another

**B.** Adopt the peer group with the manager reviewing notes monthly

**C.** Keep designated supervisors and add the peer group as a supplement

**D.** Keep individual supervision but reduce it to monthly sessions

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Peer groups offer support and fresh ideas, but no member has authority over another's work or shares accountability for client outcomes. Treating peer consultation as supervision leaves associates without anyone who can direct their practice.

**B** **Incorrect** (trap: Good but not best). Note review adds some oversight, but a monthly look at records is not supervision. It lacks the regular relationship, skill development and timely guidance on risk that caseloads with frequent suicide risk need.

**C** **Correct.** Supervision is an ongoing relationship in which the supervisor holds authority and shares responsibility for the supervisee's practice; peer consultation is advisory, and no member carries that accountability (NASW & ASWB, 2013; Kadushin &

Harkness, 2014). Associates managing frequent suicide risk need someone who can direct their work and act on safety concerns. Keeping designated supervision while adding the peer group preserves accountability and adds low-cost peer learning. It beats B because occasional note review is oversight in name, without the regular contact supervision provides.

**D** **Incorrect** (trap: Good but not best). Monthly supervision keeps a designated supervisor and saves money, so it sounds like a fair compromise. With frequent suicide risk on the associates' caseloads, though, a month between contacts leaves risk questions waiting; keeping regular supervision and adding the peer group as a supplement meets both aims.

**Takeaway:** Peer consultation can supplement supervision but cannot replace it, because no peer holds supervisory authority or shared accountability.

**Sources:** NASW & ASWB Best Practice Standards in Social Work Supervision (2013), Standard 3 (Legal and Regulatory Issues): supervisors share responsibility for services to clients; Kadushin & Harkness (2014). Supervision in Social Work (5th ed.). Columbia University Press, Ch. 9: Group Supervision
**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.2 Supervision and consultation models: individual, peer, group · **Qualifier** MOST APPROPRIATE · **Cognitive** application · **Difficulty** 2 of 3

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### 333. INT-0009 (rev 1) · Best answer: C

A supervisee who is usually confident becomes hesitant in supervision and repeatedly asks the supervisor to say exactly what to do. The supervisee is working with a client who feels helpless and keeps asking the supervisee for answers. What is the supervisor's **BEST** response?

**A.** Give step-by-step directions for the next session
**B.** Recommend personal therapy for the supervisee's dependence
**C.** Explore whether the case dynamic is being replayed between them
**D.** Note a performance concern in the supervisee's evaluation

#### Why each option is right or wrong

**A** **Incorrect** (trap: Boundary slip or collusion). Detailed directions may solve the next session but repeat the dynamic the client has set up: a helpless person being rescued. They also bypass the supervisee's own clinical thinking.

**B** **Incorrect** (trap: Hands off too soon). Personal therapy may help if a lasting personal issue emerges. Recommending it now treats a common, case-related supervisory dynamic as a personal problem before it has been explored.

**C** **Correct.** The supervisee is reenacting with the supervisor the same helpless, answer-seeking stance the client takes with the supervisee. This is parallel process (Searles, 1955). Naming and exploring the parallel helps the supervisee understand the client's experience and regain footing, and it uses the supervisory relationship as information (Kadushin & Harkness, 2014). It beats A because step-by-step answers repeat the rescuing dynamic and leave the supervisee as dependent on the supervisor as the client is on the supervisee.

**D** **Incorrect** (trap: Skips assessment). Recording a performance concern assumes a skill deficit before the supervisor understands the change. A sudden shift in a usually confident supervisee points to a case dynamic worth exploring first.

**Takeaway:** When a supervisee re-enacts the client's dynamic with the supervisor, name and explore the parallel process rather than acting it out.

**Sources:** Searles, H. F. (1955). The informational value of the supervisor's emotional experiences. Psychiatry, 18(2), 135-146, parallel process; Kadushin, A., & Harkness, D. (2014). Supervision in Social Work (5th ed.). Columbia University Press, educational supervision

**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.3 Transference and countertransference in supervision · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 334. INT-0118 (rev 2) · Best answer: C

A supervisee who usually welcomes feedback starts rejecting every suggestion the supervisor offers about one case, saying, 'That won't work either.' In that case, the client turns down each resource the supervisee proposes. The supervisee stays open to feedback on other cases. What is **MOST** likely happening?

**A.** Transference toward the supervisor
**B.** Countertransference toward the client
**C.** Parallel process
**D.** Burnout

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Transference toward a supervisor stems from the supervisee's earlier relationships with authority figures and would tend to color supervision broadly. Here the behavior appears on one case and copies that client's pattern.

**B** **Incorrect** (trap: Wrong model or concept). Countertransference is the supervisee's emotional reaction to the client, such as frustration. It may be present, but it does not explain why the supervisee is acting out the client's rejecting stance with the supervisor.

**C** **Correct.** In parallel process, a dynamic from the client relationship is unconsciously reenacted in the supervisory relationship (Searles, 1955). The client rejects every resource; the supervisee now rejects every suggestion, and only on this case. That specificity is the decisive clue. Recognizing the parallel lets the supervisor explore it with the supervisee, which can deepen understanding of the client's help-rejecting stance (Kadushin & Harkness, 2014). It beats A because transference toward the supervisor would draw on the supervisee's past relationships and show up across cases, not mirror one client's pattern.

**D** **Incorrect** (trap: Near-miss diagnosis). Burnout brings exhaustion, cynicism and reduced effectiveness that usually spread across a caseload. This supervisee stays open on other cases, which points to a case-specific dynamic.

**Takeaway:** When a supervisee treats the supervisor the way a specific client treats the supervisee, suspect parallel process and explore it.

**Sources:** Searles, H. F. (1955). The informational value of the supervisor's emotional experiences. Psychiatry, 18(2), 135-146, parallel process; Kadushin, A., & Harkness, D. (2014). Supervision in Social Work (5th ed.). Columbia University Press, parallel process in educational supervision

**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.3 Transference and countertransference in supervision · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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### 335. INT-1610 (rev 1) · Best answer: A

An associate social worker in a jail mental health program came from a job where a supervisor publicly criticized every mistake. The new supervisor gives mild, specific feedback, yet the associate apologizes repeatedly, leaves errors out of case reviews and braces for reprimand at each meeting. What is **MOST** likely occurring?

**A.** Supervisee transference

**B.** Parallel process

**C.** Supervisor countertransference

#### Why each option is right or wrong

**A** **Correct.** Transference in supervision occurs when a supervisee reacts to the supervisor as to an important earlier figure, often a past authority (Bernard & Goodyear, 2019). The associate meets mild feedback with apologies, hidden errors and expected reprimand, a pattern that fits the harsh former supervisor rather than the current one. It beats B because parallel process involves a client dynamic replayed in supervision, and this pattern traces to a past supervisor and appears across all case reviews.

**B** **Incorrect** (trap: Wrong model or concept). Parallel process is the replay of a client-worker dynamic in the supervisory relationship, or the reverse (St. Arnaud, 2017). Here the pattern follows a past supervisor and appears in every review, not in connection with a particular case.

**C** **Incorrect** (trap: Wrong model or concept). Countertransference would be the supervisor's own reaction to the associate. The stem describes the supervisee's responses, and the supervisor's feedback is mild and specific.

**Takeaway:** A supervisee who reacts to a current supervisor as to a past authority figure shows transference within supervision.

**Sources:** Bernard, J. M., & Goodyear, R. K. (2019). Fundamentals of Clinical Supervision (6th ed.). Pearson, Supervisory relationship: transference and countertransference; St. Arnaud, K. O. (2017). Encountering the wounded healer: Parallel process and supervision. Canadian Journal of Counselling and Psychotherapy, 51(2), 131-144, whole article

**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.3 Transference and countertransference in supervision · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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### 336. INT-1611 (rev 2) · Best answer: B

A hospice social work supervisor has been quietly rewriting a new staff member's incomplete bereavement risk screens instead of returning them, as is done for other staff. In consultation, the supervisor traces the rewriting to a protective pull: the staff member's recent loss of a parent closely resembles the supervisor's own. What should the supervisor do **NEXT**?

**A.** Keep examining the protective reaction in consultation
**B.** Give the staff member the same corrective feedback others receive
**C.** Share the similar loss with the staff member to build trust

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). Ongoing consultation can help, but the supervisor has already traced the rewriting to the reaction. More reflection without action leaves the incomplete screens and the unequal treatment in place. Continued consultation fits alongside the corrective step, not instead of it.

**B** **Correct.** Supervisors should evaluate supervisees fairly against clearly stated criteria (NASW Code 3.03), and supervisors are expected to keep their own countertransference from distorting supervision and evaluation (Bernard & Goodyear, 2019). Having traced the rewriting to that reaction, the supervisor's next step is to hold this staff member to the same standard, which also builds the screening skill that bereaved families rely on. It beats A because the reaction and its effect are already identified; further reflection alone prolongs unequal treatment and withholds feedback the staff member needs to improve.

**C** **Incorrect** (trap: Boundary slip or collusion). Supervisor self-disclosure should serve the supervisee's learning. Sharing the parallel loss centers the supervisor's experience, may blur the relationship, and leaves the incomplete screens unaddressed.

**Takeaway:** After recognizing countertransference toward a supervisee, correct its effect by applying the same standards and feedback given to others.

**Sources:** NASW Code of Ethics (2021), 3.03; Bernard, J. M., & Goodyear, R. K. (2019). Fundamentals of Clinical Supervision (6th ed.). Pearson, Supervisory relationship: transference and countertransference
**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.3 Transference and countertransference in supervision · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 337. INT-1612 (rev 2) · Best answer: C

An emergency department social work supervisor is under pressure to shorten patient stays. The supervisor has started cutting supervision short and giving quick orders. Now a supervisee has begun ending patient assessments abruptly and rushing patients through instructions. Which response by the supervisor is **BEST**?

**A.** Coach the supervisee to slow assessments with patients
**B.** Review the supervisee's recent assessments for missed details
**C.** Slow down and model an attentive pace with the supervisee

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Coaching on pacing is reasonable, but it treats the behavior as the supervisee's alone. Delivered within the same rushed, directive meetings, it is likely to repeat the dynamic. It fits better once the supervisor's own pace has changed.

**B** **Incorrect** (trap: Good but not best). Checking recent assessments may catch gaps that affect patients and is a fair quality step. It treats the rushing as the supervisee's performance problem, though, and leaves untouched the hurried supervision that the pattern mirrors.

**C** **Correct.** Parallel process can run in both directions: dynamics in the supervisory relationship can carry into the supervisee's work with clients (St. Arnaud, 2017). The supervisee's rushed, directive style with patients matches how the supervisor has been treating the supervisee. Changing the supervision itself addresses the likely source and shows the stance wanted with patients. It beats A because telling the supervisee to slow down, in the same hurried and directive way, repeats the pattern instead of changing it.

**Takeaway:** Parallel process can flow from supervision to practice; a supervisor first changes their own conduct in supervision.

**Sources:** St. Arnaud, K. O. (2017). Encountering the wounded healer: Parallel process and supervision. Canadian Journal of Counselling and Psychotherapy, 51(2), 131-144, whole article
**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.3 Transference and countertransference in supervision · **Qualifier** BEST · **Cognitive** application · **Difficulty** 3 of 3

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## 338. INT-1687 (rev 1) · Best answer: B

A social worker with 8 years of hospice experience recently joined a new hospice team. In supervision, the social worker praises every suggestion, repeats the supervisor's phrasing word for word and recently said the supervisor is 'just like my first mentor, who always knew best.' The social worker has stopped offering their own case formulations. What is the **MOST** appropriate response by the supervisor?

**A.** Offer more direction while the social worker settles in
**B.** Name the pattern and explore it with the social worker
**C.** Suggest personal therapy to work through the mentor relationship

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Added structure suits a worker new to a practice area, as developmental models suggest (Stoltenberg & McNeill, 2010). This worker has years of hospice experience; more direction would reinforce the dependence that is crowding out their judgment.

**B** **Correct.** Supervisee transference, relating to the supervisor as to an important figure from the past, is handled within the supervisory relationship and kept distinct from therapy (Bernard & Goodyear, 2019). An experienced hospice worker who now copies the supervisor and compares them to a first mentor is showing idealization, and it is displacing the worker's own formulations. Naming the pattern and exploring it, with a focus on the work, helps the worker reclaim independent judgment. It beats A because more direction would feed the idealization in a worker who needs no extra structure.

**C** **Incorrect** (trap: Hands off too soon). Personal therapy may be suggested if personal issues impair practice and do not shift in supervision. Starting there skips a direct conversation about a pattern the supervisor can address within the supervisory role.

**Takeaway:** Address supervisee transference within supervision by naming and exploring it in relation to the work before suggesting outside help.

**Sources:** Bernard, J. M., & Goodyear, R. K. (2019). Fundamentals of Clinical Supervision (6th ed.). Pearson, Supervisory relationship: transference and countertransference; Stoltenberg, C. D., & McNeill, B. W. (2010). IDM Supervision: An Integrative Developmental Model for Supervising Counselors and Therapists (3rd ed.). Routledge, Domains of development; supervision environments by level

**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.3 Transference and countertransference in supervision · **Qualifier** MOST APPROPRIATE · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 339. INT-1688 (rev 1) · Best answer: C

A clinical director supervises an associate who publicly questioned the director's plan at a team meeting. Since then, the director has drafted lower ratings for the associate's annual evaluation than the case reviews support. On reflection, the director recognizes resentment about the meeting as the source. The evaluation is due in 5 days. What should the director do **NEXT**?

**A.** Keep monitoring the reaction over the coming month
**B.** Tell the associate about the resentment to clear the air
**C.** Seek consultation on the reaction before finishing the evaluation
**D.** Transfer the evaluation and supervision to another manager

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Self-monitoring was the right start, and the director has done it: the reaction is identified and already shaping draft ratings. With the evaluation due in 5 days, further watching leaves biased ratings in place.

**B** **Incorrect** (trap: Boundary slip or collusion). Disclosing resentment shifts the supervisor's emotional work onto a supervisee who holds less power and whose evaluation is pending. A conversation about the meeting itself can come later, once the director's reaction is managed.

**C** **Correct.** A supervisor's countertransference, an emotional reaction to a supervisee, can distort evaluation, and supervisors manage it by getting consultation rather than acting on it or handing it to the supervisee (Bernard & Goodyear, 2019). The director has identified resentment and seen it lower draft ratings, with the evaluation due in 5 days. Consultation offers an outside check so final ratings match the case reviews, as fair evaluation requires (NASW Code 3.03). It beats D because transfer is premature: the reaction is recognized and workable, and abrupt reassignment would disrupt the associate's supervision.

**D** **Incorrect** (trap: Hands off too soon). Transfer is warranted if a supervisor cannot manage a reaction even with consultation. Here the reaction is recognized and the ratings can be corrected; reassignment would disrupt the associate's supervision before the less drastic step is tried.

**Takeaway:** When a supervisor's own reaction is biasing a supervisee's evaluation, seek consultation before finalizing it, rather than disclosing the reaction or transferring first.

**Sources:** Bernard, J. M., & Goodyear, R. K. (2019). Fundamentals of Clinical Supervision (6th ed.). Pearson, Supervisory relationship: transference and countertransference; NASW Code of Ethics (2021), 3.03
**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.3 Transference and countertransference in supervision · **Qualifier** NEXT · **Cognitive** reasoning · **Difficulty** 3 of 3

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### 340. INT-1613 (rev 2) · Best answer: B

At a youth residential program, a resident left the building at night and was injured. Required 15-minute checks had been logged but not done. The director asks the clinical social work manager to lead the response and revise the overnight safety procedures. What should the manager do **FIRST**?

**A.** Draft stricter overnight check rules for board approval
**B.** Review the incident with involved staff to find system causes
**C.** Retrain overnight staff on the check procedure right away
**D.** Gather sample procedures from similar programs

#### Why each option is right or wrong

**A** **Incorrect** (trap: Right action, wrong time). Revised procedures are the goal, but writing them before learning why logged checks were not done risks a policy that fails the same way. Drafting follows the review.

**B** **Correct.** Risk-reducing policy starts with understanding why the failure happened. Root cause analysis looks at systems and processes, such as staffing, workload and how checks are logged and monitored, rather than only individual performance (The Joint Commission, Sentinel Event Policy). Checks logged but not done signals a system gap that new rules alone may not fix (Reamer, 2015). It beats A because rules written before the causes are known may repeat the same gap between written policy and actual practice.

**C** **Incorrect** (trap: Skips assessment). Retraining feels responsive and may be part of the fix. Done before the review, it assumes the cause was a knowledge gap, when checks logged but not done may reflect staffing, workload or monitoring problems that training will not touch.

**D** **Incorrect** (trap: Right action, wrong time). Other programs' procedures can inform the revision once this program's failure points are known. Collecting them first skips the analysis of what went wrong here.

**Takeaway:** Before revising a safety policy after an incident, analyze the incident with involved staff to find system causes.

**Sources:** The Joint Commission (2024). Sentinel Event Policy and Procedures. Comprehensive Accreditation Manual, Root cause analysis (comprehensive systematic analysis); Reamer, F. G. (2015). Risk Management in Social Work: Preventing Professional Malpractice, Liability, and Disciplinary Action. Columbia University Press, Ch. 1: Professional Risk Management: An Overview
**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.4 Developing and evaluating risk-reducing policies and procedures · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 341. INT-1614 (rev 1) · Best answer: A

Six months ago, an outpatient clinic adopted a policy requiring a suicide risk screen at every intake, with a same-day safety plan for positive screens. All clinicians signed the policy and finished training. The quality committee now wants to know whether the policy is reducing risk. What should the committee do **FIRST**?

**A.** Audit a sample of charts for completed steps
**B.** Compare suicide attempts among clients before and after adoption
**C.** Survey clinicians about how useful they find the screen
**D.** Report the signature and training totals to the board

**Why each option is right or wrong**

**A** **Correct.** Evaluating a risk-reducing policy starts by checking whether it is actually followed, since outcomes cannot be credited to a policy that is not in use. Reamer's ethics audit, for example, compares written policy with what staff actually do (Reamer, 2001). Signatures and training show only that staff were informed; a record audit shows whether screens and same-day safety plans occur. It beats B because outcome comparisons come after fidelity is established, and suicide attempts are infrequent enough that 6 months of data would be hard to interpret.

**B** **Incorrect** (trap: Right action, wrong time). Outcome data matter eventually, but a before-and-after comparison means little until the committee knows the policy is being carried out. Suicide attempts are also infrequent, so 6 months of data would be hard to interpret.

**C** **Incorrect** (trap: Good but not best). Clinician views can reveal barriers, but usefulness ratings do not show whether screens and safety plans actually happen. A survey can supplement a record audit.

**D** **Incorrect** (trap: Misuses the technique). Signature and training totals show that staff were informed, not that practice changed. Reporting them as results would overstate what is known.

**Takeaway:** Evaluate a risk policy by first confirming it is followed in practice; awareness measures and outcome comparisons come later or fall short.

**Sources:** Reamer, F. G. (2001). The Social Work Ethics Audit: A Risk Management Tool. NASW Press, Audit steps and risk ratings

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### 342. INT-1615 (rev 1) · Best answer: C

An agency committee lists risk areas such as consent and boundaries. It compares daily practice with written rules, rates each risk and plans fixes. What is this process called?

**A.** Utilization review
**B.** Root cause analysis
**C.** Ethics audit
**D.** Peer review

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Utilization review checks whether services are necessary and appropriate, often for payers. It examines service use, not risks across areas such as confidentiality, consent and boundaries.

**B** **Incorrect** (trap: Wrong model or concept). Root cause analysis examines one adverse event to find the system failures behind it. The process described is a broad, proactive review across practice areas rather than an investigation of a single event.

**C** **Correct.** Reamer's ethics audit is a risk-management tool. A committee identifies ethics-related risk areas such as confidentiality, informed consent and boundaries, gathers data on actual practice, rates each area from no risk to high risk, and writes action plans starting with the highest risks (Reamer, 2001). It beats B because root cause analysis investigates one adverse event after it happens, while an audit reviews practices across areas before harm occurs.

**D** **Incorrect** (trap: Wrong model or concept). Peer review has colleagues assess the quality of individual practitioners' work or records. It does not rate agency-wide risk areas or produce an agency action plan.

**Takeaway:** An ethics audit proactively compares practice with policy across risk areas, rates each risk, and sets action plans, starting with high-risk areas.

**Sources:** Reamer, F. G. (2001). The Social Work Ethics Audit: A Risk Management Tool. NASW Press, Audit steps and risk ratings

**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.4 Developing and evaluating risk-reducing policies and procedures · **Qualifier** none · **Cognitive** recall · **Difficulty** 2 of 3

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## 343. INT-1689 (rev 1) · Best answer: A

A family services agency will begin evening home visits next quarter. The director asks a senior social worker to draft a field safety policy. Over the past 2 years, staff have filed several incident reports about daytime visits, and some workers have described near misses at staff meetings. What should the social worker do **FIRST**?

**A.** Review past visit problems with frontline staff
**B.** Adapt a neighboring agency's evening visit policy
**C.** Require two workers on every evening visit
**D.** Schedule de-escalation training for field staff

**Why each option is right or wrong**

**A** **Correct.** Risk-reducing policy starts with assessing the agency's actual risks. NASW's safety guidelines call for analyzing past incidents to find gaps in protocols and for assessing risk before field visits (NASW, 2013). The agency already has 2 years of incident reports and staff accounts of near misses, the most relevant data for designing evening visit rules. It beats C because a blanket two-worker rule may fit some visits but, set before the risks are known, could halve visit capacity without addressing the real hazards (Reamer, 2015).

**B** **Incorrect** (trap: Good but not best). Another agency's policy can be a useful model during drafting, but copied now it reflects that agency's clients, neighborhoods and staffing rather than the risks in this agency's own incident history.

**C** **Incorrect** (trap: Skips assessment). Paired visits are a recognized option for higher-risk visits. Mandated for every visit before reviewing incidents, the rule may miss the actual hazards and cut visit capacity unnecessarily.

**D** **Incorrect** (trap: Right action, wrong time). Training is a core part of a safety program and belongs in the policy. Planned first, it cannot be matched to the specific risks the incident review would reveal.

**Takeaway:** Build risk-reducing policies on an assessment of the agency's own incident data and staff experience before choosing rules or training.

**Sources:** NASW (2013). Guidelines for Social Worker Safety in the Workplace, Standard 2: Prevention; Standard 6: Risk Assessment for Field Visits; Reamer, F. G. (2015). Risk Management in Social Work: Preventing Professional Malpractice, Liability, and Disciplinary Action. Columbia University Press, Ch. 1: Professional Risk Management: An Overview

**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.4 Developing and evaluating risk-reducing policies and procedures · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 344. INT-1690 (rev 2) · Best answer: C

A community agency banned text messaging with clients to reduce privacy risk. A compliance review a year later finds that many case managers text clients from personal phones anyway. In interviews, staff explain that many clients have prepaid phones with few voice minutes and answer texts but not calls. Which policy response is **BEST**?

**A.** Reinforce the ban and discipline staff texting from personal phones

**B.** Survey clients about how they prefer the agency to reach them

**C.** Allow texting on agency phones with set safeguards

#### Why each option is right or wrong

**A** **Incorrect** (trap: Good but not best). Enforcement fits when staff break a workable rule for convenience. Here the review shows the rule conflicts with how clients can be reached; discipline would cut contact with these clients or push texting further out of sight.

**B** **Incorrect** (trap: Right action, wrong time). Asking clients is a sound step when their needs are unknown. Staff interviews already show that clients rely on texts because of limited voice minutes, so a survey delays a needed policy change.

**C** **Correct.** Evaluating a risk policy means checking whether it reduces the risk in practice. This ban has pushed texting onto personal phones, where messages are unsecured, outside the record and beyond agency control, a greater privacy risk than the one the ban targeted. Allowing texting on agency phones with safeguards meets clients' real needs, lets the agency protect the messages (NASW Code 1.07(m)) and keeps staff from exposing personal numbers, as NASW safety guidelines advise (NASW, 2013). It beats A because stricter enforcement ignores why staff text; contact with clients who cannot afford calls would drop or go further underground.

**Takeaway:** When a risk policy produces unsafe workarounds, revise it to meet the legitimate need with safeguards rather than enforcing it harder.

**Sources:** NASW Code of Ethics (2021), 1.07(m); NASW (2013). Guidelines for Social Worker Safety in the Workplace, Standard 5: Use of Mobile Phones

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### 345. INT-1616 (rev 1) · Best answer: B

In a large hospital, each social worker answers to two people. The head of social work oversees professional standards. A unit nurse manager sets daily assignments. Which organizational structure does this describe?

**A.** Flat structure
**B.** Matrix structure
**C.** Functional structure
**D.** Divisional structure

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). A flat structure has few management layers and wide spans of control. It describes how many levels an organization has, not dual reporting lines.

**B** **Correct.** A matrix structure gives staff two lines of authority, typically a functional or professional department and a program or project unit (Galbraith, 2009). Here the department director oversees professional standards while the unit manager directs daily work, so each social worker answers to both. It beats C because a functional structure groups staff by discipline under a single chain of command; the second reporting line to the unit manager is what makes this a matrix.

**C** **Incorrect** (trap: Wrong model or concept). A functional structure groups staff by profession or function, such as a social work department, under one chain of command. The added reporting line to the unit manager goes beyond that.

**D** **Incorrect** (trap: Wrong model or concept). A divisional structure organizes staff into self-contained units, such as service lines, each with its own chain of command. Here professional oversight stays with a separate department.

**Takeaway:** A matrix structure gives staff two lines of authority, usually a professional department and a program or unit.

**Sources:** Galbraith, J. R. (2009). Designing Matrix Organizations That Actually Work. Jossey-Bass, Matrix structure defined: dual reporting relationships
**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.5 How organizations are structured, function, and develop · **Qualifier** none · **Cognitive** recall · **Difficulty** 1 of 3

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### 346. INT-1617 (rev 1) · Best answer: B

A community agency plans to replace paper charts with an electronic record. Months before any training, the director shares data on missed follow-ups caused by lost paper files, holds staff forums about problems with the current system and invites concerns. In Lewin's model, which stage is the director **MOST** likely working on?

**A.** Refreezing
**B.** Unfreezing
**C.** Moving

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). Refreezing stabilizes a change after it is made, through new policies, reinforcement and routine use. The electronic record has not been introduced yet.

**B** **Correct.** Lewin (1947) described planned change in three steps: unfreezing, moving and refreezing. Unfreezing builds readiness by showing that the current way of working is not acceptable and by surfacing concerns before new behavior is introduced. Sharing data on missed follow-ups and inviting staff concerns before any training is unfreezing. It beats C because moving is the stage in which people learn and adopt the new system, such as training and go-live, which has not started.

**C** **Incorrect** (trap: Wrong model or concept). Moving is the transition itself: training staff and beginning to use the new record. The director's work comes before any training begins.

**Takeaway:** In Lewin's model, unfreezing builds readiness by showing the current way is not working; moving is the change; refreezing stabilizes it.

**Sources:** Lewin, K. (1947). Frontiers in group dynamics: Concept, method and reality in social science; social equilibria and social change. Human Relations, 1(1), 5-41, whole article
**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.5 How organizations are structured, function, and develop · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 2 of 3

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### 347. INT-1618 (rev 1) · Best answer: A

A new director at a large homeless shelter plans to move from rule-based discipline to a trauma-informed approach. Staff resist, and many say they will wait to see what one long-time staff member thinks, though that person holds no supervisory title. The board supports the change. What should the director do **FIRST**?

**A.** Involve that colleague in planning from the start
**B.** Schedule trauma-informed care training for the full staff
**C.** Announce a firm start date with the board's backing

#### Why each option is right or wrong

**A** **Correct.** Organizations run on an informal structure of influence alongside the formal chart, and informal leaders shape whether staff accept direction (Barnard, 1938). Staff say they are waiting on this colleague, so involving that person in planning builds buy-in before rollout. A trauma-informed approach also calls for collaboration and shared decision-making among staff, not only with clients (SAMHSA, 2014). It beats B because training delivered to a resistant group before influential members are on board is likely to be discounted.

**B** **Incorrect** (trap: Right action, wrong time). Training is essential to a trauma-informed shift, but it works better once influential staff support the change. Offered first to a resistant group, it may be dismissed.

**C** **Incorrect** (trap: Too directive). Board backing gives formal authority, but announcing a deadline over staff resistance relies on that authority alone. It can deepen opposition and ignores the informal leadership staff are following.

**Takeaway:** Organizational change goes better when informal leaders are engaged early; formal authority or training alone meets resistance.

**Sources:** Barnard, C. I. (1938). The Functions of the Executive. Harvard University Press, Ch. 9: Informal organizations and their relation to formal organizations; SAMHSA (2014) Concept of Trauma and Guidance for a Trauma-Informed Approach (SMA14-4884), Six key principles: collaboration and mutuality
**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.5 How organizations are structured, function, and develop · **Qualifier** FIRST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 348. INT-1691 (rev 2) · Best answer: B

At a county benefits office, intake staff spend most of each appointment checking that every form field is complete. Workers turn away applicants in urgent need over minor paperwork gaps, and staff meetings focus on audit error rates rather than whether families get help. Supervisors praise flawless files. Which organizational problem is **MOST** likely present?

**A.** Mission drift
**B.** Goal displacement
**C.** Groupthink
**D.** Creaming

#### Why each option is right or wrong

**A** **Incorrect** (trap: Wrong model or concept). Mission drift is a gradual shift in what an organization does or whom it serves, often driven by funding. The office still exists to deliver benefits; the problem is that procedural compliance has replaced that aim in daily work.

**B** **Correct.** Goal displacement occurs when rules and procedures meant to serve an organization's purpose become ends in themselves (Merton, 1940). Here complete forms and low audit error rates are what staff pursue and supervisors reward, even as families in urgent need go unserved. It beats A because mission drift is a change in the organization's purpose or population, often following funding; this office's purpose is unchanged, but procedure has eclipsed it in practice.

**C** **Incorrect** (trap: Wrong model or concept). Groupthink is a cohesive group's drive for agreement overriding critical judgment in decisions (Janis, 1982). The issue here is rules becoming the aim of everyday work, not a team suppressing dissent.

**D** **Incorrect** (trap: Wrong model or concept). Creaming is selecting the clients most likely to succeed, often to meet performance targets. Applicants here are turned away for paperwork gaps regardless of their prospects, which points to rule fixation rather than selection for success.

**Takeaway:** When procedures meant to serve clients become the organization's real aim, the problem is goal displacement.

**Sources:** Merton, R. K. (1940). Bureaucratic structure and personality. Social Forces, 18(4), 560-568, whole article; Janis, I. L. (1982). Groupthink: Psychological Studies of Policy Decisions and Fiascoes (2nd ed.). Houghton Mifflin, groupthink defined
**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.5 How organizations are structured, function, and develop · **Qualifier** MOST LIKELY · **Cognitive** recall · **Difficulty** 1 of 3

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### 349. INT-0119 (rev 1) · Best answer: B

A family service agency learns its funding will drop 15% next year. It runs a crisis response program, a parenting education class and a youth recreation program. The director must decide how to allocate the reduced budget. Which approach is **BEST**?

**A.** Cut each program's budget by the same 15%
**B.** Rank programs openly by client need and outcome data
**C.** Close the program that serves the fewest clients
**D.** Protect the programs that board members value most

**Why each option is right or wrong**

**A** **Incorrect** (trap: Good but not best). Across-the-board cuts are simple and look even-handed, but they treat unequal needs and results as equal. A 15% cut to crisis services may cause more harm than the same cut elsewhere.

**B** **Correct.** When resources cannot meet every need, the NASW Code calls for allocation procedures that are open and fair, nondiscriminatory and based on appropriate, consistently applied principles (NASW Code 3.07(b)). Ranking programs by client need, including the safety stakes of crisis services, and by evidence of outcomes applies clear principles in a transparent way. It beats A because equal cuts look fair but ignore differences in need and effectiveness, and could weaken an essential crisis service as much as an optional one.

**C** **Incorrect** (trap: Skips assessment). Participant counts alone are a narrow measure. A small program can serve the highest-need clients, such as people in crisis, so need and outcomes should be weighed before anything is closed.

**D** **Incorrect** (trap: Misreads law or ethics). Basing cuts on what board members prefer substitutes personal preference for open, consistently applied principles. Board input matters, but allocation should rest on client need and evidence.

**Takeaway:** When cutting budgets, allocate through an open process using consistent principles such as client need and outcome evidence.

**Sources:** NASW Code of Ethics (2021), 3.07(b)
**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.6 Budgeting, fiscal oversight, and allocating resources · **Qualifier** BEST · **Cognitive** reasoning · **Difficulty** 2 of 3

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### 350. INT-1619 (rev 2) · Best answer: C

Which budgeting system calculates cost per outcome achieved, such as the cost for each client who stays housed for a full year?

**A.** Line-item budgeting
**B.** Functional budgeting
**C.** Program budgeting

**Why each option is right or wrong**

**A** **Incorrect** (trap: Wrong model or concept). A line-item budget lists expenses such as salaries, rent and supplies. It controls spending but does not link costs to services delivered or outcomes achieved.

**B** **Incorrect** (trap: Wrong model or concept). In Kettner, Moroney and Martin's framework, functional budgeting yields cost per unit of service or per output, such as

cost per counseling hour or per client completing a program, not cost per outcome such as stable housing. In nonprofit accounting, 'functional' instead means sorting expenses into program, management and fundraising; neither use gives cost per outcome.

**C** **Correct.** Kettner, Moroney and Martin (2022) describe three linked systems. Line-item budgeting lists expenses by category for financial control. Functional budgeting adds total program cost and cost per unit of service or output for management. Program budgeting goes further, to cost per outcome, such as each client who remains housed, for planning. It beats B because functional budgeting stops at outputs, such as cost per case management hour, rather than results achieved.

**Takeaway:** Line-item budgets list expenses; functional budgets give cost per unit of service or output; program budgets give cost per outcome.

**Sources:** Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 12-13: line-item, functional and program budgeting
**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.6 Budgeting, fiscal oversight, and allocating resources · **Qualifier** none · **Cognitive** recall · **Difficulty** 3 of 3

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### 351. INT-1620 (rev 2) · Best answer: B

A family housing program's grant pays for two case managers. One position has been vacant for 4 months, leaving unspent salary funds. Meanwhile, several client families face eviction over small rent debts. The grant agreement requires the funder's written consent for spending outside its line items. Which action by the program director is **BEST**?

**A.** Use the savings for rent aid and inform the funder at year end
**B.** Ask the funder to approve moving the savings to rent aid
**C.** Use the savings for rent aid once the board approves it

#### Why each option is right or wrong

**A** **Incorrect** (trap: Misreads law or ethics). The goal is worthy, but redirecting restricted grant funds without the required approval uses them for unauthorized purposes. Reporting the change later does not cure the lack of approval and puts the program's accountability to the funder at risk.

**B** **Correct.** Social workers should be diligent stewards of agency resources and should not use funds for purposes other than those intended (NASW Code 3.09(g)). Line-item grant budgets exist to control spending by category (Kettner, Moroney & Martin, 2022), and this agreement requires approval for changes. A budget modification request lets the director respond to the families' urgent need through legitimate channels. It beats A because spending first and reporting later uses restricted money without authorization, even for a worthy purpose.

**C** **Incorrect** (trap: Misreads law or ethics). Board approval is a sound internal control, but this grant agreement requires the funder's written consent for spending outside its line items. An agency board cannot authorize a change to a funder's restricted budget.

**Takeaway:** Restricted grant funds are redirected only with the funder's approval, even when the new purpose meets urgent client needs.

**Sources:** NASW Code of Ethics (2021), 3.09(g); Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 12-13: line-item, functional and program budgeting
**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.6 Budgeting, fiscal oversight, and allocating resources · **Qualifier** BEST · **Cognitive** application · **Difficulty** 2 of 3

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## 352. INT-1692 (rev 2) · Best answer: B

A crisis stabilization program manager reviews the monthly budget report at midyear. Overtime spending is 40% above its year-to-date budget, and the program is projected to end the year in deficit. The executive director asks for a plan by next week. What should the manager do **FIRST**?

**A.** Draft a staffing schedule that caps overtime spending
**B.** Find out what is driving the overtime spending
**C.** Ask the board to raise the overtime budget

**Why each option is right or wrong**

**A** **Incorrect** (trap: Right action, wrong time). A schedule that limits overtime may well be part of the plan. Drafted before the cause is known, it may cap hours a crisis program needs for safe coverage, or miss a driver such as vacancies or rising admissions that calls for a different fix.

**B** **Correct.** Fiscal oversight compares actual spending with the budget and then analyzes the reasons for any variance before choosing a response (Kettner, Moroney & Martin, 2022). Overtime could reflect vacancies, rising admissions, sick leave or scheduling practices, and each calls for a different fix. Knowing the cause lets the manager propose a plan that controls cost without endangering crisis coverage. It beats C because a budget increase may be justified, but the board will rightly expect an explanation of the variance first.

**C** **Incorrect** (trap: Right action, wrong time). More funding may be warranted, for example if admissions have risen. Requested before the variance is analyzed, it lacks the justification decision makers need and may pay for a problem that scheduling could fix.

**Takeaway:** When spending departs from budget, analyze the cause of the variance before cutting, shifting funds or requesting more.

**Sources:** Kettner, P. M., Moroney, R. M., & Martin, L. L. (2022). Designing and Managing Programs: An Effectiveness-Based Approach (6th ed.). SAGE, Ch. 12: Budgeting for control, management and planning
**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.6 Budgeting, fiscal oversight, and allocating resources · **Qualifier** FIRST · **Cognitive** application · **Difficulty** 2 of 3

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### 353. INT-1693 (rev 2) · Best answer: A

A family support program gives grocery gift cards to clients in need. One case manager orders the cards, keeps them in a desk drawer, hands them out and records each distribution in a spreadsheet. A routine review finds the records complete. The program director wants to reduce the risk of error or misuse. Which control is **MOST** effective?

**A.** Give purchasing, custody and recordkeeping to different staff
**B.** Have a supervisor count the cards against the log each month
**C.** Refer the arrangement to the compliance officer for investigation

**Why each option is right or wrong**

**A** **Correct.** Segregation of duties is a basic internal control: authorizing, handling and recording assets are divided so no one person controls a whole transaction (GAO, 2014). One case manager now orders, stores, distributes and logs the cards, so an error or misuse could go undetected. Dividing these tasks creates built-in checks while keeping cards available to clients. It beats B because a count against the case manager's own log cannot catch cards that were ordered but never recorded; separating ordering, custody and records closes that gap.

**B** **Incorrect** (trap: Good but not best). An independent count is a real control and better than none. The same person still orders the cards and keeps the log the count is checked against, though, so cards ordered and never logged would not show up, and any problem surfaces only weeks later.

**C** **Incorrect** (trap: Skips assessment). Investigation is warranted when there is evidence of loss or misuse. The review found complete records, and the director's aim is prevention, so the fix is a structural control rather than an inquiry into one employee.

**Takeaway:** Prevent fiscal error and misuse by dividing authorization, custody and recording of assets among different people.

**Sources:** GAO, U.S. Government Accountability Office (2014). Standards for Internal Control in the Federal Government (GAO-14-704G), Principle 10: Design Control Activities, segregation of duties
**Area** III Intervention and Practice · **Sub-area** III.D Supervision and Administration · **Topic** III.D.6 Budgeting, fiscal oversight, and allocating resources · **Qualifier** MOST · **Cognitive** application · **Difficulty** 2 of 3